

Healthcare Global (HCG)

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Call: Aug 2022

August 17, 2022

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors held on August 10, 2022

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(oa) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on August 10, 2022, with Analysts/Investors to discuss the Unaudited Financial Results of the quarter ended June 30, 2022 .

This is also available on the website of the Company www.hcgoncology.com .

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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“Health Care Global Enterprises Limited Q1 FY -23
Earnings Conference Call”

August 10, 2022

MANAGEMENT : DR. B. S. AJAI KUMAR – EXECUTIVE CHAIRMAN ,
HEALTH CARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CEO, HEALTH CARE GLOBAL
ENTERPRISES LIMITED
MR. SRINIVASA RAGHAVAN – CFO, HEALTH CARE
GLOBAL ENTERPRISES LIMITED

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Moderator : Ladies and gentlemen good day and welcome to the Q1 FY23 Earnings Conference Call of Healthcare Global Enterprises Limited. This conference call may contain forward looking statements about the company which are based on the beliefs , opinions and expectations of the company as on date of this call. These statements are not guarantees of future performance and involve risk and uncertainties that are difficult to predict. As a reminder all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes . Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Dr. B. S. Ajai Kumar – Executive Chairman of Healthcare Global Enterprises Limited . Thank you and over to you sir.

B.S. Ajai Kumar : Thank you very much and good evening to everyone and welcome to all the present on the Q1 FY23 earning conference call for Healthcare Global Enterprise. I have with me here Mr. Raj Gore, our Chief Executive Officer and Srini Raghavan, who is our CFO and Ashutosh and Venkat from our management .

HCG is a leading oncology care provider which is patient centric , technology oriented and outcome based. Our focus has been on oncology and in conjunction with partners in some areas which has built a strong legacy as a world class oncology institution and go to brand oncological treatment and services across India. We are also into infertility as well as few multispecialty hospitals which we are also strengthening. Our focus on strong clinical talents , good infrastructure , technology and timely upgradation of the same has enhanced our ability to deliver exceptional clinical outcomes equal to or better than global standards. In the very near future, we are putting together data on head and neck and breast cancer on these outcomes and see how we can challenge the global outcomes.

HCG is today not only a service provider but has taken a lead role in research and academics. Recently we have acquired next generation sequence in genomics which is a sequencer of high end and in process of acquiring circulating tumor cell platform. These additions are already enhancing our capabilities , physician diagnosis , leading to early detection of not only cancer but also early detection of recurrent cancer and genomic guided therapy. We have a separate tumor board for this with relevant expertise. In addition, we have introduced enterprise RIS PACS in collaboration with Siemens for the first time in this country for AI based physician imaging which will enhance our research capabilities and also improve our effect on the patient outcomes and diagnosing the patients in proper staging . Further HCG is introducing first of its kind in the country , an AI enabled adoptive radiotherapy in our center of Excellence in Bangalore which will again make a difference in how we precisely treat cancer patients. Under clinical initiatives we have achieved a sizable scale in terms of case load and patient outcome across centers and continue to enhance our in-house capabilities to increase the number of clinical trials carried out at present and work with the pharma companies on drug discovery and related matters backed with significant data.

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Talking of our environmental -friendly initiatives , we have done a very successful implementation of 3.3 MW solar plant at our Bangalore facility , related to our Bangalore facility slated to the commission in a couple of months which will result in substantial cost savings also. We have already initiated a pioneering effort in genomics , metabolics and microbes to unleash

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indigenous research in India for Indians rather than extrapolating western origin data and force

fitting in the context of Indian populace. In addition, we will be transitioning away from the conventional medical record system to electronic medical records which will not only provide value for the patients, in terms of their clinical management but also analytics and data intelligence will be gathered. We are working on modus operandi to ensure that the last mile benefits of our physician and personalized medicine endeavors through tech enabled innovations like HoloLens, a phenomenal technology that marked a super processing power and wider field of reason to provide help to our patients not only in the center of excellence but also in Tier 2, Tier 3 cities. With these few remarks I now hand over to Mr. Raj Gore, our CEO.

Raj Gore: Thank you Dr. Ajai. Good evening, everyone. Very warm welcome to all of you. We are delighted to share healthy financial and operation performance yet again for Q1 FY23. We are happy to report robust growth for quarter ended June 2022. Our consolidated revenues for Q1 FY23 grew by 26% on a year-on-year basis and we've reported PBT of Rs. 10 crores for Q1 FY23 as compared to loss of Rs. 9 crores in Q1 FY22. We are also happy to report that we've been able to successfully accelerate the financial performance for HCG over the last eight quarters. Our revenue has increased by 1.7 times since Q2 FY21 on a quarterly run rate basis. Correspondingly our EBITDA in absolute terms have grown by 2.4 times over the last eight quarters and have been able to achieve consecutive PBT positive quarters.

We have been working on multiple fronts to take HCG to greater height by constantly improving holistic value that we offer to our patients and thus create value for the business in a sustainable manner. We are confident of maintaining our growth momentum going forward. Four key areas of our strategic roadmap that we are currently working on are;

1) Digital transformation. It means two things, building digital capability, to build HCG brand online, to increase digitally influence patient footfalls into our hospital. 2) Improving our processes from patients' perspective to deliver an experience that matches evolving consumer behavior and expectations. This initiative will involve omni-channel patient engagement with the help of multiple digital assets like website, social media, WhatsApp bot, patient app portal, centralized call center, integrated CRM and advanced analytics and business intelligence. We are already seeing some green shoots. For example, our website traffic is up by 2.7 times over last six months.

The second initiative is cost optimization and efficiency enhancement. We have also been working on our structural cost optimization and efficiency enhancement program. We've been able to evaluate multiple areas for enhancing workforce productivity and optimize staffing accordingly in a scientific manner. Additionally, we have also relooked at our pricing and

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collectively these initiatives should drive significant operational efficiency to improve margin. We would commit later but we are targeting a healthy margin improvement from these initiatives. The third initiative is growth of our medical value travel business. We have recasted our international business strategy starting a couple of quarters ago. Largely the strategy is focused on enhancing our footprint, both geographically and channel wise. We have set up six new information centers across our large contributing geography for creating greater brand pool. Currently we are averaging about 1.4 times the monthly average during the pre-COVID times. The fourth initiative is inorganic acquisition. We are also scouting for few inorganic acquisitions as a part of our growth strategy and a

re actively pursuing some. We will keep the market posted at appropriate time. Our history of successful partnerships and upscaling of the acquired businesses over a period gives us confidence to pursue this strategy.

To conclude I would like to reiterate, that we have strengthened our senior management team, our clinical team across the network and have been continuously upgrading our technology and capabilities in cancer care for providing all oncology related services under roof. With this I would like to handover the call to Srinivasa to take you through the financial and operational performance highlights for the quarter ended June 2022.

Srinivasa Raghavan: Thank you Raj. Very good evening to all of you. We have uploaded our Q1 '23 financial results and updated investor presentation on the stock exchanges and company's website. I do hope everybody had an opportunity to go through the same. We are delighted to share that we have been able to grow our revenues ahead of the industry growth due to the trust and brand created for HCG over decades.

On the revenue front; our consolidated revenues for Q1 '23 stood at 408 crores as compared to 323 crores in Q1 FY22, a growth of 26%. Revenue split between HCG and Milann stood at 96% and 4% respectively for Q1 FY23. Revenue growth for HCG stood at 26% YOY and Milann stood at 43% YOY. As mentioned in slide #27 revenue from the existing center stood at 298 crores, a growth of 23% on a YOY basis. Revenue from new center stood at 75 crores a growth of 23% on YOY. We are delighted to state that our new centers are inching towards maturity and are seeing good traction across geographies.

Coming to slide #33; our revenues from oncology business grew by 35% on YOY basis to 343 crores and revenue from non-oncology business stood at 58 crores. I now call your attention to slide #34 where we have for the first-time disclosed bifurcation of our operational parameters across our existing network and new network. Our company wide AOR stood at 64.6% and AOR for existing versus new centers stood at 64% and 66.1% respectively. Higher Occupancy for new centers is due to only 70% of beds were operational in new centers. AOR and capacity beds stands at 49%. Our ARPOB on company level stood at Rs. 38,454 and our ARPOB from existing network stood at Rs. 40,606 and for new centers it stood at Rs. 32,968. ROCE for existing network stood at 19% annualized for Q1 FY23 as compared to 15.4% in FY22, a growth of 360 Healthcare Global Enterprises Limited
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bps. ROCE for new centers stood at (-4.6%) annualized for Q1 FY23 as compared to (-8.3%) in full year FY22. This again is an improvement of 370 bps.

Across geography we have given our revenue breakup in slide #35. Jaipur grew by 263%, revenue from Ranchi grew by 112% and Mumbai grew by 53%. Bangalore center of excellence grew by 37%.

Our Milann business is also doing well. Revenues have increased by 43% in Q1 FY23 on YOY basis, our new registrations grew by 139%. New centers revenue growth in Milann stood at 70%.

On the EBITDA front; our consolidated EBITDA for Q1 FY23 stood at 72 crores as compared to 51 crores in Q1 FY22, a growth of 41% on YOY basis. Our EBITDA growth on quarter-on-quarter basis stood at 14%. We have also given bifurcation of our EBITDA across existing and new centers. I would request to participants to view slide #27 for further details.

We are happy to report that EBITDA from our new center is growing. EBITDA margins for Q1 FY23 stood at 17.7% as compared to 15.9% in Q1 FY22 and 17.3% in Q4 FY22. EBITDA margin expanded by 180 bps on YOY basis. As we draw synergies from various cost saving initiatives including strategic efficiency projects that Raj talked about, we are confident of improving EBITDA margin by 100 to 200 bps over the next 12 to 18 months. Some benefits of these

initiatives are reflecting in the higher margin for this quarter.

On profit before tax; we are having to report PBT positive number of 10.4 crores in Q1 FY23 as compared to a loss of 8.9 crores in Q1 FY22. We aim to be a profit-making company on a consistent basis from now onwards. Happy to share that we are PAT positive by 6 crores in Q1 FY23 which is fairly sustainable and likely to go in the coming quarters. PBT on QOQ basis increased by 11 times. PBT margin for Q1 FY23 stood at 2.6% as compared to (-2.8%) in Q1 FY22. Our net debt position excluding capital leases as on 30th June, 2022 stood at 191.5 crores compared to 293.7 crores on 30th June '21 and 190.5 crores as on 31st March '22. CAPEX for Q1 FY23 was Rs. 16.3 crores, bifurcated between existing centers and new centers at Rs. 12.2 crores and 4.1 crores respectively. Our expansion of existing facilities at Ahmedabad Phase 2 and Whitefield extension of Bangalore COE is on track and we have incurred 7 crore and 1 crore respectively. Total plan CAPEX for Ahmedabad is Rs. 85 crores expected date of operations being Q1 FY25 and for Bangalore COE is Rs. 25 crores expected date of operation being Q4 FY24. With this I would now like to open the floor for question and answer.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Shubham Ajmera from SOIC.

Shubham Ajmera: I would like to get your view on the international patients, like have we reached the pre-COVID level here and what kind of growth we are seeing here and what is the current percentage of revenue from international patients store in our total revenues? If you can give us some broad idea on that.

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Raj Gore: Thank you for this question. As I mentioned in my opening remarks, Pre-Covid period Bangalore was largely the only destination to get international patients. Post-COVID, today we have Mumbai as well as Kolkata, two more destinations and we have made significant efforts in expanding our geographical footprint as well as channel footprints, whether it's government tie-ups, insurance tie-ups, institutional tie-ups with different corporates as well as digital channels. As a result, we have seen in Quarter 1 our business is 1.4 times the pre-COVID level. I think today at a HCG level our international business is about 4%. But you would be happy to know that at our center of excellence in Bangalore it's at 15% of top line and I think we're just getting started.

Shubham Ajmera: Second question is, we have done some changes in the life of property plant and equipment. Can you please elaborate on which categories we have done it and what is the current life of such assets from the previous year?

Srinivasa Raghavan: This is a major change basically it is PET CT which we are depreciating over a period of 10 years, the Companies Act and the industry practice allows to kind of depreciate for a period of 13 years which is what we have done.

Moderator: The next question is from the line of Shyam Shrinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one team on AOR. I know you don't kind of look at it like how we look at other hospitals. But just that's been a significant jump. So, is there something from an operational standpoint that's changing we've reached like close to 65%? Just want to understand what's happening on occupancy. I don't know whether you have an outlook for it but just would like to understand this one.

Raj Gore: I think we've been mentioning in our several quarterly calls that there's a lot of effort that we have taken over last 4 to 6 quarters in improving our clinical strength in each hospital, adding more technology, adding more services. But mainly a lot of effort has been taken on our go to market

strategy, our number of empanelment with different TPAs, different corporate, different pair channels. There's a lot of groundwork that has happened. There is a lot of activation activities have happened to create awareness about HCG's capability in treating cancer patients.

I think we're seeing a consistent growth in our footfalls as a result of all these efforts. As a result, the occupancy is growing. If you see the trend in occupancy is consistent over last few quarters.

Shyam Srinivasan: Get the factual point but I'm just saying is this how should we look at it going ahead that's one because we are now breaking new ground. I don't know whether have at least seeing historical numbers of this high. How does it actually impact our business model, either in terms of what growth you can aspire to versus and the kind of margins? Because I'm assuming generally simple that higher utilization means better margins.

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B.S. Ajai Kumar: As you know Shyam in the past the question has been asked about our occupancy and we were always in the 50% or so we were said it is because a lot of outpatient therapy going on. But now as you can see on revenue growth of 26% in the last eight quarters. I think like what Raj said increasing footfall essentially this is what we were expecting. We have space, we don't have space constraints. We don't have to put more brick and mortar. This Occupancy is happening as per our expectation as the revenue grows from each center because our center of excellence and areas in Jaipur are all reaching historic growths. Naturally the occupancy will increase. It won't remain stable at 50%. That is the growth we are seeing and going forward yes, we still have 64, some consider in the 75 as the maximum. We may reach there at that point when all other centers start performing to its potential. But it also could be depending on centers, for example Jaipur now we are almost full. So, we have capacity so also in our Ahmedabad center. And Ahmedabad we are building a new center. All of this will also take care of this. But it is a healthy growth it is expected and we are obviously happy about it.

Shyam Srinivasan: My second question is on ARPOB dynamics and slightly different trend, so for example new centers we have not seen ARPOB grow but if I were to look at existing centers, they have grown well, blended is 6%. Can you walk us through some of the dynamics here? What's happening either in terms of price, mix, what is being done here?

Raj Gore: Just one more data point on the previous question that you asked. I think we have a capacity of about 1,974 beds right now. We have personalized 1,737 beds. If your question was any way or if your question was related to how much upside of growth is, as and when we increase our occupancy, we will continue to commission more beds that are already built. I don't see capacity as a constraint going forward. On ARPOB, I think that has happened on improvement in ARPOB, has happened due to 2-3 levels. One is pricing increase, second is I think we have pricing for cash as well as TPA revision. We are seeing some positive mix change. We have more hospitals which are NABH accredited which gives us additional pricing ability. As a combination, I think our ARPOB is improving.

Shyam Srinivasan: Raj, you mentioned in the opening comments, we have looked at pricing. So, cash and TPA what's the revision, is it like linked to 4%-5% medical inflation or higher?

Raj Gore: Usually year-on-year we do about 2% to 3% inflationary increase. But we have also talked about operational efficiency improvement program. Under that program we have done more structured pricing revision starting with our Bangalore market. I think that's also contributing to higher margin. Now we are in process of rolling out that initiative

across 10 more centers in the network.

So, you will see additional improvement on realization because of that.

Shyam Srinivasan: Thank you so much. Just my first question is still pending which is why the difference between existing versus new ARPOB? I'll stop there. Thank you.

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Raj Gore: So, a new center . if you look at the growth, I think Jaipur has really picked up . Jaipur, I think the mix is more in favor to scheme business and as a result of that the realization is constant. Also, the way we are approaching Jaipur is we want more and more patients to experience our services and all and the optimization will happen as and when we go closer to the utilization level , higher utilization level.

B.S. Ajai Kumar: Regarding your question of why ARPOB is increased more in our existing centers , it is because of the mix technology , price increase . Obviously, our center of excellence Bangalore will have a much higher ARPOB even as we go forward when we look at the ARPOB in a conventional way. That could be the reason also the difference is there Shyam .

Moderator: The next question is from the line of Kaustubh Pawaskar from ShareKhan by BNP Paribas.

Kaustubh Pawaskar: My question is on the structure pricing revision you just talked about in Bangalore Center. Can we just elaborate it and how you are planning to implement it into other centers?

Raj Gore: So, there are two types of pricing exercise. One is annual inflationary price increase but every 2-3 years we would like to do a more structured pricing increase. What does that mean? That means we map the competition , we look at the pricing , we map our positioning in the market , our value proposition in the market versus other players. We look at our capacity utilization for particular businesses and see if there is an opportunity to increase pricing premium. We go line item by line item in a more structured manner and identify items where we can optimize our pricing. Now that's the methodology that's specific to a particular market. Therefore, we started with Bangalore and now we will take it to other markets. It does take some time to study the market and map it and we will take it to more markets as we move forward in this year.

Kaustubh Pawaskar: My second question is on your cost optimization strategy which was explained in the initial comments of the management. So can you just elaborate on how you are planning for this cost optimization strategy because these are some of the levers which will help us to drive the margins and as sir said that you are focusing on achieving around 150 to 200 bps improvement in the margins, these are the levers which will help you to drive the margins. Can you just elaborate on your cost optimization strategy?

Raj Gore: Absolutely. You know that as an industry we've been facing increasing competition , price regulation , pressure on margin and therefore we believe that we must do everything to increase margin and the key in that is identify micro efficiencies which and aggregate helps us to improve overall margin. So, there are several levers we are looking at it. One is workforce productivity. Workforce cost is one of the major cost items in our P & L. Second is indirect cost optimization , the long tail of fixed cost. What we do in that is basically go line by line , identify duplication based non-value adding activities , redesign processes , introduce technology to improve the productivity of certain processes , do shared services , introduce shared services so that we can have economies of scale. We do a very structured program line item by line item and I think Healthcare Global Enterprises Limited
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again we've started this initiative in Bangalore clusters first and we will now take it to other markets.

Kaustubh Pawaskar

ar: My third question is on; I've gone through all the clusters. We have seen that most of the clusters have done pretty well strong growth. Maharashtra, we have seen only the growth of just 10% , any particular reason for it?

Raj Gore: I think if you look at Maharashtra, Nasik is one of our big centers. I think it's the biggest center for us in Maharashtra in terms of top line. Last year Q1, we had a significant COVID business in that hospital and this quarter that is not there and that's why the percentage is looking lower.

Kaustubh Pawaskar: It is because of the high base of the COVID led business?

Raj Gore: Yes.

Kaustubh Pawaskar: One last bookkeeping question , this quarter we have seen tax rate higher at 7.9 crores. Is there any one -off in to it?

Srinivasa Raghavan: The thing is more from a conservative point of view ; we don't take any deferred tax asset on our loss breaking business . That's the reason why we take , so that is the reason why it is like that.

Kaustubh Pawaskar: What would be a normalized tax rate for the year ?

Srinivasa Raghavan: We are at about 34.5% which is the normal tax rate 34.5% which is normal tax rate that each year effects us at this point in time. At an appropriate time, we will evaluate the lower tax of 25%.

Kaustubh Pawaskar: As you mentioned that our optimum capacity utilization is something which you are looking at all the centers. So, when you think about putting a CAPEX what kind of a utilization you have in front of you for a particular center? For example, Ahmedabad as you said that currently it is now fully utilized capacity. So, when you do an expansion or when you come out with a greenfield project next to a particular hospital what kind of capacity utilization or occupancy

you normally look into the n after which you have to exp and the capacity ?

B.S. Ajai Kumar: In the capacity utilization as we discussed previously about occupancy rate beds. Our focus going forward in the greenfield if we do will be less beds and more outpatient facility , daycare because the whole cancer care is moving towards that. When we talk about capacity utilization , it is not only about beds anymore but it is about how many linear accelerators , what is the number of patient a linear accelerator can it take, can it do 100 patients because some of the newer technology and all can be used to give that kind of capacity. When we are running for example in Jaipur , we are at capacity utilization on the two units , we are now trying to bring a third unit. So, we are reaching there. Another important thing is as we know our CAPEX is actually, we have Healthcare Global Enterprises Limited
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discussed this in the past meetings where it is now pay-for-use model majority of the times even though there may be exceptions . With this model the CAPEX actual requirement in terms of what if actual CAPEX becomes less , it is more of a sharing the revenue. With all this I think we are very well positioned to handle even a capacity needed , putting extra technology without actually putting extra CAPEX in it. So, it will be risk reward sharing. So, all of this I think will help us to grow very fast in future depending on where the need is and it is very specific. For example, we may be reaching capacity in Jaipur but we may not have reached capacity in Kolkata or South Mumbai. We will now also focus on that which areas need to be focused and the management is putting in extra effort to these areas to reach capacity.

Moderator: The next question is from the line of Ashay Jain from Jain Capital.

Ashay Jain: I have a couple of questions. Firstly, can you elaborate on digital initiatives taken to increase the revenue and how do we see the overall technology planning a major part in the increase in revenues?

Raj Gore : Thank you for asking that question . The whole genesis of

HCG is bringing the most advanced medical technology to cancer patients in India. This is something that we are pioneering. I think if you look at what has happened parallelly in other industries , digital technology has been used to meet evolving consumer needs. Consumer needs around self-service , consumer needs around online payment , consumer needs about information requirements , consumer needs about comparing different products , services but mainly providing convenience to consumers when they avail the services. This is one area; we feel that we want to really invest in through multiple channels . So right from creating awareness about HCGs oncology capabilities online . Ours is a customer base which every year you have about 15 to 20 lakh new cancer patients in a population of 140 crores. Our ability to reach to them in traditional sales or marketing methods is very limited and will not be as productive. But today we know that patients want patients and their families want to learn about their disease , want to do research online , want to make second opinion and are willing to travel any corner of India or world if they feel that's where they get the right treatment. It gives us an avenue to reach out to our fragmented , thinly dense patient group in a very effective manner. Historically we have a very good track record. We treat more than lakh patients every year. That patient base is spread all over India and the word of mouth has helped. In addition to this , this is another thing that we are deploying. The other part is we know that the mix around digital is it mainly a domain of millennial male in metropolitan cities. We know that post -COVID, the rural users of internet have surpassed the metro users of internet. We know that people with age group of more than 45 , that group is as big as millennials which is 20 to 35 in terms of internet usage. So, things are changing. The internet penetration is almost doubled over the last couple of years and now half of India's population is covered under that , it will continue to grow. We feel that with our kind of differentiated and specialized products it gives us a very good channel to reach out to masses across India , in fact across world through digital channels. So that's one thing. Second is the cancer patient journey starts before HCG and will continue while they are getting treated at HCG. They will always have this sword hanging Healthcare Global Enterprises Limited
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over them and they will have to do follow ups once they finish the treatment for rest of the life . Through digital asset it gives us the ability to make sure that their journey before HCG to during HCG and after HCG is convenient and we can help them by giving them the right advice , doing a follow up with them , sending reminders , bringing them back because early detection gives them best chance to improve their outcome. There are several aspects of this initiative. We've started working on it. We are seeing some early green shoots and we feel that with changing consumer behavior I think this will become a growing channel for us going forward.

Ashay Jain: Second question , when can we expect the new centers perform as the matured centers and will be at what scale and within what time frame?

B.S. Ajai Kumar: Sorry can you repeat your question again?

Ashay Jain: My question is when can we expect the new centers perform as the mature centers and at what scale and within what time frame?

Raj Gore : As you see , this time we have given a breakup between existing and mature centers , sorry existing and new centers. You see how the traction on new centers has been , a year-on-year growth has been fantastic , occupancy rates are going up. This bucket was loss making , now its EBITDA positive. One by one we 've turned all centers around. Kolkata is our newest center. We expect

that to be turned around this year. I think we are on the right force . Unfortunately for us the 2 years of COVID pandemic slowed down our ramp up on these new centers. But as soon as the COVID is going away we've seen immediate acceleration in this turnaround story. We expect one negative EBITDA center Kolkata, the newest center to be turned around this month. And ramp up has been fantastic. You see Nagpur ramp up , you see Jaipur , Jaipur has done fantastically. We are very confident that in next 12 to 18 months this bucket will start getting pretty close to existing tenants.

Moderator: Next question is from the line of Raj Joshi from Ace Securities.

Raj Joshi: What will be our strategy for expansion in the form of greenfield , brown field and inorganic? Can you elaborate on the same and what will be the CAPEX guidance for the next coming 2-3 years?

Raj Gore : We have refrained from giving a CAPEX guidance going forward. However, on your greenfield questions , today we have two of Mumbai centers as well as Kolkata centers which are greenfield centers for us. We are in process of ramping them up. We have also further announced two centers. One is an extension center in Whitefield for our center of excellence which is a greenfield project. Our Ahmedabad center of excellence has peaked out in terms of utilization. We are relocating in a newer site with an expanded capacity. That's another greenfield project we are doing. I think beyond that we will focus on inorganic acquisition route. As an event we find targets that we feel fit in our strategy , we feel that we can either ramp them up quickly or Healthcare Global Enterprises Limited
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turn them around quickly and we can get it at a good valuation in a value accretive manner. So that's our plan for brownfield growth. As and when we have more definitive developments , we will share it with you.

Raj Joshi: What are the steps taken to improve the revenue and the margin from the new center , especially from the Mumbai and Kolkata center ?

Raj Gore : So again both of these centers were relatively new before we entered into COVID pandemic era. The playbook for us has been very simple. Get the right clinical talent , introduce more treatment capabilities , more technology , enhance the customer base by your go to market activities with institutional tie-ups, with TPAs, insurance , PSUs, different pair channels , bring for Mumbai - Kolkata especially. I mentioned it earlier , prior to COVID we had only Bangalore as a destination to bring international patients. Mumbai and Kolkata are very well connected across the world. We are already as a result we are able to bring in international patients to these two more locations. That has helped us to improve our international business 1.4 times the pre - COVID level. We will continue to drive that growth going forward. The playbook is pretty simple for turning around these new centers.

Moderator: The next question is from the line of Priyanka Gandhi from Ace Capital.

Priyanka Gandhi: I just have a couple of questions. You mentioned about cost rationalization but can you quantify on the same , what will be the uptick in margins due to the decrease in cost and increase in efficiency?

Raj Gore : As I mentioned , we started this project with our Bangalore cluster sometime a couple of quarters ago. We've already seen just in Bangalore cluster we've seen about 20 to 30 basis point improvement in our margin in Quarter 1 alone. We will take this initiative to more locations. We expect about 100 to 200 basis points improvement in our margins as we roll out these initiatives across all centers and ramp them up to their full capacity or potential.

Priyanka Gandhi: My next question being scalability and industry growth for cancer ; can it be scaled up and across the country as a multi-specialty hospital brand?

B.S. Ajai

Kumar: As a multi speciality what? Can it be scaled up as what?

Priyanka Gandhi: Can it be scaled up across the country as a multi-specialty brand hospital ?

B.S. Ajai Kumar: We are very focused as an oncology brand. We don't feel need to really incorporate multi-specialty because when you look at oncology being focused factory . That is how we have grown from a very small enterprise in 2005 -06 to this level because we always believe that focus factory approach is what drives the center of excellence , the quality outcome and that attracts real cancer patients to come to the center of excellence . On a multi-specialty , as you know, it's usually an

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implant model where they do cardiac surgery to renal to oncology. The oncology itself rarely attract patients to come for oncology care where exceptions are there. Usually in a multi speciality patient goes with a symptom of abdominal pain, lump in the breast and ends up being cancer and gets the treatment then. But that issue is there for all the time. That is why we don't have like 120,000 patients driving through this. Through oncology is what we see there is no way multispecialty can drive that number of patients. So, we are very clear in our focus and that is how we have driven it. In future also categorically we are going to drive multi-specialty only and we feel there is enough, sorry single specialty only and we have enough need for it. Like if you look at HCG itself today, we have nearly 50 crore people can access but there is a lot of need for it to expand. We are not even in certain parts of India and also within where we are further expansion can happen there including in Tier 2 and we are one of the few which are in Tier 2, Tier 3 cities also in oncology which is not there in many specialties. We are very clear and we are very positive for the future growth of oncology specialty.

Raj Gore: Just to add to what doctor said, Priyanka if you look at healthcare is usually characterized as demand supply gap sector in India and that demand supply gap when you talk about oncology is even higher. Now you look at what HCG has been able to achieve. We already have 22 cancer centers across 19 cities, 9 states that's scalability. The market has been growing over last few years with a CAGR of about 12%. We are growing about 15% CAGR. That's HCG's brand pull, that's HCGs differentiated and specialized product. The fact that we've been able to achieve this with an oncology focused care model in big cities in Tier 2, Tier 3 cities with double digit revenue growth which is higher than market growth at (+20%) EBITDA in our mature centers with high teen ROCE, as higher EBITDA in our center of excellence more than 25%. It explains the scalability, viability and strength of HCG's unique model. I think the opportunity for growth is plenty out there for HCG. We are a market leader on many parameters and we will continue to be market leader and consolidate our position going forward.

Moderator: The next question is from the line of Rishabh Parekh from Sunidhi Securities and Finance.

Rishabh Parekh: My question was around the mature centers. Now that we are at 25% EBITDA margin and Rs. 40,000 ARPOB, 65% occupancy, what is the growth we can expect from a mature center? What are the levers that we can use to drive growth?

B.S. Ajai Kumar: See the growth in the mature centers, certainly there is the spike because of these are all becoming centers of excellence or hubs where new technology will be put in, new areas of diagnosis, genomics and others will take place. Because of that new talent acquisitions will happen and I look at for example putting in mature centers. There are a lot of mature centers don't have robotic surgery, may not have very high-end therapy and genomics. As we do this obviously the rev

enue impact will be higher and occupancy may increase up to 75%-80%. But more than occupancy the footfall will increase, and the radiation load will increase, the outpatient chemotherapy load will increase, our cost will increase. We are seeing like in our Bangalore center our ARPOB is 66,000. So, there is an opportunity for us to reach the 66,000.

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So, we have to aim for the gold as they say. Because nowadays patients also look for high end oncology centers near them. I think the fact we are building a very big center in Ahmadabad itself is a credit to that that how opportunity exists for us to expand the existing centers as hubs and even we have even talked about expanding in Cuttack making it a center of excellence how it has grown. There are and of course Mumbai I think in the next few years will become a center with clinical talent acquisition and all happening. I think the mature centers I believe still have an opportunity for growth. Particularly what Raj talked about digital coming in, all of this will only enhance the footfall and definitely it can increase. We are very positive on the mature centers growth in the future.

Rishabh Parekh: That's very helpful. Outlining the headroom that the mature centers have to grow. My second question is on ARPOB. Does the base quarter which is Q1 FY22 have the impact of any vaccination revenue which was not there in this quarter hence optically the ARPOB increase was only about 5%-6%?

Raj Gore: No that's not the reason. In Q1 we did not have any vaccinations, vaccinations primarily started from Q2.

Rishabh Parekh: Okay so Q1 FY22 did not have vaccinations.

Moderator: Thank you. As there are no further questions from the participants, I would now like to hand the conference over to Mr. Raj Gore for closing comments.

Raj Gore: So, thank you everyone for joining the call. We will keep updating the investor community on a

regular basis for incremental updates on HCG. I hope we've been able to address all your queries. For any further information kindly get in touch with us or Strategic Growth Advisors, our Investor Relations Advisors. Thank you once again. Stay safe.

Moderator: Thank you. Ladies and gentlemen on behalf of Healthcare Global Enterprises Limited that concludes this conference call. Thank you for joining us and you may now disconnect your lines.

Call: Nov 2022

November 17, 2022

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on November 11, 2022

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(oa) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on November 11, 2022, with Analysts/Investors to discuss the Unaudited Financial Results of the quarter and half year ended September 30, 2022 .

This is also available on the website of the Company www.hcgoncology.com .

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited Q2 FY-23
Earnings Conference Call"

November 11, 2022

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN ,
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CEO, HEALTH CARE GLOBAL
ENTERPRISES LIMITED
MR. SRINIVASA RAGHAVAN – CFO, HEALTH CARE
GLOBAL ENTERPRISES LIMITED

Healthcare Global Enterprises Limited
November 11, 2022

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Moderator : Ladies and gentlemen good day and welcome to the Q2 FY23 Earnings Conference Call of Healthcare Global Enterprises Limited . This conference call may contain forward looking statements about the company which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict. As a reminder all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes . Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Dr. B.S. Ajaikumar – Executive Chairman, Healthcare Global Enterprises Limited . Thank you and over to you sir.

B.S. Ajaikumar : Thank you very much. I'm sorry about this disturbance. As I was saying I'd like to take few minutes to elaborate on new technology we have launched at our Center of Excellence , Bangalore. The technology is called ETHOS Varian and it helps perform adaptive radiotherapy and it's one of the first few in the world we have established here now at our Center of Excellence. Adaptive radiotherapy is something which continuously probes deep into the tumor and intelligently adopts the treatment to the tumor configuration using an AI platform . We can use the actionable information generated from it in the treatment of subsequent patients. Our area of interest is to collect proper data and see how a tumor responds to the treatment. This technology will finally help us answer some critical questions like when do we get a complete remission, how long should the treatment's span be, questions that have challenged radiation oncologists for several decades.

Being a leading cancer care provider , we are deeply engaged in academics and research. I'd like to highlight our seminal paper publications. Talking of academics , we are very proud to announce that we have fellowship programs of over 170 fellowships and different DMV programs and all of our programs are in high demand , not only in Bangalore but also importantly in Tier-2, Tier-3 cities. As we all know in the past it is very difficult to ensure high end training in Tier-2, Tier-3 cities and also to find personnel who are willing to relocate and practice there. But we are very fortunate that now even in Tier-2, Tier-3 cities like even Ranchi , Vizag, Angul , we have fellowship programs , DNB students for training which undoubtedly augurs well for the community of these regions. We take research very seriously. Till date we have published close to 756 research papers. For October alone we have 9 publications and more importantly a significant number of them have been a podium presentation . This clearly reflects a high quality of work which is one of the best in oncology globally not just in India. We have some doctors who have also written key chapters in textbooks , we have one who has won Best Paper Awards and been in top in the clinical research abstracts. Recently we participated actively in a radiation immune conference where we won one of the best awards for it globally. This is just of our achievement in academic and research and going forward we will continue to ensure a seamless integration of clinical services , academics and research such that all our - breakthroughs to serve the large purpose in the form of better treatment outcomes and improving the quality of life for our patients. The future is very bright for HCG across all aspects of cancer management.
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We will continue to strive hard to make cancer a chronic

c disease and bring higher success to

HCG. Thank you very much. Now I hand over to Mr. Raj Gore, our CEO.

Raj Gore: Thank you so much Dr. Ajai. A very warm welcome to all the participants on the call. We are delighted to share yet another quarter of good performance. We've been delivering growth on year-on-year basis as well as sequentially for the last eight consecutive quarters now and this quarter is no different.

We are happy to report another strong financial performance for quarter ended September, 2022.

Our consolidated revenues for Q2 stood at 420 crores a growth of 19% on YOY basis. This strong revenue growth coupled with our focused efforts on cost rationalization has resulted in year-on-year margin expansion of 130 bps leading to adjusted EBITDA margin of 19.3%. Our adjusted EBITDA for Q2 FY23 stood strong at Rs. 81 crores a growth of 28% over Q2 FY22. As a result, our Q2 FY23 profit after tax on a pre-Ind-AS basis stood at Rs. 10.54 crores, up by 137% on YOY basis. Over the last few quarters, we've been regularly informing you about our efforts to drive growth on several fronts like enhancing our clinical services portfolio, increasing our clinician bandwidth and our go to market initiative to increase reach online and offline, retail and institutional accounts, domestic and international fronts. We are making encouraging progress on all these fronts resulting in a steady growth in new patient registration, higher utilization across all modalities of treatment across metro and non-metro market. While our mature centers continue to show higher than market growth rates, the growth strategies implemented for the emerging centers has started showing promising results. Kolkata and Mumbai centers have grown by 40% and 30% respectively on year-on-year basis in current quarter. I am happy to highlight here that our Jaipur center has more than doubled revenue on a year-on-year basis with more than 25% EBITDA margin, our two linear accelerators there are nearing full capacity utilization and we will be commissioning one more linear accelerator early next year. Our differentiated and specialized cancer care along with strong brand positions have enabled us to attain leadership positions in 13 out of 18 locations where we are present. Going forward we will continue to invest in HCG brand to make it the most preferred choice for cancer patients across India. We are very optimistic of improving our market share and strengthening our leadership position going forward. Now I would like to hand it over to Srinivas, our CFO.

Srinivas Raghavan: Thank you Raj and very good morning to everyone. We have uploaded our Q2 FY23 financial results and updated investors presentation on the stock exchanges and company's website and I hope everybody has an opportunity to go through the same. We are delighted to share that we have been able to grow our revenues ahead of the industry growth due to the trust and brand created for HCG. On the revenue front, our consolidated revenues for Q2 FY23 stood at Rs. 420 crores as compared to 350 crores in Q2 FY22, a growth of 19%. Our revenues for H1 FY23 stood at 828 crores, a growth of 23% year-on-year. Revenue split between HCG and Milann stood at 96% and 4% respectively for Q2 FY23. Revenue growth for HCG stood at 21% year-on-year and for Milann excluding for covid revenues in Q2 FY22 stood at 15% year-on-year. Healthcare Global Enterprises Limited
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As mentioned in slide # 25 revenue from the mature centers stood at 309 crores, a growth of 19% year-on-year basis for Q2 FY23. Revenue from emerging stood at centers 95 crores, a growth of 26% on year-on-year for Q2 FY23. We are delighted to state that our emerging centers are inching towards maturity and are seeing good traction across geography.

I now request your attention to slide #26 where we have

disclosed our operational parameters

across our mature network and emerging centers for Q2 FY23. Our company wide AOR stood at 66.4% and AOR for mature versus emerging centers stood at the 65% and 69.9% respectively. Higher occupancy for emerging centers is due to 72% of beds were operational in new centers. AOR on capacity beds stands at 57%. Our ARPOB on company level stood at 36,914 and our ARPOB from mature centers stood at 39,684 and for emerging centers stood at 30,145. The de-growth of AOR in our emerging centers were majorly attributed to a couple of centers in the emerging markets growing at a very fast pace with higher share of institutional business as compared to business from out-of-pocket patients. However, we believe this is a temporary phenomenon and should stabilize and we will see increasing ARPOB from our emerging markets as well.

Across geographies we have given our revenue break-up in slide # 27, Jaipur grew by 205%, revenues from Rajkot grew by 80%, Mumbai grew by 30%, Bangalore Center of Excellence grew by 35% year-on-year for Q2 FY23. Our Milann business is also doing well. Revenues have increased by 15% in Q2 FY23 on YOY basis excluding the vaccination revenue for Q2 FY22 on a like to like basis and new registrations grew by 14%. On the EBITDA front, our consolidated reported EBITDA stood at 74.7 crores as compared to 61.7 crores in Q2 FY22, a growth of 21%. Reported EBITDA for H1 FY23 stood at 146.9 crores, a growth of 30% year-on-year. Adjusted EBITDA for Q2 FY23 that is after adjusting the one-time value creation cost

and adjustment of ESO P expenses stood at 81 crores as compared to Rs . 63.3 crores in Q 2 FY 22, a growth of 28% . Adjusted EBITDA margins stood at 19.3% as compared to 18% in Q2 FY22, a growth of 130 bps. Adjusted EBITDA for H1 FY23 grew by 39 .6% year-on-year with margins at 18.9%, a growth in margins of 230 bps. We have also given bifurcation of EBITDA across mature and emerging centers and I would request the participants to view slide #25 for further details .

On profit after tax, we have been delivering positive PAT for the last three quarters now. PAT for this quarter stood at 7.4 crores as compared to 0.8 crores in Q2 FY22.

H1 FY23 PAT stood at 13.4 crores as compared to a loss of 8.7 crores in H 1 FY22. Our PAT pre-Ind-AS adjustments for Q2 FY23 stood at 10.5 crores as compared to 4.4 crores in Q2 FY22. PAT for H1 FY23 pre -Ind-AS adjusted stood at 19.7 crores as compared to a loss of 1.5 crores in H1 FY22. ROCE for matured networks stood at 20% annualized for H 1 FY23 as compared to 15.4% in FY22, a growth of 460 bps. ROCE before corporate allocation for mature centers stood at 24.8%. ROCE for emerging centers stood at negative 4.9% , annualized for H 1 FY23 as compared to negative 8.3% in full year FY22. This is again an improvement of 340 bps. ROCE before corporate allocations for emerging centers stood at negative 0.9%. Our net debt position excluding capital leases as on 30th September stood at Rs. 211 crores as compared to 190 crores
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as on March 31st, '22. Our expansion of existing facilities at Ahmedabad Phase 2 and Whitefield Extension of Bangalore COE is on track. Total planned CAPEX for Ahmedabad is 85 crores, expected date of operations being Q 1 FY25 and for Bangalore COE is 25 crores, expected date of operations being Q 4 FY24. With this I would now like to open the floor for question and answers.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Karan from Surana .

Karan: My first question would be on the 5 crores consulting cost that we booked in Q 2. Can you just give us some color on where this 5 crores consulting cost has been spent and what kind of

benefits are we going to see from this? That's my question number one. My question number two is that we see some sequential decline in our emerging centers revenue and some decline in ARPOB as well. Can you just give us some more color on how do we see this shaping up for us and what exactly are we doing in our Bombay and Kolkata center? We said that we are going to get new talent in and a new surgical head over there. Can you give us some color on both of these centers as well? Thank you , sir. That's it from my side.

Srinivasa Raghavan: Thanks for the question. I'll take the first question ; regarding this 5 crores that we are talking about , this is regarding the value creation activities that HCG has embarked upon. We are working on two things. One is how do we drive productivity and efficiency across the system? That is one line of activity. The second area that we are working on is on the digital front as to how we can use technology platform and digital platform to drive revenue. These are the two activities that we have embarked on and as we see the progress is happening and the cost of 5 crores is towards these activities. In terms of your question in terms of what kind of benefit this would kind of entail or result in , we expect this should result in the next year a profit overall EBITDA improvement of about 100 to 150 bps basically on the EBITDA Margin . On the second part of the question Raj would like to answer .

Raj Gore: If I understood your question , one was about emerging centers ARPOBs . As Srinini mentioned in his opening remarks , that when you open a new hospital the first goal is always to get the utilization up on different modalities. So usually , you drive more institutional business and quarter -on-quarter that mix sometimes can change between different hospitals. We're not concerned about it. We think it's a one -off and it will get back on track going forward. Can you repeat the third question that you have ?

Karan: On our strategy in both in Bombay and Kolkata centers, so what exactly are we

Raj Gore: As discussed in the past also, Kolkata and Colaba in Mumbai are our newest centers. As soon as COVID went away we started doing our go to market initiatives . We started increasing our clinician bandwidth , improving our clinical services portfolio. Those are showing results. As I mentioned in my opening remarks , Kolkata has grown 40% year-on-year, Mumbai hospitals
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have grown 30% year-on-year. We're very confident that as in subsequent quarters we will continue that growth momentum.

Karan: Just a follow up on the consulting cost , will we see this 5 crores continuing in Q3 and Q 4 as well or will this now start trending on a downward trajectory in Q 3-Q4?

Raj Gore: So, it will start coming down, by end of this financial year we expect it to go away.

Srinivasa Raghavan: In following year, next financial year this will not be there. That's why we identified it as a one-time separate expense.

Moderator: The next question is from the line of Kaustubh Pawaskar from Shrekhani By BNP Paribas.

Kaustubh Pawaskar: My question is on margin front. Of the total material, medical equipment how much is the imported equipment because what I'm trying to understand is because of this rupee depreciation will it have any impact on your cost element and that might result in margins coming lower in the coming quarters?

Raj Gore: No, as far as rupee depreciation is concerned there are two things. We don't see any margin impact because the agreement we have with some major suppliers or as we have already indicated in the past or the pay per use model. Also, we do earn lot of foreign exchange which is a natural hedge. So because of these two things we don't see any margin impact from the procurement of the equipment.

Srinivasa Raghavan:

Secondly, we do not have any dollar exposure.

Kaustubh Pawaskar: Just to understand the question previous participant asked about the ARPOB remaining flat. This is like a one quarter impact, you might see ARPOB coming in better in the subsequent quarters?

Raj Gore: Absolutely Kaustubh, thank you for asking that question. The emerging center is a bucket. You also have Mumbai there. You also have Kolkata there. Kolkata ARPOB is almost close to 50,000 per day. Mumbai ARPOB is in mid 50s. So as these three centers, two in Mumbai and one in Kolkata continue to grow in terms of their contribution to total revenue of emerging centers, the ARPOB will start going higher and higher in subsequent quarters. In addition to that these are also the two locations where we will get higher international business compared to other non-metro locations. That's another lever that we have to drive price realization as well as ARPOB. Usually what we do Kaustubh, the playbook for new hospital commissioning is, you open the hospital first you go for foot fall, first you go for higher occupancy, higher utilization on different modalities. You create a significant number of treated patients to drive word of mouth. And then you start optimizing different mixes to drive your realization ARPOB margin. So we are in that journey right now.

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Kaustubh Pawaskar: Thanks for the understanding. My last one is on as you said in your initial comments that patient registration is one of your key driver in terms of occupancy. On year-on-year basis can you give us some idea what was the increase in the patient registration? Because word of mouth and whatever technology you are bringing in. That will also help you to have more and more registrations going ahead. Any understanding on how it was in this quarter and any expectation in the quarters ahead?

Raj Gore: On a YOY basis last quarter we saw NPR registration growth in lower teens about 11%-12% at a network level. Obviously, it will vary from geography-to-geography and maturity of the center.

B.S. Ajaikumar: I think one of the things is we have not seen the full impact of our other initiatives like what Raj or Srinivasa mentioned. That impact is to be seen in the last quarter at the beginning of the next year.

Moderator: The next question is from the line of Dhara Patwa from Smifs Ltd.

Dhara Patwa: I just wanted to understand how much time does the emerging centers take to become mature? Like when can we expect East India hospitals to deliver margin of (+15%)?

Raj Gore: Thank you Dhara. Sorry if I got the name wrong. If you see the different hospitals are at a different maturity stage. I gave an example of Jaipur. Now Jaipur is already double the revenue has started delivering EBITDA margin (+25%). So Borivali similarly in Mumbai is delivering in mid -20s EBITDA margins. Kolkata is our newest center; we expect it to start breaking even early next year and then we will continue to grow that margin. 15% is probably the subsequent year, following the next year because that's our newest center.

Dhara Patwa: Wanted to understand like 88% of our revenues comes from oncology. Do we have any approx breakup like how much of that would be from medicine and how much will be from the surgical procedure?

B.S. Ajaikumar: Okay split by radiation business.

Dhara Patwa: Right.

Raj Gore: Usually we break our business in four big buckets. One is consultation and diagnostics which is about 20% of our top line, medical oncology which is chemotherapy, immunotherapy etc. that's about 35%, radiation oncology is about 20% and surgical would be about 25%.

Moderator: The next question is from the line of Karan Vora from Goldman Sachs.

Shyam Srinivasan: This is Shyam Srinivasan. Just one on slide #11. Maybe I have not seen the slide before but it

seems very interesting. Doctor can you just explain the non-metro versus metro? I think many of the other healthcare companies now are talking about going into non-metro locations but you

seem to have made a pretty decent job of it. Just if you could explain this slide please.
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Raj Gore: So, Shyam, at this slide what we are trying to say is look at healthcare opportunity in India, demand supply gap, accessibility, availability. We all know about it. But I think the bigger problem we have in this country is disparity. Most of our health care supply is in metro, big cities, state capitals. The real need to provide quality health care, bigger need to provide quality health care is outside big city. I think this is where HCG has a unique business model. What this slide basically says that we have 13 locations which are Tier-2, Tier-3, Tier-4 places like Angul like 2 to 3 lakh population, Shimoga a town, 2 to 3 lakh population. Even almost Tier-4 kind of a category. Now out of those 13 locations, 11 locations we have a number one market leadership in oncology. The way we have, the way the business model works is the result of our business model. Out of those 13 locations we are delivering EBITDA margins in the range of 15% to 28%. Some hospitals the lower side is 15% and the higher side is 28%. In terms of ROCE for these 13 locations the range is between 15% to 24%. That's because one we have an early mover advantage, first mover advantage, second our unit economics we are about 20%-25% lesser costs in terms of our CAPEX in these markets. These markets our operating cost also tends to be lower and then we go for more institutional business to drive higher volumes at relatively lower price points to make this model work for us. If you look at all the healthcare players in India I think HCG is probably the only one who has made this model work in different tiers of cities in India delivering pretty much similar EBITDA margins as well as ROCE returns across our locations. Shyam Srinivasan: Just to follow up, there Raj just on the peer mix which you also highlight, right. You take on more institutional, even government scheme patients. So ROCE you're mentioning greater than 15% still. But is there no worries around receivables from the government and how do you navigate that?

Raj Gore: I think when it comes to receivables, the trick is get your processes in terms of documentation, submission on time and follow up. Get that discipline in place. A lot of these schemes have gone now in terms of their processing online, you have portals to submit. There is better visibility coming in these channels. In fact, our DSO is consistently going down over last quarters for this particular payer mix in this market.

Srinivasa Raghavan: I think to add what Raj mentioned, Shyam this is Srinivasa here. Yes, while there could be some government challenges in terms of collection. But as Raj rightly pointed out net DSO is kind of coming down quarter-on-quarter. That's point number one. How did you say that this quarter ended September was our highest collection month in terms of overall receivable prices, so while there are challenges but there is clear focus in terms of how we drive collection and ensure that we stay on board.

Shyam Srinivasan: Last question just back on ARPOBs. I'm talking now only just matured center ARPOB. I think remember last quarter or before we did a rationalization exercise to look at how our prices are versus competition and if you are a leader, we have taken some price increases up. Is there something that, I know it used to be a 12-18-month exercise but do you think that could be a lever in the next 12-18 months also that you could do the rest of the portfolio where you have not done that exercise?

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Raj Gore: Absolutely. Just to recollect what we informed last time; we started that initiative from our Bangalore Center and we are in process of rolling it out to rest of the locations. In Bangalore Center alone we've had about 25-30 basis points impact on an overall EBITDA margin of the company by just rolling out this initiative in our Bangalore market. Now we are rolling it out as we have mentioned earlier, Srinivasa also covered it in his remarks that we are looking at about 100-150 basis point improvement in our EBITDA margin by end of once we finish the rollout of the initiative, obviously it will also help us to drive ARPOB. Our mix is very different because our locations are Bangalore market, Ahmedabad market which has our core home market where we have dominant market position for a longer period. Our ARPOB is around 75,000 per day. Mumbai as I mentioned is in mid 50s. Baroda is in mid 50s. Kolkata is almost close to 50. So as our center starts getting at a higher utilization level, we will continue to optimize mix, drive price realization, pricing as a lever and we'll continue to grow our ARPOB in the right direction. Shyam Srinivasa: Srinivasa, just following up there. So that 150 is like a 12-18 month journey or how should we look at that? Just from this activity.

Raj Gore: So, we've already got some impact. We've seen that at a quarterly level early, I think first quarter of next year we will start seeing that impact in our P&L. We are expecting the full rollout implementation of these initiatives across different hospitals by end of Q4 of this year.

Moderator: The next question is from the line of Abdul kader Puranwala from Elara.

Abdulkader Puranwala : Could you please elaborate on the status of our expansion plans at Whitefield and Ahmedabad because if I see sequentially the CAPEX incurred this quarter , it's flattish. So just some color on that?

B.S. Ajaikumar: In Ahmedabad we have started the new project. The work is going on. We expect the transition to happen by October of '23. So that is going as planned. As you know there are also certain transfer of technology , linear accelerator all of these developments . So, we hope that by October it will all be completed. It will be fully operational. Regarding the Whitefield, we have just got the approval from the local authorities , the BBMP so for the plans have been approved. We expect the project to be completed in a bout maximum 2 years that is by the 18 to 24 months once the construction work starts .

Moderator : The next question is from the line of Naman Bhansali from Perpetuity Ventures .

Naman Bhansali: You previously talked about some diversified revenue from various modalities. Could you please help me understand the margin dynamics in that re gion? That is the margin dynamics in the medical oncology , surgical oncology , in that part ? That's my first question.

B.S. Ajaikumar: We do not give margin dynamics by se rvice lines. Because unlike other specialities oncology services are integrated and majority of patient undergoes multi -modality treatment. So, that is why margin at consolidated level are more meaningful.

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Naman Bhansali: My second question was on the unit economics. So, you talked about 20% lower cost. What is the CAPEX per bed or CAPEX per center which you incur versus the industry?

B.S. Ajaikumar: CAPEX per bed, let me put it this, as far as the you're looking at Tier-II centers, right?

Naman Bhansali: Yes.

Raj Gore: Again, I would like to differentiate here. Bed capacity is not necessarily the main driver for our growth. We have four modalities OPD, diagnostic , chemotherapy , medical oncology , radiation.

These are all daycare or outpatient services ; only surgical largely is our inpatient. That mat

rix is

not necessarily relevant for us. However, if I have to give you a ballpark , our cost per bed in metro cities will be about 65 to 75 lakhs and in Tier-II, Tier-III cities it will be about 50 to 55 lakhs for a hospital size of about 80 beds.

I just want to address the previous question that we had. The reason we do not give margin by modalities is because it's not necessarily a driver for margin improvement. Cancer patients , when cancer patient walks in, we do not have a choice to the modalities that you need to treat that cancer patient , it's not a choice. It depends on what type of cancer , what stage of cancer so that's not necessarily a controllable variable for u s. It's more of a, it depends on what the patient 's state is. It doesn't make sense to track it that way because it's not really a variable that you can control or optimize or drive.

B.S. Ajaikumar: Just to add to what Raj said in a multidisciplinary appr oach what we have the team decides whether the patient should undergo chemotherapy or radiation or surgery or an integrated approach. Nearly 60% of the patients end up having all three modalities of treatment so it cannot really break up into which is a hi gh margin , which is low margin . We have to look it at a consolidated level. Perhaps what we try to look at is per patient , new patient what is it cost to do certain treatments combined so obviously there are ways the contribution factor may be more from radiation. So, we do look at all that but it is not the driving force for us to do the right treatment for the patient.

Moderator : The next question is from the line of Dipti Kothari from Kothari Securities.

Dipti Kothari: My first question was that we have been increasing in terms of revenues and margins , have also expanded over the last six to eight quarters. However, we are still not a high PAT generating company. What is your sense on P AT going forward?

Srinivasa Raghava n: If you go back to my earlier part of the thing, you know I talked about PAT before In d-AS impact. So let me set the context here. If you look at it my PAT at a pre Ind -AS level is close to 10.5 crores and if adjust for the one -time impact that we talked about on value creation , overall, PAT at a pre Ind-AS level is 14 crores . Let me explain couple of nuances in this . Point number one, basically because of the In d-AS impact the long -term lease rental model that we have.

Because of that my post Ind -AS PAT is getting depressed because of the initial year of the lease

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rentals , given the equalization of lease costs. But having said that over a period of time especially at the mid-way point this should kind of reverse. You got to look at the pre Ind -AS impact a s far as PAT is concerned. Secondly, I did talk about the value creation activities. There is one time cost of 5 crores that is sitting which is in the EBITDA and kind of flowing into the PAT number

also. If we exclude the impact of that my PAT should look better. Last but not the least since you asked about how is the future looking like we are currently at a tax regime of 35%. We are going to be moving to a 25% tax regime next year onwards. So, putting all these factors together on a pre Ind-AS PAT basis, my PAT is at about 14 crores for Q2 FY23, that's the way I would put it.

Moderator: The next question is from the line of Aditya Khemka from InCred PMS.

Aditya Khemka: Ironically my question is exactly the opposite of the previous participant. I see we have a lot of intangibles and obviously we do acquisitions and alliances that create intangible. But I don't see a lot of amortization of that intangibles as in we are amortizing either not amortizing or if we are amortizing, we are amortizing at a very slow pace and I understand the higher the amortization the more depressed the PAT will be.

But as an investor we don't really care about

PAT. We care about cash flows and the cash flows have actually been really healthy for the past 4-5 years and they continue to be healthy. So, my question to you is why don't we amortize intangibles at a faster pace so that our ROC -ROCEs can look better and we get tax exemptions, tax break on our intangible amortization and that helps us save cash. End of the day any company's job is to generate cash and PAT is just a number.

Srinivasa Raghavan: Yes, I understand. It's a good point that you are bringing up and it's quite interesting that you call it that. I also appreciate the fact that you talked about positive cash flow in the last few quarters which is a good thing. Yes, it's getting the right balance. Getting the right balance in terms of amortizing the intangibles. We are not aggressive, at the same time we are not very slow as well; we are doing it as per the law, as per the procedures. And intangible comes only in the case of new acquisitions. In the last few quarters, we had seen we had this Lab acquisition, we had this Suchirayu acquisition. Those are kind of created those intangibles but having said that it would be amortized on a basis which is that allows as per the companies that as well as, as per the accounting standard.

Aditya Khemka: Yes, but your accounting standard actually gives you a very large range of amortization. It is 10 years to 50 years that you can use to amortize intangible s?

Srinivasa Raghavan: I agree.

Aditya Khemka: My point is that instead of using 50 years or 40 years you could do it over 10 years?

Srinivasa Raghavan: Our philosophy is our amortization happens anywhere between 5 to 8 years. I think that's the kind of trend that we have been doing and that is the way we would move forward as well.

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Aditya Khemka: My second question is regarding the multi-specialty and the super-specialty makeup of our company. Obviously, I'm keeping Milan n aside here given that it's very small in the context of things. But the multi-specialty hospitals, your super-specialty obviously seems to be doing phenomenally well. On the multi-specialty side what is the plan? Do we plan to remain a company which is both multi-specialty and super-specialty or at some stage, do you have any restructuring in mind for either of these verticals?

Raj Gore: I think we have stated earlier also that our strategy is to be oncology focused organization. That's what HCG is synonymous for, that's what is our core competence, we would like to pursue oncology business going forward. Now as a legacy we have a few multi-specialty hospitals. Currently what we are trying to do is see how we can grow oncology specialty business within those multi-specialty hospitals. A good example of that is Bhavnagar. It was a purely multi-specialty hospital with pretty much no oncology business. Last few years we added a linear accelerator there, we started growing surgical and medical oncology, that specialty is growing there. We will do similar things at Rajkot. So right now, we will focus on growing the oncology share of business within this that's what our current focus is.

Aditya Khemka: Basically, your multi-specialty setups you are trying to tweak them to be more super-specialty within the multi-specialty. Is that how I should read it?

Raj Gore: Yes.

Aditya Khemka: But why not divest those assets?

Raj Gore: I think that is best for us in Bhavnagar. We're trying to replicate that in other locations also.

Aditya Khemka: But Raj you could actually divest those assets. If you look at the private market transactions in the hospital space, the multiples at which these transactions are happening are far higher than the your stock there. So, it is actually going to be value accretive for you if you divest to multi-specialty assets, get the cash, pay off the debt and use the cash to acquire super-specialty oncology assets?

vest to multi -

specialty assets, get the cash, pay off the debt and use the cash to acquire super-specialty oncology assets?

B.S. Ajaikumar: Yes, in this regard obviously this has been internally discussed quite a few times but our multi-specialty is very limited. If we have to look at it diversify and everything it requires for us to maybe put more emphasis on multi-specialty. we are looking at various models. What you have

suggested also has been discussed internally and obviously we will continue to discuss. As they say there is a time and place for everything. We'll look at it when the right time and see whether we should do some of these things what you're suggesting also.

Aditya Khemka: On the CVC stake in our company which is currently if I am not mistaken around 58% . Have they shared any timeline with you as to how long they plan to hold on to the land share of the company and the fund which has invested in your company? How long will they stay invested
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and when do they plan to liquidate and will the liquidation be one shot or multi stage staggered liquidation? How will that work?

B.S. Ajaikumar: This discussion has not taken place yet because it is too early possibly. They have been with us for little over 2 years now. Obviously, the time horizon will be 5 years or longer for any private equity. So, they are at this point obviously they have done well and they have been a continued support to us in our new whatever endeavors we have taken. It is a good partnership and at this point we want to really keep that good partnership and grow.

Aditya Khemka: One last question Dr. Ajai for you. A lot of the hospitals that we meet and speak to obviously all of them want to focus oncology as a specialty given the growing incidence of the disease and the lack of treatment options available in our country. My question to you is have you explored the opportunity of doing operating and maintenance contracts with many of these let's say standalone hospitals which might have oncology wing but may not be making much money or may not be doing very well in that wing because they just don't have the expertise or the knowledge or the technology to deal with the therapy area and that could be a very asset light model for you because it's just knowledge -based revenue. So, have you explored such opportunities? If yes what is the kind of feedback you're getting there , what is the traction in that business model?

B.S. Ajaikumar: As you know Aditya in the past, we have already looked at that models. We've also looked at the implant model . To begin with I would like to say the implant models are not workable for us because we want to be an independent dedicated cancer center. Even those asset light model what you talked about is not long lasting. The problem with this is maybe is the Indian DNA where the person through which you have got the contract at some point would like to say if we feel like we're doing good , we are generating profit they would like for us to exit they say we ourselves will do. We have seen that all happen a lot in Apollo model in the past as you know where there have been out. So, the model we feel just to take a minute and say strategically there is immense opportunity for us to grow in oncology and for us like what we have seen in Jaipur and other areas where went into market relatively competitive but just establishing a dedicated oncology center we have grown , Nagpur . We have this kind of significant opportunities , also for M&A we have some opportunities , we are exploring that seriously. So that is the way forward for us. Dedicated oncology centers either through M &A, Brownfield, to grow and also capacity utilization of our own centers. For example, some of our centers like what we discussed are capable of increasing the revenue significantly

with all the systems we put in place where they are at an inflection point. So, our focus will be on that. Our idea is to focus and bring these centers to the level we want. Even our Bangalore center , Whitefield coming up which will add significantly in the hub and spoke model we have . There is so much opportunities in strategic areas ; we want to focus on that. Of course, our focus is on quality of care to the patient. Being a dedicated oncology that is when we can deliver the real quality and real outcomes. So, this when we look at the clinical acumen we have , the type of doctors we have , 400 doctors across India and type of service we are doing and outcomes we are producing , research , academics. It has to be a dedicated oncology like what we have MD Anderson Sloan Kettering , that is the model we
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want to drive and that will give us significant revenue and upside . There is no doubt in my mind. I don't know investors may have something but I feel as a founder and entrepreneur there's absolutely no doubt in my mind how we will grow.

Moderator : The next question is from the line of Sabyasachi Mukerji from Centrum PMS.

Sabyasachi Mukerji: I have two questions. First is if I look at your mature d center's revenue profile and EBITDA margin pre-corporate expenses. The EBITDA margin pre-corporate expenses have been stable over the last six quarters at close to 25%. But if I look at your Y oY change in occupancy that has moved from 57.6% to 65% Q 2 FY23 and you have also I think there is an improvement in ARPOB as well of 5% Y oY. Why the margin has not improved , what is the reason behind this ?

Raj Gore: As we've been saying since last quarter the value creation plan that we have rolled out , we are incurring a one -time cost. Now that cost is largely allocated to mature centers in proportion of

their revenue contribution to the total revenue. You will have to take that away to see the impact. That's one explanation. If you look at if I'm correct the graph you are referring to the quarter -on-quarter trend, you have a 19% growth on revenue but only 20% growth on EBITDA. But if you look at, if I take away that one-time cost that 20% will become 26%. So, 19% revenue growth is contributing to 26% EBITDA growth for the corresponding period. That's the difference. It's a one-time cost.

Sabyasachi Mukerji: This you are saying that this will go away by the end of FY23 and FY24 will be able to see the full impact and full benefit of the cost exercise?

Raj Gore: Absolutely.

Sabyasachi Mukerji: That benefit would be close to 150 basis points you are saying. This 25% whatever we are seeing in the pre-corporate margins that will probably move to 26.5 roundabout. Is that understanding, correct?

B.S. Ajaikumar: Yes, roundabout yes.

Raj Gore: That's at a network level.

Sabyasachi Mukerji: 150 basis points you are saying on a network level?

Raj Gore: 100 to 150 bps at a network level, yes.

Sabyasachi Mukerji: My other question is related to that only. On the emerging centers if I heard you correctly Jaipur has been doing very well, probably operating at more than 20% or I think I heard you 25% operating margin, Mumbai Borivali is also operating at a good margin levels. Where do you see the operating margin trajectory of the emerging centers which is currently at 10% level. Where do you see this going in FY24?

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Raj Gore: Let me give you a trend to figure out how it will grow. Six-seven quarters ago that was at (-8%), today we are at (+10%) so that's a track record we have and like I said we will continue to drive revenue growth.

Sabyasachi Mukerji: I am aware of the trajectory. It has been very steep. The point I want to understand here is I believe the major drag in the emerging

centers; margin is Kolkata where you are kind of seeing

a 50,000 kind of ARPOB levels and eventually the emerging center's occupancy has almost touched 70% this quarter. Eventually going ahead once the occupancy level settles then probably the margin levels will go up. That is the thought process. I was coming from there and that is why the 150-basis points improvement in the network level margins I look little conservative if I say so?

Raj Gore: Let me clarify. 100 to 150 basis points impact is just on those initiatives that we have launched.

But we are driving higher than market revenue at a higher than market growth rate. The operating leverage will kick in. Our endeavor is in next 18 to 24 months, our endeavor is to get emerging center bucket, PAT coming closer to mature center bucket. That's the aspiration that we are driving for.

Sabyasachi Mukerji: The total impact in margins would be lot higher probably in the range of 300 to 400 basis points, right?

Raj Gore: That's what we are going for.

Moderator: The next question is from the line of Pallavi Deshpande from Sameeksha.

Pallavi Deshpande: I was just a little late in joining the call. On the 5 crores one-time expense, if you could just elaborate who has just been, who is the consultant and I think you mentioned it will continue in the balance two quarters, is that right?

Raj Gore: Just to recap. End of last year we engaged one of the Big Four's to start driving operational efficiencies. Operational efficiencies on multiple levers. One was strategic pricing based on our market positioning in terms of market share, competitive strength, our internal economics, utilization rates et c. So that's one component. This is more over and above the inflationary price increase that one takes every year. Second is staffing productivity, staffing norms, dynamic staffing based on variability and utilization rates in different hospitals across different seasons. Third is the fixed cost that we have, the long trail of items in the mid-line that we have. Then you have areas of improving, reducing our discount, revenue assurance et c. There are different levers that we are working on with the help of one of the Big Four's and we expect that we will roll out all those initiatives by end of the year and this cost will go away starting from next year.

Pallavi Deshpande: We can expect a similar cost in the second half 6 crores, what we saw in the first half?

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Raj Gore: It will start tapering down in remaining quarters of this year and it will completely go away by end of this year.

Pallavi Deshpande: Secondly you mentioned about the ARPOBs in the Mumbai property I think and by when do we

see them ? Any timeline for it to catch up with what you have at Ahmedabad?

Raj Gore: See that ARPOB is a function of high -end work , complex work that you do , your specialty mix , it's a function of your payer mix , it's a function of your market position where you can command premium and drive your mix optimization. That's why these centers are in emerging centers so eventually it will happen . Anyway, even if you compare mid -50s is a good ARPOB, even if you compare it with multi -specialty , there are not too many players who are more than 60 K per day. I think as the centers matures it will happen. Sure, we can give timeline for that but our endeavor is to go there as soon as possible.

Pallavi Deshpande: The South M umbai lease , it's for how long with the Tatas ?

Raj Gore: I think another 9 years but it's mutually renewable agreement .

Pallavi Deshpande: That center only directs the patients to Borivali, right? It's not a...

Raj Gore: No, it has everything and more than what Bo rivali has in terms of certain expense. That's the only other center other than our flagship at Bangalore which has a Cyberknife . It

also has Tomo.

In many ways the radiation technology that we have is best in class in entire Western India.

Moderator: Thank you . Ladies and gentlemen , due to time constrain s we take that as the last question. I now hand the conference over to the management for the ir closing remarks. Over to you sir.

Raj Gore: Once again I take this opportunity to thank everyone for j oining the call. We will keep updating the investor community on regular basis for incrementing updates on our company. I hope we have been able to address all your queries . For any further information kindly get in touch with us or Strategic Growth Advis ors our Investor Relations Advis ors. Thank you once again. Have a good day.

Moderator : Thank you . Ladies and gentlemen on behalf of Healthcare Global Enterprises Limited that concludes this conference. We thank you all for joining us and you may now disconnect your lines.

Call: Feb 2023

February 16, 2023

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on February 09, 2023

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(oa) of SEBI (Listing Obligation and Disclosure Requirements) Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on February 09, 2023, with Analysts/Investors to discuss the Unaudited Financial Results of the quarter and nine months ended December 31, 2022 .

This is also available on the website of the Company www.hcgoncology.com .

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited
Q3 FY '23 Earnings Conference Call"
February 09, 2023

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – WHOLE -TIME DIRECTOR AND
CHIEF EXECUTIVE OFFICER – HEALTHCARE GLOBAL
ENTERPRISES LIMITED
MR. SRINIVASA RAGHAVAN – CHIEF FINANCIAL
OFFICER – HEALTHCARE GLOBAL ENTERPRISES
LIMITED

Healthcare Global Enterprises Limited
February 09 , 2023

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Moderator: Ladies and gentlemen, welcome to the Q3 FY '23 Earnings Conference Call of Healthcare Global Enterprises Limited. This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

Now I hand over the conference to Dr. B.S. Ajai kumar, Executive Chairman of Healthcare Global Enterprises Limited. Thank you, and over to you, sir.

B.S. Ajai kumar: Thank you and good evening, and welcome, and a warm welcome to all present on the Q3 FY '23 Earnings Conference for Healthcare Global Enterprise. Today, I'm joined by Mr. Raj Gore , our Whole -time Director and Chief Executive Officer; Mr. Srinivasa Raghavan, Chief Financial Officer, besides the few members of the senior management team to share our operation and financial highlights for the quarter ended December '22. HCG has been not only a cancer destination for critical services but also a destination in research and academic excellence. We have been being very successful in gaining our hub -and-spoke model across the country and I'm happy to report that we have made long strides in academics and research where we nearly have 150 students strength present spread across -- so we have been very successful in creating a hub -and-spoke model across the country, and I'm happy to report that we have made long strides in academics and research where we have nearly 150 students at present spread across various oncology programs in Bengaluru and nationwide.

Recently, we have published the third edition of the dedicated journal named Precision Oncology, talking about the closing gap in cancer care, including 1 of the kind general called antimicrobial policy in cancer management with our own data, which will help to treat cancer patients with infections the right way for the first time in our own country.

We have collaborated with Intuitive to augment the accessibility of new -age surgical technology and robotic -assisted surgery across India, including non -metro cities by now going in the installation of the fourth -generation da Vinci surgical robot US-based Intuitive in Vadodara, Mumbai, Kolkata to make this technology accessible to a wider population in the country to further yielding fruitful results with better clinical outcomes. Our Ahmedabad center, I'm proud to say, has completed 700 robotic surgery, GI and hepatobiliary thoracic oncology department completed 500 robotic surgery, possibly the highest type of surgery in the country as far as we know.

In the academic and research, we are proud to announce that our doctors, Dr. Lohith Reddy and Dr. Krithika from Bangalore have been named in the prestigious National Cancer Institute of Healthcare Global Enterprises Limited
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USA. as working committee members in their immune division. This kind of recognition is taking HCG to newer higher levels globally. In clinical trials, we have been one of the few in the Phase 1 trials and several other investigator -initiated trials are also ongoing. There are many more studies in the pipeline go through a lot of iterations and the guidance of our solid ethics committee.

We have taken a leadership role in genomics -driven tumor boards and profile complete 500 gene sequencing for over 1,500 patients, which is

the largest in the country to the best of our knowledge. This has given insights into patient-centric approach, particularly for advanced and recurring tumors, not only from India, but from Africa and Middle East, making HCG a destination of cancer care. With a wealth of data emerging from cancer genome studies and data lake, which we have created, we feel this is just the beginning of our journey in the precision medicine trying to win the war on cancer, not only in early stage, but in advanced cases to convert cancer to chronic disease.

I now hand over to Raj Gore to take you through the strategic initiatives and financial highlights. Over to you, Raj.

Raj Gore: Thank you, Dr. Ajai. A very warm welcome to all the participants on the call. 4th February was the World Cancer Day, and this year's theme was Close the Care Gap. Being an oncology player, I would like to spend a couple of minutes on this important matter. The global burden of cancer is growing steadily. India is no exception to this trend. More people died due to cancer than COVID in last 3 years across the world. In 2020, there were 8 to 9 lakh deaths due to cancer in India, an estimated economic burden in terms of GDP losses is in the range of US \$11 billion, that is 0.4% of national GDP in 2020.

I would like to highlight four major gaps in India that we must address. First, awareness and prevention gap. While awareness levels about tobacco-related cancers is high in India, we still need to create awareness about other risk factors such as HPV, alcohol and obesity and drive implementation of prevention measures. Number two, screening and diagnosis gap. Only 1% or less than 1% of India's population is covered under screening programs for major cancer types, such as oral cancer, breast and cervical cancer, and more than 2/3 of cancer patients get diagnosed late at advanced stages of the disease. Number three, access to treatment gap. Population living in more than 70% districts in India do not have access to comprehensive cancer centers.

Of the 480 comprehensive cancer care centers available in the country, about 40% are concentrated in metros and state capitals. We need to add another 570 comprehensive cancer care centers by 2030 to meet the demand. Fourth, outcome gap, challenge of growing incidences is further intensified due to suboptimal outcomes compared to global counterparts across all major cancer types, due to all other gaps. While these gaps and challenges seem daunting, together, we can change that by forging alliances, uniting our voices and taking action.

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HCG has been a pioneer and the leader in addressing all these gaps with our 21 comprehensive cancer centers, largest in India. With 2/3 of these centers in nonmetro locations, we have been making quality cancer care more accessible and affordable in India since our inception. We have not only provided latest technology and treatments available across the world, but also been able to deliver outcomes compared to leading institutions across the world at a fraction of cost as acknowledged in a Harvard Business case study.

Coming back to the quarterly performance. We are very happy to report yet another strong financial performance for quarter ended December 2022. We have continued our streak of highest-ever revenue for the eighth quarter in a row and highest-ever EBITDA for 7 quarters in a row now. Our consolidated revenue for Q3 FY '23 stood at INR 425 crores, growth of 19% year-on-year. Over the last few quarters, we have been regularly informing you about our efforts to drive growth on several fronts, like enhancing our clinical services portfolio, increasing our clinical bandwidth and our go-to-market initiatives to increase reach, online and offline, retail and institutional accounts, domestic and international funds.

We are making encouraging progress

on all these fronts, resulting in a steady growth in new patient registrations, higher utilizations across all modalities of treatments across metro and nonmetro markets. In our emerging geographies, our priority is to create scale and grow revenue with an objective to improve capacity utilization and create higher visibility for HCG. In this regard, the company continues to make operating investments in engaging quality clinicians and brand building.

We have started implementing few modules of our digital platform across key functions of patient services, sales and marketing successfully. We have recorded highest-ever revenue in both digital and international business. On the margin front, we will be able to improve our margin from 17% in FY '22 to adjusted EBITDA margin of 19% in Q3. Our efforts during the last 12 months on pricing optimization and cost rationalization has started to yield and major benefit will flow in FY '24. We are confident of maintaining our leadership position in our mature markets and gaining market share and leadership position in our emerging geographies.

With this, I hand over to our CFO, Srini, for financial highlights.

Srinivasa Raghavan: Thank you. Good evening to all of you. We have uploaded our Q3 FY '23 results and updated investor presentation on the stock exchanges and company's website, I do hope everybody had

an opportunity to go through the same. We are delighted to share that we have been able to grow revenue ahead of the industry growth due to the trust and brand created for HCG. On the revenue front, our consolidated revenues for Q3 FY '23 stood at INR 424 crores as compared to INR 358 crores in Q3 FY '22, a growth of 19%. Our revenues for 9 months FY '23 stood at INR 1,252 crores, a growth of 21% year-over-year. Revenue split between HCG and Milann stood at 96% and 4%, respectively, for Q3 FY '23. Revenue growth for HCG stood at 20% year-on-year and for Milann, stood at negative 1% year-over-year.

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As mentioned on Slide 8, revenue for the matured centers stood at INR 306 crores, a growth of 17% year-on-year basis for Q3 FY '23. Revenue from emerging centers stood at INR 101.5 crores, a growth of 29% on a year-over-year basis for Q3 FY '23. We are delighted to state that our emerging centers are inching towards maturity and are seeing good traction across geographies.

I now request your attention to Slide number nine, where we have disclosed our operational parameters across our matured network and emerging centers for FY '23. Our company-wide AOR stood at 65.7% and AOR for matured and emerging centers stood at 63.2% and 71.9%, respectively. Our ARPOB on company level stood at INR 37,014 and our ARPOB centers stood at INR 40,154 and for emerging centers stood at INR 30,057.

Across geographies we have given our revenue breakup in Slide number 10. Rajkot grew by 110%, revenue from Jaipur grew by 104% and Mumbai grew by 28%. And new center of excellence grew by 15% year-over-year for Q3 FY '23. Our Milann business is also doing well. Revenue stands at INR 16.8 crores in Q3 FY '23 and new registrations grew by 53%. On the EBITDA front, our consolidated reported EBITDA stood at INR 75.5 crores as compared to INR 61.3 crores in Q3 FY '22, a growth of 22%. Reported EBITDA for 9 months FY '23 stood at INR 222 crores, a growth of 27% year-over-year.

Adjusted EBITDA for Q3 FY '23, that is, after adjusting the onetime value creation costs and adjustment of ESOP expenses stood at INR 81 crores as compared to INR 63.6 crores in Q3 FY '23, a growth of 28%. Adjusted EBITDA margin stood at 19.1% as compared to 18% in Q3 FY '22, a growth of 156 basis points. Adjusted EBITDA for 9 months FY '23 grew by 35.3% year-over-year with margins of 19%, a growth in margins of 200 basis points. We have also given bifurcation of our EBITDA across

matured and emerging centers, and I will request the participants to view Slide number 8 for further details.

On PAT, we are delivering positive PAT for the last 4 quarters now. PAT for this quarter stood at INR 7.5 crores as compared to a loss of INR 0.3 crores in Q3 FY '22. 9-month FY '23 PAT stood at INR 20.9 crores as compared to a loss of INR 9 crores in 9 months FY '22 adjusted for onetime exceptional gains or losses in FY '22. Our PAT Pre-Ind AS adjustments for Q3 FY '23 stood at INR 11.3 crores as compared to INR 3.1 crores in Q3 FY '22. PAT for 9 months FY '23, pre-Ind AS adjusted stood at INR 31 crores as compared to a profit of INR 1.6 crores in 9 months FY '22. ROCE for matured network stood at 19.7% annualized for 9 months FY '23 as compared to 15.7% in FY '22, an improvement of 400 basis points.

ROCE before corporate allocations for matured centers stood at 19.7% and ROCE for emerging centers stood at negative 5.3% annualized for 9 months in FY '23 as compared to a negative 8.3% in full year FY '22. This is, again, an improvement of 300 basis points. ROCE before corporate allocations for emerging centers stood at negative 1.4%. Our net debt position, excluding capital leases as on 31st December stood at INR 212 crores as compared to INR 211 crores as on 31st September 2022. Our expansion of the existing facilities in Ahmedabad -Phase II and Whitefield expansion of Bangalore COE is on track. Total planned capex for Ahmedabad

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is INR 85 crores. The expected date of operations being Q1 FY '25 and for Bangalore COE is INR 25 crores, expected date of operation being Q4 FY '24. With this, I would now like to open the floor to Q&A.

Moderator: The first question is from the line of Kaustubh Pawaskar from Sharekhan by BNP Paribas.

Kaustubh Pawaskar: Couple of questions. First, ARPOB for this quarter is down. So any particular reason for this?

Raj Gore: Kaustubh, Raj here. Look, as I mentioned in my opening remarks, in our emerging centers right now, we are going for volumes, we're going for scale. We're going for higher utilization of our capacity. It's reflected on a very good growth in Rajkot, Jaipur and across various emerging centers. That's the reason ARPOB is lower. But if -- I would like to draw your attention, this is

still an improvement over the Q2. So as we've always stated, first, we will go for volumes, and then we will go to improve the quality of the business and go for ARPOB.

Kaustubh Pawaskar: Right. My second question is on the consultation fees. So this has been there for past 2 quarters. So till what time it will continue to reflect on your books because we are setting it as a onetime exchange so.

Raj Gore : Correct. Correct. So this last quarter, Q4 of this financial year would be the last quarter.

Kaustubh Pawaskar: Okay. Q4 of this financial year. And your bed, till last quarter, it was around 1,794 beds. This quarter, it has increased around 36 beds to 1,873 so any new -- I mean where this addition has been done, 36 beds?

Raj Gore : Yes. So we have this operational more beds in Jaipur. These beds were always there. It's as an installed capacity, we're just operationalizing it.

Kaustubh Pawaskar: Okay. And another question is on your fertility business part. So there, we have been seeing the revenues at around INR 16 crores to INR 17 crores. And even on registration side, what I'm struggling to understand is whether there would be improvement in terms of operational impact on the business or where the company wants to focus on that part of business? So any thought focus on that.

Raj Gore : So look, I know that Q3 was flat. But if you look at year -on-year, we have about 9% -10% growth. Q3 could be just a matter of seasonality. There have been certain regulatory changes as well. So the whole industry is adjusting to new regulatory requirements.

rements. So we feel that going forward, it will continue to grow.

Srinivasa Raghavan : Presently, this government of India has got into regulatory act called ART. I think in the state of Karnataka it has been lifted probably in the month of March and April patients going to get engaged. So going forward and probably by Q1 of FY '23 -'24 could see the big numbers growth on that respect.

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Kaustubh Pawaskar: So this is a temporary thing maybe by quarter one, things should improve for this business? , Q1FY'24

Srinivasa Raghavan : Yes

Moderator: The next question is from the line of Anil Sarin from Centrum Asset Management.

Anil Sarin: So first of all, good job once again. I will start with a request that if you can postpone the call for the next day post declaration of results, that would give all of us time to go through the presentation in more detail, and we can ask better questions that way. So if you could consider that for next time, that would be great.

Raj Gore : We'll take that into consideration

B.S. Ajaikumar : We will do next day because last time, there was some request we do it on the same day, there's conflicts context and all. We certainly will take your request and see how we can accommodate, okay?

Anil Sarin: Sure, sure or at least giving a gap of a few hours, as you know, I mean, this is result season, a lot of results coming, but some time, the quality of questions would improve if we've had time to go through.

B.S. Ajaikumar : So a good point. Good point will take it.

Anil Sarin: Sure. The question I had was that there has been a steady improvement on a sequential quarter basis for the last 7 or 8 quarters that I have been seeing this, this is perhaps the ninth quarter. So -- but on a sequential quarter basis, the EBITDA margin has not improved in any material fashion, unless I'm mistaken, if you can just throw some light on that because revenue has grown, but the EBITDA margin post adjusting for these onetime costs, I doubt if it has gone up, if I'm inaccurate, you can please point out.

Raj Gore : So Anil, good question. So one reason is our onetime expenses in our value creation plan. The second reason is we are disproportionately investing in hiring clinical talent and building brand for our emerging centers to drive scale, to drive volumes across those centers. And we are seeing the results of those efforts in the revenue growth across emerging centers. So that's an upfront investment to ramp up the utilization going forward to take it to a next level.

Anil Sarin: Okay. Okay. Great. And so, post this coming quarter, that would be the last time when you will have this consultancy charges, unless you go for a new project. So next year, what kind of overall growth can one expect? And what kind of overall EBITDA margins can one expect including Milann, including all the constituent parts?

Raj Gore : Yes. So, Anil, without the onetime expenses, we are about 19% in the last quarter, EBITDA margin. And we are confident maintaining our growth momentum going forward on a quarterly basis. We usually don't give forward-looking guidance on numbers.

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Moderator: The next question is from the line of Dhara Patwa from SMIFS Limited.

Dhara Patwa : Sir I have 2 set of questions. So despite higher occupancy of 72% in the emerging centers and ARPOB of INR 30,000, which is flat on the sequential basis, still the EBITDA margins have not seen a decline to 7.5% as compared to the average of 10.5% in the previous 2 quarters. So is it the new normal should we expect it or going forward, it will increase to 10%, 12% or let's say, 15%? So what should we assume for the emerging centers ?

Raj Gore : So again, we don't give forward-looking guidance on numbers. But as I explained, a lot of

our

growth in emerging center is coming because we have higher clinical talent, we've increased our clinical bandwidth. We are investing disproportionately higher in our sales and marketing activities. As a result, we are seeing revenue growth. It's our upfront investment. The margins will follow. On question of ARPOB again, initially, we would go for scale. We would go for volume. Usually, the playbook in hospitals, is you fill the hospital first and then you optimize your mix, your pay or mix, your -- and that will drive your realization all operating performance indicators going forward. So we are following the same playbook center by center in emerging geographies.

Dhara Patwa : So should we expect at least a 10% plus margin for the emerging centers going forward? Like at least a direction like, it won't be below 7%, but it will be in the range of 10% or something?

B.S. Ajaikumar : It will start moving towards our mature center bucket slowly.

Moderator: We'll move on to the next question from the line of Aditya Khemka from InCred PMS.

Aditya Khemka: First question, if you could help us with what is the current EBITDA margin we do, let's say, at our flagship facility in Bangalore?

Srinivasa Raghavan : The EBITDA margin in Bangalore, you were talking about center of excellence, it's about post Ind AS is about 27% - 28% is our EBITDA margin.

Aditya Khemka: And this number would not include any profit overhead right? This should just be unit economics

Srinivasa Raghavan : This can improve by a couple of basis points. But as you know, we are reaching the top in terms of the EBITDA margin for the center of excellence. Okay.

Raj Gore : Correct. It's at a unit level Aditya .

Aditya Khemka Yes, it's on a unit level, it does not include corporate overhead, right?

Raj Gore : Yes. Right.

Aditya Khemka: Okay. So therefore, my question is that I believe the endeavor of the company would be to move as many units as possible closer to the EBITDA margin that we do with our flagship facility over a period of time. Obviously, it can't happen as soon as we open the facility, it takes to ramp up and mature the facility to get to an EBITDA margin of 25% plus, which would also imply Healthcare Global Enterprises Limited

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that at the time when the majority of the capacity of your 1,800 -odd beds, which is 1,500 or so 1,800 beds actually become "mature". Then at a unit level excluding corporate head, we should be at an EBITDA margin of 25% plus minus then the corporate overhead with, I don't know how much it is, but I'm guessing it to be around 5% to 8% of pay. So my question to you is, when do we see that happening? When do we see a lion's share of our beds, more than 80% of our beds maturing to a level of profitability that we do in our most flagship facility?

Raj Gore : Yes. So Aditya, good analysis. Our model, our business model has been able to deliver a mid - 20s EBITDA margin in metro markets, nonmetro markets, when the center becomes mature. So we will drive the performance of individual units to get to that level. We are looking at about 2 years span for the entire current base to move in that direction.

B.S. Ajaikumar : So as you know, we are at EBITDA without the onetime charge, we are at 19% already. This is including the corporate cost. So if you kind of take out the corporate cost at a unique level, we are already in the low 20s. For that to reach 25, it should be doable. And I think we are on track to, I can say, confidently.

Aditya Khemka: So sir, at a unit economic level, are we already at 21% 22%? Is that what you mean by low 20s?

B.S. Ajaikumar : Yes, at our economics, we are already at 21%, 22% because our 19% includes the corporate cost.

Aditya Khemka: Which is about 2% to 3% is what we're implying?

B.S. Ajaikumar : Yes, yes.

Aditya Khemka: Okay. Okay. So I get it. So this is not unreasonable.

B.S.

Ajaikumar : I said, on a lighter, we can take your assessment of 5% , It is not 5% but I am saying on a light note

Aditya Khemka: No that's awesome if you're doing it on a leaner organization, it's actually excellent, because I

understand hospital business flagship facility would actually include a lot of corporate costs, this gets called out at the EBITDA, I get that. I get all that. So I understand that you're at 2% to 3% corporate cost so which means Dr. Ajai or Dr. Raj essentially, it means that it is not unreasonable for a shareholder today to believe that in the next 2 to 3 years, we will be 200 to 300 basis point expansion in EBITDA margin, am I right?

B.S. Ajaikumar : Yes. It's certainly not unreasonable and hope, though we can't guarantee, we can even better that that is what we will strive for. That is where a lot of effort is going on in terms of operational efficiency and doing. But if all the centers come to mature, and we have good efficiency as we are planning and our revenue growth is there with fixed cost remaining, and as you said, our corporate cost, we should be able to restart and hopefully even better that.

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Aditya Khemka: That's awesome. That's very good to know. My second question is essentially on the cash flow. So I heard the net debt numbers. And correct me again if I'm wrong, this would be excluding lease liability. I think it is around INR 210 crores to INR 212 crores for the past 2 quarters, am I right?

B.S. Ajaikumar : Yes. That's right.

Aditya Khemka: So where is the free cash flow? I mean, ideally, the net debt number should have gone down given that we have had a very profitable quarter -- what are we investing in? And this is coming off after your PAT, so obviously, your investment in manpower, center of excellence, R&D scientists, all that is the cash flow after those investments. So are we -- so where are we making those capital expenditures, which is not helping us reduce our net debt further.

B.S. Ajaikumar : Okay. Point number one, I think it's largely flat as you see over the last couple of quarters. But having said that, we have invested in revenue-generating activities. For example, we have invested about close to INR 18 crores in solar in our Karnataka center. That is giving us some savings in our electricity bill. We have also invested in a high-end equipment in our center of excellence which we talked about in the last quarter. And we are also investing in robotic surgery. So these are the places where we are investing and as I mentioned earlier, these are all revenue-generating capex, which will kind of give benefits to the business in the future quarters.

Aditya Khemka: I understand that. So could you give me capital expenditure run-off for the 9 months this fiscal?

B.S. Ajaikumar : About INR 96 crores is the capex for 9 months. INR 56 crores.

Aditya Khemka: INR 96 crores okay, then it makes sense. I understand. My last question is on the technology side. So I think Dr. Ajai, would be the right person to respond. So this da Vinci robots, the first edition that you are acquiring. First of all, the acquisition of these robotic machineries, I'm assuming these are on lease and not outright purchases?

B.S. Ajaikumar : Yes. We have cleared the first-of-a-kind model with them. It is not really a lease. It is our model of pay per use where I think first time da Vinci to best of our knowledge instantly where we have without any minimum guarantees making, there is a model where we share in our revenue and certain fixed amount, and we found that it's doable, and that is we can penetrate Tier 2, Tier 3 cities apart from doing where we expect the volume to grow over the next year or 2. So this is the model we are trying it out with the new da Vinci 3 units basically.

Aditya Khemka: Understood. So we are using this da Vinci model -

- so the da Vinci robots to my understanding

for first benefit is obviously cosmetology because the arms are flexible and you can have better holes in the body of the patient than let's say laparoscopy.

B.S. Ajaikumar : The da Vinci came about 2 decades ago because of the precision surgery. We talked about precision variation, targeted, but surgery was always not precise, extensive surgery was done.

With the da Vinci unit, what happens if you get that microscopic look and we use the da Vinci handles to really do precision where the normal tissue is not hit. The normal tissue doesn't suffer.

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and it is precise excision of the tumor or whatever your target is, thereby, the patient recovers fast, complications are less. So overall, as you said, the cosmetic scar also will be less.

So based on all this, more and more da Vinci users have come compared to the time for neuro-oncology first; for doing prostate was the first time it came. Now we are doing significant like I said in my opening remarks, we have done in Ahmedabad 1 of the highest renewal of esophagus using da Vinci. It's a remarkable feat because it is -- almost in 3 days, the patient can go on literally no pain and no complication. We used to have these leakages at the anastomotic side, we don't see all that. So it is a near perfect you can reach in terms of surgery and outcome will also be good, unless it hits on the normal tissue.

And again that also, we are beginning to do without any scar imagine doing a head and neck

surgery, overhead, where there is no scar and a good outcome. So we are also having signed up an agreement with the da Vinci where we will be doing not only training, but we, together, will create some innovative platforms. So that is what I personally as a doctor am looking at how will we innovate new techniques, new technologies.

Aditya Khemka: Sorry, I have two more questions on this platform, Dr. Ajai. Firstly, how many clinicians are running doctor to surgeon? Do we have certified by da Vinci to operate and use the da Vinci system?

B.S. Ajaikumar : Today, in Bangalore, we have about 5 surgeons who are certified, and we are starting -- we have our own training program with da Vinci. In Ahmedabad, we have another 5 people, 4 to 5 people who play in the northern head and neck for GI. And we have others Doctors who's already waiting for da Vinci, and they are 2 or 3 trained in Mumbai were ready to do surgery now. So we have quite a few, and we intend to increase this number not only for HCG doctors, and we want to make sure even outside doctors can come and try precision so that essentially, the patients at large can benefit. That is what we are looking at, okay

Aditya Khemka: That's enough from objective. Last question on the da Vinci platform. Intuitive also has this new system or this new sort of a diagnostic machine that helps in diagnosing lung cancer early or the smoking population. Any thoughts on that, Dr. Ajai, have you explored that option with Intuitive?

B.S. Ajaikumar : No, we have not explored that. We will certainly look into that and explore. We have explored this lower level of CT scan for smoking population. We are thinking of launching that on Sundays, where we can do lower CT scan, because this is a big trial, which was done in US years ago, very successful. So we have been working with the radiology group. We are bringing in possibly a new radiologist who's experienced in that. But da Vinci part, I have not explored. I will certainly look into this.

Aditya Khemka: Yes, they call it the lung biopsy or something, I think. When I studied it, I found it really exciting. So maybe something

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B.S. Ajaikumar : I know what you're talking about, it's the robotics -guided lung biopsy, where there is very minimal i

ntervention. But there are parallel units available. So certainly, we look at it, but they did make a -- come and make a presentation a few months ago. So we are evaluating what is suitable for our people here, okay?

Aditya Khemka Makes a lot of sense. And more power to you guys to help our country and our society.

Moderator: The next question is from the line of Reshab Sisodiya from Sameeksha Capital.

Reshab Sisodiya: Sir, one question on the growth guidance that we have. So given that you are reaching almost 70% occupancies now, and our new bed capacity that you mentioned is coming up by the end of FY '24. So how confident are you for the growth momentum for the company? And at the same time, where do we see our ARPOB growth going ahead for both the centers, emerging as well as matured?

Raj Gore : The occupancy numbers we reports are on operational beds. Not all the beds in some of the emerging centers have been operationalized. If we take this occupancy on an installed bed capacity, it becomes 56%. So we don't expect bed capacity being a constraint in our growth, especially since our chemo modalities is a day care modality, radiation doesn't require bed. It's only the surgical patients who require largely overnight admission on inpatient beds. So we don't see bed being a constraint for us going forward in terms of driving growth.

B.S. Ajaikumar : I just want to add what Raj said is absolutely right. I just want to say, in oncology, historically, it has been more and more outpatient and less hospital stay, as you look at the ALRs, it tends to come down as we move forward year -on-year. So that is the reason the bed is not the way we feel to measure the growth. There are other parameters we should take into consideration to look at the growth.

Reshab Sisodiya: Okay. Sir, that's helpful. Sir, on the Mumbai center. So the current growth given as compared to other centers like Jaipur, Rajkot, they have very high growth. So why is the Mumbai center having a bit of low growth even when we acquired new assets recently?

Raj Gore : So I think the Mumbai growth looks subdued because last year, we had a significant vaccination revenue in our Mumbai center. These were the emerging centers, COVID came, we couldn't go out and build brand HCG among the communities that we serve. Vaccination was a good opportunity to respond to nation's call as well as reaching now to the community to build our brand. So we do that very well. If you adjust that, this growth will look better. The other part is in Mumbai, we go for a better pay or mix compared to Rajkot and Jaipur. So it's more cash TPA corporate driven growth rather than government -driven growth. So that's also another reason that this growth will be a better quality business growth yielding better ARPOB eventually and better margins.

Reshab Sisodiya: Okay, sir. Just the last question. From an entire company, not just for the cancer part. So what would be the payer mix, if you could just break it down?
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Raj Gore : So about almost 65 -70 will be cash plus insurance. And then remaining will be government as well as corporate business.

Reshab Sisodiya: Okay. So are we facing any repricing issues with insurance companies or with corporate? And do we see that any further repricing ahead that would help us in the ARPOB growth going ahead?

B.S. Ajaikumar : Sorry, can you repeat your question?

Reshab Sisodiya: I'm asking given that insurance in corporate parts, are we looking at increasing the rates with them and that could help our ARPOB next year, growth?

Raj Gore : Yes. So usually, this comes for renewal every 2 years. So it will come up for renewal next year in some of the centers, it's a center -by-center thing. So after the contractual period, we go for a rate increase. In general, if you look at historically, we have a larger presen

ce outside big cities,

outside metro cities. Many of our new centers are in bigger cities relatively, 2 in Mumbai, in Kolkata, Nagpur, these are better markets. So as this share of the emerging centers grow as a share of total revenue, our ARPOB will automatically start growing to a higher level.

Moderator: The next question is from the line of Urmil Shah from Aegis Federal Life Insurance.

Urmil Shah: Sir, my first question is, you mentioned a 56% occupancy on installed bed, that's at the company level or if you could split between mature and emerging centers?

Raj Gore : So 56% on a total capacity. Yes.

Urmil Shah: Sure. So what would be the same number for mature and emerging centers?

Raj Gore : So whatever you see on the slide as far as established and the adjustment is concerned, there -- I mean, there, we don't have any difference between operational and capacity beds. It's primarily the new centers where there is a difference of about 250 beds, where we have installed 250 less in our emerging centers. Thereby, if you take capacity utilization in terms of beds at emerging center, it is around the 56%.

Urmil Shah: Sure, sure. And from a medium -term perspective, up 65%, which is occupancy for the mature centers that's -- what should be doable -- of course, it will be depending on each center. But from a 3-5-year point of view, your current bed should be reaching that kind of occupancy?

Raj Gore : Sorry, which is your question. Is it a bed question or --

Urmil Shah: So you mentioned that the occupancy level in your mature centers, it's similar for when calculated on operational beds or installed capacity. My question was as regards to emerging centers from a next 3 to 5 -year point of view, they should also be getting to a 65% kind of occupancy level? And in your case, generally, at what kind of occupancy levels you would further need to add capacity in a particular center?

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Raj Gore : Yes. So we do expect emerging centers to start reaching to mature center occupancy. Usually around 75% when the occupancy is 75%, we start looking at how do we create capacity. So one of the things you must understand, as I mentioned earlier, that it's only surgical specialty, which largely uses overnight inpatient beds. That average length of stay going down with minimally invasive technique robotics. So what you -- what we've seen in our experience, what is happening is we are converting inpatient beds into daycare beds. On daycare base, we have ability to increase number of shifts in the same day. So for us, bed is not as much a concern as one would think in a multi -specialty environment.

Urmil Shah: Sure, sure. Sir, my second question was more on the emerging centers. So we've discussed about the margin performance over there. So what could be the drivers for getting the margins, if not to the mature center levels, but at least to mid -30s, what could be the drivers keeping into account the current ARPOBs and the AOR, which we are currently doing?

Raj Gore : Yes. So Urmil, as we mentioned, right now, we've invested in clinical talent. We're bringing them out in our outreach markets. We're investing in branding and marketing sales activities, basically driving more new patients, driving utilization of different modalities, driving capacity utilization, driving revenue. That's the sure short way to improve margin in these centers.

Urmil Shah: Sure. And just to dissect on the AORs of emerging centers. Of course, this is an average. So does the emerging centers also have a few centers where the AOR would be 75% plus?

Raj Gore : So yes, like for example, Jaipur, where we just mentioned earlier that we commissioned more beds because we were reaching higher occupancy. So yes, in that bucket, we do have a center, which is a higher capacity -- higher occupancy.

Urmil Shah: Sure. Sir so

then in such centers like there are 2 centers, how different is the profitability as compared to the emerging center category as a whole?

Raj Gore : So Jaipur in a span of year has gone to EBITDA margin of mid -20s post Ind AS.

Urmil Shah: Sure, sure. Okay. Yes. Sir, my last point was on the free cash flow question, which was there .

So this year, we are having the investments for new high -end equipment also in talent also on the consultant fees, which are there. Next year, if we look into -- we've already talked about the capex for the capacity addition. So adjusted for the one -timers and the investments which have gone in this year, would it be fair to assume that that's the bare minimum increase in free cash flow on NOPAT that should come in?

Srinivasa Raghavan : Yes. See, the way to look at it is we will have to look at our ratios , our debt -to-EBITDA ratios, which is at a very, very acceptable level as well as the overall business is concerned. So if you see, we are at about 2.3%, and we should be around that it would slightly come lower as soon in the future quarters. But having said that, the key point is there is some amount of capex , while there won't be any GPP costs next year, there will be some amount of capex towards maintenance capex growth oriented capex , both will be there in the future years as well. But the key point to note is this will not come out of any additional extra borrowing, we will be able to manage out

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of our internal accruals , point number one . And number two , we will be able to maintain some of our key ratio at an acceptable level, which is accessible to the market.

Moderator: The next question is from the line of Sagar Shah from Phillip Capital.

Sagar Shah: Yes. Actually, I just had one question. Actually, we were looking to open our 203 -inpatient beds in the time to come. So can you specify your time that in which -- something like in 6 months or in 12 months or the next 18 months, are we going to open up the bid? And my second question was related to our growth drivers. For FY '24, what would be our key revenue drivers for the same?

Raj Gore : So, Sagar, I mean, the answer to your first question will change from hospital to hospital. But as a total bucket, in next around 18 months, we'll have to deploy all our installed capacity, which is not operational as we operationalize it. In terms of revenue growth drivers, look, the growth drivers for next year will continue to be the same growth drivers that have entered this year also.

B.S. Ajaikumar : And just to add on that the growth drivers, obviously, we are looking at the ones which are mature. So those which are not mature center, we grow faster, obviously, than those who are mature that, that could be and also the technology we are investing and they like what our CFO said new technology we have brought, those will be also drivers for the future.

Sagar Shah: So basically, you are saying the improvement in the utilization of the emerging hospitals, improvement in the volumes of new patient volumes in the emerging hospitals would be the same growth driver for the company in FY '24, right?

B.S. Ajaikumar : Yes.

Moderator: The next question is from the line of Pallavi Deshpande from Sameeksha Capital.

Pallavi Deshpande: Yes. Just on the part where you mentioned about having talent, et cetera. Will we be done with that this year or will this be a continuous process on the clinical talent side

Raj Gore : Pallavi I don't think we'll ever stop adding clinical talent. In new centers, you will first make sure that your clinical services portfolio is complete. When those start growing, their bandwidth starts getting choke, you will add more clinicians under each as the program evolves we'll start creating vertical specialization, head and neck, breast, etcetera. So this is a continuous p

rocess.

We'll continue to invest in talent going forward.

B.S. Ajaikumar : And just to add to that, for us, being a very specialty focused, as you know, the way oncology is growing is in multidisciplinary clinic tumor boards. But this, we definitely not only have they have training doctors and also have additional talent. And that is what we are doing, not only in big centers, but even in Tier 2, Tier 3 cities. Our talent pool is increasing and definitely increased, and that will definitely add to our overall growth, and we have proven that again and again.

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Pallavi Deshpande: My second question was on the ROCE. So ROCEs of the emerging centers, we're just seeing a small decline quarter -on-quarter is not material, but we are giving adjusted direction seems to -

-

B.S. Ajaikumar : So, as Raj said, it is not material. And for the fee as Raj said, we have sort of invested more on

operating expenses related to clinical talent, hiring and business promotion. This will start to -- into our higher revenues going forward however, it is temporary as you rightly said .

Pallavi Deshpande: And sir, you mentioned about the mix of the government and corporate at 30, wouldn't that -- wouldn't the [Part of the question was not audible].

Raj Gore : So Pallavi, historically, our presence outside metro cities was larger, where we had a mass market strategy, low price, higher volumes with government schemes. Many of our emerging centers are in bigger cities. We have, for example, we have 2 in Mumbai, 1 in Kolkata. They will have a better pay or mix, better ARPOB, better realization. So as share of emerging centers to the total revenue goes up, all these parameters will start improving going northward.

Moderator: Ladies and gentlemen, due to time constraints, we will take one last question from the line of Naman Bhansali from Perpetuity Ventures.

Naman Bhansali: Just 1 request was that you used to give an FY '22 a good breakup of ARP OB, EBITDA margins in occupancy on a region -wide basis. So it would be great if you could continue giving that in the presentation, if possible. So that was a request. And my first question relates to the international patients. How much of the business as a percentage of contribution was international patients? And what do we do to attract these patients? Are these all organically driven or are we hiring any agencies to attract these patients from international borders? And the second question is on the industry and on HCG that how is HCG different from a proton and other hospitals in the industry who also have a high -end oncology machines like CyberKnife and etcetera? And what risk do this industry in oncology face in terms of newer technologies being introduced in coming future? These are my 2 questions.

Raj Gore : Naman, this is Raj here. Let me take the first part and Dr. Ajai will take the second part on the difference between different technologies. So 1, we'll take your suggestion into consideration about the breakup. Second, international business, look, we drive international business through all channels that are present to us. There are agencies, there are direct institutional tie -ups, there are tie ups with international insurance agencies and very direct business generation, which we do by taking our clinicians to these cities, to these markets.

If we have information centers in this market, we have permanent team members in these markets. So it's a combination of both. For us, international business is doing really well. We had about 1.5 to 1.6x of our pre -COVID high numbers. In fact, last quarter was our highest ever. We are close to about 5% of total revenue in our international business. I'll request Dr. Ajai to take your question on Proton and

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B.S. Ajaikumar :

Yes. In terms of Proton, as you know, Proton has been around since 1970s, and it is mainly because efficiency and precision and a lot of issues are in the maintenance of Proton apart from the cost and other things. There is no exit and entry dose. But as the time has advanced, now we have some units which are better than Proton. In fact, HCG considered Proton a few years ago.

And I myself being an oncologist and radiation oncologist, I was looking at it. But after careful evaluation and looking at the global usage of Proton why it is coming down, attending the conferences and all, primarily because alternatives had come. And 1 of the things we have installed in Bangalore of course, we have CyberKnife, which is precision medicine radio surgery, 1 in Bangalore, 1 in Mumbai, and doing extremely well in terms of physician and retreatment also can be done with literally very little exit dose.

Now the advancement on this has been the Ethos, which is adaptive radiotherapy using the model of AI -based. And this has been a game changer for us. We have recently installed it. We have done a significant number of patients in the 3 months. And what we are seeing in our impact, I am involved in partially in the research with Dr. Lohith and Krithika. What we have seen is, as the tumor changes, day to day, we can make the change in the tumor lithography and also change the treatment. And we are also studying what is the exact dose required. So this is a study in progress called radiosensitivity index. So with this kind of major developments, technology obviously becomes very important.

Added to that, we are looking at how do we connect genomics to technology, where certain tumors respond well, certain tumors do not respond. Why? What is the genomic story behind it?

So we can decide on the proper dosing of not only radiation, but also targeted therapy, immunotherapy, everything. Today, cancer treatment, as you may know, is multidisciplinary. And our outcomes based on this has been very good, very encouraging, not only in the early stages, but even in advanced stages. Today, lung cancer patient can live for even with advanced stages in to 8 to 10 years. A patient with breast cancer similarly can live for 8 to 10 years. So a lot of changes have taken place. Technology is going to complement. AI is going to complement the way we see it, and that is why in HCG, we have become really progressive on

it. We are spending significant our money also and just to see how this can translate to precision patient therapy. You are absolutely right in terms of technology going to play a very big complementary role for doctors to see what should be the right treatment for the patient at the right time. I always believe first -time right treatment will give us the best prognosis. And this, obviously, technology, virtual tumor boards are going to be big factors in future along with AI, of course, as we go forward.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to Mr. Raj Gore for closing comments.

Raj Gore : Yes. Thank you, everyone, for joining the c all, and all thos E your interest in HCG. Hope we've been able to address all your queries. We'll continue to keep you updated on a regular basis on Healthcare Global Enterprises Limited
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developments in HCG. For any further information, please get in touch with us or Strategic Growth Advisers, our Investor Relations advisors. Thank you once again and have a good evening.

Moderator: Thank you. Ladies and gentlemen, on behalf of HealthCare Global Enterprises Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Jun 2023

June 02, 2023

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on May 26, 2023

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(a) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on May 26, 2023, with Analysts/Investors to discuss the audited Financial Results of the quarter and year ended March 31, 2023.

This is also available on the website of the Company www.hcgoncology.com.

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited
Q4 FY '23 Earnings Conference Call"
May 26, 2023

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. SRINIVASA RAGHAVAN – CHIEF FINANCIAL
OFFICER – HEALTHCARE GLOBAL ENTERPRISES
LIMITED

Healthcare Global Enterprises Limited
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Moderator: Ladies and gentlemen, good day, and welcome to the Healthcare Global Enterprises Limited Q4 FY '23 Earnings Conference Call. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions, and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen only mode. And there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on a touchstone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. B.S. Ajai kumar, Executive Chairman of Healthcare Global Enterprises Limited. Thank you, and over to you, sir.

Dr. B S Ajai Kumar : Thank you very much, and good morning to everyone, and a very warm welcome to all present on the Q4 FY '23 Earnings Conference Call for Healthcare Global Enterprise Limited. Today, I'm joined by Mr. Raj Gore, our Chief Executive Officer; Mr. Srinivasa Raghavan, Chief Financial Officer, besides a few members of the HCG senior management team to share our operation and financial highlights for the quarter ended and year ended March 2023.

HCG is a leading chain of hospitals focused on oncology care, which is patient-centric, technology oriented and focused on quality outcomes for its patients. Our focus is to provide the best-in-class cancer treatment on a pan-India basis with superior outcomes with quality of life for our patients.

HCG is not only focusing on providing patient-centric outcome-based cancer care treatment but is equally focused on and has taken a lead role in the field of research and academia. Given the quality of our innovation, research and patient centricity, HCG ensures access to world-class cancer treatment and services for our patients. At HCG we actively participate in clinical trials, invest in ground-breaking research to advance the frontiers of cancer treatment. Our tumour board initiative is supporting platform that brings together a multidisciplinary team of oncology experts to discuss complex cancer cases, share insights and develop personalized treatment plans.

Employing breakthrough technology, including advanced adaptive AI, therapy, robotics, and genomics coupled with our pay-per-use model for LINACs enable us to provide best-in-class treatment at cost-effective prices. Our relentless fight against cancer is founded on our continuous research and technological innovation towards exploring new therapeutic avenues to move up the value chain of clinical excellence and lasting outcome.

HCG is first in Asia to complete 105 clinical runs for over 1,000 patients of comprehensive genomic profiling. The results are encouraging and with the wealth of data emerging from cancer genome studies and data lake created, we feel this is just the beginning of our journey in the Healthcare Global Enterprises Limited

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precision medicine and to win war on cancer, not only in early cases, but also in advanced cases to convert cancer to chronic disease.

We are also first in India to launch self-circulating tumour cell testing where just by a blood test, we can detect circulating cancer cells. Self-search is the first and only system which has received FDA approval for precise enumeration of circulating tumour cells in the patient's blood for diagnostic procedure.

Self-search helps in early detection and screening and also in detecting the response to therapy, informed patient care decisions, real-time treatment, and monitoring. At HCG, we always believe in raising the

bar and staying ahead of the curve, we strive to provide the best possible treatment to our patients. I now hand over to Raj, Mr. Raj Gore to take you through the strategic initiatives and financial highlights. Over to you, Raj.

Raj Gore : Thank you so much, Dr. Ajai. A very good morning to all the participants on the call. We are extremely happy to announce our stellar performance for FY '23. We've ended FY '23 with revenues of INR1,691 crores, a very robust growth of 21% with increased profitability. Our adjusted EBITDA for FY '23 has increased by over 31% on Y-o-Y basis and stood at INR321 crores. Our efforts on operational efficiency, coupled with operating leverage has resulted into expansion of adjusted EBITDA margin by 139 basis points, which stands at 18.9% for the year FY '23 compared to 17.6% in the previous year, FY '22.

We ended FY '23 on a strong note with our consolidated revenue for Q4 FY '23 standing at INR442 crores, year -on-year growth of 21% and our adjusted EBITDA margin standing at 18.8%. We've been able to create a strong foothold in our mature geographies and have been able to make inroads into our emerging markets. In Q4 FY '23, our revenues from mature centers have increased by 19% year -on-year, and revenues from our emerging centers have increased by 32% over the same period.

The adjusted EBITDA margin is lower by 30 bps in FY '23 Q4 compared to the previous quarter primarily due to upgradation or replacement of radiation machines at 3 of our locations, Ongole, Ranchi and Shimoga, which has resulted in reduction of margin by 40 bps. These machines are under installation and are expected to be operational during Q1. In addition to driving higher utilization of existing capacity, we continue to invest in superior clinical expertise, capacity creation and brand building to fuel future growth and further fortify our leadership position in the industry.

To ensure our growth momentum going forward, we have increased our sales and marketing spend, which we think is an investment in our future in Q4 FY '23 by 0.5% of top line over the previous quarter and by 1.2% of top line over the same quarter previous year. Especially in our emerging geographies, our operating investment in engaging quality clinicians and our go -to-market activities are showing good results.

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We are happy to report that our onetime value creation cost will end from this quarter, and there will be no further adjustment for the same on EBITDA going forward. These transformational activities done over the last 6 quarters has started adding value and have set us up very well on a path of profitable growth going forward. Our revenue through digital channels has grown by more than 3x in FY '23 over FY '22, crossing 6% of total revenue in Q4 FY '23.

We've also recorded our highest ever annual revenue from medical value travel with a year -on-year growth of 91% and reaching to 1.5x of pre -COVID highest level. We've been consistently delivering growth across our network and have grown faster than the industry over the last 8 quarters.

Our track record of consistent performance reflected in highest ever revenue and highest ever EBITDA over last 9 and 8 quarters in a row, respectively, is a testimony of meticulous planning and rigor in execution we have shown as a team. This unparalleled growth journey over many years is primarily due to HCG's pan -India network, specialized and differentiated cancer care model and local market leadership across locations, which we have established over past several years.

To conclude, I would like to say that HCG remains to be the trusted health care partner for every individual battling cancer. We are honoured to have earned the trust of our patients and the communities we serve and are committed to upholding that trust every day. Through our relentless endeavour , we

assure our patients the best of the treatments available across the globe

and live up to promise of adding lives to years. With this, I hand over to our CFO, Srini, for financial highlights.

Srinivasa Raghavan : Thank you, Raj, and very good morning to everyone. We have uploaded our Q4 FY '23 results, an updated investor presentation on the stock exchanges and company's website and I do hope everybody had an opportunity to go through the same. We are delighted to share that we have been able to grow our revenues ahead of the industry growth due to the trust and brand created by HCG.

On the revenue front, our consolidated revenues for Q4 FY '23 stood at INR442 crores as compared to INR365 crores in Q4 FY '22, a growth of 21%. Our revenues for FY '23 stood at INR1,694 crores again registering a growth of 21% year -over-year. Revenue split between HCG and Milann stood at 97% and 3%, respectively, for Q4 FY '23. Revenue growth for HCG stood at 22% year -over-year, whereas Milann witnessed flat revenues.

As mentioned in Slide 9, revenue from the mature centers stood at INR317 crores, a growth of 19% on a Y -o-Y basis for Q4 FY '23. Revenue from emerging centers stood at INR109 crores,

a growth of 32% on year -on-year for Q4 FY '23. We are delighted to state that our emerging centers are inching towards maturity and are seeing good traction across geographies. Moving to Slide 10. There are the key operational KPIs for our company. New registrations formed 21% of our revenues. Number of new registrations grew by 18% at 80,000 in FY '23
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versus 68,000 in FY '22. 37% of revenues came from chemo sessions and volume -wise, the figure stood at 133,000 in FY '23 versus 104,000 in FY '22, registering a growth of 28%. Radiation formed 18% of our revenues and capacity utilization stood at 66% for FY '23 versus 59% for FY '22.

Total radiation patients treated stands at 5,100 in Q4 FY '23 as compared to 4,700 in Q4 FY '22, a growth of 9%. And 21,000 in FY '23 as compared to 18,000 in FY '22, a growth of 17% year -over-year. Inpatient bed occupancy stood at 60% for FY '23 compared to 53% for FY '22.

I now request your attention to Slide 11. There, we have disclosed our operational parameters across our mature networks and emerging centers for Q4 FY '23. Our company wide AOR stood at 65.1% and AOR for mature versus emerging centers stood at 64.8% and 65.7%, respectively. Our ARPOB on company level stood at INR39,800, and our ARPOB from mature centers stood at INR41,400 and for emerging centers stood at INR36,000.

Coming to the next slide, for full year, our AOR stood at 65.4% and for mature centers it stood at 64.3% and for emerging centers, it stood at 68.5%. On ARPOB, we did ARPOB of INR38,000 for mature centers and ARPOB of INR40,500 and for emerging centers, the ARPOB stood at INR32,200.

Across geographies, we have given our revenue breakup in Slide 13. Kolkata grew by 142%. Revenues from Rajkot grew by 58% and Ranchi grew by 51%. Bangalore centers of excellence grew by 24% year -over-year for Q4 FY '23. For our Milann business, revenues for FY '23 witnessed a growth of 6.8%, and new registrations increased by 13.6%.

On the EBITDA front, adjusted EBITDA for Q4 FY '23, that is after adjusting the onetime value creation cost and adjustment of ESOP expenses stood at INR83.1 crores as compared to INR67.6 crores in Q4 FY '22, a growth of 23%. Adjusted EBITDA margin stood at 18.8% as compared to 18.5% in Q4 FY '22, a growth of 26 bps.

Adjusted EBITDA for FY '23 stood at INR321 crores, a growth of 31% year -over-year, with margins at 18.9%, a growth in margins of 138 bps. We have also given bifurcation of our EBITDA across mature and emerging centers, and I would request the participants to use Slide 9 for further details. Our consolidated reported EBITDA stood at INR76.3 crores for Q4 FY '23 as compared

to INR63.2 crores in Q4 FY '22, a growth of 21%.

Reported EBITDA for FY '23 stood at INR299 crores, a growth of 26% year -over-year. On PAT, PAT for this quarter stood at INR8.3 crores as compared to a profit of INR5.9 crores in Q4 FY '22. Full year '23 PAT stood at INR29.3 crores as compared to PAT loss of INR3 crores in FY '22 adjusted for onetime exceptional gains or losses in FY '22.

Our PAT pre -IndAS adjustments for Q4 FY '23 stood at INR11.2 crores as compared to INR9.4 crores in Q4 FY '22. PAT for FY '23 pre -IndAS adjusted stood at INR42 crores as compared to a profit of INR11 crores in FY '22. ROCE for mature network stood at 22.9% annualized for Q4 FY '23 as compared to 18.7% in Q4 FY '22, an improvement of 420 bps. RO CE before corporate
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allocations for mature centers stood at 25.2%. ROCE for emerging centers stood at negative 3.6% for Q4 FY '23 as compared to negative 8.3% in Q4 FY '22. This is again an improvement of 470 bps. ROCE before corporate allocations for emerging centers stood at negative 2.4% Our net debt position, excluding capital leases as on March 31, 2023, stood at INR198 crores as compared to INR190 crores on December 31, '22. Our expansion of existing facilities in Ahmedabad Phase 2 and Whitefield extension of Bangalore COE is on track. Total planned capex for Ahmedabad is INR85 crores, expected date of operations being Q1 FY '25 and for Bangalore COE is INR25 crores expected date of operations being Q3 FY '25. With this, I would like to open the floor for Q&A.

Moderator: Our first question comes from Dhara Patwa with SMIFS Limited.

Dhara Patwa Shah: Sir, I have a few questions. Still we are seeing increased significant growth in Africa and East India. So what are the reasons for this? And will the traction continue going forward for these 2 regions.

Raj Gore : Yes. Thank you, thank you for that question. In East India, our newest or youngest emerging

center, Kolkata, had shown 142% year -on-year growth in Q4, and we are really happy how it's progressing in the right direction. In addition, our mature centers, Ranchi, and Cuttack. Ranchi is showing 51% year -on-year growth last quarter. Cuttack, which is a very mature center, large center, it is showing 32% year -on-year. So both emerging, mature centers are showing high double -digit growth, which is contributing to show such a good growth.

In Ranchi, now as I discussed earlier, we are also putting -- upgrading the LINAC machines with higher capabilities will further help us to do more high -end, higher realization radiation treatment. Africa, we have commissioned a new linear accelerator which is first of its kind in East Africa. And I think as a result, we are seeing good growth momentum in our Nairobi Center, and of course, I mean, all this is sustainable, you will continue to see it going forward.

Dhara Patwa Shah: Okay sir, understood. Sir, in North India, which are the locations that we are covering like which are the hospitals there we are covering?

Raj Gore : It's only Jaipur.

Dhara Patwa Shah: Okay, only Jaipur ?

Raj Gore : Which is our emerging center , yes.

Dhara Patwa Shah: Sir, in earlier con call, you highlighted that now once we reach an optimum occupancy level in the emerging markets, we will see improved ARPOB growth, sir I guess our emerging markets are already at a good occupancy level of 68%. So should we expect an ARPOB increase from here on?

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Raj Gore : Yes, absolutely. So look, emerging centers, we are at 68.5%. But again, we have not operationalized all our beds in emerging centers. So as we keep reaching higher capacity utilization on beds, we'll keep operationalizing more and more beds, but yes, directionally, that's what we had -- that's the guidance we had given. If you look at our annual

ARPOB for emerging

centers, it looks like it's decreased over last year. But look at how we've ended the year . If you look at Q4, we have gone back at 36,000 level. It's the -- it's what we said that as the occupancy keeps going up, we'll start optimizing our payer mix and our procedure mix, business mix, and therefore, the ARPOB will continue to grow going forward.

Dhara Patwa Shah: Yes, sir, that's helpful. One last question on Slide #10, where you have highlighted the capacity utilization for LINAC. So just wanted to see what is the peak utilization for LINAC in any hospital -- cancer hospital. So we are at already 65% -- yes.

Raj Gore : So -- on -- doctor you can go ahead.

Dr. B S Ajaikumar : No, on the linear accelerator, capacity utilization, depending on the type of fractions we give, we can go up to 120 to 130 patients, but nowadays, with the different modalities of treatment like hypofractionation and all, we can even treat up to 140 new patients per month on the linear accelerator. It again depends on the type of linear accelerator and the technology we use.

Some of the linear accelerator that we use complicated technology like IMRT, IGRT, so we may take a little bit more time, so the number of patients may be less. So it's always depends on the mix of 3D CRT, but now the world is -- we are moving towards more high-end like IMRT, IGRT, so with that and the hypofractionation, we are very comfortable with the high -end linear accelerators we have, treating up to 120 patients on the machine per day.

Raj Gore : Thank you, Dr. Ajai. Just to add to that. From a capacity utilization perspective, we can go to 90%, 95% easily including downtime, holidays, lesser load on weekends, et cetera. So 90%, 95%, we've consistently done in our mature centers.

Moderator: Our next question comes from the line of Kunal Randeria with Nuvama.

Kunal Randeria: So sir, my first question is actually related to Slide 9. So when I see this slide, it seems the company has 2 different growth trajectories. Your mature centers are actually doing fairly well, growing at a very steady, consistent pace at both revenue as well as the -- your EBITDA trajectory. But the emerging centers, the EBITDA trajectory seems to be all over the place. Sir, just want to understand what is the growth for these emerging centers, see because your occupancy, your ARPOBs in Q4 are not too far away from the mature ones. So I'm just wondering where the profitability in some of these emerging centers will come from?

Raj Gore : Yes. So thank you, Kunal, for that question. See, the -- with any new hospitals, the way you do it is you may have 100% build capacity, but you don't operationalize total capacity, right? So you will open up part capacity. You will staff, you will add clinical talent, you will get to an optimum utilization on that operational capacity then you will again release additional build

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capacity to operationalize it. Then you'll have to again add clinicians, again, start your go-to-market, so that's the nature.

So it's a step-by-step growth. What we have done is our -- as we kept hiring clinicians and that's an upfront cost. So the cost comes first and then the revenue growth comes later. That's one factor. Second, our investment in brand and our go-to-market comes first and then the growth comes. So because of the nature of this stepwise upfront investments and then the subsequent lagging revenue growth, this trending looks a little different from hospital to hospital in that bucket. And that's the result you see on the slide.

: Ashutosh Kumar : Just to add one point there so on the capacity front or the utilization front, 65.7% occupancy is on our operational beds, and we have given a note in our presentation that only 75% of our capacity beds are operational right now. So on capacity beds, our utilization is

only 50%. So we

still have a lot of headroom there.

Kunal Randeria: Right. Okay. So -- and -- but then on your -- at least on your capacity beds, you are at optimum level, right, at around 65%. So I mean, my question is more -- I'm not too worried about the revenue side of things, but more in terms of the profit because see as you operationalize some of the other beds also, there will be some increase in cost, right, from clinician costs, some staffing costs, utilities, so on and so forth. So it just seems that you are now stuck in the 7% to 10% kind of margin bracket for the moment.

Ashutosh Kumar: See there are some costs, which probably would not be increasing commensurate with the increase in operational beds. For example, as Raj mentioned, that in our emerging centers at Mumbai and Kolkata, the focus has been to invest on clinical expertise and sales and marketing upfront. Now these costs -- first cost is in our clinician. The clinician cost, I don't think would go up in the same proportion as incremental beds become operational, and you will start seeing operating leverage on that.

Second is sales and marketing, which is like since the company has been performing really well, we have been investing on sales and marketing costs disproportionately higher in our emerging centers to fuel future growth. And within the emerging centers, our primary focus is in Mumbai and Kolkata regions

Kunal Randeria: All right. Okay. Got it. Sure. Okay. Now my second question is I would like to understand a bit more about your capex. Now the way I look at it is, it can be divided in 3 buckets. One would be your normal maintenance capex on all the centers that you have now. Second would be like you're adding new tech to the existing centers, something like the Self-search that you mentioned. And third would be what you would spend on expansion. So if I were to take a slightly longer-term view, right, so let's say, between FY '25 to FY '27 or something like that, could you highlight your plans that how would you be allocating the resources across these 3 buckets that I spoke?

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Ashutosh Kumar: See I mean the current year capex is about INR135 crores, our maintenance capex is in the range of INR55 crores to INR60 crores. We have also spent incremental capex in the current year FY '23 towards technology upgrade. So we have got the high-end radiation equipment, which is adaptive therapy ethos in our center of excellence in Bangalore. Then we have invested in solar power plant in Karnataka which would help us in reducing our operating costs.

Having said that, the company has laid out 2 greenfield projects, which the company is right now executing. One is in Ahmedabad Phase 2. And second is in Bangalore at Whitefield. We have also provided the capital outlay towards these 2 projects in our presentation. For Ahmedabad project, we have capex of INR85 crores, whereas in Bangalore the capex is INR25 crores.

Kunal Randeria: Got it. And maybe just to simplify it a bit more from me, let's say, if we were to spend INR100 crores in a year on capex, how much would be maintenance? How much would be made from the new technologies that you're investing in your existing centers?

Srinivasa Raghavan : So I mean while we have not given guidance per se on the growth capex. However, our maintenance capacity remains in the range of INR55 crores to INR60 crores.

Kunal Randeria: Okay.

Dr. B S Ajaikumar And all over you may want to add here that capex nowadays with high-end capex equipment, we are doing pay-per-use models. So because of that, our actual capex requirement has drastically come down. And going forward, we see this as the new way of even replacement capex, which we have done for high-end linear accelerator. Right now, we have done very successfully for high-end linear accelerators. In

future, we may also do for other equipment's.

So right now -- but based on what Ashutosh said, our replacement capex will be the most

requirement. And even that if it comes under pay -per-use, our requirement becomes less and less as we go forward.

Kunal Randeria: Sure, sir. That's very interesting because then would it be fair to assume that if capex will be lower in the outer years and maybe at the cost of compromising on some margins somewhere because pay -per-use I'm sure that will go to the previous quarter.

Raj Gore : Yes, it could be.

Moderator: Our next question comes from the line of Sabyasachi Mukerji with Bajaj Finserv.

Sabyasachi Mukerji: So first question is a clarification on the margin front. So if I look at your mature center revenue trajectory and the absolute EBITDA trajectory, the revenues have gone up sequentially, and it has been a phenomenon for the last probably 8, 9 quarters, that is very commendable. But from Q3 to Q4, the absolute EBITDA has dropped. And I think you have mentioned this because of some upgradation of LINACs, I missed the point, but this is structural because we are going for Healthcare Global Enterprises Limited

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a pay -per-use model, and hence, the rental will kick in, in the P&L and margins will look lower. Any color on that?

Raj Gore : No, it's not structural. As I said earlier, we have certain radiation machines in our mature centers, which are going through replacements. We -- in this entire digital transformation journey, we are also increasing incremental cost towards IT applications, licenses. That's the reason. There is no structural issue in this. Radiation, see radiation is a high-contribution business in all our modalities between surgical, medical, radiation. Radiation has the highest margin.

And therefore, any drop in radiation affects the total contribution margin that much more and as -- but what we are doing is in all these 3 centers, we are putting a high-end machine, which will give us capability to do high-end modalities, which have a higher realization and therefore, there will be improvement in your realization of radiation revenue in all these 3 locations. It will be more margin accretive going forward.

Sabyasachi Mukerji: Okay. Got it. So how many LINACs you have now total? And how many of them are in pay -per-use models?

Raj Gore : Yes. So we have 5 which are pay -per-use model out of 32 LINACs. Most of these LINACs have been installed years ago. It's only last couple of years, we have started moving towards pay -per-use model. And we expect going forward that all our replacements will start moving into pay -per-use model. And therefore what Dr. Ajai was saying that our capex requirement will go down.

Sabyasachi Mukerji: Understood. Follow-up to the margin question, so basically, if I look at a full year basis, the operating metrics that you have shown in Slide 10 that has improved strongly every aspect of the operating margin -- in the operating metrics. But if I look at the overall margins, overall EBITDA margins of the company it has adjusted somewhere around 18.8%, 18.9% and strongly refuses to move beyond 20% and probably missing piece of the puzzle here is your emerging centers still operate at a 7%, 8% kind of EBITDA margin, where the mature centers is doing at 24%, 25%. My question is, when can we see these emerging centers move up the ladder and revenue?

Raj Gore : See as a bucket, I think it will be about 18 months. But let me give you more details. Jaipur is already in mid -20s. Borivali is in mid -20s, so some of the emerging centers are already at that margin level. As a bucket in another 18 months, we should get to the mature level.

Moderator: Our next question comes from Nitin Agarwal with DAM Capital.

Nitin Agarwal: Two questions, sir, on this pay -per-use LINAC model versus an outright buyout. In your assessment, what

is the difference in ROCE between the two from -- how does the financial the economics really work out for you in both the models?

Ashutosh Kumar: Yes. So I mean, Theoretically, income from a single patient on our pay -per-use machine is ROCE accretive because there is no capital involved. Now how does this play out when it comes to profitability is that almost about around 20% to 25% of what we generate from patient care Healthcare Global Enterprises Limited

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goes to the manufacturer. However, what we have also done and observed is that due to upgrade of these machines from the previous versions, we are able to give high-end therapies to our patients. So a lot of costs, which is going to the manufacturers also get traded off due to higher recovery from the patient. Further the pay -per-use model includes all maintenance cost and future upgrades

Dr. B S Ajaikumar : Nitin, what Ashutosh is trying to say is that the pay -per-use model is also good for a few things. One is when the beginning you may have a low volume of patients. So you are not really

investing in the capex even with the lower volume, we're okay because this per patient you're paying, you've not invested and you're not -- your debt is not increasing and you're not paying any -- debt payment is not happening.

Point number 2 is because of the high -end units we are getting, we are able to do more of IGRT IMR T in even Tier 2, Tier 3 cities, where the revenue model is pay per -- the patient pay model is higher, because of that, our contribution factor will be high. So essentially, that usually whatever we charge is usually paying off what extra -- paying off what we may have to pay to the manufacturer.

So essentially, it makes us a win-win situation for us, working with the manufacturer with no investment from our part and no -- like no CMC, no custom duty. So everything comes -- we believe it comes in our favour when you look at it, we have done deep steady on this. That is the conclusion we have come to. Long term, it really works out well. And after so many years, like 8 years or so, it essentially becomes our own unit.

Nitin Agarwal: Okay. And doctor, broadly speaking, in your assessment over the next say 2 to 3 years, how much capex would be saving on in terms of cash outlay based upon our expansion plans?

Dr. B S Ajaikumar : Yes, if you look at replacement capex on LINACs, we may have to replace another 7 to 8 linear accelerators. Am I right, Ashutosh? Close to that, at least \$22 million?

Ashutosh Kumar: Yes.

Dr. B S Ajaikumar : And not only replacement, the new instalments when you look at second linear accelerators, we may be in the region of 10 to 12, so you are talking about each unit about \$2 million. So you are looking at approximately \$20 million, \$25 million.

Nitin Agarwal: Okay. Okay. That's helpful. And secondly, on the mature centers, while we did just talk about the fact that Q4 had some one-offs because of which numbers are a little down, but these EBITDA for these centers has been in the 74%, 77% range -- INR70 crores range for the last 3 or 4 quarters, all through FY '23, so Raj, where do we see this number? I mean do -- is there a scope for a meaningful delta on this number? Or this is where we're probably starting a bit of a plateauing situation in this -- in the mature centers on the EBITDA front?

Srinivasa Raghavan : Before Raj takes up, just on the numbers, while you said INR74 crores, INR75 crores, those were -- those are reported numbers. And if you look at the adjusted numbers, it has moved up

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from INR75 crores to INR76 crores to INR83 crores, so there is a delta of INR6 crores, INR7 crores from quarter 1 to quarter 4. Now in ...

Raj Gore : Yes. And as I explained, this is not -- this quarter is not the end goal for us. It's not the last over

r,

right? We are -- as I mentioned earlier, we are continuing to invest in our sales/marketing expenses. Our sales/marketing expenses are almost 1% more than last year this year. And that's the right thing to do because I believe that we have -- we not only have expertise, we have spare capacity. There is enough demand supply. It's important that we create visibility of HCG brand so that we grow footfalls going forward. That's the right thing to do for the business on an ongoing basis.

Nitin Agarwal: Got it. And Raj sir on the last one on the INR14 crores, INR15 crores that you spent on the consulting expenses this year, what kind of payoff do you see on that? And when do you start to see it coming through?

Raj Gore : So we had already guided the market that we were expecting our margins to go up in the range of 150 basis points. Of that, we have sort of right now at least 60%, 70% of that benefit we have already seen in the current year and some of the benefits like now would commence as we go forward in the next few quarters.

Nitin Agarwal: Okay. And if I can squeeze the last one. On any -- what are the -- what is the way forward for us on the fertility clinic business? How are we seeing that business -- is it a strategic business for us, is it core? Are we looking to invest in this business incrementally?

Dr B S Ajaikumar Yes. As we have said, we are now -- because of the COVID and post recovery took us some time, we have reorganized the fertility and we are also not going to do any of the merger we have planned with Ovum. So we feel there is a significant upside for us to focus on the facility only. So with this, we are looking at good improvement this year, and we have put a lot of systems in place.

And we want to be definitely dominant in Bangalore, so our focus going forward will be in Bangalore. So going forward in the next few quarters, we'll definitely update how we are doing but we do see quite a few positive signs for our growth. As you know, we have made it very clear at some point we want to divest. So we will decide at what points to divest.

Moderator: Our next question comes from Shyam Srinivasan with Goldman Sachs.

Shyam Srinivasan: Just one on the outlook for fiscal '21 -- '24, Raj, Dr. Ajai, how should we look at revenue growth. Firstly, we have done 21% this year. And if you could help us disaggregate into either changes

in ARPOB that you foresee, we have about 4% increase ARPOB overall. So just want to see how we should look at ARPOB versus the utilization improvements.

Raj Gore : Yes. So as you said of 21% revenue growth , 4% is ARPOB, rest is volumes. If you see even NPR, which is the new patient registration which is the most important metric for us because 1 patient coming in, getting treated for multiple modalities and the lifetime value, so we have had

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a very good 18% growth on a year -on-year in NPR. So largely, it's led by volume. As I mentioned earlier, first, we want to drive capacity utilization before we start optimizing all other financial metrics.

So we'll continue to do that. On going forward, we don't actually give forward -looking guidance but if you look at our track record for years and including last several quarters, we have outpaced the industry growth and we will -- our endeavour is to continue on that journey and grow at a higher rate than the industry growth.

Shyam Srinivasan: Yes, that is helpful. So what is the industry growth you think 15%, 14%?

Raj Gore : So cancer market is growing at 11%, 12% annually CAGR over a few years, of -- I mean last 5 years, it's grown about 11%, 12%. It's expected to grow at the same rate going forward, about 12%. So that's the cancer market growth in India right now.

Dr. B S Ajaikumar : And also, Shyam, I want to just add one thing. See, when we look at the growth for us historically for HCG, we

have been very strong in radiation oncology.

Shyam Srinivasan: Right.

Dr. B S Ajaikumar : And even now, our radiation oncology is a big strength, where the contribution factor is very high. And when we look at the places like even Mumbai, Jaipur, we are adding more units and similarly in Vizag. So a lot of our centers, including mature centers and also growing. We are adding more units. So we feel our growth in terms of our radiation chemo administration will excel quite a bit in the coming year. That is where we are going to see a significant growth happening.

Shyam Srinivasan: Got it, sir.

Raj Gore : And Shyam, I think we have shared this in the past also that while we continue to grow organically, we are also looking at inorganic opportunities. We had several targets that we were evaluating. We have made good progress and hopefully, we'll be able to share more details in coming months on that front.

Shyam Srinivasan: Understood. If I were to look at Slide 13 and just looking at your regional cluster breakup, where do you think there is scope for further improvement? I'm just looking at headline numbers, like Maharashtra has only grown 6%, let's assume, right? So just the ones where growth has been slower for the full year -- or Andhra Pradesh for the quarter, let's assume. So is there something regional specific, cluster specific strategy that we have in the next 12 months?

Raj Gore : Yes. So growth, the -- while the mature centers continue to grow at a steady state, steady rate, the growth will come from emerging centers. Emerging centers , we have 2 in Mumbai, Maharashtra, 1 in Kolkata and then you have Rajkot, Jaipur, which are showing very good traction. You mentioned Maharashtra, it's showing 6%, but if you look at, I think that's largely

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because in Maharashtra, in Nashik, in both Mumbai hospitals, we had large COVID and vaccination revenue last year.

If you look at our oncology revenue growth year -on-year in Maharashtra, that 6% will become 15%. If you look at -- if I can draw your attention to year -on-year growth in Q4, it's reflecting 19% because by Q4 last year, COVID and vaccination was not there. That is a real reflection of our oncology business growth there. If you look at Mumbai, we are -- in Mumbai, in Q4, we have grown about 24% for the year in Mumbai for oncology business, like -to-like, we have grown 41% over the previous year.

So Mumbai is growing very well. Mumbai, Kolkata, emerging center as a bucket will be the future growth drivers going forward. And as I mentioned -- as we have been mentioning, that 2 of our top centers of excellence, Bangalore, and Ahmedabad, we are adding capacity in terms of new facilities. We are adding 1 in Whitefield to continue to grow our market share in Bangalore. And Ahmedabad, we are shifting to a newly modern build 200 -bedded facility by beginning of next financial year. And that will ensure that these 2 centers will continue to play a major role in our growth going forward.

Shyam Srinivasan: Got it. My last question is just on the -- I remember 12, 18 months back, we did a price

benchmarking exercise of our prices with competition, and we took some price increases up. Do we foresee that will likely come back again? Is there pricing power you think where you can take prices up. The ARPOB growth, like you said, has been 4% only. So is there an element of price increases we can take in '24 or '25, let's assume.

Raj Gore : So we had -- 12 months ago, we had done it -- actually 15 months ago, we started doing that exercise in Bangalore. During this last financial year, we have taken it to the rest of 10, 12 major facilities. So that's been rolled out recently. We don't see it happening again soon, but we continue to do our inflationary price increase every year in -- effective from first April, which we have already done. I t

think as I stated before, we do have capacity in many of our centers. We do want to go for driving volumes first. In the meantime, we are upgrading technology. We are upgrading our clinical strength and we are investing in building brand, combination of all 3 when the capacity starts reaching to optimum level should give us our pricing ability to charge premium, considering that anyway, in 2/3 of our locations, we have a market leadership position locally in oncology market. So we are working towards that. As of now, the focus is on volumes, volumes, volumes, driving capacity utilization. But naturally, it will move in that direction as we start reaching optimum utilization.

Moderator: Our next question comes from Aditya Jhavar, an Investor.

Aditya Jhavar : Yes. I have a question on the cash flow statement. I see there is a receivable write-off of INR31-odd crores and there are again doubtful debts for, I think, 2 to 3 years, INR11-odd crores. So could you throw some light on this part?

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Srinivasa Raghavan : Thanks, Aditya, the thing is we have receivable policy positioning. So based on that, on the credit customers, we do make provisions based on the receivable, based on the collection trends.

So the write-offs that you see in the books of accounts is basically the provision that we have created in the past. And over a period of time, we have verified it is compatible that's what we have cleaned it up as part of this exercise.

But having said that, it's not that we cleaned it up on dealer that's the end of the story. We will keep monitoring this and as and when there is an opportunity to put these collections, we will continue to do so. It's more a financial discipline exercise and our efforts to kind of drive the receivables ensures that we are behind this and collect as much as possible continues to be on.

Venkat P Just to add to what Srini said, since you picked up on the cash flow the net impact because of the bad debts and the provision is a write-back to the PBT because it's not a cash outgo, just making it clear.

Aditya Jhavar : But in the business, I'm still not able to understand receivables how it got stuck like INR31-odd crores is a very big amount. So could you again briefly explain here?

Srinivasa Raghavan : Yes. Listen Aditya this is not a 1-year phenomenon. It's over a period of time. We are looking at last 5 to 6 years. So as I said, on all our credit customers, we have been creating provisions based on the aging of those customers. And based on that, the provisions have got accumulated over a period of 5, 6 years that INR35 crores we are looking at is a provision that has been created over a period of time that we have realized is not collectible at this point in time. So based on a good financial discipline, we decided to write this amount, write up this amount.

Aditya Jhavar : Okay. And my second question is on the margin strength. If I see the core strength in margins, we have coming around 10% to 11%, which has been for the past 2 years. But as we know that in the hospital sector, generally, when the capex comes over, we see that the margin should trend upwards. But from the last 3 to -- from the last 6 to 8 quarters, we are sitting near 10% to 11% on margin. When we can see that bump up in the margins, if you can give in maybe 2- or 3-years perspective, like emerging centers should contribute how do you see this going forward?

Raj Gore : So Aditya I mean, we are not able to relate to your 11% margin because our reported adjusted - sorry, our adjusted EBITDA margin is 18.8%.

Aditya Jhavar : I'm taking out rental also here. If you Post IndAS. Pre IndAS. Yes.

Ashutosh Kumar: pre-IndAS?

Aditya Jhavar : Yes.

Ashutosh Kumar: pre-IndAS is 15%.

Srinivasa Raghavan : Pre-IndAS cannot be 11%, Aditya. It should be a higher percentage. But the larger question is

how do we drive margins across the emerging countries.

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Aditya Jhavar : Yes, yes.

Raj Gore : So Aditya, I mean, if you look at last 2 years, 8 quarters, we've grown from 13% to almost 19% in terms of margin, post IndAS. Our existing centers have gone from 18% to almost 22%. right? It's the new center bucket that was dragging it down earlier. It was negative, now it's coming positive. I have -- we've been explaining that while we reached to a certain level of margins 3, 4 quarters ago, we decided that we want to go for scale.

And therefore, we started spending our clinical talent and investing in go-to-market building brand to make sure that we ramp up our hospitals faster than what we have done historically. Our emerging markets are new markets to us, we have to grab market share from someone else. So we decided to go on a front foot aggressively to grow these volumes.

I think what is happening the margin trend is a reflection of these decisions, which we feel are the right decisions for those businesses, right? Now fundamentally, we've shown again and again that the model delivers good margins in mid -20s, whether it is irrespective of the location that they are in, whether it's new -- whether it's metro, whether it's non-metro, Tier 3, Tier 4. We've proven it multiple times. It's a matter of fact when all cylinders start firing in the emerging bucket that it starts moving in the right direction towards the mature center margin.

Srinivasa Raghavan : I think in fact Raj has given a time frame of about 18 months to kind of for the emerging centers to fall in line with the mature centers. And I think that's a good starting point.

Raj Gore : And as I said, Jaipur in last 2 years have gone to mid -- early 20s. Borivali is gone to 20s, right? So even in the emerging center bucket, there are hospitals which are already reaching that level.

Aditya Jhavar : Okay. That helps. That helps. And the last question I have is on the LINAC machines. So currently, we have 31 odd LINAC machines, right? But correct me if I'm wrong. Correct?

Raj Gore : Yes.

Aditya Jhavar : Okay. Now we have current capacity utilization as 68 -odd percent, but you said that we can move up to 90 plus percent. So how do we increase here capacity utilization? And how fast we can ramp up here? Or it depends on the more CIC, the number of patients also going up. so which is a good amount, then how do the dynamics change here, the increase in capacity utilization of LINAC. And for the 2 to 3 years, how much more we are trying to add that also in LINAC how many machines because you said we are going for pay-per-use model and this one here.

Raj Gore : Yes. Yes. So Aditya, if you look at the page on operating metrics, it shows that year-on-year, our utilization has gone from 59% to 66%. On quarter-on-quarter, it's moved from 60% to 65% in Q5 -- in Q4. In -- this is an average utilization for the entire bucket. We have many hospitals where we are already 90%, 100% utilization. For example, Borivali, Borivali we've been clocking 95%, 100% utilization. We had a bunker. We want to add 1 machine. The machine is commissioned as we speak. This month, we have commissioned the second machine. So the Healthcare Global Enterprises Limited

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answer lies hospital by hospital. We have identified hospitals where we already are hitting utilization, right? Ranchi is one more example. Jaipur, Jaipur we have 2 the utilization is in higher 80s.

What we've learned from our -- some of the past experiences that we don't need to wait for it to reach 90%, 95% because there is a long lead time to order machine and install it. It takes 6 to 9 months for that. Therefore, as we start hitting 80%, 85% in respective hospitals and wherever we have bunkers will start ordering machines and deploying it. Where we don't

have bunkers, we'll start creating bunkers.

As we speak right now, we have about 4 machines that are -- 5 machines that are getting installed in various places. Borivali, Ranchi, Ongole and Jaipur. That's what is happening right now, and these machines will get commissioned in this 2, 3 months period, current period. We have also identified some of the other hospitals where we would like to add machines. For example, Nagpur, for example, Kolkata. So we have a list of units where we would like to now increase capacity by adding a LINAC. We have planned for the year and the subsequent year. And this pay-per-use model if there is a possibility to deploy it at right time without incurring capex in a margin-accretive and a ROCE-accretive manner going forward.

Moderator: Our next question comes from Rishabh Tiwari with Allegro Capital Advisors.

Rishabh Tiwari: My question was regarding the pre-IndAS numbers, which was being discussed in the last question as well. Earlier, we used to report it used to be around INR65 crores odd number in IndAS impact annually, what would be the ballpark number for this FY '23 here?

Srinivasa Raghavan : This is the IndAS impact is INR70 crores annually. So if we are looking at INR321 crores for the full year, pre-IndAS numbers probably would be about to INR250 crores.

Rishabh Tiwari: About -- sorry, sir I lost you.

Srinivasa Raghavan : INR250 crores.

Risha bh Tiwari: Okay, sir. So Pre -IndAS would be higher than the post-IndAS number, is it?

Raj Gore : No, no, no. Our post -IndAS EBITDA number for last year is INR321 crores. If you want a pre-IndAS like-to-like number, as we reported earlier, you will have to remove about roughly INR70 crores from it, which takes it to about INR251 crores.

Moderator: Our next question comes from Yogesh Tiwari with Arihant Capital.

Yogesh Tiwari : My question is basically on right -of-use assets, basically related to balance sheet. So you're right of use assets have declined in FY '23. Also your other intangible assets has also declined. So what would be the drivers for it, sir?

Raj Gore : Yogesh, can you repeat your question? There is a lot of disturbances in the background.

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Yogesh Tiwari: Yes, my question is the right -of-use assets has declined in FY '23 from about INR400 crores in FY '22. So what would be the driver for the decline? Also, there is a decline in other intangible assets? So if you can share some thoughts on it.

Venkat P So Yogesh, last time when for the year -end call, we had explained this thing that we revised our policy of capitalization of right of use assets, we had taken only the lock -in period of most of the assets, which were only 8 to 10 years. And we wrote back, or we reduced the ROU assets of the impact of 10 years capitalization. That's why you saw INR300 crores reduction last year on account of this, plus, as we keep paying the rent, as we know, ROU is nothing but the capitalization deficient rentals. And as you keep paying rent, this will also organically come down. That is the reason why you are seeing a decrease in the current year also.

Yogesh Tiwari: So sir, the decline in rental would be related to which facility?

Srinivasa Raghavan : No it is not -- it is relating to all the facilities where we are paying rent, so which is almost across all the locations. So a better rightly explained, as the year goes by, the capitalization would keep coming down, and that's the reason for the reduction.

Ashutosh Kumar: It would capitalize rent .

Yogesh Tiwari: Okay. Sure. And sir, one last question on the upcoming capex . So at Whitefield and at Ahmedabad. So as you told earlier, it takes about 18 months for an emerging asset to become mature. So can we assume that the 2 upcoming assets at like Whitefield and Ahmedabad, it will start giving the targeted ROCE by the second half

of FY '26?

Srinivasa Raghavan : Let's put it this way. Ahmedabad is not a [a new center We have just lifted in the existing hospital and putting it at a new place because we have run out of capacity at existing center. [inaudible 1:06:38]. So given that we should be able to maintain the momentum, rather improve the momentum as far as this center is concerned. As far as Whitefield center is concerned, it's an addition to our existing center of excellence, so we are optimistic that we should be able to pick up volumes very quickly from the induction stage itself.

Yogesh Tiwari: Sure. So just on the timeline , like we expect that to, for example, Whitefield to complete in the first quarter of '25, so we can expect like FY '26 would be the major contributor for this keeping in mind the timeline ?

Srinivasa Raghavan : See one of the contributors , it's one of the contributors as Raj and Ashutosh explained, there are many activities that are happening, result is expansion. We are also looking at M&A activities. We are also looking at emerging centers kind of coming at par with new centers . All this will be a major driver to propel the overall growth of the organization.

Raj Gore : And just to add to what Srinivasa said, the -- our center in Whitefield in the niche center primarily from the facilities that we want to consolidate and improve our market leadership in Bangalore.

It's going to be a small center compared to what we already have in Bangalore, and it is more

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aimed towards like increasing our market share in Bangalore, going to be primarily catered radiation and medical oncology center that we do have some surgical OTs and big beds as well. However, it's going to be add -on center to our center of excellence. So it's going to be -- it's not going to drag us as far as paper debit is concerned, other will add to the incremental revenue and as Srinivasa said, Ahmedabad is going to be just a shift of the hospital earnings not going to change the economics of the business.

Moderator: Ladies and gentlemen, we have reached to the end of the question -and-answer session. I would now like to hand the conference over to Mr. Raj Gore for closing comments.

Raj Gore : Yes. Thank you. Once again, I take this opportunity to thank everyone for joining your call, keeping -- keep having this interest in our organization, our growth story, we'll keep you

updating on a regular basis for any incremental updates we have on the company. I hope we've been able to address all your queries. If there are more queries, please get us -- get in touch with us or our Investor Relations advisers, Strategic Growth advisers. Thank you once again and talk to you next time again. Thank you.

Moderator: Thank you. On behalf of Healthcare Global Enterprises Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2023

August 18, 2023

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on August 11, 2023

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on August 11, 2023, with Analysts/Investors to discuss the Unaudited Financial Results of the quarter ended June 30, 2023.

This is also available on the website of the Company www.hcgoncology.com.

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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Q1 FY '24 Earnings Conference Call"
August 11, 2023
MANAGEMENT: DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. SRINIVASA RAGHAVAN – CHIEF FINANCIAL
OFFICER – HEALTHCARE GLOBAL ENTERPRISES
LIMITED
MODERATOR : MR. SAGAR SHROFF – SGA

Healthcare Global Enterprises Limited
August 11, 2023

Moderator: Ladies and gentlemen, good day, and welcome to the Q1 FY '24 Earnings Conference Call of HealthCare Global Enterprises Limited. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Dr. Ajai Kumar, Chairman for HealthCare Global Enterprises Limited. Thank you, and over to you, sir.

B.S. Ajai Kumar: Thank you very much, and good morning, and a warm welcome to all those present for the Q1 FY '24 Earnings Conference. I'm here joined today in our corporate office by Mr. Raj Gore, Chief Executive Officer; Srinivasa Raghavan, Chief Finance Officer; and also the senior management team. I will be sharing our operational financial highlights for the quarter ended June '23.

While we have always looked upon value as being a center of excellence for several years, similarly, we are now trying to establish center of excellences in Mumbai, Ahmedabad and in future in Kolkata. And in this regard, we are also looking at further growth of HCG centers across the country. And I'm very happy to report that we have made an acquisition in Indore, the SRJ CBCC Hospital Indore.

This is a comprehensive cancer center, 50-bed facilities, and it is scalable up to 100 beds in future with facilities offering medical, surgical and radiation oncology. It's the largest private player and leader in Indore in private section. And one of the things we have looked at in the future in M&A, is how do we enter Central India. We have a strong presence, as we know, in Maharashtra, Gujarat, Karnataka and in the Eastern India. This will give us a presence in Central India, and it will propel the growth for us in this part of the country, particularly with the hub-and-spoke model we use.

With these acquisitions and a few others we have done, and we are in the process of doing in the future, we will continue to dominate the oncology market in this country. Coming through the medical excellence, what we are focused on in the field of economic and research, as we all know, HCG has been a pioneer in excellence, data collection and outcome focused. In future, this will improve the outcome for our cancer patients and of course, improve the modalities of treatment.

We are also very proud to say that we have established multiple robotic surgery units across India, and this will once again provide the best of care to our cancer patients across the country. This will bring about a change where our data in the future, particularly the work we are doing
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in the areas of genomics, circulating tumor cells. We continue to position ourselves as a cancer destination.

With these few words, I would like now to hand over to Mr. Raj Gore, CEO of HCG, for his comments on the strategies going forward and operational performance. Raj?

Raj Gore: Thank you, Dr. Ajai. A very warm welcome to all the participants on the call. Allow me to initiate my comments by elaborating on the recent advancement in this quarter.

After consistently delivering organic growth over the series of many quarters, we have stated our clear intention to accelerate HCG's growth with inorganic prospects. We are delighted to share that we have initiated

our journey of expansion through inorganic means, successfully completing the acquisition of two centers situated in Indore and Nagpur. These are strategically significant and attractive locations in central part of India, and both assets have an already established market leadership position. With our time-tested and highly replicable playbook of driving sustained post-acquisition growth across geographies, we are confident that these two hospitals will contribute to HCG's growth in the forthcoming years.

Speaking on Nagpur. You note that we have been operating comprehensive cancer care centers since 2018 in partnership with Dr. Ajay Mehta, who is a minority partner. However, there existed a complex structural arrangement between two entities, HCG and NCHRI LLP, which is our operating hospital; and NCHRI, which is 100% owned by Dr. Mehta.

Nagpur is a strategic market for us located in Central India. It promises significant growth in future supported by large capacity. We have a market leadership position that we have already

worked hard towards, thereby creating a compelling case to simplify the structure by acquiring stakes from Dr. Mehta in both the entities. This is expected to result in enhanced operational efficiency and generate larger economic benefits.

With our already existing presence, strong presence in Maharashtra, Gujarat, East India, our pursuit of footprint in Central India would strategically position us across larger India.

Discussing the performance during the first quarter of fiscal year 2024, we recorded higher ever revenue for the 10th consecutive quarter of INR 461 crores with a 13% year-on-year growth.

This growth was propelled by 22% year-on-year growth in our emerging center, highest ever quarterly revenues in medical value travel and digital channels and overall improvement in operating metrics.

We recorded EBITDA, excluding ESOP, of INR76.4 crores for Q1 FY '24 with 5% Y-o-Y growth. This lower-than-expected EBITDA growth was due to subdued incremental EBITDA on account of two factors, apart from the annual inflationary cost increase. First, investment in clinical talent. As indicated in previous quarters, we are aggressively pursuing to increase our market share in emerging markets primarily driven by clinical talent acquisition, which is visible in the growth of our revenues.

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Second, delayed LINAC installation and upgradation, downtime and installation delays of LINAC machines at five locations resulted in loss of more than 300 operating days on radiotherapy in Q1. This impacted our revenues from radiation oncology, which is a high-margin segment affecting our overall margin profile. However, four out of these five LINACs have already become operational during quarter 2. We expect this to be normalized in second quarter. In summary, we anticipate improved performance in the coming quarters with the operational LINACs and the operating leverage effect contributing positively to our revenue and margin dynamics.

With this, I hand over to CFO, Srini, for financial highlights.

Srinivasa Raghavan: Thank you, Raj, and very good morning to everyone. We have uploaded our Q1 FY '24 financial results and updated investor presentation on the stock exchange's and company's website, and I do hope everybody had an opportunity to go through the site. So let me take you through a few highlights regarding the sales.

On the revenue front, revenue split between HCG and Milann stood at 96.5% and 3.5%, respectively, for Q1 FY '24. Revenue growth for HCG stood at 14% Y-o-Y to INR444 crores in Q1 FY '24. As mentioned in Slide 10, revenues on the matured centers stood at INR331 crores, a growth of 11% on Y-o-Y basis for Q1 FY '24. Revenue from emerging centers stood at INR113 crores, a growth of 22% on Y-o-Y for Q1 FY '24. We are delighted to step at our emergin

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centers reaching towards maturity and are seeing good traction across geographies.

Moving to Slide 11. These are the key operational KPIs for our company. Our chemo sessions increased by 9% Y-o-Y taking the total chemo sessions to 35,000 in Q1 FY '24. LINAC capacity utilization stood at 69% for Q1 FY '24, versus 67% for Q1 FY '23. Onco-Bed occupancy stood at 66% for Q1 FY '24, compared to 65% for Q1 FY '23.

I now request your attention to Slide 12, where we have disclosed our operational parameters across our matured network and emerging centers for Q1 FY '24. Our company AOR stood at 66.9% and AOR for matured business emerging centers stood at 67.1% and 66.4%, respectively. Our ARPOB on company level stood at INR39,686, and our ARPOB for mature center stood at INR41,253 and for emerging centers stood at INR35,766.

Across geographies, we have given our revenue breakup in Slide 13. Kolkata grew by 54%, Rajkot by 51% and Ranchi by 27% for Q1 FY '24. For our Milann business, revenues for Q1 FY '24 witnessed a degrowth of 5% in revenues. On the EBITDA front, our EBITDA excluding ESOP for Q1 FY '24 stood at INR76.4 crores with EBITDA margin of 16.6%. We have also given bifurcation of our EBITDA across mature emerging centers. I request the participants to view Slide 10 for further details.

Other insights and EBITDA have already been shared by Raj, and let me move on to PAT. PAT for this quarter stood at INR7.6 crores as compared to a profit at INR6.1 crores in Q1 FY '23.

Our PAT pre-IndAS adjustments for Q4 FY '23 stood at INR11 crores, as compared to INR9.4

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crores in Q1 FY '23. RoCE in our matured networks stood at 20.2% annualized for Q1 FY '24, as compared to 19% in Q1 FY '23, an improvement of 120 bps. RoCE before corporate

allocations to a mature centers stood at 24.5%. RoCE for Emerging centers stood at negative 5% for Q1 FY '24, as compared to negative 4.6% in Q1 FY '23.

Our expansion of existing facilities of Ahmedabad Phase 2 and Whitefield expansion of Bangalore COE is on track.

Total planned capex for Ahmedabad is INR85 crores, expected date of operations being Q1 FY '25 and for Bangalore COE is INR25 crores, expected date of operation being Q3 FY '25.

With this, I would now like to open the floor for Q&A.

Moderator: Thank you very much. We will now begin with the question -and-answer session. We take the first question from the line of Kaustubh Pawaskar from Sharekhan. Please go ahead.

Kaustubh Pawaskar: My first question is on the EBITDA -. So Raj, you just mentioned that there were two of the one-offs -- five of the LINAC machines were not operational during quarter. So could you help us to understand how much was the impact because of this loss of operation? Because sequentially or year-on-year, if we look into the utilization of the LINAC machines, both were higher on quarter-on-quarter as well as Y-o-Y basis. So just want to understand why there is a loss in terms of margin or business?

This is Kaustubh here. Just want to continue the question on the EBITDA front. So Raj, you just mentioned that there was issues related to LINAC machines, five of the machines were not operational and that had an impact on the profitability because these are the high margin generating businesses for you. So I just want to understand what could be the impact because of this non-operational five LINAC machines. And from quarter 2, whether things would normalize in terms of profitability, or will it take some time?

Raj Gore: Yes. Regarding the Linear Accelerator Machine, three of them were a replacement. So when you bring down the machine, there's a time lag, we - expected they will be done - in the first quarter itself. Now they're all operational in the second quarter.

So we are now not going to see any impact in the second quarter. And certainly, the numbers

what we have seen in June are very promising. So we should be able to reach our expected level in the second quarter. Obviously, we will see the further growth of these new machines, which are coming in, - the second, third and fourth quarter, which will hopefully meet - meet our - expectation -.

And also, as - mentioned, we are beginning to invest in areas like in Mumbai with high talent.

So talent acquisition has also cost us in quarter 1, and that was part of the reason we had this margin issue. But we are now catching up, and we should be able to do well as we stated in the second and third quarters.

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Kaustubh Pawaskar: Right, sir. But talent acquisition, this is something which will continue because you want to expand to newer markets, and that is something which is part of your business strategy. So I guess that part will continue going ahead as well?

Raj Gore: Talent acquisition is part of the strategy, but when we bring talent at a rather higher cost I think it's cost versus talent. These clinical talent have come at little bit more expensive than what we normally do, particularly in a competitive market like Mumbai. And that is why our cost there has gone up. but that will get normalized as we move forward. And now as the revenue increase because of these talents, we see an increase in the revenue coming from this, and that will get it normalized. payout to our doctor to our overall revenue ratio.

Kaustubh Pawaskar: Right. And sir, one last one on the debt part. So debt has gone up on the books, and you mentioned the reason that because of acquisition, the debt has gone up. Should we expect any further increment in debt considering the ongoing capex, which is there on books? Or from here we should expect the debt levels gradually to come down or remain at same position what they are at the current level.

Srinivasa Raghavan: Yes. The thing is the debt level that you are talking about is planned for the acquisition that Raj and Dr. Ajai spoke about. But internally, the way we have put our budgets together is most of our capex should be self-funded. So the debt number that you are seeing, I think we should be able to kind of sustain it.

Moderator: Thank you, sir. We take the next question from the line of Jay Modi from EIML. Please go ahead, sir.

Jay Modi: A couple of questions from my side. So first was around the chemo sessions. In previous quarters, we've seen the growth to be higher at around mid-teens to high-teens also this quarter, we've seen 9% growth. So any particular reason for this?

Raj Gore: Can you please repeat your question?

Jay Modi: So my question was around chemo sessions growth. In earlier quarters, we've seen this growth to be higher at mid-teens to high-teens. Whereas this quarter, we've seen only a 9% growth. So any particular reason for this?

Ashutosh : These chemo numbers were higher in the previous quarters because some of the centers in the

emerging markets were ramping up. For example, Jaipur and Nagpur which is our emerging centers have led to the higher volume growth in chemotherapy. Some of the emerging centres have exponentially grown last year and we are experiencing normalization of growth in these centres. However, we expect incremental volume growth in chemotherapy shall be driven by Kolkata and Mumbai going forward.

Jay Modi: Okay. And do these sessions have any element of price increase? Or what is the volume growth ideally translating to revenue growth for us?

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Ashutosh : We take an annual inflationary price increase to the extent of about 4 to 5%. However, what we have experienced adoption of high end molecules in medical oncology. Greater adoption of immunotherapy in medical oncology is leading to higher average realization. I would request

Dr. Ajai to further dwell on this.

B.S. Ajai Kumar: Yes. On this matter, we are seeing a change in the approach how we manage our oncology patients. There are two things here. One is we all know cancer patients are now living longer.

And cancer has become a chronic disease. So as cancer patients, particularly repeat cancer patients, when they come back and when we do the first-time right treatment, there are certain number of people, who will recover.

And particularly for them, and even for first time people as technology advances and as more genomic studies and all come, when we become more patient specific, we are beginning to see treatments like immunotherapy becoming very prominent apart from genomic testing. The immunotherapy drugs or patented drugs they are quite expensive.

So you will see a definite ramp up in our oncology section, where our revenue is increasing. Our percentage of oncology contribution in medical oncology is also increased. So this is what we are beginning to see. And this may be only the beginning because in the future, the therapies like immunotherapy will increase significantly.

Jay Modi: Got it. And in chemo sessions, do we, so in chemotherapy, do we have any element of scheme-based patients where in the government puts the bill or it is largely the private patients, which forms the majority of share?

B.S. Ajai Kumar: Even in scheme patients, it is gradually happening because now the states like Orissa to some extent, Gujarat, probably Maharashtra are beginning to recognize the importance of this kind of targeted therapy and even though they are expensive, they are beginning to approve. Of course, it is all related to the budget of these people, what the states have in health care.

So they will have a final say, yes or no. But in the long run, I think states will have to recognize.

And at some point, of course, they will become generic, that will become far, far cheaper for governments to approve. But we are beginning to see that trend.

Ashutosh : And if I can just add, the question you asked is whether chemotherapy is being delivered to scheme patients. So just to mention that, our scheme patients distribution across the modalities are more or less are similar, however it is little skewed on a positive side in chemotherapy, primarily led by our Tier 2 centers, where we see late stage patients who typically undergo medical oncology treatment due to spread of the disease.

Jay Modi: Got it, and so what is the ideal mix between, what is the share of scheme patients in an overall revenue, scheme based patients?

B.S. Ajai Kumar: When you talk about only schemes, which is like a, like a, Ayushman scheme and...

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Raj Gore: When we look at BPL and state schemes, share of those schemes is about 20% of our total revenue.

Jay Modi: Got it. That is also said. Sir, next question was around Milann. So we've seen a lot of competitive pressure building up, a lot of players coming in, a lot of deals happening. So could you speak a bit about the competitive intensity that you are seeing on the ground and also any particular reason, why we've seen a steep decline this quarter?

B.S. Ajai Kumar: As far as Milann is concerned, as we know, we are quite a dominant player in Bangalore and we are sticking to that dominance in Bangalore and we have put a lot of systems in place to grow.

Unfortunately, there was a slight drop in the Milann revenue because of the regulatory surrogacy which formed an arc which the government regulators had taken time.

Now that has been cleared, all of our centers have a certificate and we are beginning to see a healthy ramp up happening. In fact, July it has already happened. So we are focusing on Milan to make it center of excellence, good quality outcomes, and this kind of approach in Bangalore

only. Our future growth

primarily will be in Bangalore.

And we have a center in Delhi, which has not been doing good. We are shutting down that center in Delhi. The Chandigarh Centre is doing ok, they are putting some efforts to improve. But apart from that, the greater Bangalore area, we will be growing. And we don't see that much competitive pressure.

We have others, who are also doing it in Bangalore. But because, I guess, can see long term we have our doctor, Dr. Kamini Rao, I think in the future, we will be able to come back and grow and certainly reach a leadership position.

Jay Modi: Got it, and typically what is the cost of setting up one Milann Center? Is it a very cost intensive affair?

B.S. Ajai Kumar: Oh no, it is not cost intensive, it costs about roughly INR3 crores to INR5 crores is the cost, right Raj?

Raj Gore: Yes.

B.S. Ajai Kumar: You want to say anything Raj?

Raj Gore: No.

Jay Modi: Okay, and my last question was around the margin. So a couple of years, we've seen -- we've undergone the consolidation that we've had and we were trying to consolidate our centers and become far more profitable than what we've been in past. Now this year, we've seen two acquisitions and couple of Brownfield expansions taking place. So do you do these expansions or acquisitions in any way impact our margins in shorter term because it will naturally take us some time to ramp these up to our optimal level.

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Ashutosh : We have charted out the mature hospitals expansionary plans and at an appropriate time, we would guide the investors with further details . However, since it is vertical expansion of our existing facility, we expect operating leverage to play out and margins should expand. Further, the assets we are targeting is expected to be accretive in profitability at the same time should have provision to attain higher scale .

Jay Modi: Okay, that is helpful, sir. Thank you and all the best.

Moderator: Thank you. We take the next question from the line of Karan from Monarch AIF. Please go ahead, sir.

Karan: Hi, sir. Thank you for the opportunity. So, sir, we have been guiding for margin improvement since Q2, FY '23 earnings call. And if I zoom out a little bit, it was mostly being derived from three aspects. So number one was our cost efficiency measures derived from consultants coming in and making us more efficient, leaner, better pricing in different emerging centres etcetera. Number two was obviously our operating leverage playing out and making many emerging centres going towards maturity and doing better EBITDA margins. And then third was obviously our consultant costs waning off, which were one off. So the margin expansion what we were hoping for, or to the trajectory, we were hoping for was around to the tune of 150 bps to 200 bps on an adjusted EBITDA basis.

However, we have only done around 16% report and adjusted EBITDA margins for us for Q4 FY '23, was around 18.8%. So I just want to know more contours, more color on what really went down this quarter and why were we flattish in absolute EBITDA and perhaps lower to what may be in 18.8 so 200 bps in Q1 FY '24 from an adjusted EBITDA perspective.

Because this is counterintuitive to the consulting cost that we took on and operating and efficiency benefits, which should have flown in. And just follow up on that would be, further cost of items, can you just highlight some cost and items and other expenses which are the most impacted? And what does that mean for us in terms of, mean reversion to 19% and then expanding into 20% - 21% does that look much harder to achieve in FY '24?

Ashutosh : Thank you Karan Let me attend your question one by one . So you're right that our adjusted EBITDA was close to about 19% in the quarter four of last year which has come down in the current quarter primarily due to lower radiation

business Just to reiterate what Raj and Dr. Ajay

said on radiation business , we have lost over 300 operating days on LINAC due to prolonged installation time of new machines.

If you refer to our prior quarterly reports , radiation business used to constitute about 18% of our business, which has gone down to 17%. And that we believe is temporary phenomenon in quarter 1 and should pick up going forward. Now that 1% of business is close to about, INR4.5 crores. and contribution margin in our radiation business being highest, which is close to about 80% has adversely affected our margin

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Other than consumable cost all other radiation related costs including doctors cost are all built in. As we start treating patients in newer machines, we expect a higher operating leverage playing out in coming quarters. Further, the annual inflationary cost including salary hike kicks in the month of April which typically suppresses the margin in Q1, however sequential revenue growth in coming quarters helps margin expansion as the fixed cost typically are very stable in subsequent quarters.

Third is on what Raj said on the investment in doctors which primarily is happening in our emerging markets. While build up of clinical team started in quarter 4 of last year, it was substantially done in quarter 1. With all this we expect the incremental revenue to have larger impact on EBITDA.

We are confident that, this is one of the quarters where two to three variables have gone against us, and we should be able to bounce back from the coming quarter.

On one-time consultant cost related to value creation plan have all been done in the last fiscal year and we have not had any cost related to this in the current quarter. As far as the margin benefits are concerned, almost about 70% - 80% of those benefits were accrued in the previous financial year only and continue in the current year. We expect 20% of the expected benefits to pan out in the current year.

Karan Surana: Got it, sir. Got it. So sir, on the emerging center side, we are doing EBITDA margins of close to 9% to 10%, depending on which quarter we see, right? Can you just tell us what levers we have to take it towards the 15% kind of mid-teens or low teens kind of an EBITDA margins going forward?

Ashutosh: The acquisition of NCHRI and minority stake of our partner in Nagpur should add to our EBITDA margin of the emerging buckets to an extent of about 180 basis points to 200 basis points.

Now the other levers are primarily operating leverage in nature. Because we do believe that 80% or 90% of our investment in clinical talent, as well as market making is done. As the revenue plays out, we should be able to see this operating leverage coming in.

B.S. Ajai Kumar: I mean isolate a few of the well performing centers, our Borivali center is already 20% plus margin. Jaipur center is also in close to 20% margin. It's primarily -- I mean, our whole focus is on two centers in the 22, 24 centers portfolio is Kolkata and South Mumbai. I mean these are the two newest centers, and we are not right now focusing on margins on those centers. We are focusing on revenue growth.

We are happy with the revenue growth which is coming in, and we do believe that EBITDA margins will play out in the future quarters.

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Karan Surana: And sir, we feel similarly for our Kolkata as well as South Mumbai center, because we've been investing in clinical talent and trying to get that up to EBITDA breakeven and obviously, a much better EBITDA on those centers. How are we on those centers currently?

Ashutosh: Losses of Kolkata and South Mumbai together impacts about 1% to 1.25% of overall EBITDA margin of the company. Break even of these centres itself shall help expand margin to a

greater extent

Karan Surana: Understood. And sir, what would be the time line for that according to you? How do you feel that internally?

B.S. Ajai Kumar: We should be able to achieve that in our fourth quarter of this year.

Karan Surana: Understood. And sir, just one last question from my side. Can you just highlight what was the total acquisition cost for both Nagpur and Indore? I understand that we've levered our balance sheet around INR100-plus crores for this acquisition. But I'm assuming that some bit of it will have been spent through internal accruals as well. So what was the total spend on both these acquisitions that we have done?

Raj Gore: So if we see what we are saying is our net debt would go up by INR107 crores, while the funding in the company is having enough sufficient cash, and it also has debt lines. So both sources of fundings are available to us. However, because of that INR107 crores of cash going out our net debt should increase by INR107 crores.

Karan Surana: Okay. Understood. And sir, I was just going to the press release on Indore, and I couldn't see a reported EBITDA on the Indore center. Can you just call out, sir, what reported EBITDA is for the Indore center currently?

Ashutosh: The company is also in the middle of doing some more deals. So, right now, we are not in a

position to sort of disclose some of those parameters as it may compromise our position in the deals. That's why we have disclosed the outcome of both the acquisitions together and we expect our EBITDA should be higher by higher by INR14 crores to INR15 crores per annum, and the resulting net debt is about INR107 crores pursuant to these deals.

At an appropriate time, once we get into those situations, we'll disclose asset wise EBITDA as well.

Moderator: The next question is from the line of Aditya Khemka from InCred PMS.

Aditya Khemka: Just a question, can you elaborate a little of the clinical talent that you guys are investing in?

What are these people doing? Why were they hired, and how is it that they will help people to increase their revenues? If you could just elaborate a little more on that B.S Ajaikumar : Is that question about what type of clinical talent? Aditya, we have -- as we know, Mumbai, particularly, we have been really trying to acquire significant clinical talent in the field of medical oncology and particularly in surgery. And in this field, we are very happy to say that we Healthcare Global Enterprises Limited

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have been able to recruit one of the star vertical oncologists from U.K. , he was the Director of the breast program in Nottingham, which is a big cancer center.

Dr. Sachin Trivedi has joined us. He has been full time since June. And so that is one of the things as we go forward, we will see that he is being positioned as a Director of Medical Oncology in the region. And his job is to really not only recruit talent for his department, apart from breast cancer, he'll be looking at entire solid tumor oncology. And also he'll be building a team.

In this regard, we are also happy to say Dr. Viji Krishnamurti , who is a - breast surgeon or renowned breast surgeon trained in stone capping in New York has also joined us for South Mumbai. And her role is to really work on the breast cancer and see how we can make it a center of excellence for breast cancer in South Mumbai.

Both of these talents apart from several of the associated talent we have recruited. So we are trying to create further comprehensive center which is doing well. And our South Mumbai center, which has got different talent and technology to really look at a center of excellence. That is where we are positioned and going forward. And also, we will cater to international market.

And apart from this, to make it a center of excellence in Mumbai, Borivali, we have just installed a robotic therapy, which will also i

mprove our talent acquisition has already done in terms of uro-oncologists. So with this, we will also see significant growth in the surgery as well as, of course, it will be truly a center of excellence with all the available technologies being present in under one roof.

Aditya Khemka: Got it, sir. So these talents that you have acquired is basically doctors, majority of it is doctors or all of it is doctors?

Raj Gore: Mainly, is that the question? The talent is what we have said on the main doctors.

B.S. Ajai Kumar: The talent is mainly doctor, but actually now we are a big training center. So all of this will propel people to come for the residency programs and improve. So with the center of excellence will have all the residency program associates. So it will go and we will be looking at data from it outcome. So many things will have when we have a talent like this. This will definitely improve.

And we are also, along the same lines to improve this kind of acquisition of talent and win, we are also putting the third robot in Kolkata. So all of this will be a significant benefit for us for our cancer patients, of course, that is the most important thing, but we'll also add to our top line and also improve efficiency

Aditya Khemka: Got it. And sir, I think one of the concerns that investors have had for some time now is stories or streets say that your primary investor, which is CVC might look to exit in the near term. Can Healthcare Global Enterprises Limited

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you throw any lights on that, anything that you would like to share with us? If there is a near - term exit of the private equity?

B.S. Ajai Kumar: At this point, as we know CVC said that they are there for long term and they may exit whenever time they think is appropriate. It is best that you ask them what do they think. As far as I am concerned and Raj is concerned, that the entire management team, we are really working hard to expand our oncology and they have been very cooperative in it. I think they have been a very good partner financially and also strategically for us. So we are really enjoying this partnership

we have for further growth of oncology.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: So my first question is on the Indore facility, what you have required. So if you could highlight what is the kind of surgical revenue on oncology front towards the center is doing coupled with that on some details on the operational part like the ARPOB and occupancy, if you could share, please?

Raj Gore: Which center?

Abdulkader Puranwala: The Indore facility?

B.S. Ajai Kumar: What is the question? Indore facility has both surgery, radiation and medical oncology. It is a comprehensive cancer center. And it is rated actually number two on surgical oncology in that city. And there is also scope for further expansion.

And we are also in the process, as we know, we cover partner, we have an agreement to build a bigger center, cancer center with Dr. Rajesh. So once we complete this acquisition, which we have done, we will begin to grow here. And now the surgical number of surgeries here will increase once we go into the new center. And our goal is to become the premier center in Madhya Pradesh with this acquisition and the future growth plan we have. Raj, you want to add?

Raj Gore: Indore center has the number one market share in private cancer care in Indore. It's a very established center. In all modalities, it has a top three position. It has a very high percentage of good margin business. It doesn't do any scheme business. It has cash TPA business. It has -- in an existing facility, we can grow this revenue -- and we have a plan to add another center in coming two years so that we have a path to grow it sustainably in future and dominate that market as in a mobile marketplace.

Abdulkader Puranwala: Got it. Sir, secondly, on this impact of addition of the LINACs on your EBITDA margin, what you called up for. I think on the last call, you also indicated that you may be adding seven to eight more at a future date. So is that still on track? And will that happen in FY '24? Or you plan it to do as when there was some necessity coming up?

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Management: Yes. So apart from these five machines, we are also evaluating adding a machine in Kolkata in Nagpur, which will probably come and Vizag, three of our well-performing centers on radiation, which will probably come beginning of next year.

So, we are on track. We are already preparing the bunkers, etcetera, ready. And we should be able to grow that radiation business in these three centers in the coming years, the next year.

Moderator: Thank you. We take the next question from the line of Ankit Agrawal from Yellowstone Equity. Please go ahead.

Ankit Agrawal: Thank you for taking my question. My first question is on the Nagpur center, we were already owning 76%, and now we have gone to 100%. Is that right?

Management: So, there are two developments in terms of Nagpur. As Raj mentioned in his earlier speech, the structure was little complex, there was an infrastructure company, and there was an LLP. LLP was the operating company. There was a revenue share arrangement between these group. Now we are buying the stake in the infrastructure by which we'll be able to consolidate the entire operations, which will give us better leverage on operational efficiencies also economic benefits.

Management: We are 26% of our partners has been acquired fully by us, 100%. So, both places will become 100% owners. That is the bottom line.

Ankit Agrawal: Okay. So that's why there is some EBITDA contribution because otherwise, the operational EBITDA would already be in there in the consolidated numbers?

Management: Right. That's right.

Ankit Agrawal: Okay, understood. That's all I had. Thank you.

Moderator: Thank you. We take the next question from the line of Mahesh Attal from Attal and Associates. Please go ahead, sir.

Mahesh Attal: Sir, my first question would be regarding your debt metrics. I mean what is the plan to deleverage the balance sheet going ahead? And where do you see the debt levels by end of this year, financial year 24 and financial year 25 and coming to your ARPOB numbers from the existing centers, where -- when can we -- I mean, expect this to reach the levels marginal levels? I mean, how exactly are we mean pushing it to come to the mature centers, levels and when do we see the numbers coming in? My two questions will be this.

Management: Yes. Let me first take the debt part of the question and the second part will be taken by Raj. See this is the debt numbers as it stands today if you see what you have shown in the investor presentation. I'm talking about where that bank debt is about INR217 crores and we are trying to add about another INR107 crores largely from these acquisitions. But don't look at debt in isolation. These acquisitions as previous management members said they're all coming with a positive EBITDA margins and that is only kind of support this debt.

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And if you look at it on an overall basis, as I mentioned, we are looking on most of our capex from our internal accruals, any debt will be as far as this one would be only for the acquisition which will be both revenue as well as EBITDA accretive. So, we are largely looking at to keep a debt level in the range of about 2 to 2.5x net debt -to-EBITDA. That's the kind of leverage we are looking at, and that is the way we are looking at it.

Management: Mahesh on your question -- go ahead, Mahesh.

Mahesh Attal: Yes, sir just one question there. So when we

say that we are going to -- I just wanted to know

what could be our spend per bed when we are going for an expansion, let's say, we are starting a new hospital tomorrow and then what would be the cost per bed and what would be the payback period for that particular bed, let us say per bed occupancy what will be the payback period at this ARPOB level, let's say I calculate it on?

Management: Mahesh at this point, we are not doing any Greenfield. Like what Srinu said, we are only acquiring hospitals, which already have a positive revenue and EBITDA. And so this will be based on, obviously, at a lower EBITDA times, that is what we are acquiring demand. So, it will be definitely beneficial for us in terms of further expanding it and defining the dominant in those regions.

So, we are not doing any Greenfields, to give you any details about what will be the cost of hospital expansion and all. We are not doing Greenfield, okay. At this point except what we call it in Bangalore, whatever expansion we are doing in Whitefield, it is part of the Bangalore dominance expansion. So we are not calling with truly Greenfield. It is the expansion of our existing centers. That is how we are dealing.

Similarly in Ahmedabad also we are building a new center because we run out of space. so that will have a huge positive potential as we go forward.

Mahesh Attal: Yes. So sir, the metrics behind when the Brownfield expansion is that are we looking at hospitals which have ARPOBs in line with our existing or existing centers or they are little lower and they're trying to bring them up? How exactly are we looking at it from now from here on?

Management: So, Mahesh these are the investments that is primarily what we have today. As Dr. Ajai said that we are not looking into majorly we are not looking into Greenfield. Any acquisition which we are taking. What we are seeing is it should be at least at par accretive to our return on capital employed.

Now when that happens, and typically, and you also touched upon payback period, we are expecting based on the headroom available and whichever targets we are taking, we should be able to have a payback within in 6 years to 7 years at the capital level. Now once that happens, and you have enough headroom available to grow, that makes a viable business sense for us. So, this is how broadly we are looking at any future investment or Brownfield.

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Mahesh Attal: Right. Because sir, also one more thing that operating leverage in our business will come only when you're actually operationalizing the beds, right? I mean just wanted to understand that where exactly the operating leverage in our business comes from?

Management: Absolutely right. I mean, when you operationalize your bed, your bed occupancy goes up, your revenue goes up and then operating leverage kicks in. And we have been saying that we have an isolated focus on certain centers which is pulling down the entire portfolio.

If you look at our existing centers, the margins are healthy, the return on the capital employed is healthy. And if you just -- Kolkata and Mumbai Center, then we are well within a decent satisfactory position both in terms of profitability and return and that's why we have been stressing up on these two centers all the time that a -- we are not looking at profitability.

We feel that if we do good in terms of increasing our market share in those regions like doing the right things when it comes to market making and clinical application, profitability will follow soon in coming months and as we said, we are already sort of seeing the improvement in EBITDA, and we should be able to do that in quarter 4 of current year.

If you look at our slide number 10, we have been on an increasing path coming on growing path in EBITDA. So our EBITDA in quarter 4, from emerging center was INR81 million, i

t has gone

to about INR99 million. And we expect this would keep growing in the future quarters.

Mahesh Attal: Fine sir. All right. Thanks a lot and all the best.

Moderator: Thank you. The next question is from the line of Karan from Monarch AIF. Please go ahead, sir.

Karan: Hi sir. Thank you for the opportunity again. Sorry to ask this again, but I'm still confused on the total acquisition cost. I understand that you gave that breakup of the debt plus the cash. But if you can just call out the total acquisition cost to company that would be with debt, with internal accruals, etcetera, the total cost would be that would be really helpful, sir.

Management: So, just to say, our net debt is going to increase by 107. Of that INR107 crores, INR30 crores of debt is what we will be consolidating from NCHRI. If you take out that debt of INR30 crores, the next INR67 crores would be primarily towards acquisition cost, not INR67 crores -- 77 crores would be flow out from the system, either in the form of debt or cash going down, that is a total consideration being paid for the two acquisitions.

Karan: Understood. So INR 77crores we are actually playing because INR30 crores debt we're getting on our balance sheet from the Nagpur center? Is my understanding correct, sir? And that's how we're coming to that debt figure?

Management: That's right.

Karan: Okay. Thank you, sir. Thank you so much.

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Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.

Management: Once again, thank you for your interest and support to HCG. We are happy to have further conversations if you want any more clarification, please feel free to reach out to our Investor Relations. Thank you for joining. All the best.

Moderator: Thank you. On behalf of HealthCare Global Enterprises Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Nov 2023

November 17, 2023

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on November 10, 2023

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(oa) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on November 10, 2023, with Analysts/Investors to discuss the Unaudited Financial Results of the quarter and half year ended September 30, 2023.

This is also available on the website of the Company www.hcgoncology.com .

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited Q2 and H1 FY
'24 Earnings Conference Call"

November 10, 2023

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 9th November 2023 will prevail.

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED

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Moderator : Ladies and gentlemen , welcome to the Q 2 and H1 FY '24 Earnings Conference Call of HealthCare Global Enterprises Limited.

This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participants' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. B. S. Ajai Kumar, Executive Chairman for HealthCare Global Enterprises Limited. Thank you, and over to you, sir.

Ajai Kumar: Thank you very much , and good morning, and a warm welcome to all present on this Q2 H1 FY '24 Earnings Conference for Healthcare Global Enterprises Limited .

Today, I am here joined by Mr. Raj Gore, our Chief Executive Officer , and Ms. Ruby Ritolia, Chief Finance Officer . Ruby has been recently appointed as CFO , and she brings with herself an unparalleled experience in the field of finance and investor relations and will join us in our journey to take HCG to its desired goals .

Along with them , we also have a few members of our HCG Senior Management Team . At this time, I would also like to thank our Srinivasa Raghavan, our former CFO , for his valuable services for the company , and wish him luck for his future endeavors .

Since our inception , we started with a single cancer hospital with an aim of providing world class cancer care service to people of our country . Today , we proudly reflect on our substantial growth , having expanded our presence across the country , to become one of the largest providers of cancer services , not only in the country , but globally .

HCG has challenged the practice of relying on multi -specialty hospitals for cancer care .

Achieving sustainable outcomes is feasible when patients receive optimal treatment delivered with topnotch talent , knowledge , technology , and infrastructure at the right time . World's renowned cancer care institutes are those which specialize only in cancer care , and we are one such.

For HCG focusing on single specialty helps us to concentrate our expertise and our resources , our dedicated deployed in understanding , diagnosing , and treating only cancer . This focused factory approach enables us to stay at the forefront of advancement in oncology , attracting top Healthcare Global Enterprises Limited

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talent , and also fostering talent , and collaborative environment dedicated to tracking the complexities of cancer .

Clinical excellence is the hallmark of successful cancer institutions or any medical center , and we are such . Our commitment to excellence in cancer care has been unwavering , and we stand proud as a beacon of hope for patients and their families . In a landscape where the prevalence of cancer is ever increasing , our mission has been to provide not just treatment , but holistic

approach to healing and adding life to years .

Further , we aim to expand our footprint strategically , reaching underserved areas and ensuring that quality of cancer care is accessible to all. We will continue to invest in research and development academics and pushing boundaries of medical science to discover new treatment , and of course , improve the quality of life to all our patients .

Lastly , our journey is not just about building hospitals , and it is about building a legacy o

f

innovation . We are confident of making HCG an epitome of excellence in fight against cancer in India .

I now hand over the call to Mr . Raj Gore , our CEO for HCG for his comments on the strategies going forward and operational performance for the quarter gone by . Thank you. Raj?

Raj Gore: Thank you , Dr. Ajai. A very warm welcome to all the participants on the call . Let me take this opportunity to wish all of you happiness , good health , prosperity on the occasion of Diwali on behalf of HCG .

HCG has consistently led the way in the battle against cancer in India , constantly adapting the latest advancement in technology and treatment methods through ongoing research and development in the extensive realm of cancer care . We have effectively demonstrated our expertise in efficiently operating single specialty hospitals nationwide , achieving superior outcomes and aligning our treatments with global standards .

Speaking of the quarter gone by, we are happy to report yet another quarter of strong performance . Our revenues for quarter 2 in FY '24 grew by 16% with EBITDA excluding ESOP cost growing by 14% and PAT growing by 84% on year-on-year basis . Our performance is a culmination of our brand strength in cancer care to generate higher footfalls and operational excellence , which has been delivered by our excellent team .

Speaking of our recent acquisitions , we have successfully consummated transactions at NCH RI in Nagpur and SRJ CBCC Hospital in Indore . Our strategy in Indore involved entering a non - metro city with a high-quality asset and integrating it under the HCG banner . This was a strategic move given the absence of major players in the region for cancer care and a growing market for cancer treatment .

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We are in the process of effectively integrating systems and processes across these hospitals with HCG , envisioning significant synergies in terms of management efficiency , treatment upgradation , procurement optimization , and increased patient traffic bolstered by our local marketing initiative and the strong pan India brand presence .

Our emerging centers , that are the centers which opened after 2017 , have been demonstrating strong performance and are progressing towards maturity . In particular , our revenues in Mumbai and Kolkata have shown impressive growth of 42% and 41% respectively , year-on-year, resulting in faster expansion of our oncology market share compared to our peers.

We are happy to announce that we have launched an additional dedicated women's cancer wing in our center in South Mumbai . Furthermore , we have been enhancing the infrastructure of our established hospitals by introducing significant upgrades .

In the most recent quarter , we operationalized three new LINAC machines . We have observed a rising demand for radiation therapy , and there exist a gap between this demand and the available supply of this treatment modality throughout India . In response to this, we have formulated a plan to install additional six LINAC machines in various hospitals over the next 18 months , aiming to bridge the gap between demand and supply .

Further , all our LINACs, which were under installation in Q1, have been operationalized in Q2 except Ongole which has been done in the month of October .

In addition to LINAC machines , we have also operationalized three robotic surgery machines at our hospitals in Baroda , Mumbai , and Kolkata . This expansion is in direct response to the increasing demand for high quality cancer care ensuring that patients have access to the best possible treatments , ultimately leading to superior outcomes .

To conclude , I want to emphasize that our substantial infrastructure improvements , our innovative hub and spoke model for delivering accessible

cancer care nationwide , and our

ongoing research endeavors aimed at advancing treatment options , all contribute to our confidence in elevating the HCG brand to the level of top tier global hospitals . We anticipate that these efforts will ultimately enhance company's financial performance in the years to come . With this, I hand over to Ms. Ruby , our CFO , for financial highlights .

Ruby Ritolia: Thank you , Raj. Good morning , everyone . Firstly , I would like to thank HCG and the Management team to add me to the HCG family . I am honored to be part of the cause of this esteemed organization to fight cancer and bring the global standard of cancer care in India .

We have uploaded our quarter 2 and H1 FY '24 financial results and updated invest or presentation on the stock exchange and compan y's website , and I hope everybody had an opportunity to go through the same.
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In reviewing our performance for the quarter , I am pleased to report a remarkable 16% year -on-year growth culminating in a top line figure of 487 crores. Our HCG Center contributed significantly generating revenue totaling 469 cror es. Milann achieved 18 crores, reflecting a 7% year-on-year growth .

Our operational matrix , crucial indicators of our performance , are showing positive trend across the board . Chemo sessions witnessed a robust 8% year -on-year increase , reaching a total of 36,500 in the quarter . While our radiation services experienced healthy growth , the decline in capacity utilization from 67 % to 61% is attributed to our proactive expansion efforts and incorporation of three new linear accelerators during the quarter . We have strategic plans in place to optimize our average utilization and a targeted ramp up by the end of the financial year . Inpatient oncology and occupancy stood at 56% for the quarter as compared to 61% for quarter two of the previous financial year on account of expanding bed capacity by 64 numbers and better bed turnaround resulting into lower average length of stay. Overall , average occupancy rate for the quarter stood at 63.6% as compared to 66.4% in the same quarter of the previous financial year. ARPOB stood at 42,054 as compared to 36,914, registering a growth of 14% on a Y-o-Y basis .

Let me speak about our performance for matured and emerging centers . Matured centers continued the study trajectory growing healthily at 13% driven by healthy volume growth across modalities . ARPOB grew by 10% to reach at 43,460 , and despite the addition of beds , the ARPOB, the average occupancy rate increased by 14 bi ps to reach 65.1% for the quarter . Our annualized ROC E for matur ed center s stood at 21.2% and ROC E pre-corporate allocation was 25.5% for the quarter of Financial Year FY '24.

Coming to emerging centers , I am pleased to announce a substantial 29% year-on-year growth in this segment . We are observing a consistent update in our emerging centers marked by increased footfall across various cancer treatment modalities . The average occupancy rate for emerging centers declined 60.1% . However , this was resultant of mi xed optimization , particularly at our Jaipur Center in North India .

The optimization is also reflected in our 28% year-on-year growth in average revenue for occupied bed, indicating a positive trajectory of our operations . Annualized ROC E for emerging centers stood at minus 3.9% , and ROC E pre-corporate allocation stands at 0.8% for the quarter of Financial Year FY '24.

In our Investor Presentation , we have retailed the revenue breakdown across various geographies . I would like to emphasize on some key highlights .

Nagpur experienced a remarkable 60% increase in revenue . Kolkata saw a substantial growth of 42%. Mumbai exhibited a strong growth of 41% . Ranchi grew by 48%

in quarter two of

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Financial Year FY '24 compared to the same period last year . These numbers underscore our robust performance and market presence in these regions .

On the EBITDA front , our EBITDA excluding ESOP grew by 14% Y-o-Y and stood at 86.4 crores as compared to 76 crores in the same quarter previous year . PAT for this quarter post Ind-AS stood at 13.6 crores as compared to 7.4 crores in quarter two of Financial Year FY '23. Our PAT pre-Ind-AS adjustment for the quarter stood at 17.1 crore as compared to 10.5 crore registering a growth of 63% Y-o-Y. Our CAPEX for H1 of Financial Year FY '24 stood at 64 crore. Net debt excluding leases stood at 310 crores as at September 2023 .

We have also given b ifurcat ion of our EBITDA across matured and emerging centers and detailed numbers on the H1 financials in our uploaded investor deck for further details . At last, I would like to open the floor for further question and answers .

Moderator: Thank you very much . We will now begin the question -and-answer session . The first question is from the line of Karan Vidan Chandra Surana from Monarch AiF. Please go ahead.

Karan Surana: Sir, first, my first question would be , can you provide specific details on the number of new beds and facility expansions which are projected to come and become operational in FY '25?

Ajai Kumar: Can you just repeat the question ?

Karan Surana: Could you provide specific details on the number of new beds and facility expansions that are projected to come and become operational in FY '25?

Ajai Kumar: I think this is around 200 to 250 be ds. However , we can connect offline to get exact details .

Karan Surana: And sir, my second question would be on our EBITDA margin expectations, right. So given our prior discussions and earnings call, we have been anticipating achieving approximately 20% post Ind-AS EBITDA margins, right? However, in recent quarters, we have seen that we may be undershooting that guidance a little bit. So sir, can you discuss the feasibility and maybe that 200 to 300 b eds, you know, that gap that we are seeing, what the feasibility of this is reaching in the second half of FY '24, especially like we would, I am assuming we would be expecting a 15% kind of an EBITDA growth for FY '24. So sir, any color or any light on that when we can achieve that kind of an EBITDA margin? And why exactly are we undershooting that? What cost line items are we seeing, you know, impacting that number, sir?

Ajai Kumar: Regarding the EBITDA margin as we have told in the last call also, we are moving upward. As you can see, there has been a substantial growth in the margin compared to the first quarter to this quarter. We expect the same trend to continue, and we feel, you know, towards the end of the fourth quarter at beginning of the first quarter of next year, we will be close to 20% margin. And you had another question I think on the PBT. Was there another question?

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Karan Surana: So just my last question I would squeeze in is that, what would be our EBITDA pre-Ind-AS for first half of FY '24? And could you provide the lease cost figures for the same period?

Ajai Kumar: Our annual lease cost is close to about 75 crore. So that you may just it's about 3% to 4%, I guess, 4%, 4.5%.

Karan Surana: 4% of revenue, sir? 3.5%, 4% of revenue?

Ajai Kumar: Yes. About 3.5% to 4%. 3.5%.

Moderator: Thank you. The next question is from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa: Sir, in the last call, you highlighted that, you know, we hired some senior Onco surgeons at a Mumbai unit. So what's the progress over there in terms of revenue? And the high growth which

we are seeing in Maharashtra, so is it because of Mumbai and Nagpur? Or is there anything more to go? That's my first question.

Raj Gore: Thank you for that question. You are right. We have always said that we want to invest in our clinical talent as well as our go-to-market efforts in markets like Mumbai and Kolkata. You can see the results. This quarter, Mumbai has delivered 41% year-on-year growth in revenue. Kolkata also has delivered 42%. Nagpur, in fact, has delivered 60% year-on-year growth. That was our rationale behind the acquisition that we made last quarter. I think you will continue to see the upward trend.

In my comment, I also mentioned that we are gaining market share in Mumbai faster than, you know, most of our peers. We will continue to add additional capabilities. In Borivali Hospital, we have added one additional linear accelerator because our prior linear accelerator was fully utilized. As that ramps up, again, it not only gives us revenue but improved margins. We have also operationalized a robotic program, our first robotic program in Borivali. And in Colaba, South Mumbai Center, we have launched a dedicated women's cancer care center to meet the exclusive needs of our women cancer patient. So, we are very confident and very optimistic of our growth in Mumbai market.

Dhara Patwa: Thanks for that explanation. And one question was a general question. Since oncology as a market is growing. So, I guess, everybody now is investing on the equipment for robotic surgery and radiation therapy, like whenever we see in your any call, they all are investing in it. So, do you think that could affect HCG going forward? I understand that we are a total cancer care hospital, and there are not more of immunologist, people who could understand immunotherapy, those kinds of surgeons are present, and we have that. So will that be so that going ahead will have edge over immunotherapy and other, but we won't have edge in the radiation or robotic surgery because everybody is investing, so the volumes might get spread across these hospitals?

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Ajai Kumar: Yes, I feel that with the growing, you know, as we know, with the population dynamics of India, as people become aged, also number of cancer patients increasing, we believe the number of centers which are there compared to the population growth is still not in parity.

Having said that, we being a dedicated oncology center, what we have shown in the last 15 years, that our growth will be far more than a multi-specialty putting up a linear accelerator or a robotic because, you know, usually the patients who go to multi-specialty, let us say, they go with an abdominal pain. They are diagnosed with cancer. They stay there. But for a dedicated, it is a globally proven fact what I am saying, But if you have a dedicated cancer center, people all from other hospitals, multi-Specialty hospital, referral from doctors come to the dedicated cancer

center . Normally , they don't go to the multi -specialty center . So we believe because we are not only in Metro cities but also in tier 1 and tier 2 cities , we don't have a really credible competitor for us in these areas .

Regarding the linear accelerators , that is our greatest strength . As Raj Gore said, we are installing more units because we are running in majority of places capacity utilization . This kind of capacity utilization , you know, if you look at it rarely seen in a multi -specialty hospital . Other thing is , we are one of the pioneers now going into the tier 1, tier 2 cities with robotic . So that also , as we go forward , will put us in a very leadership position . So I feel honestly , people can follow us . We are not followers , and we are

in such a leading position . Our growth as we have indicated will sustain and grow , and as our new centers also grow , we will continue to be in a dominant position in oncology .

And more importantly , it is not just equipments . It is also clinical excellence , which is important . We have 400 oncologists under HCG , and nobody can really talk about it like that . And in our own Bangalore center , we have 100 oncologists dedicated for organs specific . We are also big in research genomics .

So it is like somebody told me , you know, one of the other hospital to do something like HCG , for us it will take minimum several decades . So, in the past , it has not happened . In the future , with our focus factory approach , I think we are very confident we will continue to grow and provide this kind of excellence in quality care to our patients .

And recently , there was a major conference in Mumbai where HCG was a major contributor participating in various meetings , keynote address , and research activities . So we are now certainly a dominant force in this field .

Dhara Patwa: Thank you for the detailed explanation . Just last bookkeeping question . Sir, what will be the effective tax rate for FY '24 for us ?

Ruby Ritolia: So in the current quarter , we have it at around 50% , but for the full year, we will be somewhere between 40 % to 50%.

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Dhara Patwa: 40 to 50, okay . That's what my last question is .

Moderator: Thank you. The next question is from the line of Ravish Shah from Opal Securities . Please go ahead.

Ravish Shah: So, I had two questions . First would be , how have our emerging centers been performing ? And are the Mumbai and Kolkata centers at EBITDA breakeven level ? And when can they achieve the whole company level EBITDA margins?

Raj Gore : So, as I mentioned earlier , we are seeing very fantastic response to our efforts in driving revenue growth . Mumbai has delivered 41% year - on-year growth . Kolkata has delivered 42% year -on-year growth . We have done further seeding by adding technology capability , physical talent in these two markets .

Our international business is also ramping up in these two markets . Just to give an idea , this last quarter was our highest in international business , but we are actually nearly doubling our pre - COVID quarterly rate in our international business .

You know, South Mumbai is an excellent facility with excellent technology , excellent clinical talent trained in UK and in U.S. We are seeding , we are opening information centers in Middle East, in Bangladesh , in Africa . So we feel that those two cities will continue to benefit from our international business going forward .

If you look at overall emerging centers, the growth has been in mid-20s, nearly mid 20s year-on-year. The profitability has improved, and you will see , you know, I think , we will continue this positive trajectory going forward .

Ravish Shah: My next question would be can you throw some color on the acquisitions that we recently did in Indore and Nagpur? How well are they doing ? And what are our strategy for those assets that we have taken ?

Raj Gore : So Indore, as we mentioned in our announcement , it's a 50 bedded comprehensive cancer care center , purely , you know, this is the only private comprehensive or this is the first private comprehensive cancer care center in the market . I think we concluded the transaction in the beginning of October . Currently , you know, we are focusing on upgrading our clinical capabilities talent wise as well as technology wise .

We have a value creation plan across improving revenue to optimizing cost through integration or plugging it in on our HCG capabilities , network capabilities . It's early days but we are seeing good

response from our clinicians , our employees , as well as market . As we ramp up our brand marketing effort , we will see growth in that market .

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Moderator: Thank you so much . The next question is from the line of Ankit Pand ya from InCred Asset Management . Please go ahead.

Ankit Pandya: Sir, I have two questions regarding the Indore Hospital . Sir, can you give us some ballpark number on what is the margin profile of the hospital ? And also you have mentioned that there is space to further increase the capacity by 100 beds . So what will be the CAPEX regarding this 100 beds? And by when do you think you will be investing for the 100 beds?

Raj Gore : So, thank you for that question . As you can see from our Investor Presentation , Indore Hospital does about 30 Cr annual revenue . The margin profile is in mid -teens. We see a lot of opportunity to improve that margin as we integrate that with HCG Network . It is a 50 bedded hospital . We have a plan to add 50 more beds. The CAPEX for those will be about 40 to 50 crores, and it will come in next 2.5, 3 years . It will get completed in next two to three years . And that will position us in future to scale up in those markets .

Moderator: Thank you . The next question is from the line of Viraj Shah from Shah Investments . Please go ahead.

Viraj Shah: Sir, I have two questions . Are we looking out for any other inorganic opportunities ? And if yes, then what would be our ideal size of that as set in terms of bed and location ? And any new geographies ?

Ajai Kumar: Yes we are , as you know, Mr. Shah, we are now gone into Madhya Pradesh for the first time , and with the Nagpur being there , we will now consolidate . As we have said in the past , we are definitely looking for strategic locations , primarily across the country , and we believe , you know, if we see some opportunities , certainly we will work and make the information known . At this point , we are not doing any Green field except our Bangalore , which is an extension of our Bangalore centered with White Field . Otherwise , we want to look at any M&A activities and future , primarily we like to work with maybe established oncology like what we did with CBCC on those models . We will try to see whether there is an opportunity for us to grow in several regions of the country . As it is now, we have nearly 24 oncology centers , and obviously , you know, India is a big enough country for future growth , but we have to consolidate and then look at our situation and see which is an applicable India activities we can do.

Viraj Shah: And second is , what are we doing to increase our footfall and patient count in our new centers? We understand that HCG has a strong brand strength in the core regions , but what about outside the core regions ? How do we generate more footfalls ?

Raj Gore : So our emerging centers , we have shown , you know, 29% year -on-year growth , and we expect it to continue going forward . And I already , in my previous question , I explained that in Mumbai and Kolkata , which are Metro markets where there is established competition . In spite of that , Healthcare Global Enterprises Limited

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our revenue growth has been more than 40% , and we are rapidly gaining market share in those markets which shows the strength of HCG brand even in the new markets that we have entered .

Moderator: Thank you so much . The next question is from the line of Karan Vidan Chandra Surana from Monarch AiF. Please go ahead.

Karan Surana: Sir, can you just disclose the absolute EBITDA figures for the Indore and Nagpur centers as individual entities that we just acquired ? And what margins are they operating at , sir, if you can just give some color?

Raj Gore: Nagpur

actually we consolidated which is, so as I mentioned earlier , Nagpur , we see full impact for a quarter for about 2 Cr gain, 2.1 Cr gain out of that transaction going forward in EBITDA.

Indore, as I mentioned , it's about a 30 crore top line asset with about EBITDA in mid -teens.

Karan Surana: So anything between 14%, 15%, sir? That would be reasonable assumption , EBITDA margin, sir?

Ashutosh : Yes. EBITDA margin will be in the range of 17 % to 18% at Indore. And just to mention when Raj said 2 million , sorry , 2 crore impact for the quarter two , we had only partial impact in quarter two. So the full impact would come from quarter 3 onwards.

Karan Surana: So, basically , we are expecting Q3 , Q4 EBITDA to consolidate much better than Q1, Q2. Is my assumption okay , sir?

Raj Gore : Yes. Q4 definitely better . Much better .

Moderator: Thank you. The next question is from the line of G . Asha from wealth Securities . Please go ahead.

G. Asha: So my question is, what are our views on the Milann Center ? Are we going to diversify it ? And what are the plans for it ? Do we not see any substantial performance happening in that business ?

Ajai Kumar: No, Milann Center as we have said before , post-COVID, we went through a very tough time , and we are now working on it to bring some improvement , and as you can see , we are on the

right path. It has come , it is coming up the ladder , and we are looking at it in the future for divesting , but we cannot give any firm timeline for it . But certainly , we want to see how it can grow , and we have worked very hard to consolidate with the doctors , improve the outcome . We are now focusing on quality outcome with existing centers . We have also decided to close down a center to bring about efficiency in other Delhi centers . So all of this will only improve our EBITDA, and our margins , and also the quality of care . So we will , of course , inform when we make a firm decision about the future of Milann.

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G. Asha: I have another question . So growth for the matured and emerging centers have been good , but the same has not reflected in our margin flow . When can we start seeing the operating leverage playing out?

Ashutosh : So, see, if you refer to the slide where your emerging centers ' revenue has grown by 13% and EBITDA has grown only by 7% , we had done some operational efficiency exercise in last financial year which wherein we did some rejig in terms of centralization and decentralization . That has resulted into reduction of our corporate cost . While it is not visible at the unit levels , we have our overall corporate cost has gone down , and that's why you would see that at overall level , our margin impacts are not much .

Having said that , as we have been growing our revenues on quarter -on-quarter basis , as Dr . Ajai mentioned that we should be looking at close to 20% margin by quarter for our end or at the beginning of next financial year.

Ajai Kumar: I think just to add to what Ashutosh said, as you know, our mature centers ' EBITDA margins are over 20% . In fact, our Center of Excellence is in the high 20s . So, what is happening is the centers which are new , once they breakeven , automatically , it will pull up the overall margin for the entire group , and beyond breakeven , we expect that to reach higher maybe in 11 % to 15% . Once it reaches that at a group level , we will reach over 20% , and that is what we are predicting will happen either in the last part of the last quarter or the beginning of the first quarter of next year.

G. Asha: I have one last question . So there are certain rumors about CVC exit . Can you throw some light on this , like what are their plans , and what is happening on this?

Ajai Kumar: I was wondering when somebody will ask

that question . I just want to say that as a shareholder , they have every right to come in , and if they feel they have got their return , they will look at exit. Best persons to answer that will be CVC . For us, really , it is nonissue . They have been a good partner . We have worked with them very well . They came at a very difficult time , and as you can see , we have grown very well with partnering with them , and I would not like to say any further comment in this matter .

Moderator: Thank you so much . The next question is from the line of Karan Vidan Chandra Surana from Monarch AiF. Please go ahead.

Karan Surana: Sir, one more question I had was on the total debt that we have right now . We have a net debt of around 310 crores as of September '23. So, sir, do we have any plans of , you know, maybe deleveraging it or maybe our debt , net debt will continue rising up when we look for inorganic kind of opportunities ? So that's question number one .

And sir, question number two I had was on the emerging centers . So I see that a lot of our emerging centers have been operational for four to five years , right. So just getting some sense Healthcare Global Enterprises Limited

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that when can they be classified as mature centers ? Because I am assuming that 4 to 5 years is a good time period for any centers to kind of mature and become , you know, get into like high teens kind of EBITDA margins etc. So, if you can just call out , sir, what would be a new centers and, you know, number of new centers and the year of operational they have been operational , you know, so that would really help , sir? That's it from my side.

Ashutosh : So, Karan, to answer your first question on leverage , today , our net debt of 310 crores results into a net debt to EBITDA of sub 1, which is less than 1 x. And I think , I mean , we have a lot of headroom there , and whatever because we are sort of able to internally generate and fund our maintenance CAPEX . Any growth CAPEX or any inorganic expansions would lead to increase in the net debt for which we feel there is a lot of headroom available with us . So this is as far as the net debt condition is concerned and how it can go pan out in the future .

So you also sort of touched upon the question on the vintage of the hospitals , the way we look at for our emerging centers . What we have been doing so far is that we have been consistently categorizing certain sets of centers post 2017 so that the profile , EBITDA profile of those centers can be reflected appropriately , and people can compare its progress on quarter -on-quarter basis .

While you are right that these are not new centers per se, we have classified them as an emerging center . The latest additions in our portfolio have been at Kolkata and South Mumbai Center , which is like right now over three years old.

Moderator: Thank you so much . As there are no further questions , I would now like to hand the conference over to the management for closing comments .

Raj Gore : Once again, thank you everyone for joining , having an active discussion . I want to thank you for your continued interest in HCG . We are confident as a management that we have the right differentiated specialized cancer care product for India . We have been a leader and pioneer , and we will continue to grow our market share in all our markets . The future is definitely brighter .

Once again , thank you , and I wish you a Happy Diwali . Have a good festival season .

Moderator: Thank you. On behalf of Healthcare Global Enterprises Limited , that concludes this conference . Thank you for joining us , and you may now disconnect your lines.

Call: Feb 2024

February 16, 2024

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on February 09, 2024

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on February 09, 2024, with Analysts/Investors to discuss the Unaudited Financial Results of the Company for the quarter and nine months ended December 31, 2023.

This is also available on the website of the Company www.hcgoncology.com.

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited
Q3 & 9M FY '24 Earnings Conference Call"
February 09, 2024

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 9th February 2024 will prevail.

MANAGEMENT : DR. B.S. AJAIKUMAR –EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED

Healthcare Global Enterprises Limited
February 09, 2024

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Moderator: Ladies and gentlemen, welcome to the Q3 and 9 Months FY '24 Earnings Conference Call of HealthCare Global Enterprises Limited.

This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company. As on the date of this call, these statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict .

As a reminder, all participants' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone. Please note that this conference is being recorded.

Now I hand the conference over to Dr. B.S. Ajai Kumar , Executive Chairman of HealthCare Global Limited. Thank you, and over to you, sir.

B.S. Ajai Kumar : Thank you very much, and good morning to everyone, and a very warm welcome to those who are all present on this Q3 9 -month FY '24 earnings conference call for HealthCare Global Enterprise.

I just want to take a few minutes and give a little bit of background how things are evolving in India and how this has made a difference for HCG. India is on the cusp of significant democratic change, leading to doubling of population age over 60 years by 2050. Cancer incidence is expected to increase by 77% during this period, a significant increase . HCG with its network of our present 22 cancer centers and expansion plan is poised to play a dominant role in serving the diagnostic treatment needs of the forecasted demand and also play a role in preventive aspects of cancer.

HCG's performance for the quarter serves as an evidence of its progress in advancing towards focused cancer care, where research and collaboration are the key drivers for improving patient outcome. By leveraging this unique value propositions, we are able to provide targeted treatments such as adaptive radiotherapy and immunotherapy to our patients.

Moreover, our integration of technology into patientcare process, including computational work and AI platforms, allows us to be continuously on the leading edge of the change creating a differential in the way the cancer has evolved, and this has truly made us a global leader in oncology, not only in terms of service but R&D work.

Our outcomes today , I can probably say are equal to the best top centers in the world. At HCG, we have always embraced technology and innovation to be in the forefront of advancement in cancer care. We have consistently adopted cutting -edge solutions to ensure that we provide the highest quality health care to our patients.

Our ability to deeply connect with patients is built on foundation of compassion, trust that is unmatched which is what truly sets HCG apart from others in our field. We remain committed to continue making a meaningful difference in the lives of those we serve in making cancer truly a chronic disease.

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Embracing a collaborative approach, our tumor board discussions prioritize patient -centric conversations, utilizing multidisciplinary clinic across our extensive network. Entering 2024, our dedication to excellence in cancer care remains a high priority.

We are focused on our goal of making quality cancer care accessible to all through continued investment in research and development, we endeavor to push the boundaries of medical science, uncovering novel treatments and enhancing existing ones to offer the best possible outcome to every patient.

I now hand over to our CEO, Mr. Raj Gore, for his comments on the strategies going forward and operational performance for the quarter gone by

y. Raj?

Raj Gore: Thank you, Dr . Ajai. Good morning, and a very warm welcome to all the participants on the call. Last Sunday on February 4, the world observed World Cancer Day. I want to emphasize the crucial role of awareness and early detection in cancer care. Awareness regarding key risk factors like HPV, alcohol and obesity remains very low in India and less than 1% of Indians undergo screenings for major cancers, such as oral, breast and cervical cancer.

Unfortunately, over 2/3 of cancer cases in India are diagnosed at advanced stages contributing to a higher mortality to incidence ratio compared to other nations. The first line of defense against cancer is awareness, and it is instrumental in saving lives . We welcome the government's initiative to promote cervical cancer vaccination for girls aged 9 to 14. This is a pivotal step towards raising awareness and taking preventive measures to reduce the cervical cancer cases burden in India.

Now I would like to update all of you on some key growth initiatives of the company. Few quarters ago, we had embarked on our digital transformation journey to create an omnichannel end-to-end patient engagement platform. Our vision is to increase accessibility to HCG by making it easier for cancer patients across India to find and reach HCG care from wherever they are.

I'm happy to introduce our first -of-its-kind app, HCG Care, specifically designed to serve needs of cancer patients and one -stop solution for patients to navigate seamlessly from MDT consultations to second opinions, diagnostics, pharmacy needs, rehab services and home care support and post discharge follow -up, ensuring a smooth and a personalized journey.

With this, we are confident of building a long -term relationship with our patients to be their structured adviser over a lifetime. With over 30% market share and growing, we have a dominant leadership position in our home city of Bangalore.

To further reinforce our dominance, I take great pride in announcing a new 100 -bedded hospital , a comprehensive cancer care center in the thriving North Bangalore market, a strategic move aligned with our philosophy of going deeper in the market of our strength. The hospital is expected to be operational in 15 to 18 months.

As you remember, we also have a project in Whitefield in East Bangalore that is under construction right now. With the network, encompassing 4 strategically positioned hospitals and Healthcare Global Enterprises Limited

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three daycare centers across Bangalore, we remain steadfast in our mission to provide advanced and comprehensive cancer care solutions to our discerning community.

Our recent acquisition in Nagpur and Indore have progressed in line with our plan, and right -- we are rightly placed to consolidate our position in the Central India region. As we move forward, we are confident of offering quality health care to our patients through integrated physical and digital health systems, accompanied by advanced technology and research and more importantly, make quality health care accessible to all.

With this, I hand over to Ms. Ruby, our CFO, for financial and operational highlights.

Ruby Ritolia: Thank you, Raj. In reviewing our performance for the quarter, I'm pleased to report 11% year -on-year growth, culminating in a top line result of INR470 crores despite a seasonally muted nature of quarter 3.

Some of the growth highlights for the quarter are as follows: we operationalized 4 radiation machines in 9 months and with their successful ramp up, the revenue mix for the quarter has improved leading to revenue from radiation modality growing at 13%, faster than the overall revenue growth

Investments in building superior clinical talent and business promotion has started showing results in two of our focus markets in Kolkata and Mumbai, which has delivered 57% and 17% growth, respectively.

EBITDA losses at Kolkata has reduced by half on a quarter -on-quarter basis from INR2.2 crores in the previous quarter to INR1.1 crores in the current quarter of quarter 3 FY '24.

Other centers that have notably increased revenues are Nagpur, showcasing an impressive 59% rise; and Ranchi, exhibiting a robust 31% growth in quarter 3 of FY '24. The average revenue per occupied beds are of notable increase standing at INR42,800, reflecting a significant 15% year-on-year growth.

We have increased 59 beds during the quarter across our network hospitals. On the EBITDA front, our Y -o-Y EBITDA witnessed a growth of 7%, reaching to INR82.6 crores, compared to INR77.3 crores in the quarter 3 of FY '23. Notably, contribution margin improved during the quarter driven by a better revenue mix. However, due to the seasonal nature of the quarter, operating leverage did not come into full effect.

Our cyclotron operations in Chennai were adversely affected by floods resulting in a 30 bps impact on EBITDA. Additionally, strategic decision of coming out of the last shop -in-shop

model led to a scale-back of operations in M S Ramaiah Bengaluru (Bangalore) , affecting profitability for the quarter.

PAT for this quarter stood at INR5.7 crores as compared to profit of INR7.5 crores in quarter 3 of FY '23. The decline is on account of depreciation for acquisitions of Nagpur and Indore. Our capex for 9 months of FY '24 stood at about INR118 crores. Net debt stood at INR367 crores as on December 2023. We are very comfortable with the current debt levels and project a better cash flow in the coming quarter.

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With this, I would like to open the floor for question -and-answers.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Kabir from Blue star. Please go ahead.

Kabir : I wanted to know what are the strategies for Milann going forward? Are we looking to hive off this business completely?

B.S. Ajai Kumar : Yes. I think we have made the statements in the past. We are working on it. As we know, post COVID there was a downtime, and we are putting efforts to bring it to the level where it was pre-COVID. And we are looking to see at the right time to divest. So certainly, we'll inform at an appropriate time

Kabir : Okay. And when do we expect this to happen, like...

B.S. Ajai Kumar : It is difficult to give timeline right now because we are working to see at what time we can maximize our value

Kabir : Okay. Okay. Now my second question is with Dr. Ajai. So there have been rumors about CVC exit. And everyone expects them to exist some time. So just wanting to know what's their future at HCG to provide health care?

B.S. Ajai Kumar : So the way I see this is CVC is an investor who came to HCG. you should really ask them to see at what point they exit. But as far as we are concerned, we are interested in the HCG to grow.

And as a shareholder , if they feel they have made requisite return, they will try to exit. But really the comment should be made by CVC on this. So at this point, I would like to not say anything about this aspect. But I'm sure you will be able to contact them and get comments from them.

Moderator: And the next question is from the line of Pritesh Chheda from Lucky Investments. Please go ahead, sir.

Pritesh Chheda: Yes, sir, in your matured center where the growth rate is about 8%, are you constrained by any capacity there? And second, when I'm looking at your occupancy number, so despite your initial comments on oncology and the demand in India, the occupancy looks lower. So the single-digit growth and occupancy, if you could give some more comments there?

Raj Gore: Yes. So this is Raj. Let me take that question. First, I'll answer your occupancy question. I think this occupancy is based on next 60 additi

onal beds as we have added some beds in Baroda and some minor beds in other hospitals.

Pritesh Chheda: Sir, what is the bed addition, you said?

Raj Gore: 60 beds.

Pritesh Chheda: On a total count of?

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Raj Gore: Close to 2,000. I think it's mentioned on the footnote of the same slide. Second question, you asked about the capacity constraint, if there is any capacity consent on any of our mature centers?

We have addressed this before also, let's take one by one.

K.R. Bangalore, which is our center of excellence, we had informed you earlier that we have added about 20 -plus beds in a newly premium wing. our occupancies are around 65%, 70%, we still have enough headroom there in all modalities, including beds. You also know that we have a project in Whitefield for comprehensive cancer center, 25 bedded, Bangalore is one of the affluent and fast -growing market which will be ready early next year in FY '26 or calendar year '26.

We just announced another project in North Bangalore towards the airport . City is growing very fast.It is an attractive market and the city is growing in this region . So that addresses Bangalore, which is our most mature market.

Second mature market is Ahmedabad, where we have 100 -bedded facility, which is doing very well on all parameters. As announced earlier, we are relocating to a newly constructed premium state-of-the-art hospital in first quarter of the coming year, new financial year. And therefore, that hospital will continue to grow for the near foreseeable future.

So these are the 2 big markets. I'm just giving it an example. However, for every hospital, whether mature or emerging, we have a 3 -year plan where we have enlisted capacity requirements and we are taking pre-emptive action to make sure that if radiation is coming to capacity, we will add one more machine. If beds need to be added, we will add beds. If OP Ds need to be added, we will add OP Ds. So we don't see any capacity constraints for next 3 to 4 years in any of our mature centers going forward.

Pritesh Chheda: Sir, your answer mentioned about on incrementals. So when I look at the 8% growth as of now and based on your answer, can I conclude that at least today, you're constrained by capacity?

B.S. Ajai Kumar : No, no. See...

Pritesh Chheda: No. So if you are not constrained by capacity, why in the mature centers, the occupancies are lower and why then the growth rate is 8%?

Raj Gore: See, to simplify this, what we just informed is that we have added 45 beds in our Baroda center.

So that has increased the bed capacity. Second question on revenue – emerging center of 8%, in our CFO speech, she said that we have scaled down our operations a t MSR that contributes to about 3% to 4% of growth at our emerging centers. Adjusted for that , our emerging center's growth would be closed about 13%.

B.S. Ajai Kumar : So I want to say this, I think your question mainly is in last year we had an occupancy of 65.7%, and we are now seeing 58%. It is exactly for the reason what Raj explained that we have expanded our beds because we feel when you reach close to 70, overall, y ou're reaching a capacity, more or less 70 - 75%. But now with the need for customers in center of excellence, others -- we are seeing the need for expansion. So that need will be met from this expansion, and that is why we are doing it. So this, I think, is positive, that is showing the growth we are h aving.

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Pritesh Chheda: Okay. My second question is, sir, with respect to the pre -Ind-As EBITDA margin. What is the Pre Ind -As EBITDA margin in our business on a 9 month and a quarter basis?

Raj Gore: The rental impact of capital leases is close to about INR22 crores. You need to adjust for INR 22 crores to arrive at Pre Ind -As EBI

TDA margin.

Pritesh Chheda: Sorry, I couldn't understand. So INR78 crores is a reported EBITDA minus INR22 crores, so you take it about INR56 crores.

Raj Gore: Yes, that's the number.

Pritesh Chheda: Okay. And lastly, this 2,000 beds which is there today, what beds will we see at the year -end and what beds we'll see at the FY '25 and '26 based on our expansions?

B.S. Ajai Kumar : Over the next 3 years, we will be adding about 350 beds, including our Ahmedabad project and Whitefield projects.

Pritesh Chheda: And at what capex?

B.S. Ajai Kumar : Please refer our presentation , for Whitefield facility, it's INR25 crores and for Ahmedabad 200 - bedded facility, it's INR106 crores.

Pritesh Chheda: So INR106 crores plus INR25 crores, about INR130 crores, INR140 crores for 350 beds?

B.S. Ajai Kumar : No.. This is related to these two projects only. We have already spent about 48 cr in our Ahmadabad project.

Pritesh Chheda: And what assets turn?

B.S. Ajai Kumar : See, the Ahmedabad Hospital is already operational hospital. We are just shifting to a newer facility. It's a hospital, which has EBITDA margin in high 20s, ROCE around 30%. It was a hospital where have capacity constraints for future growth . And in another quarter, we will go to an expanded facility.

Moderator: The next question is from the line of Bino from Elara Capital.

Bino: This Whitefield facility would be a stand -alone facility, right?

Raj Gore: Yes. Whitefield facility is a stand -alone comprehensive cancer center facility. We are focusing on women's cancer diseases. We'll be treating -- primarily focusing on women's cancer there. Yes.

Bino: Okay. And how many bedded would be that?

Raj Gore: 25.

Bino: 25, okay. And the Ahmedabad one would be 85 bed, the expansion that you're doing?

Raj Gore: We are moving from 100 -bedded to a 200 -bedded facility.

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Bino: So 100 additional beds are coming up. Yes, understood. There was a need to additional 100 beds that you had planned for Indore, which I'm not now seeing in your slides, is that plan still on?

Raj Gore: No, sorry, can you repeat your question?

Bino: Sure. Earlier, you had also guided to adding 100 more beds in the Indore facility in a couple of years' time. So I couldn't find that in your slides on the capex program. So that plan is on?

Raj Gore: Yes, you're right. We would like to increase our capacity eventually in Indore. I think it's a Phase II plan. We recently, a quarter ago, we have acquired this hospital. And as and when we have -- we form the plan, we will share it with you.

Bino: Understood. And sir, for this 100 -bed facility in North Bangalore, what's the timeline? Again, I can't find that on your slide.

Raj Gore: Yes. So we are looking at operationalizing this facility in 15 to 18 months from now.

Moderator: The next question is from the line of Abdulkader from ICICI Securities. Please go ahead.

Abdulkader : Sir, just if you could shed some more light on how Indore has been in this particular quarter? And if I refer to your slide among the clusters, which you have shared, where does Indore fit into this cluster, so that we could better track this from the next quarter onwards?

Raj Gore: Yes. it's added to North India cluster. So as you know, 3 October, we took charge of our Indore operations. This is a market which is -- Madhya Pradesh is a very attractive market, many cancer patients from Madhya Pradesh travel to Mumbai as well as Ahmedabad. Mumbai, Ahmedabad, we have our own hospitals; Nagpur, we have our hospital, which is doing very well.

We have a sense of that market, and we understand the potential. We have already a brand presence from patients in that market. This is an important entry into a newer state for us. Right now, we have an integration plan for this hospital. So let me give you an update on what is going

on.

So far, we have been able to plug this hospital, a newly acquired hospital on our entire digital platform, whether it's HIS, ERP, CRM, our websites, etcetera. So we've been able to successfully integrate from an IT perspective. This is a hospital, which is an older hospital. We have done an assessment from infrastructure as well as clinical perspective.

We have undertaken value creation plan at Indore as project . Right now the project is underway. when we enter in a new market, we want to ensure that the facility that we have, the clinical services and the quality that we have is as per our HCG standards, we are upgrading that facility, including rooms, ICU, facade, et cetera. That project is almost about to complete in this month, . And so the integration is going as per plan, and we're very excited about this opportunity.

Abdulkader : Sure, sir. Understood. And sir, then just commenting on your capex. So if I see the 9 monthly capex for matured center, that has already surpassed where you were at FY '23. So for next year, how do we look at this capex number on the spending side as well as on the debt front? And on Healthcare Global Enterprises Limited

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debt again in this quarter, Q-on-Q has gone up slightly. So is it entirely due to the spending on Indore or any other factors if you'd like to highlight?

Raj Gore: Indore acquisition investment has not been included in the capex table. However, the capex in mature center is primarily on account of the expansion we are doing in our Ahmedabad center.

So far, it's spent about INR48 crores in Ahmedabad that is included in our mature center's capex.

Now question on your capex going forward, I think we have indicated in our previous calls as well that our maintenance capex remains in the range of about INR70 crores to INR75 crores, and that is what one can expect. This is apart from the growth plans that we have.

Ruby Ritolia: And yes, you're right, in the net debt number, it has slightly inched up because of our acquisition in Nagpur and Indore, partly funded by debt and partly funded by our internal accruals.

Abdulkader : Sure. Understood. And one final one, if I may. So with Ahmedabad now coming in Q1 of FY '25, I mean, how do we see the cost element going up? And are we still confident of maintaining this 16%, 17% EBITDA margins? Or how should we look at the entire cost profile going ahead?

Raj Gore: Yes. So very good question. So look, let me again refresh everyone's memory on Ahmedabad.

This is our second center of excellence. It's performed exceptionally well. As I mentioned earlier, our EBITDA margins are in the higher 20s. Returns are very good. It's -- we have a very strong team of clinicians, and we have a very dominant market share in Ahmedabad.

We will release the additional capacity gradually as and when we need to operationalize it, start it accordingly so that we are not incurring costs first and getting revenue earlier. So we'll try to balance that equation. And we -- I think we are very excited. I think it will be a really showcase facility for HCG going forward.

Moderator: The next question is from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

Dhara Patwa: Sir, will we previously provided guidance of achieving 20% EBITDA margin, either by Q4 or Q1 FY '25 be upheld? If yes, then what will be the factors driving the anticipated margin expansion of 200 to 300 basis points going forward?

Raj Gore: So yes. In our CFO speech, you would have heard that our EBITDA margin little got subdued in the current quarter, and it's due to operating levels not being played out. One good thing to

inform here is that as we had announced in the previous quarter that we are installing new machines in our various centers. Those have already started showing results. And happy to announce that while the overall -- on quarter -on-quarter basis, overall revenue

has gone down,

but our radiation business has picked up, that helps our contribution margin to go up.

As we go in quarter 4 and when the revenue comes back, we feel that the higher contribution margin coming from better service mix should help our gross margin improve, that will improve our profitability. Along with that, I think we are focused towards maintaining our fixed costs and overhead the way it is. That would help us reach desired EBITDA margin going forward.

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Dhara Patwa: So we are still upholding that guidance of 20% by -- at least in the next 2 quarters, right? Like from 17.5% margin, we will at least achieve 20% -something in the next 2 quarters or something, right?

B.S. Ajai Kumar : I think just to go back and clarify, now we have acquired Indore. We have done all this because those will pull down our EBITDA margin to some extent because Indore is not going to deliver the 20%. So it will bring it down. So we have to take into consideration that our Whitefield project also becoming functional.

So we'll put all together. Obviously, we are striving towards a 20% margin. But we feel we may fall short of it this coming few quarters because of the reasons I mentioned. And we also are very cognizant of some of the government functions increasing. We have taken some measures in that area also. We want to make sure a significant percentage of our cash and private pay. All of these are happening. So I would like to state that our 20% may not be achievable in the first quarter or second quarter, but we'll certainly strive to achieve the same.

Dhara Patwa: Sure, sir. That's helpful. And one more question. What could be the peak occupancy for our mature centers that beyond that, we might think of adding more beds? So will it be something like 90%, like the other multi-specialty hospital or after 80% occupancy, we might think of adding new beds or something?

B.S. Ajai Kumar : Historically, I don't know which multispecialty hospital can say 90% is the peak because most of them when they reach in their late 70s, will start looking for expansion because there's a turnover, particularly in oncology where we have daycare people coming and going, it is very important to have the beds and not put them on a waiting list because particularly oncology patients, as you know, it's not a good idea to put anybody on a waiting list or defer their treatment.

So in this area, historically, the function hours has been when you reach close to late 70s, we look at whether it is now occupancy we need to look for. And we also look at the daily sensors in the hospital. Is there a waiting list? Is there an issue? So we get feedback from the center heads and all to decide. So sometimes it is a moving target, but we certainly feel with all this 20 years experience in oncology -- more than 20 years, that late-70s is when we should start looking for, okay?

Dhara Patwa: Okay. Sure, sir. Sir, occupancy emerging centers now have reached a good occupancy rate of 60% and ARPOB is also similar in line with the mature centers. So -- but we haven't seen much improvement on the EBITDA margin side. So at least in the next 2 years, can we see some improvement on the EBITDA margin side for our emerging centers at least 15%, 16% or something like that?

Raj Gore: Is it a question on the emerging center or...

Dhara Patwa: Emerging center.

Raj Gore: So if you look at last 4, 5 quarters, emerging centers have been steadily growing its EBITDA.

And I think our margins have also improved. In CFO's speech, we also heard that Calcutta Center

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has reduced its losses quarter -on-quarter by half. In the previous quarter Calcutta losses were close to about INR22 million that has come down to INR11 million. And in the current quarter, our

emerging center's EBITDA is close to about INR13 crores. If I can draw your attention to slide nine, -- our EBITDA has grown by 66% against a revenue growth of 19%.

Dhara Patwa: Correct. So if I want to see this at a consol level, emerging -- mature centers will grow steadily and emerging centers will pick up more growth going forward. That's how it should be, right?

Raj Gore: Absolutely. Absolutely.

B.S. Ajai Kumar : I just want to clarify your question about occupancy of emerging centers. See, we have stated it earlier, our capacity is much larger. We have not operationalized entire capacity in the emerging centers. And therefore, it's showing higher as and when hospital by hospital, we feel that we need to operationalize already -installed capacity, we will continue to release that . Won't see any capacity constraint in terms of number of beds in any of our emerging centers.

Raj Gore: And just to add one more point, our emerging -- sorry mature centers are already delivering 20%-plus EBITDA margins.

B.S. Ajai Kumar : I think that is an important -- the answer to your question, that we have reached and if you look at our center of excellence and all, we are close to 30%. So -- and our ARPOB is over Seventy thousand . So our goal is to see the mature centers also reach that level. And I think like what you heard from Raj and our CFO, Ruby, I think we are on the track to achieve that, which will obviously bring us to the 20%.

The only caveat here is our new centers coming, you too should be well aware of, like Indore or Whitefield and later on our North Bangalore, they could have some drawdown in terms of EBITDA margin. But we are confident that will be compensated by the growth happening in the emerging centers.

Raj Gore: Yes. I think Dr. Ajai made an important point about ARPOB. If you look at our ARPOB this quarter, it's about 16% higher than same quarter last year. We've always insisted that historically, most of our hospitals have been in Tier 2 and Tier 3 cities. When we embarked on our expansion journey in the last 5 years, most of our emerging centers are in cities that are bigger and with a population which is more affluent. And we kept saying that as the share of emerging center revenue goes up, our ARPOB will continue to grow, and we've demonstrated that on previous many quarters. If I have to give you a sense of our ARPOB, our centers of excellence in K.R. is in higher 70s ARPOB.

Our center of excellence in HCC is about 90,000. Now to give an idea of our emerging centers, our Colaba center is in higher 80s ARPOB; our Borivali is in 50s; in Kolkata, we are setting 60; in Baroda, we are at 60; in Nagpur, we are closer to 50. As these centers continue to grow the way they have grown this year in terms of top line as well as profitability, our overall number will continue to look better and better on all operational parameters.

Moderator: The next question is from the line of Shyam from Goldman Sachs.

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Shyam: I joined a little late. So if this question has been asked, I apologize. But just on the disruptions that happened, one at Chennai as well as the shop -in-shop that we have now scaled down, right? How should we look at like Q4? From an occupancy perspective, have they improved now? And what is the kind of outlook for utilization that we look forward, say, for fiscal '25? That's my first question.

Raj Gore: Yes. So see, Chennai, let me just remind everyone, we have a cyclotron facility As you remember, November there was a big flood in Chennai . Our Cyclotron bunker got flooded . So we have to take a downtime on it due to flooding. It's still down. We expect it to be up by end of this month, and it's a highly profitable business. As soon as the cyclotron facility is up, I think we will get back to the normal operation. On MSR facility, as we said earlier, it's a s

maller, 30 -

odd bedded shop -in-shop facility in a medical college hospital setup, it was one of our last shop -in-shop stability.

And as you know, if you look at our track record last 3 years, we scaled down or exited some of the shop -in-shop. What we've also done is in our dominant market in hospitals that are performing well we've consolidated our position. You look at Suchirayu, well performing, we took a majority. You look at Nagpur, well forming, we acquired that.

So our endeavor is to continue to consolidate in our dominant market and well -performing hospitals. As you know, Bangalore is our home market. We have insight of all major players having oncology presence, multiple hospitals. We have a dominant 30 -plus per cent market share.

This MSR facility, a smaller facility was in the vicinity of North Bangalore. We felt that it's time to put our own facility premium on this airport road, which is a 100 -bedded facility because 30 beds, you can't do much. With 100 -bedded facility, we feel we have a growth part in North Bangalore as well as total Bangalore market share going forward. So it's a strategic call to get into North Bangalore and move to our own facility, and we are very excited about it.

Shyam: Yes. Just the outlook on how should we look at AORs going forward?

Raj Gore: AOR, Shyam, I mean, you know that Q3 is usually because of activities footfalls are down. And in general, in the industry, it's a little muted. I think it's a blip. We are already seeing business back to our growth trend in the current quarter, we -- I mean it's just 1 quarter seasonal mutation.

Shyam: Got it. Helpful. Just second question is on the competitive landscape. And I remember, I think a

year back maybe, we did a complete price rationalization exercise, right, or the price comparison benchmarking exercise. So my question ties into the ARPOB dynamics, right?

You've given us some different centers with higher ARPOBs. This quarter again, ARPOB has grown healthily. So I just want to understand the outlook for ARPOBs as we go forward. Is there an element of price changes upwards that we can think of as well? And just the competitive dynamics...

B.S. Ajai Kumar : Yes. I think one of the things we have to remember is like what Raj also said, we have a differential. Our ARPOB in our main centers, like particularly Bangalore, Ahmedabad is high.

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And we do not believe that there is -- because of our technology mix and the type of patients we deal, those also being a destination.

So naturally, our ARPOB will be higher than the competitors. And we are also maybe somewhat equal or when the technology is not there, which we have, we've quickly higher. But that is something we feel if we continue to grow as we bring in more technology, more multidisciplinary clinics and also provide better excellence of medical care.

For example, if you look at our mortality rate, some of -- if we look at it. We had only 7,200 discharges happen in the month of November. Our mortality rate was below 1%. And we have brought it down -- at one time 5 years ago, it used to be 3.4%. So center of excellence, -- first time the right treatment makes a difference.

So that involves, obviously, costs. That is why the ARPOB could be high. So can we compare it to the multi-specialty hospitals what to provide? I don't think you can because being a specialized oncology service, ours will be little bit different compared to multispecialty, which is also providing cancer care, okay?

Because our cancer care patients who come are also complicated because when they go to multispecialty hospital, get their first sign of treatment. When they recur, they come to us as a destination. My own clinic is an advanced and recurrent tumor, in complicated cases. So naturally, it will make a difference. And also a technology, like CyberKnife

, Digital PET also

will we make a difference.

Whereas if you go to the Tier 2, Tier 3 cities also, I think the difference may not be that much because it is smaller centers. So when you aggregate all of this, our ARPOB growth in the future will be very good. And also, there will be a differential between the major cities, particularly our center of excellence and others compared to the other competitors.

And we feel one more thing, Shyam, is we have to really, in my view, it's my personal view, stop looking at the competitors, really look at where is the oncology center of excellence, where the growth is going to happen. As I said in my beginning statement, the oncology cases are going to increase. One in nine in India have cancer and breast cancer is increasing. It is also affecting the female population.

So we should focus really on what is the type of quality of care we are providing, are we the quality, like that is what we should focus and that is where I think even like Raj mentioned, our dominance in Bangalore, Ahmedabad will play out.

Raj Gore: Yes. Thank you, Dr. Ajai. Just to add to what Dr. Ajai said. Shyam pricing is just one of the levers to drive ARPOB. High-end treatments, as you know, we have introduced robotics in three more centers. That gives us the ability to differentiate as well as increase realization per patient and reduce the ALOS. ALOS is the second factor.

As surgeries are becoming less invasive, we are able to turn around the patients faster on the same bed. As we are introducing more linear accelerators, radiation machine, we are able to drive radiation business. So combination of high-end treatment, faster turnaround, lower ALOS,

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I think will be main levers more than pricing for us going forward, which will also help us to drive volumes going forward.

We will see about 5% to 7% improvement in our year-over-year is something that we should expect going forward.

Shyam Srinivasan: Got it. And just lastly, I'm just being greedy here, is there any growth guidance that you would like to share for, say, the medium term in terms of how we should look at top line growth going forward?

B.S. Ajai Kumar : Shyam, we've never given growth guidance in the past. What we've always said is our endeavour and our track record has been to grow faster than market, and we will continue to drive that

going forward.

Shyam Srinivasan: Sir, help us, what is market growing at, sorry?

B.S. Ajai Kumar : Market is growing about 10%, 11%, and we plan to grow at a higher rate. I think we have a good track record on our existing hospitals. We have good growth pipeline. Combination of that, we will -- I'm sure, will beat market growth rate going forward.

Moderator: And the next question is from the line of Ankeet Pandya from Incred Asset Management. Please go ahead.

Aditya Khemka: This is Aditya Khemka here. Sir, I'm just little concerned with the revenue growth rate that we have reported in the mature centers. I understand that you expanded that in the mature centers and therefore, that has impacted occupancy and margin. But what I'm failing to understand is the mature center revenue growth rate of 8% year-over-year.

If you look at what the other counterparts are reporting, that seems to be a significant gap between the growth rate that HCG is doing and your peers are doing in the hospital space. So can you talk about why mature centers grew only 8% despite your standing there?

B.S. Ajai Kumar : Yes. So I think we mentioned it earlier also. We have scaled back our operations in one of the facility in Bangalore, which was categorized as mature center. There is an impact in Chennai due to flooding. This, again, was classified under mature center bucket. If we adjust for these two, that 8% becomes 12%. I don't see issue in our growth rate.

Again, this is something that has

happened this quarter. I think we have a good track record and driving mature center growth year-on-year. And you will see in Q4, it's coming back to normal rates going forward.

Aditya Khemka: Understood, sir. Secondly, just to understand the margin profile. Again, I'm comparing it to the other hospitals that we know of and they report. Even EBITDA margins for most of these hospitals are 25% and north of 25%. And these are also national chains like you are. They are also expanding like you are. They also have some new centers, emerging centers like you have. So -- and it's not like a quarter or two that we you're lagging behind their operating margins. I mean, they have been consistently doing more than 25%. And we are consistently hoping for a

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20% number from HCG. So is there something in the business model that leads to a lower margin compared to last year? Or is it that we have too much overhead? Or is it that the cost of physicians add in our business model compared to what these guys have?

B.S. Ajai Kumar : Let me answer this question in two parts. One is there are hospitals which are based in big cities who could have a higher EBITDA margin. And I don't know why you made a statement all hospitals at 25%. I don't think all hospitals are above 20%. So even look at some major corporate hospitals which are across India, in different cities, their EBITDA margin is low. And as you come up with more growth opportunities and put more new hospitals, your EBITDA margin will come down.

So 25% is the limit, but I feel that is only anomalies in terms of few hospitals showing that individual hospitals. For example, if you take the GI hospital as far as I know they have a good margin, maybe 28%. But we have a good margin in Bengaluru (Bangalore) . We have 30% margin, Bangalore Center Of Excellence. Our center of excellence in Ahmedabad has close to 23%.

Our established center for your information, is at 24% EBITDA margin, established centers and particularly in cities it is higher. -- our Cuttack center, for example, produces 30% EBITDA margin, it is there for a long time.

. I think our business model is very robust. We are in a growth mode. As you mature, some growth comes in, it will cause us to change.

But as I said before and as Raj also said, I think we are looking at reaching that 20%, but it will be push and pull because of performance of new centers, So that is how the investor community should look at it. They should look at it and see how would be and what is the future? Like what happens when all the centers reach 25%, 20% in the next three to four years, where we will be? So we should look at it the way rather than making a generalized statement in my view.

Raj Gore: Yes, add to Dr. Ajai. As you rightly mentioned, our established centers or mature center are delivering 24% margins. And as our emerging centers continue to grow, we should be able to expand our overall EBITDA margins. Just a point of reference maybe, if you look at last year same quarter, our EBITDA margin was less than 7% which has grown to 10%. As this 10% keeps growing towards and keep inching towards mature center, we should be able to expand our EBITDA margin.

B.S. Ajai Kumar : A center like Indore can come which is great for long-term opportunity, that can pull down your EBITDA for a while. So that is how we have to calculate this. Otherwise, we have to do only M&A activities, where all of them give us EBITDA margin of say 25%, then only we acquire

it, then where is a growth, where is opportunity for growth for any investor? Do you see any growth, if you...

Aditya Khemka: No, no, I understand what you're saying, and I appreciate it. Sir, one last question on the EBITDA margin. So I just wanted to understand, you gave examples of certain centers, which had 30% EBITDA. I understand these are your cen

ter excellence centers, right? So is there a difference in the peak potential margins of your center of excellence centers and some other centers, which
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were not the centers of excellence. If yes, could you quantify what a center of excellence could achieve and what another center could achieve?

B.S. Ajai Kumar : So I think as we said, center of excellence reaching 30% is extraordinary in the single speciality. That has to be appreciated. I think, will that sustain? Yes, you can sustain. But do we want it to be higher? I think that is not our objective . We are satisfied in the higher 20s to 30. But as the technology improves, as the mix improves, as the footfall improves, our growth will happen. In the centers of excellence, we have witnessed growth happening significantly, that growth will continue to happen.

Aditya Khemka: One last question, sir. On the payer mix, so what percentage of our revenue would be out of pocket, what could be private insurance and what would be public insurance?

B.S. Ajai Kumar : Our Cash TPA and Corporate business is in the range of 70 -75% And as Raj mentioned, Raj maybe you want to comment that we are trying to obviously maintain this or improve our cash and payer mix more, right? Do you want to comment?

Raj Gore: Again, just to -- most of our new centers, emerging centers, are in bigger cities, where affordability as well as insurance penetration is higher. Because of our historical presence, larger presence in smaller cities, our mix was in a certain way. But as the emerging centers start contributing more and more towards our revenues, our mix will improve going forward.

Aditya Khemka: So when you say improve that, you need more out -of-pocket and lesser public or do you mean more out of pocket and lesser private and public?

Raj Gore: No. So I think we should not look at it cash only. I think we should look at cash plus TPA. Because TPA insurance -- as insurance penetration is growing, cash will get cannibalized as people will have more and more insurance policies. I think we should club insurance and cash together and target at that segment.

B.S. Ajai Kumar : I just want to add. What is our wishlist? Our wishlist is obviously we don't want patients to spend out of pocket. There is no question. If you look at advanced countries, majority of them are cashless. I was practicing there for 28 years. I never bothered about what is the cost because the insurance company would pay, you would do the right thing.

So for us to do the right thing for the patient, ultimately dropout rate to be less, everybody should be in the private insurance. So that is what we should strive for. But fortunately, post COVID, what we have documented is that the private insurance has increased. That means people have become more concerned about health care and until then I can attest to that, the people were not bothered about the insurance.

They thought they were immortals, but now people are beginning to realize, they are coming into that. There are more insurance companies that are also opening. So I think it is definitely a positive trend for health care industry. Not just insurance penetration is growing, but I think the covered amount is also growing.

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Raj Gore: Yes, yes, historically, it was about INR3 lakhs, now it is closer to INR10 lakhs. So it's more and more -- we have understood the importance of having sufficient cover. I think it's a good time for entire industry going forward.

Aditya Khemka: And Raj, would it be fair to say that public insurance patients would be at a lower EBITDA or whatever gross margin compared to private insurance/out -of-pocket patients?

B.S. Ajai Kumar : Yes. I think one of the important things to realize is it is state -by-state. You look at Odisha, for

example,--the rates are better and payments are more or less on time, that is why our EBITDA margin is good, and the patient care is also extraordinary, which is what we focus on, what is it can provide.

For example, you take a drug like Herceptin, which is needed for, let's say, breast cancer patients.

With the insurance company, they won't pay, how can you say for a 28-year-old woman, I won't give you the right drug. So it is, we can say, a more significant moral issue, ethical issue for us. But what Odisha has done, to some extent what Gujarat has done and Maharashtra is, they are - some of them are even covering immunotherapy.

So I think we are moving in the right direction state by state. . So some states have a good EBITDA margin, but some states are very bad, very poor. So we need to now develop those states which are not good to encourage them. We are showing them the comparison with Odisha and other states to the state government and see that they also concur in paying this. But the standard answer which they give out is, we don't have money. So we need to address that, obviously.

Raj Gore: Also just to add, I think the beauty of HCG model is -- let's take Andhra. Andhra is a state which pioneered state schemes right? But because of higher turnover, higher asset turn, we've been able to -- even at a government rate, we are able to deliver 20 -plus margin, 20 -plus percent EBITDA margin in Vijayawada, Vizag, similar case in Bhavnagar. So I think you just need to figure out how to treat your model to make it work for you.

The reality is some of these Tier 2, Tier 3, Tier 4 cities, it's a government scheme which brings affordability and quality care to these patients. And as HCG, as cancer care to other, we want to find a model to make sure that our care is affordable and accessible to them in this market. Otherwise, it wouldn't be.

Moderator: And the next question is from the line of Rishabh Tiwari from Allegrow Capital. Please go ahead sir.

Rishabh Tiwari: Our understanding was that the Ind AS adjustment was around INR17 crores till FY23. Has it increased significantly to INR22 crores? Is it because of...

Ruby Ritolia: It has increased to INR24 crores in the nine months of FY24.

Rishabh Tiwari: INR24 crores for the nine months?

B.S. Ajai Kumar : Yes, yes.

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Rishabh Tiwari: Even that doesn't look right, because INR17 crores is quarterly till last year, right?

Ruby Ritolia: Sorry?

Rishabh Tiwari: Our understanding is that INR17 crores was the Ind AS assessment for the quarter, right, for last year?

Ruby Ritolia: In the nine months, it is INR73 crores.

Rishabh Tiwari: Okay. That is also a significant increase, right, from INR17 crores going to INR24 crores?

Raj Gore: See, since we acquired the assets of Nagpur and Indore, those -- these centres have also come under ROU capital leases.

Rishabh Tiwari: Okay. Okay. What is the corporate costs right now that we are incurring per quarter?

B.S. Ajai Kumar : Yes, about 4%.

Rishabh Tiwari: And can you also comment on the performance of Borivali and South Mumbai centers?

Raj Gore: Yes. So look, Mumbai has been doing very well. We have Mumbai growth about close to 20%.

Borivali has done fantastic. Its EBITDA margin is 20 -plus percent. We were running out of capacity in our LINAC. We have added one more LINAC in the last quarter. We've also added robotics in Borivali. Colaba, we had earlier indicated that we want to position it as a destination for international patients, we had tremendous success in the last 3, 4 months in attracting international patients to Colaba.

For Colaba, international patient is almost -- mix is about 30 -plus percent now. I think both we are on track. As combined HCG in Mumbai, we are one of the few players -- one of the two players who have been able to grow our market share in o

ncology over the last two to three

years. So I think Mumbai is an important market for us strategically. It's a competitive market.

We -- I think we have the right plan, and we are growing as per our expectations in these two markets.

Rishabh Tiwari: What margins are you able to achieve in the last quarter in Bombay?

B.S. Ajai Kumar : So Borivali is over 20 % EBITDA margin . I think in Colaba, the South Mumbai centre , we are still loss -making. We are expecting in a couple of quarters to get to a breakeven point. Our focus right now is, as we indicated earlier is to build clinical talent . We started building brand. We started driving specific products, --focussing on specific markets specially international markets . The centre is being positioned as destination for international patients in the field of cancer care. In addition, we have opened a women's wing specifically focusing on women's cancer, breast cancer and cervical cancer in this unit. This makes us hopeful that both centres combined, we will grow our market share in Mumbai.

Rishabh Tiwari: Understood. Just one more question on Borivali. This 24% is pre -rent, right? What is the rent cost that we're incurring for Borivali?

Raj Gore: INR92 lakhs per month.

Rishabh Tiwari: INR92 lakhs. Okay. And just one more question. What is the CapEx that we are planning to spend in North Bangalore?

B.S. Ajai Kumar : Actually, it's a 100 -bedded facility where we are -- yes, it's -- I mean, as you know, our current center of excellence we started our journey here. As we grew, we started taking more and more buildings. It's not a pre -planned, pre -designed hospital. As we became more successful, we just added buildings there. In North Bangalore, we want to make a statement by building a state -of-the-art advanced premium facility, just like what we are doing in Ahmedabad. It's 100 -bedded facility, the expected capital outlay for that is about INR90 crores.

Moderator: And there are no further questions. I would now like to hand the conference over to management for closing comments.

B.S. Ajai Kumar : Once again, I would like to thank all of you for your active interest, participation in HCG's journey. We are -- as you see, we are very excited about our growth initiatives. We are -- we feel that as a dominant oncology player in the country, it's up to us how we take HCG cancer care across India to more and more markets. We will continue to make cancer care accessible and affordable across India. Please stay in tune -- stay tuned. And thank you very much, and have a good weekend.

Moderator: On behalf of HealthCare Global Enterprises Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Jun 2024

June 05, 2024

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on May 30, 2024

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(a) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on May 30, 2024, with Analysts/Investors to discuss the Audited Financial Results of the quarter and year ended March 31, 2024.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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"Healthcare Global Enterprises Limited Q4 & FY'24 Earnings
Conference Call "

May 30, 2024

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on May 30, 2024 will prevail.

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN ,
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER , HEALTHCARE
GLOBAL ENTERPRISES LIMITED
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER ,
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. ASHUTOSH KUMAR , HEALTHCARE GLOBAL
ENTERPRISES LIMITED

Healthcare Global Enterprises Limited
May 30, 2024

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Moderator: Ladies and Gentlemen , Good Day and Welcome to the Q4 & FY'24 Earnings Conference Call of Healthcare Global Enterprises Limited.

This conference call may contain forward -looking statements about the company which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not guarantees about the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. B.S. Ajaikumar, Executive Chairman of Healthcare Global Enterprises Limited. Thank you and over to you , sir.

Dr. B.S. Ajaikumar: Thank you very much and good morning to everyone and a warm welcome to the present Q4 and '24 Earnings Conference Call For Healthcare Global Enterprises . I am joined today by Mr. Raj Gore – our CEO , and Ruby Ritolia – CFO and my senior management team along with SGA , our Investor Relations Advisor.

The past year has been another remarkable chapter in our growth history . We are proud to have positively impacted the lives of those have entrusted us with their well -being . The achievements have been made possible through the efforts and tireless dedication of our doctors, nurses, staff and other stakeholders . Having spent decades in cancer care industry both in US and in India , I needless to say, there's a contrast between the Western countries and India. In the beginning we used to see a lot of patients with advanced stages . But today I'm happy to say even bigger cities we are seeing patients in early stage, and we are seeing good quality of treatment as well as very good outcome.

Given the population of India and more recognition of cancer and lifestyle disease, there is definitely going to be a rising incidence of cancer in the coming decade. With the reported incidence 2.2 million annually, the actual incidence is expected to be much higher, maybe 1.5 to 3 times higher as the statistics are definitely all around.

At HCG, we are ceaselessly fighting the war against cancer. We are the only organized cancer care chain in the country, with the state -of-the-art operation extending to tier one and tier 2 cities . Thanks to our state -of-the-art technology , including digital PET scans, digital pathology, precision radiation therapy, cutting edge robotic surgery system , we deliver the highest standard of care to our patients.

We recently added Robotics which will transform precision medicine in the era of genomics. We believe precision medicine is the targeted therapy in the future and genomic evaluation is going Healthcare Global Enterprises Limited

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to play its major role. And having done genomic analysis over several thousand patients, we have certainly become leaders in this to the targeted therapy based on the genomics.

We plan to introduce MR LINAC Radiation Therapy in Bangalore, which would be a notable addition to the vastly superior radiation treatment offering at a HCG which will help us revolutionize radiation therapy in the state.

This cutting-edge technology integrates magnetic imaging with linear accelerator to improve the clinical outcome, decrease side effects and reduce treatment span.

There are several clinical milestones we have achieved during the year. To mention a few, we have performed record number of minimal access head and neck surgery in our center of excellence in Bangalore.

We have administered one of the rare procedure called Neuro SAFE

a nerve

saving prostatectomy. We have done robotic assisted breast axillary thyroidectomy which is the first of its kind in Mumbai.

We have also done a significant number of 3D Planning Large Tumor Resection in our Bangalore division with having an engineering department for the same.

We're happy to report that our mortality rate as we measure while we are one of the few measuring it has now come under 1% over the years, the feat we are immensely proud of and definitely meeting or beating the global standards.

Research and development is an integral part of HCG DNA. Our clinical trials and research initiatives are paving the way for new and improved treatment modalities. We have established an Institutional Research Committee to fund investigator-initiated trials, which is one-of-a-kind in the Indian private health sector. This commitment to R&D nurtures innovation and leads to industry-leading academic excellence in HCG.

We continue to collaborate with marquee companies and projects like predictive analysis, analytics, computational work and key technical design support system by leveraging our AI-ML technology.

We are the best-in-class medical talent from around the world, ensuring that our patients benefit from the expertise, some of the best minds in oncology.

Our commitment to improving cancer care in India remains steady fast. We are dedicated to raising awareness, enhancing early detection and providing advanced treatment option to transform the perception and reality of cancer care and make cancer care a chronic disease. Together, we will continue to strive for a better outcome and a brighter future for those affected by this disease.

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I will now hand over the call to our CEO, Mr. Raj Gore, for his observations and strategies going forward, and also the summary of the operational performance for the quarter gone by. Raj?

Raj Gore: Thank you, Dr. Ajai. Good morning, everyone. A very warm welcome to all the participants on the call.

We are very proud to report a strong performance during Q4 '24 with all-time high annual revenue which grew 12% with EBITDA growth of 21% translating to 19% EBITDA margin.

This exceptional growth serves as a testament to our enduring commitment to excellence in cancer care.

HCG has positioned itself as the destination for cancer care with superior clinical outcomes underpinned by advanced technology and commanding market-leading positions across 16 of 18 cities.

The company has decoded the oncology business model in India with robust performance both in metros and non-metros.

We continue our dominance in key existing markets like Bangalore, Ahmedabad and Cuttack along with turnarounds for centers like Nagpur, Jaipur, Borivali and now Kolkata.

Further more, there is a massive potential across the key established and emerging centers that still remain untapped with potential to grow faster than the market over the next few years, which would help us to improve our return matrix going forward.

For the Fiscal Year '24, ROCE performance for the company has been 10%, whereas our established centers operate at much superior ROCE of 21%. Based on vintages, we see that the nascent centers, although have low or negative ROCE currently, they can significantly improve ROCE to operate in line with longer vintage centers. With the Kolkata center generating positive EBITDA, our conviction on delivering strong returns has only increased.

There are multiple levers in place to keep dominating the oncology market in India, which we have captured in the last couple of slides in the first section of our investor presentation. Now, I would like to briefly talk about some of the key strategic initiatives. Over the years, we have taken multiple steps to enhance our operations and improve our profitability. To point out key strategic initiative

s during the year, after consistently achieving organic growth for 3-4 years

now, we are now poised to expedite HCG's expansion through strategic acquisitions.

In addition to our expansion efforts in Indore, we are committed to further strengthen our presence in Bangalore. We are currently in the process of establishing two hospitals with total

125 beds in North Bangalore and Whitefield area, slated to be operational in the next 12 to 15 months. These state-of-the-art facilities will enhance our capacity to cater to the growing cancer needs of the region.

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Furthermore, to enhance the patient experience and streamline access to healthcare services, we have introduced HCG Care Smart App suite, including a Patient App exclusively designed for unique needs of cancer patients. This innovative platform provides patients with seamless access to treatment options and their medical records with a click anytime from anywhere. In addition, the app would also help us consistently engage with our patients to monitor adherence to treatment plan and post-treatment follow up to improve long-term outcome. Already, the Smart App suite has benefited over 56,000 outpatients with active participation from more than 300 doctors on this digital platform.

Our digital revenue has grown 75% year-on-year in FY '24 and will continue to be an important driver of growth in future.

With this, I hand over to Ms. Ruby, our CFO for Financial Highlights.

Ruby Ritolia: Good morning, everyone. In reviewing our performance for the quarter, I'm pleased to report 12% year-on-year growth, culminating in a top line figure of 495 crores. For the full year of FY'24, the revenue stood at 1,912 crores, witnessing a growth of 13% year-on-year.

Our operating metrics, key indicators of our performance have shown substantial all-around improvement in this quarter.

Chemotherapy sessions increased by 15%, OPD footfall rose by 19%, and while our radiation experienced healthy growth. The decline in capacity utilization from 65 to 61 is due to proactive expansion and the incorporation of four new linear accelerators.

ARPOB for the quarter stood at 42,700 as compared to 39,700, registering a growth of 8% on a YoY basis.

The revenue of our established centers experiences a 11% year-on-year growth and revenue from emerging centers grew by 15% on a YoY basis, with EBITDA for emerging centers growing at 117% on a YoY basis.

We are observing a consistent uptick in our emerging centers marked by increased footfall across various cancer treatment modalities. Our two prominent emerging centers in Mumbai and Kolkata have demonstrated robust performance. Specifically, our Kolkata Center had an impressive year-on-year growth of 20%, while our South Mumbai center achieved 37% growth. Additionally, our Nagpur center has outperformed expectation recording a remarkable 48% year-on-year growth.

On the EBITDA front, our EBITDA grew by 21% YoY and stood at 94 crores for the quarter.

PAT for this quarter stood at 21.3 crores as compared to 8 crores in the previous year same quarter. Our CAPEX for the 12-month period stood at 187 crores and net debt excluding leases stood at 358 crores as on March 2024. This includes acquisition of Nagpur, Kolkata and Indore.

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and we have spent 116 crores towards our growth CAPEX mainly on our two facilities, Ahmedabad and Whitefield.

As we speak, Ahmedabad construction is almost complete, and we will be transitioning very soon.

We have also given bifurcation of our EBITDA across matured and emerging centers. I would request participants to refer the investor presentation for further details.

With this, I would like to open the floor for questions and answers.

Moderator: We will now begin the question-and-answer session. The

first question is from the line of Nishith from Chrys Capital. Please go ahead.

Nishith: So, I just wanted to ask the revenue growth drivers for FY'25 between new beds, ARPOB occupancy.

Raj Gore: If I've understood your question, you are asking about revenue drivers for the current year?

Nishith: Yes, between new bed additions, ARPOB occupancy.

Raj Gore: So, look, our revenue growth in last few years is primarily driven by two-thirds by volume growth and will continue the same trend going forward, it will be largely driven by volume. While we have seen improvement in ARPOB, as you know, during the year we have deployed bed capacity, linear accelerator capacity, OT capacity, we have added clinicians and strengthened our go-to-market efforts. We are also in this year moving to a larger facility in our center of excellence in Ahmedabad. So, we have deployed additional capacity. We have increased our clinical bandwidth; we strengthened our go-to-market which will continue to help us get more patient

footfalls and therefore grow at a healthy rate which is better than the market growth rate.

Nishith: Just wanted some color on outlook on margin for FY '25 and beyond ? Our EBITDA margin overall for the company in Q4 is around 19%, whereas for F Y'24 is 17.8%. So , is Q4 the new normal for margin and what will drive margin improvement if at all?

Raj Gore : So, if I can just take you back, look, first half of the year , we communicated that our margins were subdued because investment in clinical bandwidth and then downtime transition time to add our linear accelerators . Throughout this year subsequently , you have seen improvements in our EBITDA margin . EBITDA margin that we see in Q4 is on account of better service mix and a payer mix and operating leverage due to a strong revenue growth . We expect that to continue going forward. And as I mentioned ea rlier, there are lots of revenue growth leve rs to drive volume -led revenue growth going forward . Our utilization as Ruby mentioned in her presentation on beds it's about 56% , on linear accelerator is 61%. So , I think we are very well poised to drive revenue growth and that will help us to get operating leverage . One of the strongest performance is our emerging centers. As you heard, Kolkata has started contributing

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positive EBITDA. It will continue to grow going forward. South Mumbai has reduced the losses and is expected to start breaking even sometime in the middle of the year . These two were earlier EBITDA drag due to losses and now going forward as they start contributing to the EBITDA margin, we are very confident that we will continue our EBITDA margin journey in a positive direction going forward.

Nishith: So, even the emerging EBITDA which is in Q4 is 14%, you're saying is sustainable , right, because fiscal year is somewhere close to 9%?

Raj Gore : So, just to recap, we have many centers in this bucket. Most of the centers have been contributing EBIT DA and have been growing . The two youngest hospitals, the Kolkata and South Mumbai were the drag on our EBITDA margin in the past. Kolkata in Q3 and Q4 has started contributing positively and will continue to reduce the losses , in South Mumbai we will break even in the middle of the year. So , as a result, the emerging center bucket will start moving in the right direction throughout this year.

Nishith: And could you share how Indore has panned out for us?

Raj Gore : So, look, as we said, Indore was our strategic acquisition in a new market, Madhya Pradesh . Central India has the one of the lowest penetration or lowest density of comprehensive cancer care center per million population. It's about 45 , 50 lakh population. You have one cancer care center versus about 16 , 17 lakh population per cancer care center in our Southern region. So, we made this acquisition s

tarting with the second half . Our first priority was to integrate it on HCG platform in every possib le way and invest in this asset to bring it to HCG quality care. We had started the construction work or renovation work . We have upgraded IC U facilities, we have upgraded private rooms , we have invested in medical equipments , invested in upgrading OT equipment , we have started empanel ing more TPAs , our go -to-market initiative has been strengthened . It's on the right track so far as per our integration plan and we will continue to share the progress going forward. So, far so good in terms of our progress on integration of this new asset.

Nishith: Any thoughts on inorganic acquisition?

Raj Gore : We have been saying that after 3 -4 years of consolidation and a consistent quarterly strong performance , few quarters ago, we said that we will look at acquisition as a strategic lever to grow or expand the company. We have already done Indore acquisition. At any point of time, we are evaluating several key assets , and as and when we get to concrete concluding stage, we will be happy to share with everyone.

Moderator: The next question is from the line of Dhara Patwa from SMIFS Limited. Please go ahead.

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Dhara Patwa I just have three questions. One is, what is your bed expansion strategy for the next two to three years ? And suppose if you want to develop a new hospital, so what is your criteria to select the geography for that expansion?

Raj Gore : Thank you for asking that question. See, over the years, we have created a dominating presence in our current 18 , 19 locations. Our first priority is to invest in these assets and create capacity in terms of beds, OT s, linear accelerators, so we continue to dominate our presence and continue to grow our market share in our current locations. We have already announced and shared with you that in Ahmedabad, we are moving from a below 100 beds to 200 bed capacity , newly built to our specifications, v ery premium advanced cancer care center in next month. So , we're doubling our capacity there. We have added about 20 beds and two OTs in the last year in our

center of excellence in Bangalore . We have announced two new projects in Bangalore market, one in East Bangalore in Whitefield with 25 beds, comprehensive cancer care center , and one in North Bangalore , about 100 beds , comprehensive cancer care Center which will become operational in another 12 to 15 months. So, we're doubling down in our both strong markets . In total, across our current hospitals , we are looking at adding about 350 to 400 beds in our existing hospitals in the next four to five years. Most of that will get completed in the next three years. So, that's the plan on our current Brownfield expansion in our current market . As I mentioned that we continue to look at M&A opportunities. In the past, we have had a brilliant track record in acquiring cancer care centers and creating value. So , we continue to look at it. We started our journey last year with Indore. We are looking at opportunities. Basically, our criteria for that is we're looking at comprehensive cancer care centers in markets which are attractive markets in terms of cancer incidence, affordability, household income, density of comprehensive cancer care centers . These assets are usually 70 to 80 beds. We are looking at assets which preferably are EBITDA -accretive from right in the beginning and we can acquire it at a good valuation. So , that's the criteria for our M&A. In markets where we do not see M&A opportunities, but we feel that there are markets of strategic importance , we may look at Greenfield, especially in our current states where we dominate like Maharashtra, Gujarat . As and when we have something concrete plan on that front , we will share with you.

Ashu tosh Kumar : In addition t

o 350-400 beds we are having, we also have about 200 additional installed bed capacity, which we have not deployed. So, along with adding new capacity, we will also be deploying the existing installed capacity where the CAPEX is already spent.

Raj Gore : If you look at our current occupancy is around 65% , 67% on our operational beds. Our capacity - on-capacity beds, including the 200 -odd beds , Ashu mentioned, which we have not made operational is about 56%. So, in all capacity parameters, we have enough headroom in our current hospitals to continue to grow for the next five years.

Dhara Patwa : My second question was what is the average life of a LINAC machine and the replacement cost for the same?

Raj Gore : Sorry, are you asking about average life of a LINAC machine?

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Dhara Patwa : Yes, like how much is the duration that we could use it -- it is 12 years, 15 years, something like that?

Dr. B.S. Ajaikumar : The linear accelerators normally there for about 10 to 14 years. Most of the new technology has been in the software upgradation. The basic hardware platform has remained the same in the last 10-years or so . So, as new technology evolves, as you know with the AI and high technology, more and more it will be software upgradations. So, the basic unit may remain the same with upgradation. So, our thinking at this time is , it will last anywhere between 12 to 15 years , the average linear accelerator hardware. Of course, the software and all can even last longer, but we are in the transition process as far as the linear accelerator is concerned , because we are talking about adaptive therapy, precision therapy, MR LINAC , so many new things are happening. So , it is undergoing a tremendous change . As an oncologist, I do believe this will last for a long time.

Dhara Patwa : And lastly , what will be the effective tax rate for FY '25?

Dr. B.S. Ajaikumar: Sorry, can you ask that question again?

Dhara Patwa : Effective tax rate for FY'25, ETR?

Ruby Ritolia: Somewhere between 35 % to 39%.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one on the opening remarks around the whole aspiration to grow faster than the industry , so we have done about 12 % , 13% this year in terms of top line growth. When I look at some of the large NCR -based players who report oncology separately, they have grown 21 % to 25%. I know they may be an outlier, but some of their relative absolute sizes of revenue have now reached almost pretty close to us. So , just want to understand what's the reason why we may be actually growing lower than some of these peers , is it just that they are in expansion phase, and we have not , just want to understand some of the competitive dynamics ?

Raj Gore : We have grown about 12% year on year -on-year. However, I want to just point out that the last couple of quarters ago, we have announced that we're going to scale down our large shop and shop center in North Bangalore, MSR . So, that business has not been with us for the last six months. If we adjust for that, we have grown about 14% year -on-year in the last year. That is about the market growth rate, which is about 12% on an average oncology market. Now , obviously year -on-year growth rate is also a function of the base that you have, and I think many multi -specialty players have started focusing on oncology recently and therefore, their year -on-year growth is a function of their existing base that they had. We have a large presence; we have been in this business for a long time and therefore our base is much higher . Some players have very high concentration in specific markets, right? So , there the realization plays a larger role in

growth versus volume -led growth. I think as a strategy, we want to continue to focus on vo

lume -

led growth, which is a sustainable growth, while continue to improve realization, but primarily

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volume -led growth. So, look, as a leading oncology player, we have enough levers and as

explained our plan is robust to maintain our leadership position which is volume -led growth,

and we will continue on that path. More than that, I can't comment on other players, yes.

Ashu tosh Kumar: While Shyam, as you rightly mentioned, they have been on an expansionary path, we have been

in a consolidation phase. We started deploying additional capacities and in some of our key

centers like Ahmedabad. So, Ahmedabad, Cuttack and some of the other large centers, we have

started deploying capital, increasing the capacity which has been done and we should start seeing

growth momentum from these centers which will rub on the overall growth of the company as

well.

Shyam Srinivasan: Just a second question, actually a related question, question number one, in terms of doctor

attrition because some of your competitors are expanding, have you seen attrition or any of those

parameters that you look at from an HR or retaining our key talent perspective, has that seen an

uptick or a change or what are we doing to retain our top talent?

Dr. B.S. Ajaikumar: Shyam, as you know, HCG has always been very proud of our doctor group, and our top doctors'

attrition is very low, less than 5%. So, we have been very proud and even today because of the

way we empower the doctors, we are involved in academics and research. Our attrition has not

been a major issue and we don't expect it to be an issue at all. And another thing I want to tell

you is we are also in training, we have our own training programs in fellowship programs,

residency program and because of that we have significant number of doctors coming out of the

training in medical oncology, radiation, surgical oncology. Because of that and we are an

institution and I always like to compare it to where I trained MD Anderson, wherever you went

in US, you found doctors trained in MD Anderson. And it is a proud thing that doctors are there

working in different institution trained by that. So, similarly where we go now, we see doctors

are trained in HCG are there. So, we don't have any issues. Doctors who want to even sometimes

leave ... because we have an internal training mechanism and because we brand HCG more than

doctor and we collaborate with doctors and work, we don't see that as a foreseeable issue at all.

And the people come to it to HCG has a quality institution as a destination. So, that is how we

have been able to grow and maintain our status as the leader and we will continue to do that.

And we are very clear that most of the major doctors have been with us for a long time and will

continue to be there and we don't see any issues in.

Shyam Srinivasan: Just my second question is if you could double click on both I think the case studies that you've

presented in the investor presentation are very helpful, but I just wanted to go through like South

Bombay, what's you now are looking at international medical tourism as an avenue. So, just want

to understand what's the size there, what are the plans for further enhancing our offering at South

Bombay, what's been the response even from local patients?

Raj Gore: As you know, South Mumbai, our South Hospital is a premium hospital at a very good location.

We have a differentiated technology there. It is still the only hospital in Western India with a

CyberKnife and Tomotherapy under one roof. As mentioned earlier, we have invested in our

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clinical talent. We have medical oncologist who focuses on breast oncology. He used to be a

director of breast oncology program in UK, in Nottingham Hospital. We have a breast surgeon

who's com

e back after getting trained in Memorial Sloan Kettering. So, we have a very good

talent on medical side, surgical side radiation, we were always strong in that department. So, we

have a product now which is a premium product, differentiated technology and with full-time

clinicians with a very high pedigree. I think this is the perfect ... and Mumbai is more connected

to the rest of the world in most cities in India. So, this is the perfect recipe to attract international

patients. And just to explain, CyberKnife gives a lot of advantage in terms of treating cancer

patients which cannot be treated by other LINAC machines. But one of the biggest advantage of

CyberKnife when international patients travel to India, you can treat the patient with a hyper

fractionation. Generally, in linear accelerator, you will go through 25 to 30 fractions, and you

have to stay 6 to 8 weeks for that, whereas CyberKnife can do it in a matter of days and therefore

the length of stay for international patients in India becomes much lesser and therefore out of

pocket expenses goes down. So, not only do you have a superior outcome, but your out-of-

pocket expenses go down. So, it's well positioned to target international patients. We have been

working on our go-to-market efforts in certain markets like Middle East, Oman and East Africa. We have seen currently 30 % to 35% of our center revenue coming from international. At the same time because of these differentiated products, we are targeting a wider geography, not just South Mumbai, but a Greater Mumbai, Maharashtra and strategically connected locations to Mumbai who historically train into Mumbai like Northeast states there, some of the cities in central India. So, we continue to spread our net wider in terms of go-to-market to get the right segment for the products that we have in this. We have significantly reduced our losses this year in this center, and we are expecting it to break even sometime middle of this year.

Shyam Srinivasan: Similarly, on Borivali, your presentation talks about positive EBITDA in Fiscal '24 and revenue going at a CAGR of 22%. So, all the changes regarding I think we have made management changes as well. So, how is that seeing fruition now when I look at Borivali? Any numbers you want to share at this point of time given that we have now reached probably some scale there?

Raj Gore: Borivali has been a well-performing asset. Actually, it exactly follows our unit economics. Last year we have added one more linear accelerator to augment our radiation capacity there. We added our surgical robot there. We have created a vertical specialization in surgical team. We have onboarded surgical talent. It had shown 20 %-plus growth. It delivers a mid-20 EBITDA margin, and we have enough spare capacity on all modalities there. So, I think we are perfectly poised to gain market share from others and continue to grow at a very aggressive growth rate going forward in Borivali in Mumbai.

Shyam Srinivasan: My last question is just on the balance sheet. In terms of our debt, I know, debt went up year-over-year in Fiscal '24, but what are the plans going forward, will we use this dry powder to kind of do M & A like we may have hinted, but just wanted to understand how should we look at any plans for debt reduction?

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Ruby Ritolia: So, at the current levels, we are very comfortable with all our currently covenants we have enough headroom. We are exploring looking at inorganic growth and that Raj talked about, and we will be looking at those. In terms of funding requirements for that, we will be funding it through both internal accruals as well as external debt funding.

Moderator: The next question is from the line of Ankit Pandya from InCred Asset Management. Please go ahead.

Ankit Pandya: Sir, I have a few ques

tions. So, starting with on the CAPEX front, so what will be the organic CAPEX for the next one to two years?

Ashutosh Kumar: As far as the maintenance CAPEX is concerned, we have always guided, and our maintenance CAPEX will be around 65 to 70 crores and 70 crores is the number which you saw for FY'24 as well. On the growth Capex, Largely any new projects which is coming up, we have been presenting it before you. So, other than what we have laid out in Ahmedabad and Whitefield in Bangalore, we also announced North Bangalore Center. North Bangalore, the total CAPEX is estimated is around 90 crores which largely would be spent in the current year and of course in the next financial year.

Ankit Pandya: You mentioned that in some of the centers you will be adding more capacity over there. So, in FY'25 to '26, how many Brownfield capacity expansion can we expect apart from Ahmedabad and Bangalore that has already been announced?

Raj Gore: I think other than Ahmedabad, I mean that's primarily the Brownfield capacity increase that will come this year. Rest will probably follow in the year after that majority, I mean.

Ashutosh Kumar: We are doing some beds and OT expansion in our existing centers. For example, our center in Vizag, we're doing expansion in Cuttack. So, there are beds or associated infrastructure addition which is happening in couple of centers and large one would probably happen in next financial year in Cuttack.

Ankit Pandya: So, that will be roughly how many beds that we will be increasing?

Ashutosh Kumar: Sorry, what is the question?

Ankit Pandya: How many beds capacity you will be increasing in the Whitefield?

Ashutosh Kumar: In Bangalore, we are adding about 125 beds.

Raj Gore: But that won't come this year, that will be next financial year. So, in between both projects, North Bangalore and Whitefield will be added in about 125 beds, which will get commissioned in the following financial year FY '26.

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Ankit Pandya: Sir, can you just talk about your inorganic CAPEX also, inorganic plans of acquisition that we will be doing? So, which region and how much you'll be willing to invest in inorganic opportunities?

Raj Gore : I think we are uniquely poised to be a consolidator in this space . There are some active comprehensive cancer care centers across the country , while from a management bandwidth, we would prefer them in existing markets or adjacent to our market s. We will be opportunistic if we get a good acquisition target in a newer state. So, with M&A , you have to be opportunistic. Our investment in these assets will be proportionate to the size of the asset. So, we will be able to tell you as and when we have something concrete. So , it's difficult to comment on the size of the acquisition.

Ashutosh Kumar : It will depend on obviously the visibility of the capital, and we are mindful of the debt level, which we can sustain, and we have our internal guidance on the debt levels , we will be within that.

Ankit Pandya: Sir, one more question on the tax rate. If I look at your cash tax, the tax that you pay on and it is visible on the cash flow statement . Last year on PBT of 45 crores, we paid 23 crores tax, that's 50% of P BT and this year on a PBT of 68 crores, we have paid 47 crores of cash , that's like 70% of our P BT. And then on the P &L, you are guiding for an effective tax rate of 35 % to 39%. I thought the corporate India tax rate has fallen to 25% a few years back. So, what is it that is keeping our cash tax and our P &L tax so high , I mean 70% tax rate on our cash tax basis seems unreal. So , can someone explain to us why our cash tax rate tends to be like 60 %, 70%?

Ruby Ritolia: So, we have various entities and under which our businesses are there . There are entities separately where we incurred losses. And since they ar

e separate entities, we don't get that

benefit at a consol level, and we don't recognize the effort that asset on that. And that is the reason why our effective tax rate is higher. You're absolutely right that we're moving towards 25% at the corporate level. However, there will be a journey . Once these entities becomes profitable , we will start seeing a reduction in our effective tax rate.

Ankit Pandya: So, on the entities which are profitable, are we paying exactly 25% tax?

Ruby Ritolia: Yes, that is right , we are paying 25% tax. So, we have about 47 crores of losses that is netting off our overall profitability.

Ankit Pandya: What I was asking was that if I take the 47 crores of tax paid in FY '24 and I multiply it by 4, that gives me almost 180 crores of profit before tax on our profitable entity. Would that be correct?

Management : So, we have for the year entities making 115 crores of profit , on which they have paid 24% , 25%, which comes to 27 crores profit, and we have 47 crores of loss-making entities where there are no tax credits taken. So, if we adjust for the 47 crores, we would be at 26 % , 27%. Right now, we are at 39% because we are not taking the tax rate of 47 crores. You can do the math .

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Ankit Pandya: I'm not following the math here. I'm sorry. I'll have to drill a bit more on this. So 67, 68 crores of consol profit, right, add that 47 crores of losses that we are going to be included in this profit , so that gives me 120 -odd crores of profit . Even on 120 -odd crores of profit -making entities , we are paying 48 crores of tax. So, that's 40% tax, is that correct?

Management : 27 crores of tax , you see the net tax. Please make it above the deferred tax also.

Ankit Pandya: Net of refund the tax is 47 crores on your consol cash flow. I'm talking about cash tax.

Ruby Ritolia: No, that includes the TDS also, that is not the tax charge , you should look at the P&L because the balance sheet includes two things. One is the cash tax as well as the T DS, which gets deducted from the payments that we collect from our credit payers. You should not include that.

Ankit Pandya: So, what is this TDS?

Management : TDS is tax deducted at source under Section 194J. Whenever we bill our credit payers while taking the payment, they will deduct tax at 10% before taking the payment or in some places we have got low TDS which will be anywhere between 3.5% to 4.5%. But still there is the tax deduction which normally will get 3 % in a cycle of two to three years. So, those are not the cash taxes , that is what impact we feel them that is only a cash flow issue, which is with the government, with the income tax department, which gets refunded after two to three years.

Ankit Pandya: But this has been going on for the last 2-3 years, right? So, this 22 crores tax that we paid last year -

Management : So, it will go on because we have credit business, which is of our total revenue. So, it will keep going on like that. So, once our PAT is 10% of the revenue, you will see netting off that part .

Right now we are at 2 % to 3%. So, we have some way to go there.

Ankit Pandya: Last question is on the multi -specialty hospitals that we have , Raj. This question is for you. On the three multi -specialty hospitals that we have , what's the strategy now going forward and what are the current sort of margins that those facilities are doing for us?

Raj Gore : So, Aditya, thank s for that question. We have been pretty clear on our communication on this front. We had four multi -specialty hospitals. Bhavnagar is now oncology , is a dominant specialty.

We added a linear accelerator a few years ago. We are looking at adding another linear accelerator there . So, oncology will continue to contribute more and more in Bhavnagar. Rajkot , again, last year we started surgical oncology and medi

cal oncology. Currently, we are in the process of building a bunker. So, on Rajkot, we will also have a comprehensive cancer care facility and oncology will become a dominant specialty going forward. In Suchirayu again we have a very strong market share there. It's a very, very good referral center for North Karnataka. We are currently building a bunker for linear accelerator as well as PET scan next to the Suchirayu Hospital. Eventually, we have an opportunity, we have another comprehensive cancer care center in Hubli. Eventually, when we need to add more capacity, we cannot add that capacity

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in our existing Hubli cancer center. However, this is a plan roadmap for going forward. Since we are adding a linear accelerator in PET Scan next to Suchirayu we have a growth headroom. Whenever we need additional capacity and then oncology will start becoming a dominant specialty within Suchirayu. The last multi-specialty hospital is in Ahmedabad, HMS, which will continue to be a multi-specialty hospital. There is no space there to add oncology in that hospital.

Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

B Pathiparampil: A couple of questions. Following up on the earlier question, in Ahmedabad where we are adding beds, what is the existing capacity utilization there or occupancy there and what's your expectation regarding the new 100 beds? So, is it going to get filled in six months?

Raj Gore: So, I wish I can fill 100 beds in six months in any market. But yes, look, Ahmedabad, again, it's a hospital that we started more than 12, 13 years ago. It had less than 100 beds. The capacity constraint more than beds, it was ICU beds and OTs, we have had five OTs. As you know, this is a surgery dominant cancer center. We have a very strong team of more than 20 surgical oncologists there with vertical specialization and a very high volume, strong robotic surgery center. Since it is a surgery dominant center, OT capacity and post-op ICU beds became a problem for us or capacity constraint for us and that's when we decided to invest in a new hospital, which is 200 beds. There we will be moving sometime next month; we are doubling the OT capacity from 5 to 10 OTs, and we have further headroom to add OTs whenever we need. Same for ICU. So, we are very excited. This is a center that is built to our specification, a very premium center, because we have all the clinical bandwidth and currently the number one market share, I think we are best poised to ramp it up quickly. How fast can we ramp it? Well, we are shooting as fast as we can, but as stated earlier, we have always said that we will grow at higher than market growth rate in every micro market. Here, we have an opportunity now to increase our market share. We are already #1 market share with 30%-plus market share. So, we have an opportunity. So, we will go all out and see. It's difficult to predict how fast we can fill that at this stage.

Ashutosh Kumar: To give some historical growth pace around that value input. So, in FY'23, Ahmedabad center by 20%, in FY'24 in spite of all the capacity challenges we grew by 14%. We do expect the growth momentum to continue, and this growth probably will get accelerated once we get into the new centers. We have had a capacity challenge. And you can take guidance from here of the growth rate and I mean make a sense of how faster we will fill those beds.

B Pathiparampil: Second, typically at least three years from plan to operationalizing a new facility or a new bed, so after this Bangalore 125 beds addition in FY'26, it looks like we don't have a plan to grow further our capacity. Do you have anything in mind, even in the early stages where you'll be doing something significant Brownfield or Greenfield for addition

n beyond FY'26?

Raj Gore: It's difficult to hear you. Is this a question about Bangalore?

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B Pathiparampil: No. Overall, your bed growth plan because we don't have a plan in place beyond FY'26, next year, we have 125 beds addition in Bangalore, beyond that we don't have a plan in place, so the question.

Raj Gore: No, we have a plan. We just spoke about it a little while ago. So, let me start from the current base. Today, we have 56% utilization on our capacity beds. So, we have about 44% beds unoccupied right now in our existing hospitals. So, that's number one, first headroom. Then in our strategic markets like Bangalore, we have added 20 beds last year. We have the ability to add, we have a plan to add another 25, 30 beds in our existing center in the next two years. We are also adding two more hospitals in Bangalore, one in Whitefield and one in North Bangalore with total 125 beds, that will get commissioned in FY'26. Now that's enough bed capacity for Bangalore to continue to grow for the next five years. I would like to point out that we have also have three daycare centers in this market which have done very well, and we will continue to add couple of more daycare centers in next 12, 18 months in Bangalore, so that we have a deeper penetration in greater Bangalore market and grab higher market share. The second key market is Ahmedabad. We just spoke about it where we are doubling our capacity. So, we have a

headroom and a runway to continue to grow for the next few years. Similarly, market -by-market, we have added 35 beds in Baroda , we are adding another 25 , 30 beds in Vizag next year , and then we have a plan to add another 25 , 30 beds in Vizag in two to three years , in Cuttack we will be adding about 60 to 70 beds. The remaining all our hospitals we have enough current capacity which is unutilized which will continue to fuel the growth for the next five years. We don't see any capacity constraint in our hospitals with this plan baked into our five-year plan. We don't see any capacity constraint for at least the next five years. And as we ramp up, we will continue to monitor utilization and figure out if we need to add subsequent capacity in any of these markets . So, to summarize today, we have enough capacity to grow , we are clearing the capacity bottleneck, and we will have a runway to grow for the next five years. And if we feel that in the next 2 -3 years we will continue to monitor it and continue to deploy more capacity in our established centers . These are the centers we have a dominant market share, and we will not let the capacity come between our growth in future.

Ashutosh : So, we do have visibility and within the next three years we will be deploying 500. We also have done an assessment of which are the centers which probably would go out of capacity after three years. That would also need an addition of about another 200 to 300 beds. So, in five years' time you have a visibility of adding about 800 to 900 beds.

B Pathiparampil: So, these are mostly Brownfield , you're saying?

Dr. B.S. Ajaikumar: I just want to add one thing here. See , in oncology, you should never focus only on beds. As we go forward , oncology is more outpatient, more time -related treatment and also we should look at the growth in oncology , not clearly just measured by the beds alone. While the beds will be needed but the proportionate growth of oncology will be more without even considering the beds, the way it has moved in the last few decades because our average length of stay, as you can see, has come down significantly, more targeted surgeries, more targeted treatment So, all

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of this will have a more foot fall is what we have to look at rather than the bed strength. But as we explained , bed strength

will increase, but one should not measure only by the bed strength.

Because for example if you have hub-and-spoke model , you are more clinics like what Raj mentioned, outpatient clinics and there will be no beds, but it will still infusion centers which are contributing to the revenue and the growth . So, it has to be looked at in a more analytical way in oncology rather than just the beds.

Raj Gore : And as you know, we have added a few linear accelerators in the last few years. Today , we have 36 linear accelerators . On that, our utilization is about 61 % , 62%. So, there also we have enough headroom to keep growing going forward.

Moderator: Ladies and gentlemen, as that was the last question, I would now like to hand the conference over to the management for closing comments. Over to you , sir.

Raj Gore : So, once again, thank you so much for your interest in HCG. As I mentioned in my opening remarks, we have had a fantastic performance in Q4 with about 12% year -on-year growth, 12% year-on-year growth on revenue, 21 % on EBITDA , our margins are hitting 19%. The team has worked very hard to add clinical bandwidth, clear capacity constraints, drive our go -to-market, we are very optimistic in driving our growth in the current quarter and in the current year .

Looking forward to see you next quarter on this call. Thank you.

Moderator: On behalf of Healthcare Global Enterprises Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines .

Call: Aug 2024

August 19, 2024

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on August 09, 2024

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligations and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on August 09, 2024, with Analysts/Investors to discuss the Unaudited Financial Results of the Company for the quarter ended June 30, 2024.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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“HealthCare Global Enterprises Limited Q1 FY25
Earnings Conference Call”

August 09, 2024

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges on 9th August, 2024 will prevail

MANAGEMENT : DR. B. S. AJAIKUMAR – EXECUTIVE CHAIRMAN ,
HEALTH CARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CEO, HEALTH CARE GLOBAL
ENTERPRISES LIMITED
MS. RUBY RITOLIA – CFO, HEALTH CARE GLOBAL
ENTERPRISES LIMITED
MR. ASHUTOSH KUMAR – VP, STRATEGIC PLANNING
& CORPORATE DEVELOPMENT , HEALTH CARE
GLOBAL ENTERPRISES LIMITED

HealthCare Global Enterprises Limited
August 09, 2024

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Moderator: Ladies and gentlemen, good day and welcome to the Q1 FY25 Earnings Conference Call of HealthCare Global Enterprises Limited.

This conference call may contain forward-looking statements about the Company which are based on the beliefs, opinions and expectations of the Company as on date of this call. The statements are not the guarantees of the future performance of the Company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” your touchtone phone.

I now hand the conference over to Dr. B. S. Ajaikumar – Executive Chairman of HealthCare Global Enterprises Limited. Thank you and over to you sir.

B. S. Ajaikumar: Good morning and a very warm welcome to everyone on the Q1 FY 25 Earning Conference Call for HealthCare Global Enterprise.

Today, I am very pleased that we are joined by Mr. Raj Gore – our CEO ; Ruby Ritolia – our CFO ; and our Senior Management Team , along with our Investor Relations Advisor, SGA. We have commenced FY 25 as planned marked by robust Operational and Financial Performance across our network. Our centers are performing as expected or even better, reflecting our commitment to medical excellence. I am confident that this momentum will sustain our growth trajectory in the years to come.

A key milestone during the quarter was expansion of our footprint through acquisition of Vizag - based Mahatma Gandhi Center Hospital & Research Institute. This center, under the leadership of Dr. Murali K.V., a distinguished oncology surgeon in Andhra Pradesh, has established a strong brand in the region. Vizag will undoubtedly play a key role in shaping our growth strides , particularly since we already have a good center running there. We consolidate our presence to become the single largest player in the region.

On the industry front, there have been a lot of discussions in the past and of course, we saw healthcare allocation to the budget, which was rather minimal, but there were some in terms of the drugs, there was some leeway given for the customs duty, which may be helpful for some of our cancer patients. But more importantly, there has been some circular, which I read yesterday on the government has done away with requirements for clinical investigation for new drugs. And by this regulatory approval in US, UK, Japan, Australia, Canada is all that is needed. So, this is a very important milestone because new drugs can channel to India very quickly. That is one of the things we were lacking in the past. So, with this, there is a clinical trial waiver. And HealthCare Global Enterprises Limited
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because of this, hopefully as oncologists for us, we will have access to new drugs immediately, which will certainly help to give better outcome to our patients.

In this regard, I would like to add that every three months there has been change in the how we manage cancer patients. New drugs, targeted drugs, antibody conjugate drugs, and so many developments are happening in personalized medicine. Certainly , we are at a crossroad to

become one of the major centers in the world to treat cancer patients. And HCG has taken a lot of initiative, as some of you are already aware, in research, in academics, conducting tumor boards across over 27 centers, and also I am happy to say we have truly become a destination. I am extremely proud of it, with nearly 400 oncologists, what we have. And we are also into innovation.

We are doing a lot of computational work with Accenture as our partner to identify what really the way to treat a spe

cific patient with a specific cancer and the idea is to treat the patient the first time the right way and we have truly excelled in it. So one of our data suggests we are equal or better than some of the best centers in the world. And I am immensely proud of HCG team. It is collective effort over every HCG-ian that is set to serve the larger cause of our ultimate stakeholder who are our patients with conviction and credence. In this, we have definitely called out at a set of destination of world class entity in our country. I would like to now express my gratitude to all of our stakeholders for their continued support in this wonderful journey. I would like to now hand over the call to Mr. Raj Gore, our CEO, who will provide insights into our strategies moving forward and share the operational performance of the quarter. Raj !
Raj Gore : Thank you, Dr. Aja i. A very warm welcome to all the participants on the call. We are very pleased to report another quarter of strong financial and operational performance.

Our revenue for Q1 FY25 is all time high at Rs. 526 crore, reflecting a growth of 17% adjusted for discontinued centers at MSR in Bangalore. HCG centers, excluding our fertility business, also recorded an all-time high revenue of Rs. 511 crores, reflecting 18% year-over-year growth adjusted for discontinued centers at MSR in Bangalore, resulting from strong volume growth and improved realization.

Our consistent effort on operational efficiency has resulted into reduction of ALOS from 2.13 to 1.98, thereby creating additional capacity and higher ARPOB, which has increased by 12% year-on-year.

Let me highlight some recent reclassifications of our established and emerging centers. We have now classified only three centers as emerging ; two in Mumbai - Borivali and Colaba, and one in Kolkata. The rest have been reclassified as established centers after a detailed assessment. We believe these reclassified centers are approaching established status. The centers in Mumbai and Kolkata are still evolving and we have implemented multiple initiatives to enhance their revenue growth and margin profiles. Notably, the Borivali and Kolkata centers have shown robust growth trajectories. This quarter, the Kolkata center has performed exceptionally well, having a revenue HealthCare Global Enterprises Limited

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growth of 73% year-on-year, turning from red to green in terms of profitability, resulting into EBITDA of positive 27 million compared to 25 million loss last year, same quarter, which translates to 14% EBITDA margin. We expect substantive operating leverage in these centers as the revenues grow, culminating into better EBITDA margin in next 12 months.

Another key highlight for this quarter is our proposed acquisition of an 85% stake in MG Hospital in Visakhapatnam at an enterprise value of Rs. 414 crores. Established in 2005, MG Hospital is the leading private comprehensive cancer care provider in Vizag holding approximately 30% market share. It boasts a robust oncology focused infrastructure with two LINAC machines, one PET/CT, one robotic surgery system, and a dedicated bone marrow transplant unit and 200 beds. Since its inception, MG Hospital has performed over 30,000 radiation therapy treatments, 150,000 chemotherapy sessions, 400 plus robotic surgeries, and more than 20,000 complex cancer surgeries. It has a strong financial track record with revenue growing at CAGR of 15% over the last 10 years. The hospital has consistently delivered an EBITDA of over 30% with an EBITDA margin of 35% for FY24. It is a net debt-free Company with a pre-tax ROCE of over 30% and approximately 70% cash flow conversion from EBITDA. Going forward, we leverage the HCG brand to enhance our revenue profile through improved procurement, operational efficiencies, and synergies in overhead expenses. We will also cross deploy learnings across our h

ospitals to further drive business efficiencies. We believe that the acquisition of MG Hospital will consolidate our market share in Vizag, positioning us as the undisputed leader in the region with a combined market share of 46%, promising significant synergy and growth potential ahead. We are extremely delighted and happy to launch our customer app. Some of the unique functionalities are first of its kind in the industry and goes in sync with our vision of adding life to years when it comes to cancer care.

In the 1st Quarter of rollout, we have been able to extend cancer care through our app to almost 8,000 unique patient IDs. In our journey to provide value, we have also moved ahead in e-pharmacy model for our patients as committed in the beginning of the year. We have been able to service 28,000 plus orders on e-pharmacy platform. These are early green shoots, and the

responses have been very encouraging.

Overall reach for our app downloads stands at 8000 plus at the end of Q1 FY25 and has exceeded our expectations. As part of our strategy, we have also invested in expanding our medical tourism business, focusing on attracting international patients. Our hospitals in major cities like Mumbai, Bangalore, Kolkata, and more recently, Ahmedabad, provide us with a locational advantage in attracting medical tourism patients. This segment is particularly valuable as it reflects on the confidence international patients have on HCG as cancer care destination. In Q1 FY25, our international business generated Rs. 20 crores highest for any quarter. This segment has been growing rapidly and we are optimistic about maintaining this momentum moving forward. Over the years, we have established strong local market leadership in each of our locations. This has been achieved through a steadfast commitment to excellence, continuous innovation, and deep understanding of the unique needs of the communities we serve.

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Lastly, I would like to highlight that we are witnessing higher revenue growth momentum in the current year compared to the last year reflected in HCG's revenue growth of 16.7% in Q1 FY25 compared to 13.7% overall growth in FY24, adjusted for our discontinued center at MSR in Bangalore. Initial month performance in Q2 is healthy too, and is expected to be better than our expectations.

With this, I hand over the call to Ruby to take you through operational and financial highlights.

Thank you.

Ruby Ritolia : Thank you, Raj. Good morning, everyone.

I am delighted to share our performance for the quarter which shows a strong 14% year-on-year growth, building our top line to Rs. 526 crore for the 1st Quarter of FY25. Our HCG centers, excluding Milan, have delivered an impressive 15% year-on-year revenue growth driven by consistent volume increase across all modalities. This solid performance is reflected in our key operating metrics, which have shown significant improvement across the board this quarter. OPD footfalls increased by 10%, complemented by a rise in patient inquiries and intake through our digital channels. We achieved a 17% year-on-year growth in chemotherapy sessions, a critical metric for our medical oncology offerings. Even with the addition of four new LINACs, compared to the previous period, we have successfully maintained LINAC capacity utilization at 65%. Our pay-per-use model continues to provide flexibility for quicker installation with minimal capital expenditure.

With higher volumes and a solid 61% utilization of operational beds, we are witnessing improvement in both volume and realization across all modalities. Revenue from Milan stood at Rs. 144 crores, a dip of 12% on account of discontinued operation in our Delhi center.

Regarding the performance of our established and emerging centers, they continue to deliver market-beating growth with established

shed centres growing by 14% and emerging centres by 33% year-on-year for the quarter. Our focused approach to enhancing the performance of our Kolkata Centre has yielded positive results. Over the last four quarters, we have seen increased revenue at our Kolkata Centre, with operating leverage playing a significant role. Kolkata grew by about 73% in revenue in Quarter 1 YoY. And based on this, achieved a double-digit EBITDA margin of 14% in quarter one of FY25. Our South Mumbai Center is also on track to break even this year.

Some of the other highlights during the quarter include an ARPOB of 44,340, reflecting a 12% increase year-on-year. Increased operational efficiencies, reducing the ALOS for patients to less than two days also enabled the growth in ARPOB. Our Adj. EBITDA for Q1 FY25 was Rs. 93 crores compared to Rs. 77 crores in Q1 of FY24, marking a 21% growth. Supported by robust growth, EBITDA is led by a high degree of operational leverage in our business. With rising revenues supported by a stupendous performance in the month of July, we anticipate that margin will grow at an even faster pace moving forward. While salary revisions and other ancillary costs

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typically impact the 1st Quarter, we expect higher operating leverage in the coming quarter on higher revenue growth as evidenced in the previous years. Our EBITDA for Emerging Centres has also now stabilized and is growing. We have maintained positive EBITDA for these centres for over four quarters and are optimistic about increasing margins on a quarter-on-quarter basis. PAT for the quarter was Rs. 12 crores, up from Rs. 7.6 crores a year ago, representing a 59% growth. With increased revenues leading to higher EBITDA impact, we believe we will continue to enhance our PAT performance. ETR for the quarter was 28.2% driven by reduced losses in subsidiaries, mainly turn around in Kolkata, Nagpur, and Vadodara. We expect ETR from here

onwards to be in the range of 30% to 32% for the rest of the year.

CAPEX for the quarter was Rs. 80 crores and we expect the CAPEX to be in the same range including investments in the new facilities like Whitefield, North Bangalore during the year. We have added 200 crores of ROU on account of the new facility in Ahmedabad which we operationalized in the last quarter and as well as the up-and-coming North Bangalore facility. Additionally, we announced MG Cancer Center and Research Hospital acquisition in Vizag earlier this quarter and we will be ready to consolidate post completion of condition precedents. Including the pro forma EBITDA of MG Hospital, we have successfully surpassed the Rs. 100 crore mark in EBITDA for Quarter 1 of FY25.

We have uploaded a detailed presentation for other operational and financial KPIs on the Stock Exchange and Company's website. And we may request you to visit those for further details.

With this, I would like to open the floor for questions and answers.

Moderator : Thank you very much. We will now begin the question and answer session. The first question is from the line of Gautam Rajesh from Everflow Capital. Please go ahead.

Gautam Rajesh: I have two questions. My first question was what sort of a margin profile do you expect in the existing business for the full year excluding the acquisition? Do you think we can still get a 20% EBITDA margin for the full year?

Ashutosh Kumar : So, the EBITDA margins, as Raj mentioned earlier on the call, that we have reclassified some of our relatively newer centers to emerging (rectified with correct data) buckets. They are still at a lower EBITDA margin. As revenues increase, we expect the EBITDA margin of these centers to increase, resulting into overall EBITDA margin expansion in our established bucket.

B. S. Ajaikumar : The question is, what he asked was, will you be close to 20% ?. Am I right? That was your

ur

question, no?

Gautam Rajesh : Yes, for existing businesses, excluding acquisitions, do you think we can still reach that 20% EBITDA for the full year?

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Ashutosh Kumar : We should expect similar range.

B. S. Ajaikumar: Between 19% and 20% is what we expect.

Gautam Rajesh: Thank you. My second question was what sort of growth do you see in the existing hospitals or established businesses for the full year perspective?

B. S. Ajaikumar: I think the growth trajectory will continue as it is which was established around 13 % to 15 %.

Ashutosh Kumar : Q1 revenue growth of established centre is 14 % we should be in similar range.

B. S. Ajaikumar: And it even increase , but it will be between 13% to 15% range.

Moderator : Thank you. The next question is from the line of Nitesh from Chrys Capital, please go ahead.

Nitesh : So, I have a couple of questions. So, you mentioned that Q2 started on a positive note. So, is it fair to assume that we can see a 15% YoY revenue growth in Q2 and possibility of Rs. 100 crore plus EBITDA ?

Ashutosh Kumar : So, the early indications that you see, I mean, historically, our quarter two performance over quarter one has been almost about 5% to 6% . And we are seeing similar or better growth what we have experienced in the historical period. And that should result into 14%-15% growth which we just mentioned in.

Nitish : Understood. And when should this reflect in our PAT growing to a meaningful level? Because right now we're on a reported PAT of Rs. 12 crores for this quarter. When should we just flow down to our PAT numbers?

Ruby Ritolia : Some of our subsidiaries which are loss -making are currently bringing down the PAT growth.

But now as they turn profitable, we are also able to take benefit of better ETR. So, if you see our current quarter ETR is about 28% , significantly down from the previous year and we expect to maintain ETR at these levels. But of course, it will not be an immediate bump, but it will slowly and steadily move in the right direction.

Moderator: Thank you. The next question is from the line of Hemant Agarwal from Leo Capital, please go ahead.

Hemant Agarwal: My first question is for Dr. Aja i. So, there has been news in the market of private equity promoter looking to exit, right? So, would the existing promoters and management also be looking to exit as a part of that transaction?

B. S. Ajaikumar: No, I really would not like to comment anything. As far as I have made public statements, at this point I don't have any intention of exiting. And regarding our CVC exit also, they'll be the ones

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to answer. But right now, our goal is to focus on the growth of HCG medical excellence. That's what we are doing. So, I really cannot comment on those things.

Hemanth Agarwal: Sir, my second question is, so has there been any increase in our competitive intensity for oncology hospitals in general, like multi-specialty hospitals seem to be in a very good up-cycle with lifetime high margins and our margins haven't even yet reached 20% mark. So, is there something from a market perspective which is pulling down the growth and margins of our Company?

B. S. Ajaikumar: I think when you look at us as a single-specialty hospital and with the way we have navigated over the years, I think we believe as a single-specialty, we have done very well. With our margins reaching close to 20%, we are progressing well. I really cannot comment on what are the margins of multi-specialty? Considering our new centers, and there are several of them, their margins are expanding very fast. As we know, our Mumbai Center and Calcutta Center are all doing good. And recent acquisition of the Vizag Center which is EBITDA margin accretive would add to the margin profile. ... But

I cannot really say whether oncology is better or multispecialty better. But I do believe our focus being only oncology and medical excellence, that is how HCG came and we are really on the path to creating this as a destination with good margins. And you will also see in the coming years, this performance will also translate into good return on capital employed as all the centres stabilize.

Raj Gore: This is Raj here. I just want to add what Dr. Aja i said. Look, I think with HCG's focus on cancer care, we have actually pioneered this field and showed the way, right? And the focus that other players have put in oncology is not something that is happening this year or last year. It's been happening over the last few years. If you look at our revenue CAGR for the last 4-5 years, it's one of the best in industry in spite of increased focus on oncology. Multiple markets, multiple of our locations, we have been able to grow our market share in spite of increased interest from other players. I think we have shown the way, the way we have pioneered in cancer care. We want to continue focus on giving the best possible care to our patients with the best possible outcomes. And we don't necessarily think or too much worry about competition, focusing on oncology. What has worked for us? We believe that we will continue to work for us with our focus on innovation and research, and we would stay focused on it.

Moderator: Thank you. The next question is from the line of Abdul kader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala: Could you throw some light on the lower ALOS what we had in this particular quarter? Any color as to what has driven this? And is that something what you're targeting less than two days going ahead as well?

B. S. Ajaikumar: No, I just want to say ALOS is a hall mark what we always focus how to reduce the ALOS and particularly with the number of days for surgery coming down, you know today even complicated surgeries, we discharge the patient in matter of one or two days and lot of them are HealthCare Global Enterprises Limited
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day surgery. And as we know the radiation oncology is all day care. And even chemotherapy now, admission for chemotherapy is done only for few diseases like treatment of colon cancer. But otherwise most of it is day care. So, all this trend moving towards limited admission required, our ALOS is coming down, which I think is very good. That is our target to be below two. And I think we have achieved it and will continue to pursue this matter the way we have done. That is a mark of excellence in my opinion. Raj, you want to add?

Raj Gore: No, I agree with Dr. Aja i. I think if you look at our ALOS trend over the last few quarters, we have continued reducing our ALOS. It's a function of our focus on minimally invasive surgeries, robotic surgeries, better efficiency to minimize our patients' stay in our hospitals. And I think we will continue to go in this direction going forward. It has created more capacity for us because of better efficiency and also has helped us to improve our ARPOB by about 12% YoY this quarter. So, I think we are moving in the right direction, and we will continue to move in that direction.

Abdulkader Puranwala: And the second question is on this Vizag acquisition. So, on consolidation, would you classify that as an established center?

B. S. Ajaikumar: Yes. It will be an established center, and also it will be positive for us all around.

Abdulkader Puranwala: Understood. And so the guidance what you provided earlier on this call of 19% to 20% EBITDA, does that factor Vizag? Or you were talking about the existing bids what is there in the network?

B. S. Ajaikumar: It will not factor Vizag.

Moderator: Thank you. The next question is from the line of Dhara Patwa from Smifs Limited, please go ahead.

Dhara Patwa: Sir, can you

throw some more light on this MG Hospital, like what could be the revenue which we would be expecting in the next three, four years from this unit, and what kind of ARPOB this unit has?

Ashutosh Kumar : So, the current level of revenue quarterly is close to about Rs. 30 crores plus. And that is resulting into 35% EBITDA margin. We are not looking at our MG Hospital in isolation. As you all know, we do have an existing center in Vizag. And the center is also operationally running very well for us. The center together with MG, this would constitute almost about 46% of market share. And we expect in that market to grow about 10 % to 12% going forward.

Dhara Patwa: Sure, so I understand the hospital has EBITDA margins of 35%, despite being in cancer care, which is a highly capital intensive industry. So, could you share something, like what are they doing differently to get these kinds of high margins? Is it the operating leverage which they are getting or maybe the low cost doctors, which is now helping them to keep the cost in check?

Any color on that?

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Ashutosh Kumar: Yes, so as you rightly said, the scale at which Mahatma Gandhi Hospital is operating, it is helping them to achieve 35% EBITDA margin. We do have some of our centers which is generating close to 30% margin, which is operating at similar scale to MG hospital in Vizag. They have higher operating leverage given the scale compared to HCG as a group which has mix of established and emerging centers, which is resulting into a high operating margin.

B. S. Ajaikumar : I think there being a single center, obviously there will be no corporate costs or other costs associated. And secondly, it is a surgeon, Dr. Murali, who is running it. And we believe that is like what Ashutosh said, some of our centers like Bangalore, our center of excellence also is close to 30% margin. So, it is in the range of what the well-established centers are and that is why we feel it's a good acquisition for us.

Dhara Patwa: One last question, will we be doing any additional CAPEX on the equipment side on this Vizag unit or will this start it as it is? Or do we need that more specialties or modalities should be added in this unit?

B. S. Ajaikumar : At this time, we are not thinking of adding any other exact CAPEX at this point. It is fully done. They have a robotic unit also. So, they have all the necessary equipment such as PET scan, linear accelerators, everything. We are actually adding a linear accelerator to our Vizag center. So, together we four linear accelerators, two PET scan and one robotic. So, it will be well capitalized unit combined together for next several years.

Moderator: Thank you. The next question is from the line of Nitish from Chrys Capital, please go ahead.

Nitish: My question is on our Ahmedabad Phase II. So, how is Phase II ramping up and have we commenced operation as the expected date of operation on slide 36 states Q1 FY 25?

Raj Gore: So, we are very happy that the project is almost complete. It's a beautiful infrastructure, very modern, very advanced. We have already moved our OPD services to that hospital towards the end of 1st Quarter. We have moved out daycare services. Very soon, we will be moving our inpatient services. So, we are well on track with our revenue growth in Gujarat market.

Nitish: Okay, thank you. Another question is, our finance costs have gone up in Q1, around Rs. 8 crores year-on-year and Rs. 7 crores Q4 to Q1. What's the reason for this increase in finance costs and are there any one-offs over here?

Ruby Ritolia: Yes, sure. So, the finance cost, if you see, 1) there is an ROU that got added in the current quarter as well as last quarter. This includes the ROU that we created in Ahmedabad, in North Bangalore. Then we also added previously in Kolkata when we renewed our agreements so

there

is an impact of that. Plus, in the current quarter also, we have funded about Rs. 50 crores more into CAPEX. Of Rs. 80 crores of CAPEX, in the current quarter, Rs. 50 crores we funded through debt. There is an increased finance cost on that account which is coming in. Secondly, there is a positive impact in the previous quarter when we did a Kenya shilling reinstatement we had to

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reverse. There is a Rs. 2.5 crores of benefit which is lying in the previous quarter which is quarter four of FY24.

Moderator: Thank you. The next question is from the line of Viraj Shah from Shah Investment, please go ahead.

Viraj Shah: So, how are we seeing our international business as we have seen a good growth this quarter? Can you throw some light over there? Any outlook, front or something like that?

Raj Gore: Yes, so as we said, this has been our highest ever quarter. We at a current run rate, we are almost 2x of what our pre-COVID international business was. As you know, as earlier stated, we have four cities that we focus on as destinations for inbound international patients. Bangalore, which has always been with lion's share; Mumbai 2 hospitals, beautiful advanced infra; Kolkata with close proximity to Bangladesh; Myanmar; Nepal; and Ahmedabad with some deeper connections in Eastern Africa. So, with our continuous efforts over the last two to three years to

develop more markets, more countries in SAARC , in Middle East, in Africa, doing more activities, taking our doctors there for more frequent visits, doing CMEs, outreach efforts, having more institutional tie-ups in these markets with insurance companies, institutional tie-ups.

Viraj Shah: Yes, there was one more question. So, any outlook on further inorganic opportunities in medium term?

Raj Gore: Are you asking for international?

Viraj Shah: No, inorganic opportunities.

Raj Gore: You are talking about inorganic.

Viraj Shah: Yes, inorganic opportunities.

Ashutosh Kumar: We are evaluating inorganic opportunities. We continue to do so. However, there is nothing which is very mature at this point of time.

B. S. Ajaikumar: At this point , we have done acquisition of Vizag and Indore, we are trying to consolidate. Of course, we will be looking at some inorganic, if strategically feel we need to do. As and when it happens, of course, we will inform the markets.

Moderator: Thank you. The next question is from the line of Jeet Shah from Equity Analyst. Please go ahead.

Jeet Shah: I kind of missed the comments about CAPEX in the initial remarks. Can you just give me a guidance for the next two years?

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Ruby Ritolia: For the current quarter, we did a CAPEX of about Rs. 80 crores. And for this particular year, we will maintain a similar range in the current year for the rest of the quarters also. And this is on the back of some of our Brownfield facilities and the new facilities which are coming up.

Moderator: Thank you. The next question is from the line of Dhruv Shah from Dalal & Broacha. Please go ahead.

Dhruv Shah: Sir, just one question on your debt reduction plan. So, from when can we expect your debt to start going down?

B. S. Ajaikumar: Yes, as you know, our debt once the acquisition of Vizag is completed , will go up initially by Rs. 200 crores and later on there is another Rs. 150 crores after 18 months to complete the 85% acquisition. Internally we have decided that to be at the right time we will be seeking the board . The board has been informed and at the right time we will be seeking approval for primary which will bring down the debt significantly. And we will of course convey at the right time what will be the size of the primary. Our intention is to certainly keep the debt to equity ratio at a certain range so that

it is within what internally we have accepted. So, Ashutosh, you want to comment on that anything?

Ashutosh Kumar: So, As Dr Ajai said, internally we have decided to keep our debt to EBITDA level to around 2.5X to 2.75X on a pre-IndAs basis. . And the same was communicated in the market early on. Considering these leverage levels, we will be raising primary, to bring debt levels post vizag hospital to desired levels as Dr. Ajai suggested.

Moderator: Thank you. That was the last question for the day. I now hand the conference over to the management for closing comments. Over to you, sir.

Raj Gore : So, thank you once again for joining us and thank you for your interest in HCG. As mentioned earlier, we have had a fantastic performance in the 1st Quarter of this year. We have accelerated our growth, topline and bottom line compared to last year. We have seen good momentum in the first month of second quarter and we are very confident that we will continue in this growth trajectory going forward for the remaining year. We request you to keep in touch and join us during the next quarter. Thank you so much and have a good day.

Moderator: Thank you. On behalf of HealthCare Global Enterprises Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Nov 2024

November 19, 2024

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on November 11, 2024

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on November 11, 2024, with Analysts/Investors to discuss the Unaudited Financial Results of the Company for the quarter and six months ended September 30, 2024.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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Q2 & H1 FY '25 Earnings Conference Call"
November 11, 2024

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges on 11th November, 2024 will prevail

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER –
MR. ASHUTOSH KUMAR – VP – STRATEGIC PLANNING
& CORPORATE DEVELOPMENT
SGA, INVESTOR RELATIONS ADVISOR

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Moderator: Ladies and gentlemen, welcome to the Q2 and H1 FY '25 Earnings Conference Call of Healthcare Global Enterprises Limited. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company and it can involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference call is being recorded.

Now I hand over the conference to Dr. B. S. Ajai Kumar, Executive Chairman of Healthcare Global Enterprises Limited. Thank you, and over to you, sir.

B.S. Ajai Kumar : Good morning and welcome to our Q2 earnings conference call for HealthCare Global Enterprise Limited, today, I am joined by Mr. Raj Gore, CEO; Ms. Ruby Ritolia, CFO; and senior management team, along with SGA, our Investor Relations Advisor.

I am proud to announce a robust financial and operational performance for Q2 FY '25, a testament to the dedication and expertise of our exceptional capabilities, team of doctors and support staff. For over 2 decades, HCG has served as a beacon of hope for millions, establishing itself as one of India's most trusted healthcare brands. It is because of our team's commitment that today, we manage one of the highest volumes of complex cancer cases across diverse geographies, consistently delivering excellence even in the most challenging cases. This strong performance for the quarter not only reflects our operational capabilities, but also the resilience and compassion that define HCG.

At HCG, we recognize that cancer is a complex and multifaceted disease with over 600 types identified, and each requiring a unique focused approach to diagnose and treat. Our models are based upon personalized patient-centric care, ensuring that each individual patient receives precise treatment tailored to their specific cancer type and stage.

This involves leveraging specialized teams that include organ and modality-specific oncologists who collaborate to deliver comprehensive multimodal care. We are also, in this regard, becoming leaders in research and academics. And our R&D department is one of the most growth-oriented departments where we are seeing a number of people being published. In fact, recently at the conference in International Conference in Chicago, ours was the only Indian presentation at the podium presentation, and we are very proud of it.

Our commitment extends beyond treatment with an end-to-end approach that includes lifelong, after-care support, recognizing that cancer management is a journey that requires continuous vigilance and care. HCG's integration of advanced technologies, genomics, artificial intelligence reflects our dedication to staying at the forefront of cancer research, ensuring that our patients benefit from the latest innovations in cancer care.

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In this regard, I'm again proud to announce that we have signed agreements with TRUCAN in regard to working with them on fundamental biological research and also with Accenture where we are going to be doing computational work with them, which will help in identifying patients who require precise treatments. And based on that, we can bucket to see which patients require the right treatment at what time.

Lastly, our hub and spoke model plays a crucial role in extending specialized and advanced care across regions with

central hubs providing expertise and support a network of satellite centers. This structure ensures that even patients in remote areas have access to top-tier cancer treatments, contributing significantly to our operational success and most important, is bringing out better outcomes for these patients. As you know, in the past, the outcome has been a big answer, and we are very proud to say, today, HCG's data clearly says in some of our outcomes, are equal or better than the global standards.

In closing, our strong performance this quarter reflects HCG's commitment to excellence in cancer care. With a dedicated team, advanced technology and a focus on patient-centric treatment, we are well positioned to continue delivering high-quality care to patients across diverse regions. We remain committed to expanding our reach, enhancing our services and upholding the standards that have made HCG one of the most trusted names in healthcare. We look forward to building on this success for the years ahead. I would now like to hand over the call to Mr. Raj Gore, our CEO, who will provide insights into our strategies moving forward and share the operational performance of the quarter. Raj?

Raj Gore: Thank you, Dr. Ajai. A very warm welcome to all the participants on the call. Today, I'm proud to announce that our performance for Q2 and H1 FY '25 has reached new heights, marking the best quarterly results in HCG's history and outperforming the industry growth.

We have achieved revenue of INR553.5 crores for the quarter, reflecting a robust growth of 14% compared to the same period last year. Likewise, our revenues for H1 FY '25 have also increased by 14%, standing at INR1,079 crores compared to H1 FY '24.

Our adjusted EBITDA crossed INR100 crores first time, reaching INR104.2 crores, witnessing a strong growth of 21% for the quarter on a year-on-year basis, and margins have shown significant improvement, reaching 18.8% in Q2 FY '25, compared to 17.8% in Q2 FY '24.

Pro forma revenue growth, including Vizag acquisition, stands at 20%, with EBITDA margin of 19%. This increase reflects higher revenue and cost efficiency with operating leverage positively impacting our performance. Additionally, we have focused on digital initiatives which have enhanced our operational efficiency and streamlined processes, contributing further to our margin growth.

This growth is a result of our collective efforts, driven by focused commitment to delivering clinical outcomes that match the best institutions globally right here across India and not just metro, but also non-metro locations.

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The Oncology business has experienced impressive 20% growth after adjusting for the exited center in MSR, Bangalore last year. Revenue growth has been strong across all markets where HCG operates, reducing concentration risk and reinforcing our market leadership in 16 out of 18 locations.

The domestic business has performed strongly, more than offsetting a 17% decline in international revenue due to geopolitical challenges in a few markets, particularly in Bangladesh. We expect the situation to start normalizing by Q4 of the current fiscal year.

Looking ahead, we have been working on several key strategies to accelerate and sustain growth of HCG. After several years of steady organic growth, HCG is now focusing on accelerating its expansion through strategic acquisitions. In the past year, the company has completed 2 acquisitions, one in Indore and the other in Vizag, reinforcing our earlier stated position of adding 200 to 300 bps of growth through acquisitions.

Alongside our growth initiatives through acquisition, we are prioritizing brownfield expansion across our network with significant focus on our key markets in Bangalore, Ahmedabad and Cuttack. We successfully operationalized our new state-of-the-art 200-bed

ded comprehensive

cancer care center in Ahmedabad in second quarter. The transition has been smooth with minimum impact on our business.

We are currently developing 2 state-of-the-art hospitals with a total of 125 beds in North Bangalore and Whitefield area in Bangalore. These facilities are expected to be fully operational by early FY '26. We plan to add 60 beds in Cuttack with the potential to expand to 100 beds in future.

Construction is set to begin in the current financial year with operations expected to start by early FY '27. We aim to expand in our existing markets by operationalizing over 900 additional beds across the network within the next 3 years, including this year's additions. Of these, 350 beds are fully invested, but not yet operational.

Now let me share an exciting update on our digital transformation initiative that we embarked about 2 years ago. To recap, there were 2 primary objectives behind it. First, with around 20 lakh reported new cancer patients diagnosed every year among the population of 142 crores, ours is a thinly dense widely fragmented target market. Taking advantage of increased Internet usage and evolving consumer behaviour, we want to make it easier for cancer patients across all

India to find HCG care by creating online awareness and omnichannel access to quality care. This is a huge opportunity for HCG to move from being a network of current 18 physical micro markets to pan -India cancer network with a combination of physical and digital presence. Second, we don't treat cancer, we treat patients with cancer. Cancer patients' journey starts before HCG hospital and continues after HCG for rest of their lifetime. We want to stitch together an omnichannel, seamless journey and a relationship with our patients with the help of digital technology. The objective is to build a long -term relationship with our patients to be a trusted adviser over a lifetime and not just care provider at one point of time.

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We are pleased to share that our digital initiatives have significantly boosted our patient funnel, raising digital channel revenue to 14% of our overall revenue in quarter 2, up from 4% in Q2 of FY '23. By strengthening our patient acquisition capabilities through HCG -owned digital assets like websites, social media handles and online campaigns, we have been able to grow digital revenue 4.6x in metro and 6.3x in non -metro locations during this 2 -year period. Our aim is to achieve 25% of revenue through digital platforms over the next 3 to 5 years.

Lastly, on our imaging centers, we've been focused on boosting revenue from these centers. I'm pleased to share that our center in Kolkata has shown particularly strong performance of 66% growth in revenue. This growth has not only increased our margins, but also contributed to an overall revenue boost, helping us absorb fixed costs and drive operating leverage, which has positively impacted our margins.

Our South Mumbai center has faced challenges due to the decline in international business due to geopolitical situation in key markets, though domestic business continues to grow. To offset this, the company is expanding into new domestic catchments. While international business is expected to recover by Q4 of fiscal year, we remain confident in the center's potential for a turnaround.

Recent acquisition of MG Hospital in Vizag has been progressing in time with our plan, and we are rightly positioned to consolidate our presence in the region. As we look to the future, we remain focused on innovation, patient centered care and expanding our reach to bring hope and healing to more cancer patients. Together, we are not just building a stronger organization, but a brighter future for all those we serve. Thank you for your continued trust and support on this journey.

With this, I hand it over to

Ms. Ruby, our CFO, for financial highlights.

Ruby Ritolia: Thank you, Raj, and good morning, everyone. As Raj already enumerated, I am pleased to report that our performance this quarter demonstrates strong momentum with a robust 14% year -on-year growth, driving our top line to INR554 crores for the quarter ended -- for the second quarter ended FY '25.

Excluding Milann, our HCG centers delivered an impressive 15% Y -o-Y revenue growth, fueled by consistent volumes increase across all modalities. Specifically HCG oncology centers, on a like-for-like basis, adjusting for a closure of M S Ramaiah Hospital, achieved an outstanding 20% growth year -on-year.

Our key operating metrics have also shown significant improvement this quarter. OPD footfalls, which are a vital indicator of volume growth and accounting for 18% of our revenue, increased by 9%. In Medical Oncology, we saw a 14% increase in chemotherapy sessions, reflecting the continued expansion of our patient base. Capacity utilization for our LINAC machine reached 70%, up from 65% in the previous quarter, 3 machines currently being under replacement.

Through our innovative pay -per-use model, we were well positioned to expand LINAC installation in new locations in the near future.

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Additionally, our patient bed occupancy rate remaining strong at 61%, underscoring our commitment to optimize facility utilization and improving patient access to care.

Turning to our core operations, both our established and emerging centers have delivered strong performance this quarter. Our established centers continues to demonstrate steady growth with a 13% year -on-year revenue growth alongside a robust 20% growth in EBITDA. At the same time, our emerging centers have exhibited exceptional momentum, achieving a 32% increase in revenue for the quarter with EBITDA expanding fivefold.

Notably, our Kolkata center has posted an impressive 66% revenue growth, while our Mumbai centers have collectively grown by 15%. We are also on track for our South Mumbai centers to

breakeven this year. These markets are pivotal for us as they are located in Tier 1 cities with large catchment areas and are key hubs for international patients.

The outstanding growth in our emerging centers is a result of multiple initiatives implemented across locations, which is further bolstered by operating leverage that is steadily enhancing our financial profile. We are confident in our ability to maintain this upward trajectory as our strategic initiative coupled with enhanced operational efficiencies, continuing to drive sustained growth and strengthen our overall financial performance.

Coming to ARPOB. Total ARPOB grew by 7.4%, standing at INR45,188. Established centers witnessed an ARPOB growth of 7%, standing at INR43,394. And ARPOB for emerging centers witnessed a strong growth of 10%, standing at INR72,652. Capex spends for the quarter was INR52 crores, which includes the CyberKnife replacement in Bangalore. We expect the capex to be around INR250 crores to INR300 crores in FY '25.

Other income for the quarter includes onetime insurance claim and EPCG settlement to the tune of INR6 crores. Our ETR for the quarter is at 25%, and this is on account of profit accruing in subsidiaries, thereby minimizing the tax credit losses that we have.

This is the performance summary that we had to share. If you have any further questions, please let us know.

Moderator: Thank you very much. Our first question is from the line of Sagar Tanna from Alchemie Ventures. Please go ahead.

Sagar Tanna: Congratulations on a good set of numbers. I think you've been delivering what you've been talking since the last few quarters. I have 2 questions. One is, what was our loss in Bombay Hospital in the first half?

Raj Gore: Loss in terms of EBITDA?

Sagar Tanna: Yes.

Ashutosh Kumar :

The Bombay Hospital, both put together posted a profit of INR 10 million in quarter 2 and INR 25 M illion for first half

Sagar Tanna: Which is South Bombay and Borivali, right?

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Raj Gore: Yes.

Sagar Tanna: No. Just the South Bombay hospital, what would be the total loss for the first half?

Ashutosh Kumar : The South Bombay's first half would be close to about INR4.3 crores,

Sagar Tanna: INR4 crores odd, right? Okay. Second thing you've also mentioned there is some slump sale or divestment of the business to our wholly -owned subsidiary. Can we explain this transaction and how it impacts and what is the rationale behind this?

Ruby Ritolia: This transaction that we have mentioned as a resolution to the Board meeting has been on account of us looking at our Triesta and Cyclotron business, moving out of the HCG fold into a separate entity from where we can look at growing it beyond the HCG networks also. So this is more on account of business realignment where onco comes under one company and the expandable business, which we can look at expanding beyond our own network, which is the lab and the Cyclotron business can move to a separate entity . It also allows greater freedom with a separate entity structure.

Sagar Tanna: So what all scope of activities will we undertake here?

Ruby Ritolia: So largely, it will be the lab business and the Cyclotron. Lab business, which is under Triesta.

Sagar Tanna: Got it. So it will be more like a stand -alone radiology, pathology lab kind of a business? Is my understanding correct?

B.S. Ajaikumar : No. I think the main reason is, as we know, diagnostics has become very important in any field, particularly oncology, with re -expanding into genomics and personalized treatment. What Ruby was trying to convey is, there is a scope for expansion beyond HCG. So it requires a focused approach, and that is why we decided to take this action. So it could grow as Triesta brand name. And similarly, the Cyclotron, which is a production of FDG, it's a production unit, which can produce FDG for non -HCG also. So these 2 areas have been brought together. Their growth has been good and they can continue to grow beyond HCG growth. That is the intention we have to see that it can grow. .

Sagar Tanna: So currently, how many centers do we have?

B.S. Ajaikumar : You're talking about HCG centers?

Sagar Tanna: No, these Triesta centers?

B.S. Ajaikumar : All of the Triesta centers are there wherever the hospital are, like 24 centers, we have Triesta centers all over. They're all attached to hospital. Now we are trying to see how we can bring in work from outside our HCG hospitals. That is the intention.

Sagar Tanna: Got it. So it will not be like a stand -alone, and it will continue to be a part of a hospital network or part of the hospital infrastructure itself? Is my understanding...

B.S. Ajaikumar : At this time, that is the goal.

Moderator: The next question is from the line of Ankush Mahajan from Axis Securities.

Ankush Mahajan: So sir, this is once again that in the existing center, EBITDA is on sequential basis, is on the lower side. So the question that still arises that on existing centers, when would we will build a double -digit ROCE? I really appreciate, sir, if you throw more light on this side because continuously, we are focusing on this part, still existing centers are not making ROCE, and the EBITDA is continuously decreasing. So would you throw some more light on it, sir, that if you compare the ARPOB of the Max and Fortis, they are quite on the higher side. We are still at a - gap of ARPOB. That is a major reason. We want just your view on it, sir, that ROCE on the existing centers.

Ashutosh Kumar : Ankush, this is a question on existing centers or em

erging centers? Emerging centers, right?

Ankush Mahajan: Emerging centers, emerging centers.

Ankush Mahajan : No, no. Existing center. Existing centers are already... I am talking about existing centers, sir. This is more than 2 years now.

Ashutosh Kumar: Look, just a clarification, sorry for repeating it. Is this question on emerging centers or existing centers?

Ankush Mahajan: Emerging. Emerging centers, sir.

Ashutosh Kumar: Yes. So there are 2 things here we need to look at it. One is, we have changed the definition of our emerging centers. Now only 3 centers are part of our emerging center, which is Mumbai, 2 centers and Kolkata, 1 center. We have been reporting strong performance on Kolkata from last 3, 4 quarters.

As you see an impact at the current year, we are expecting to add almost about more than INR10 crores of EBITDA in the current center. Last year, Kolkata Center delivered a negative growth of almost about 20%. This year, we are on an H1 annualized basis, it's a positive ROCE already.

This ROCE in Kolkata Center, is expected to be in double digits or in high teens. So this is showing the path -- Kolkata is showing the path in terms of where the ROCE could be.

There has been a dramatic change in terms of return profile of that center. So this is one market within the emerging center. So we are very hopeful with the progress the way we are making in Kolkata.

The second one is, in Mumbai centers, we are progressing well as far as our Borivali center is concerned. We are generating decent ROCE there in single digits. We have our own challenge as far as South Mumbai is concerned. So in our entire portfolio, be it emerging or existing, we have 1 center which we need to turn around, which is South Mumbai. That center is also heavily invested with capex in terms of technology.

There are 2 radiation machines, together, they account for almost about INR30 crores, INR35 crores. So overall, we had made an investment of INR100 crores in South Mumbai. So this is Healthcare Global Enterprises Limited

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one center -specific issue, and we are very hopeful that as the Kolkata and Borivali center grows, our ROCE should pan out. We have our task in hand as far as South Mumbai is concerned, which we expect to turn around within a couple of quarters.

Raj Gore : If I can address your ARPOB.

Ankush Mahajan: Yes.

B.S. Ajaikumar : Yes, go ahead please .

Ankush Mahajan: Sir, my question was, if we compare with the industry leaders, like Max and fortis, they have a higher ARPOB. So can we reach to these levels? What are the aspirations?

Raj Gore: Yes. So as you know, historically, we've had a larger presence in Tier 2, Tier 3 markets. The centers that we have opened in the last 5 years are largely in Mumbai, Kolkata and better and bigger quality of markets like Nagpur and Baroda. We have moved from about INR35,000, INR36,000 ARPOB 2 years ago to INR45,000 ARPOB in the previous quarter. That's a blended ARPOB for the whole network. But if you look at our ARPOB in, let's say, Bangalore or Ahmedabad, it is in the mid 80s. If you look at our AR POB in Mumbai, Kolkata, it's in higher 60s or lower 70s. If you look at our ARPOB in Nagpur, Baroda, which are growing very well, they are in 50s.

As these centers continue to grow and contribute higher percentage revenue share to the total revenue, our ARPOB will continue to improve in the north direction. So once again, we've made a very good progress on ARPOB in last 2 years. Our big market ARPOB is actually comparable

or better than many other players. And as we grow, our ARPOB will start moving in the north direction.

Moderator: Our next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Raj, you mentioned in your opening comments around your digital initiatives. I mean 2, 3 things. One is, a, can you just help us understand, the strategic importance

of increasing digital sales in

the business? And I mean, what kind of impact does it have on the profitability? I mean, is it a more profitable stream of business than the nondigital part of the business? And does it really contribute much to the metro part of the business or the non-metro part, if you can just probably throw some more light on these aspects.

Raj Gore: So thank you for that question because this initiative is really important strategically for us, this initiative. As I mentioned in my commentary, we know from our experience that India as a cancer market is growing at a very high rate. The availability of comprehensive cancer care is skewed towards metro cities, and it is not available if you go even 50, 100 kilometers away from metros.

Now we know that if there is any specialty where patients don't mind traveling large distance, it's cancer. Historically, we've seen, the public hospitals or trust hospitals, which were cancer-focused, like Adyar Cancer Center, Kidwai cancer center, Rajiv Gandhi, Tata, have drawn patients from all across India.

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Now we see that HCG, as the dominant player in cancer, we should be able to tap into the entire Indian market. And the best way to reach out to these cancer patients and create awareness and make sure that we convert them into need is through their mobile phones or through their laptops. So therefore, I think what we are doing is currently, we are sum of our 18 micro markets, physical micro market with our digital initiative, with online presence and awareness, we have our ability to have a pan-India virtual and a physical market. We know that a consumer behavior is also evolving at a very rapid rate, where -- forget cancer, even to go to a restaurant to buy any product, to experience any service, we reach out to Internet and we do our background work.

When it comes to life and death in cancer, patients are definitely going to exercise that option. That gives us an opportunity to get into their consideration set because after all, we are the largest player. And with online presence, it's easy to get into their consideration set. Once we get into their consideration set, what we have is the omnichannel platform, websites, social media campaigns, social media handles, call center.

With a newly implemented CRM center and a digitized call center, we have the ability to convert every query, every lead in few clicks to our OPD consultation chamber of our oncologists. And then we have a longitudinal visibility from an initial interest leading to OPD consultation to diagnostic, to treatment, to post successful treatment, follow up in 3 months, 6 months, for rest of their lifetime through our CRM capability.

So in many ways, it's not necessarily a digital transformation, it's actually a transformation of our relationship with patients from an episodic relationship to a lifelong, trusted adviser relationship, that gives us ability to be there for cancer patients and meet all their needs before hospitalization, post hospitalization, and give us the ability to nudge them to come back after post successful treatment for follow up, so we can ensure that cancer hasn't come back. And unfortunately, if it comes back, we have the ability to detect in time, which is a key to delivering better outcomes and bring them back for treatment again.

So it's a very strategically important initiative. As I mentioned, we usually make a mistake of associating Internet usage to metro cities. What we have seen is some of the markets, we have very good response in nonmetro markets. Our metro markets have grown 4x. Our nonmetro markets have grown more than 6x.

Now what we are doing is adding vernacular capability in our call centers, website, social media, YouTube channels, which, I feel, will give us even a deeper penetration in Tier 2, Tier 3 ma

rkets

where there is a dearth of cancer treatment options and therefore, gives us the ability to bring them to our physical locations.

So that's our overall strategy for digital. We have grown to about 14% in just 2 years from a 3%, 4% of our total revenue coming from digitally influenced footfalls. We are very confident, in the next 3 years, we can take it to 25%. Today, digital revenue is our second largest hospital if I look at that. And we feel, going forward, that will continue to be our largest hospital in terms of patient footfalls.

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Ashutosh Kumar: Just to add to Raj, your other question was on profitability from patients sold through digital means. The primary difference, as far as the profitability is concerned between digital and nondigital sourced patient lies in the cost of acquisition of the patient.

So as you would know that we have 2 sources of the patient, one is B2B. B2B is primary to our network particular in doctors, where our costs somewhere lies close to about 6% to 7%. In comparison to that, the digitally -driven patients, the cost of acquisition of those patients are in the range of 1.5%.

Within digital channel, I mean, there are 2 primary sources. One is organically sourced patients and second is digital campaign -led patients. Our cost of acquisition of patients through digital campaign -led is a little higher compared to our organically sourced patients.

What we have seen over the last 3 to 4 quarters is that the source -- I mean the patient volumes for our organic channels are increasing compared to a campaign -led channel. So as we move forward, this 1%, 1.5% also is expected to go down in future. So we're very open in terms of -- since it is a very highly profitable channel of our business, as it grows, that should also add to our overall margin.

Raj Gore: Thank you, Ashu. And there is one more point. If you look at industry after industry, the disruption has happened into digital technologies in customer acquisition front. We've seen e-commerce platforms doing backward integration and acquiring brick -and-mortar businesses. This is our way to go forward. We have the strongest network of brick -and-mortar hospitals. This is our way to move forward and create a virtual channel and own the relationship with cancer patients directly without any middle person. Whether anyone refers them or not to us, we want to have that ability to create awareness and get into their consideration and own the relationship directly with our cancer patients throughout their lifetime.

Moderator: Our next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: I joined a little late, so if it's been asked, just pardon me. So just on the MG Hospital acquisition, they have about 30% share. I think you also have a reasonable amount of share. So I just want to understand from a micro market of Vizag perspective. And when I look at some of the growth estimate, I don't know why it's fiscal '24 E for -- in your Slide 29, but growth has been 5%, 6% only for this particular hospital.

So how should we look at growth for this, once we get it acquired into our system? Do you think there are scope for improving the growth? and the margins, when I look at it, I know you've talked about it in the past as well, but they're sliding down. So anything that you think we need to do differently for this acquisition?

Raj Gore: Yes. So Shyam, thank you for that question. This is a new question this time. However, we addressed some of your concerns in our previous calls on MG acquisition, but I will again explain. We have had a presence in Vizag from 2017, '18. We have a 17% market share. #1 player has been MG with more than 30% market share. We felt that Vizag is a top 10 GDP growing city market in India.

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So it's an important location for a state of Andhra Pradesh with port, airport connectivity and focus on industrial growth. We felt that we had a perfect opportunity to consolidate our market share in this city and state where we already have a larger presence.

Now coming to your question of MG growth, what we have observed is there are 2, 3 levers that we have available. One, we have identified the way to add about 25 beds in these centers, that will ensure growth.

We will -- on the other hand, in our own center, add about 30 to 35 beds this year and another 30 to 35 beds in the coming 2 years. So between these 2 setups, we will be able to add about 60 to almost 90 beds in next 2 to 3 years. That will ensure that both businesses and our market share in Vizag continues to grow.

We have identified synergy levers on the revenue side. Both hospitals have independent, deeper penetration in terms of your go -to-market capability, referral network. Both hospitals have very strong capabilities on clinical side, but there are complementary capabilities that we can spread over both centers rather than just one center to drive better utilization of our medical equipment and clinical program and get better productivity on our sales and marketing machinery. I think as a combination of that, we are confident that we'll continue to grow both businesses and combine the business in this market going forward.

Shyam Srinivasan: Sir, when you look at debottlenecking, like you said, when you are trying to add these, what is our current AOR? Or I know it's less relevant for oncology, but I just want to understand, is that

-- what is the current AOR at MG?

Raj Gore: Yes. So I think MG would be about 70% -75%. And that is probably the reason why it's been stagnating or slowing down in last previous years, and that's what we are debottlenecking going forward.

Shyam Srinivasan: Yes. Just the second question on the overall company. If you could exclude MG, I just want to understand how should we look at like overall growth for the remainder of the second half top line? And what could happen to margins? If you could give us some directional sense, please.

Raj Gore: Yes, as I said, we have -- in H1, grown at a 14%. Our oncology business has grown at 20%. If we adjust the exit from MSR Bangalore Hospital, which was there last year. So I think we are outpacing the industry growth. We'll continue to move -- maintain that momentum in Q3 and Q4. We are very confident.

As you know, --in both quarters, Q1 and Q2, we've had about 100, 110 bps margin expansion. We will continue to move forward on our margin expansion journey. And before you joined, we shared that we feel that we will -- today, if you look at a pro forma basis, we are 19% EBI TDA without MG, we are about 18.8% adjusted EBITDA. We will continue to go forward on our margin expansion and will inch towards 20% by Q4.

Moderator: The next question is from the line of Yash Darak from RSPN Ventures.

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Yash Darak: With respect to other income, INR5 crores of other income came to be a one -off. So are we expecting the other income to run down a bit during the year? Or is it going in the same range?

Ruby Ritolia: This onetime is only for this quarter. We reported a PAT of about INR18.2 crores, INR18.3 crores, and which, if we were to adjust for these onetime, would go to about INR15.3 crores, INR15.4 crores. However, as we know that in quarter 3 and quarter 4, we get a better operating leverage and margin expansion happens. That would also flow to the PAT. So this onetime expenses were one time. These will not continue.

Yash Darak: Okay. And can you give an ETR guidance for the full year? Are we expecting to pay more tax in Q3 and Q4?

Ruby Ritolia: So ETR, we will be around in the range of about 28% or so. We should be b

elow that. So we'll

continue with a better ETR percentage. For the first time, we hit 25% this quarter, which was on account of largely our subsidiaries turning into profit and thus we, not losing our tax credits over there. We hopefully want to continue for the rest of the year also, it should not go beyond 28%.

Yash Darak: So in the Ahmedabad facility, is it completely operationalized? Or are you expecting it to ramp up slowly?

Raj Gore: No. So did I hear the name correctly, Ahmedabad?

Yash Darak: Yes, yes.

Raj Gore: Yes. So just to recap, what we've done in Ahmedabad is we had about 100 -bedded facility, -- region where we have a dominant market share. While we continue to grow, the capacity was a bottleneck. In the last quarter, we moved to a newly built to our spec modern premium facility, which is 200 bedded. We have moved from our old facility to the new facility. That transition happened in Q2. with this additional capacity we will continue to ramp that facility going forward, which will give us an ability to grow at a higher rate going forward.

Yash Darak: Just a last question. The total capex for the year is around INR300 crores. By now, how much have we already incurred? And the average cost of debt.

Ruby Ritolia: In quarter 1, we incurred about INR80 crores, and current quarter, we incurred about INR50 crores. So about INR130 crores, we have already incurred.

Yash Darak: And the average cost per bed ?

Raj Gore: the capex is an amalgamation of your maintenance capex as well as growth capex . So it's difficult to give you an average per bed because it's a mix of both.

Ashutosh Kumar: And. Just to add for the newly -- new beds which we are adding, we have separately disclosed in our presentation for Ahmedabad and North Bangalore.

Moderator: Our next question is from the line of Priyank Parekh from Abakkus Asset Managers LLP.

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Priyank Parekh: Just wanted a clarification on the number of beds that we would be having third quarter onwards. So currently, we have 1,809 beds. So next quarter, we will be operationalizing, I mean, consolidating Vizag 100 beds. And I think 100 beds from Ahmedabad. So is it fair to understand that we'll be having close to 2,000 beds in the next quarter onwards? Operational beds?

Ashutosh Kumar: So as far as the patient beds are concerned, we have included in quarter 2, of the 200 beds, we

have operationalized about 118 beds in Ahmedabad. We may be adding may be 15, 20 more beds in terms of operationalizing them. But more than that, we are not operationalizing. So as the volume grows, we will continue to operationalize more beds. In the next 2 quarters, we may be adding only about 20, 25 more beds and addition of MG Vizag beds.

Priyank Parekh: Okay. So 1,900, 30, 50 kind of beds.

Ashutosh Kumar: Yes.

Priyank Parekh: Okay. Understood. And, sir, on pre -Ind AS EBITDA level, what should be the right figure to date for FY '25?

Ashutosh Kumar: Is that pre -Ind AS question? What was that, sorry?

Priyank Parekh: Yes, pre -Ind AS...

Ashutosh Kumar: You're not audible clearly.

Priyank Parekh: Sir, now am I audible?

Ashutosh Kumar: Yes.

Priyank Parekh: Yes. Sir, you say that we'll be inching towards 20% post -Ind AS EBITDA for this year. What should be the pre -Ind AS number corresponding to that?

Ashutosh Kumar: We have a rental cost of about INR25 -26 crores per quarter to be adjusted.

Moderator: Our next question is from the line of Dhruv Shah from Dalal & Broacha.

Dhruv Shah: Sir, my first question is with your focused strategy on expanding through inorganic acquisition of brownfield. So approximately, what kind of size of these acquisition are you looking at, say, in terms of value and, say, in terms of bed capacity? And what is the payback period that you're expecting from it?

Ashutosh Kumar: So typically, the

stand -alone cancer centers which becomes our targets are in the range of 80 to 100 beds. And the acquisition price is in the range of 10x to 12x of the EBITDA of the operating hospitals. And our payback period for these acquired hospitals is in the range of 6 to 8 years, depending on whether it is our strategic value or financial value.

Dhruv Shah: And are you looking at new geographies? Or are you looking to expand in the current geographies only?

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B.S. Ajaikumar : I think I think our expansion policy, as what we have taken as a call, is to do in the current geography as much as possible. We are, as you know, in the Bangalore , we are in Gujarat as well as in Maharashtra area as well as in and around Kolkata. So initially, we want to expand existing center around that areas. We also have a plan of developing oncology outpatient clinics. That is our goal right now to expand in existing areas, more or less. Ashutosh, do you want to add anything?

Ashutosh Kumar: No, no.

Moderator: Dhruv, does that answer your question?

Dhruv Shah: Yes, yes, yes. Just one more question. So currently, close to 2,000 beds we have. So by FY '26 and FY '27, what kind of bed capacity should we look at, including both your...

B.S. Ajaikumar : I just want to say, I know before Raj will also say a few things on this. As we know, oncology is more and more becoming outpatient. Always, it has been and it is trending towards more. Our average length of stay actually is below 2 days. So when we look at it, our focus will be more on footfalls, outpatient care on that. So unlike a regular multi -specialty hospital, once you look at the oncology center, as total in terms of what is the exact number of footfall of patients. And that is what we'll be focusing on. But naturally, the bed strength will increase, but more focus is on the footfall.

Ashutosh Kumar: just to add to what Dr. Ajai said that it is very evident from the average length of stay within our hospitals going down, that is becoming more and more outpatient. And on the expansion beds also, we have a decent number of day care beds and not all inpatient beds.

Just to reiterate what Raj said earlier in his speech, is that in the next 3 years, we are going to operationalize incremental 900 beds, including the beds which we have added in Ahmedabad. Of the 900 beds, we have already invested in 350 beds. The incremental 550 beds is what we'll be investing in the next 3 years.

Moderator: Our next question is from the line of Nitish Rege from ChrysCapital.

Nitish Rege : So as you mentioned earlier on the call that we've moved locations in Ahmedabad. So -- and this will involve shifting of large equipment. So can you quantify how much of this onetime expense have you incurred for this?

Ashutosh Kumar: So yes, we have done this. And I think broadly, we have been very meticulous in terms of transitioning our equipment. I think the one -time cost is in the range of INR1 -INR1.5 crores of primarily shifting the medical equipment, the large -- the bigger equipment, LINAC and PET/CT. This is a range of onetime cost which we have incurred.

Nitish Rege : Okay. And also, is there any impact of severe rains on LINAC being down during the quarter?

B.S. Ajaikumar : Actually, the LINAC is not going to be that much down because one of the LINACs is functioning in the existing center, which is outside the purview of where we are exiting. That's
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a separate center. Bunker is there. And so that will absorb most of the patients -- have already absorbed. So there has been very little downtime in this.

Ashutosh Kumar: Only around 20, 25 patients, we have been treating less. We transferred all the patients to other machine tomotherapy. But given that capacity limitation of tomotherapy, we could

take 20, 25

less per month. Some impact was there.

B.S. Ajaikumar : Actually, we put them on wait list and observed all of them.

Raj Gore: That's right. Going back to your question, there was some rain disturbances in Gujarat region during last quarter. And when it comes to LINAC, we have upgraded our LINAC in Bangalore, one LINAC and one CyberKnife during the last quarter. They are fully operational by end of quarter. So now we have upgraded CyberKnife and LINAC in our Bangalore hospitals.

Nitish Rege : Okay. Sorry, could you quantify the impact, please?

Ashutosh Kumar: So in terms of revenue, I think broadly about INR 2.5 crores revenue impact would be there in the quarter

And the EBITDA impact, I mean, radiation typically is our high margin business. The EBITDA impact would be in the range of about 70% -- 70%, 75% after revenue impact.

Moderator: Our next question is from the line of Nancy Yadav from Allegro Capital Advisors.

Nancy Yadav: I wanted to understand the capex place for the previous half year that has gone by. If you could tell me the total amount has been used for what all?

Ashutosh Kumar: As Ruby mentioned, around INR54 crores of capex has been spent in quarter 2. Of that, INR18 crores of capex has gone towards the growth capex . And we made a large replacement in our Bangalore center, as Raj mentioned. We replaced our CyberKnife at Bangalore center. That cost us about INR25 crores. So INR25 crores plus INR18 crores is the major spend, which is INR43 crores, the remaining are all maintenance capex .

Moderator: Thank you. Ladies and gentlemen, in the interest of time, that was our last question for today. I would now like to hand the conference over to the management for closing comments.

Raj Gore: So once again, thank you for joining us and showing your interest and encouragement to HCG. We are really happy to announce the results. We're very confident that we're going to maintain our momentum going forward and continue to grow the business profitably going forward.

Thank you once again, and wish you good luck.

Moderator: Thank you. On behalf of HealthCare Global Enterprises Limited, that concludes this conference. We thank you for joining us, and you may now disconnect your lines.

Call: Mar 2025

March 06, 2025

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on February 27, 2025

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on February 27, 2025, with Analysts/Investors to discuss the Unaudited Financial Results of the Company for the quarter and nine months ended December 31, 2024.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

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Q3 & 9 Months FY '25 Earnings Conference Call"
February 27, 2025

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges on February 27, 2025, will prevail

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. ASHUTOSH KUMAR — VP – STRATEGIC PLANNING
& CORPORATE DEVELOPMENT – HEALTHCARE
GLOBAL ENTERPRISES LIMITED
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to the Q3 and 9 Months FY '25 Earnings Conference Call of Health care Global Enterprises Limited. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions, and expectations of the company as on date of this call. These statements do not guarantee the future performance of the company and may involve risks and uncertainties that are difficult to predict. As a reminder, all participant line will be in listen-only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star the n zero on your touchtone phone. Please note that this conference is being recorded.

Now I hand over the conference to Dr. B. S. Ajaikumar, Promoter of Health care Global Enterprises Limited. Thank you, and over to you, sir.

B. S. Ajaikumar: Thank you very much, and good morning to everyone. As we all know, HCG has been in the news recently and most of the news have been in the public in the last week. I would just like to reiterate as we know that there has been a change where CVC has sold its majority of the stake to KKR. And at this point, we welcome KKR as our partner. Since most of the news is out in the public, and we don't have any more to add to that, I will not discuss further about this matter. And coming to the -- as we know, with KKR coming in, and we are certainly looking at a long-term horizon for how HCG can grow. As we're all well aware, oncology is growing rapidly in India. And particularly for a group like us who are dedicated to oncology with comprehensive cancer centers and setting up scopes and infusion centers.

It is a good growth opportunity strategically for us as we move forward. With already having close to 25 centers, opportunity for us to grow are significant to provide the right care to the patient at the right place and at the right time.

As I always say, cancer is a disease, which is, again, a disease which needs very personalized focused therapy. And in this regard, I always believe first time right treatment is critical. Because of that, precision oncology has taken hold today. And what we look at is how can we treat the patient the right first time to get the best benefit.

And in this regard, we like to minimize recurrence of cancer as we move forward. Enormous information is coming out in oncology care today. In the past, while we had randomized trials, we used to treat patients, say, 50% will respond or 50% will not respond, 50% used to get unnecessary treatment.

But today, with what we are able to deliver in precision oncology, with era of genomics, era of proteomics, and all of this has only expanded the role of comprehensive cancer centers to see the right treatment for the patient is done. And this has really come in the way of how HCG can deliver precision medicine. And because of this, HCG has integrated the part of research and academic into it.

Today, I believe no oncology center can really do the right thing for the patient unless they are involved in research, data collection, and academics. And HCG is in the forefront of that.

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Technology, artificial intelligence, how we work on computational work, which we have partnered with Accenture will go a long, long way in developing this precision medicine to our own Indian patients.

Another angle I want to bring to you, most of you are well aware, Indian genomics is different from Caucasians. So, a lot of these trials which have been done in the past, treatment be given may be related to Caucasian. In this regard also, we have become forefront of analyzing

genomics. Over 2,000 patients we have

done, identified where the issues are and how we can do right treatment the first time.

So in this, I would like to say with new partners coming in, taking on long-term horizon, we will certainly focus apart from what areas are there for mergers, acquisition, through growth of HCG centers itself, which you will hear in the future, we will also be focusing significantly on academics and as well as research and precision medicine.

So I hope in my view, HCG has a great opportunity for this kind of growth, and we will continue to pursue this with over 400 oncologists being with us as well as a significant research division within us.

With these few words, I would now like to hand over to Mr. Raj Gore to talk about further on the operational and other issues. Thank you very much.

Raj Gore: Thank you, Dr. Ajai. A very warm welcome to all the participants on our Q3 FY '25 earnings conference call. We are proud to announce that we have once again been able to clock highest ever quarterly revenue of INR559 crores in the last quarter, indicating a strong growth of 19% on Y-o-Y basis.

Despite being a seasonally weak quarter due to festivities and holidays, we have been able to increase our volumes across modalities, indicating a growth in our market share across regions. Adjusted EBITDA margin stood at 16.5% for Q3 FY '25, with adjusted EBITDA of INR92.3 crores, a growth of 15% on a year-on-year basis and a PAT growth of 23%.

The oncology business post MG Hospital Vizag acquisition grew by 24%. Our emerging centers continued to perform well in Q3 with 25% year-on-year growth. Kolkata centers grew by 40% and South Mumbai center grew by 28%. South Mumbai center witnessed strong performance despite challenges in international business due to geopolitical issues, which we expect to recover by the upcoming quarter and will be key for the center's turnaround.

During the quarter, we consolidated operations of MG Hospital in Vizag. This acquisition has been instrumental in enhancing our footprint in the region, allowing us to further expand our services and strengthen our presence in one of the key markets for cancer care. We are confident of the robust growth in these centers with improving performance on the back of strong brand creation, quality clinical talent, and increased awareness programs for cancer care and early diagnosis.

At HCG, we believe that world-class cancer care should be patient-centric, accessible, and sustainable. Our asset-light model enables us to expand efficiently, ensuring that the cutting - Healthcare Global Enterprises Limited

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edge treatment reaches more people without compromising quality. By integrating advanced technology, precision medicine, and compassionate care, we are not just treating cancer, we are redefining the patient experience. As we continue this journey, our focus remains on empowering patients with best possible outcomes while driving innovation and transforming cancer care across India.

Thank you for your continued trust and support in this journey. With this, I hand it over to Ms. Ruby, our CFO, for financial highlights.

Ruby Ritolia: Thank you, Raj, and a very good morning to everyone. As Raj highlighted, I'm pleased to share that our performance this quarter continues to reflect strong business momentum and sustained growth. We have achieved an impressive 19% year-on-year revenue growth, bringing our top line revenue to INR559 crores for the quarter. Furthermore, for the 9 months ended December 31, 2024, we delivered a solid 16% growth with total revenue reaching INR1,638 crores.

A key highlight of our performance has been the exceptional growth across our core HCG centers, excluding Milann, which recorded a remarkable 21% increase in revenue and 16% EBITDA growth year-on-year, which is 20% EBITDA margin. Highlighting our oncology centers, that is excluding Milann, multi

specialty, and the new acquisition of MG Vizag, we recorded 18% increase in revenue and 14% EBITDA year-on-year with 21% EBITDA margin. These numbers reaffirm our ability to consistently outperform industry growth, driven by our commitment to clinical excellence, operational efficiencies, and an unwavering focus on patient care. We are confident in our ability to deliver stronger growth for the quarter 4 of FY '25 with significant growth anticipated in both revenue as well as EBITDA.

One of the primary drivers of our sustained success has been the strong performance across multiple modalities, supported by increasing patient volumes and improved capacity utilization. Our investment in medical infrastructure, specialized treatments, strong clinical talent, and patient care continues to drive positive outcomes.

Additionally, during the quarter, we successfully consolidated revenue from Mahatma Gandhi

Memorial Hospital, which contributed INR25 crores, with an impressive margin of 24%. This integration further strengthens our portfolio and aligns with our long-term growth strategy, enabling us to scale operations while maintaining strong financial discipline.

I would also like to highlight that the restructuring of our diagnostic business under the brand name Triesta has been successfully completed. This will help foster large scale and wider reach to noncaptive businesses as well. Our key operating metrics have demonstrated significant improvement this quarter.

OPD footfall, which is a crucial indicator of our conversion funnel, increased by 9%, highlighting rising patient trust and engagement. In medical oncology, we recorded a 19% growth in chemotherapy sessions, underscoring the continued expansion of our patient base. Capacity utilization of our LINAC machine reached 60%. And despite the addition of 7 new LINAC machines in last 12 months, our ability to maintain this utilization rate showcases the Healthcare Global Enterprises Limited
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efficiency and robustness of our operations. Additionally, our patient bed occupancy rate improved from 52% in quarter 3 of FY '24 to 55% in quarter 3 of FY '25, reinforcing our commitment to optimize facility utilization and enhancing patient access to world-class oncology care.

Turning to our core operations. Both our established and emerging centers have demonstrated strong performance this quarter. Our established centers continue to show steady growth, recording a 20% year-on-year increase in revenue along with a 14% growth in EBITDA. At the same time, our emerging centers have delivered exceptional results, achieving 25% revenue growth and an impressive 65% increase in EBITDA for the quarter.

Notably, our Kolkata center led the way with an outstanding 40% revenue growth and EBITDA growth by 42x, while our South Mumbai center posted strong 28% increase and our Borivali center registered 11% growth. These markets play a pivotal role in our expansion strategy as they are located in Tier 1 cities with large catchment areas and serve as key hubs for international patients. Their continued growth reaffirms our ability to cater to increasing demand while delivering advanced oncology care to a broader patient base.

Coming to ARPOB, total ARPOB grew by 3.5%, standing at INR44,284. Established centers witnessed an ARPOB growth of 2.8%, standing at INR42,798, whereas emerging centers witnessed a strong growth of 12.3%, standing at INR66,000. Our capital expenditure for the year is estimated to be INR275 crores, of which INR172 crores has already been deployed. Our effective tax rate for 9 months ended 31st December 2024 is 3% on account of deferred tax recognition in our subsidiary to the extent of INR12 crores.

This is the performance summary we had to share. Now I would like to invite the participants for any questions on the performance.

Moderator: The first question is from the

line of Aditya Khemka from InCred PMS.

Aditya Khemka: Congratulations to the team on the deal. So, a couple of questions on the contours after the deal.

So, CVC, we understand will continue to hold 9% stake and Dr. Ajai obviously retains a stake of 10.8 -odd percent in the company. So, could you just help us understand if KKR makes you open offer of 26% post buying 51% in the company, and in terms of the total holding between CVC, KKR, and Dr. Ajai, will probably potentially go up to 95%, 96%, right?

Because it's 10 plus 9% plus potential 77% with KKR, including the open offer price. So how should we read that? I understand that Dr. Ajai and CVC are now classified as public shareholders. Does that also mean that in terms of operations of the business..

B. S. Ajaikumar: No, no, I...

Aditya Khemka: Sorry, sir. Go ahead, please.

B. S. Ajaikumar: I just want to correct you.

Aditya Khemka: Yes, sir.

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B. S. Ajaikumar: The CVC will be classified as public shareholders.

Aditya Khemka: Okay.

B. S. Ajaikumar: I will be still a Promoter. So, the way it works is that as part of the transaction, KKR will acquire up to 54% of equity in HCG from CVC. Upon completion of the transaction and open offer, KKR is expected to hold an equity stake of between 54 -77%, and this is based on what happens in the open offer. I will be also classified as co-promoter. So we will look at it at that situation.

In order to clarify, CVC will be only at 9.9% or even lesser depending on what happens with the

open offer, and they may come down depending on the open offer, which we cannot really calculate now, as it all depends on how many people participate in the open offer. So I will leave it up to that. But essentially, together with KKR, we will be holding close to maximum promoter holding, it can come up to 77%. Then we have to decide what to do, okay?

Aditya Khemka: And sir, in terms of your role now at the organization, the press release mentions that you are no longer in executive capacity. So does that mean you'd be more like a financial investor with the company with limited routine operations, or how does that work?

B. S. Ajaikumar: Yes. Let me clarify that because having started being a founder of HCG and started and being in operations, Executive Chairman, I felt at this point, my interest has always been in research, patient clinical excellence and academics. So I will be Chairman of the Board, and with clear involvement in clinical excellence, research, and academics.

I will also be a co-promoter with them. And, of course, as promoters, we will be meeting and have a strategy for the growth we discuss with KKR as we move forward on a monthly basis. So we will have certain arrangements. But my role, certainly, I will not be -- as of now also, my role in operations is very limited. So certainly, I will be limited or not in operations, sir. As we move forward, I will not be in operations -- in the daily operations, okay?

But my interest, as I said, is in the research, academics, and I will be spending more time on that, taking the genomics, proteomics, all these things, data collection. Data is very important in oncology, as we know. How are we doing, again, in terms of outcomes. So I will focus on, publications and outcome.

And that is what I trained in MD Anderson several years ago. Outcome of cancer patient is the most important. If you know our outcome, how we deal with advanced and recurrent tumors, that itself is what makes it a center of excellence. So I'll be focusing on creating the center of excellences so that oncology patients feel comfortable in getting treatment in HCG throughout our HCG network.

Moderator: The next question is from the line of Gautam Rajesh from Leo Capital.

Gautam Rajesh : I had two questions. How do you see the margin trajectory shaping up over the next year? That would be my first question.

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Ashutosh Kumar: This is Ashutosh from HCG. So, while we address this margin for the future years, I would just like to take this opportunity to also talk about the margins the way it has panned out in quarter 3 and how do we look at it in the upcoming quarters and then next year. Just to bring back the attention, our EBITDA margin in quarter 2 was 18.5%, which came down close to 16% in Q3. And that has been primarily as we all know that quarter 3, because of festivities, we undergo little bit of seasonality and the revenue drops, which impacts operating leverage negatively. And as we move on to Q4 and our revenue scales up and our modality mix normalizes, we expect the margin to expand upwards of what we have reported in quarter 2.

We should be improving our EBITDA margin from these levels on our existing portfolio. And as we have announced that we would be operationalizing 2 new brownfield centers in Bangalore, those are expected to generate some operating losses. However, it should get traded off largely by the EBITDA margin expansion happening in the current portfolio. So the margin from current levels, we should see expanding in the next financial year, close to about 1% to 1.5% compared to FY25

Gautam Rajesh: It will expand up to 1% to 1.5% is what you're saying?

Ashutosh Kumar: Yes, including the losses what we would be having from the new centers.

Gautam Rajesh: Understood. And my last question was, what rate are we expecting on same-store asset growing? And what rate do you expect these to sustain?

Ashutosh Kumar : Sorry, could you come up with your question again, please? It was little muffled.

Gautam Rajesh: Okay. What rates are existing same-store assets growing at? And what rate do we expect that to sustain?

Ashutosh Kumar : Our established centers have been growing upwards of 12%, 13%. And as we all know, the market growth rate had been close to about 11%. We have guided the Street that we would be growing faster than the market. -- our established centers should also grow faster than what the market growth rate is As we have made significant investment in our existing assets to ramp up the growth.

And today, we are not constrained by capacity at any locations other than 1 or 2. we are making investments to ensure that there is no bottleneck in terms of capacity. So I think we should continue to grow upwards of 13%, 14% in our established centers.

Gautam Rajesh: Can you repeat that, it got muffled, continue to grow at...?

B. S. Ajaikumar: 13%, 14% going forward.

Moderator: The next question is from the line of Sagar Tanna from Alchemie Ventures.

Sagar Tanna : If you can just mention how the Bombay centers are progressing both Borivali and the South Bombay center in terms of the EBITDA performance? And what was it last year? And what do
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we expect in the coming year? You mentioned 100, 150 bps EBITDA expansion. Would this also come from the turnaround in the Bombay centers, if you can elaborate on that?

Ashutosh Kumar: That's absolutely right. There will be a significant contribution coming in from our emerging centers. So just to remind you, we have classified 3 of our centers as emerging centers, one in Kolkata and 2 in Mumbai. Kolkata, we had announced that we had already broken even in quarter 1. We continue to take it positively from there.

As far as our Mumbai centers are concerned, Borivali is continuing to do good. What we have been trying to focus on is primarily on South Mumbai. As far as South Mumbai profitability is concerned, we did not see reduction of profitability of South Mumbai center in quarter 3, primarily because we had a subdued international business for the reasons Raj explained in his opening speech.

ech.

However, we do expect the reduction of losses in South Mumbai by at least INR1 crores, and by quarter 1 of next financial year, we should be seeing the center breaking even and expand from there. So obviously, the emerging centers, which is growing today at the rate of almost about 25%, we expect similar growth to continue and should result into reduction of losses and have an upward trajectory in terms of EBITDA margin.

Sagar Tanna : So sir, for the South Bombay center for the full year basis, FY '25, what kind of EBITDA loss are we estimating?

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Ashutosh Kumar: South Mumbai it will be in the range of almost about INR10 crores, INR10 crores to INR12 crores.

Sagar Tanna : Which was approximately INR35 crores loss last year, if I remember correctly. Is my memory right?

Ashutosh Kumar: No, it was not as high as INR35 crores. It was in the similar range

Sagar Tanna : Got it. And you expect that to break even next financial year?

B. S. Ajaikumar: It is coming down and yes.

Moderator: Sir, does that answer your question?

Sagar Tanna : Yes.

Moderator: Okay.

Sagar Tanna : Thank you.

Moderator: Thank you. The next question is from the line of Yash Sinha from MIPL.

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Yash Sinha : Hi. First, congratulations on the acquisition and also for the great performance this quarter. I broadly wanted to understand two things. First, from all the guidance that you've been giving over the last 3 or 4 quarters, does anything specific change with the new acquisition? And second, I wanted some color on your capex guidance for the next 2 to 3 years. How is it looking like every year? And what are the specific areas that you're focusing on?

B. S. Ajaikumar: Can you clarify what are you talking about in terms of new acquisition?

Yash Sinha: With the KKR acquisition coming in, will there be any material change in your business plans than what you've been guiding over the last, say, 3 or 4 quarters or so?

B. S. Ajaikumar: I think, at this point, we have not had any detailed discussion with KKR. Only thing I can say is, since they are coming in as a new investor now, we will certainly be looking at a long-term plan, which we will lay out as we move forward and develop the partnership with them. And so, at that point, we can certainly elaborate when we have concrete plans.

But as of now, our priority is to stay on course to grow the existing centers and also to see that whatever recent acquisition we have done will meet our expectation and continue to focus on operational excellence, clinical excellence, and improve the margin as what Raj, Ruby, and Ashutosh have said, okay?

Yash Sinha: Got it.

B. S. Ajaikumar: And there was a second question.

Yash Sinha: Second one was about, I just wanted some guidance on your annual capex for the next 2 to 3 years and specific areas where this capex will be focused?

Ashutosh Kumar: So, we had guided on the capex front earlier on, and we are executing our plan accordingly. Primarily, in the current year, FY '25, and next financial year, FY '26, is where we are expecting

lumpy capex to happen, which would be in the range of almost about INR275 crores to INR280 crores per annum. Of that, close to about INR100 crores per annum would be about our maintenance capex.

whatever growth plans we have laid down as on today, including the new centers, which would be operationalized in FY '26 and the expansion which has happened in FY '25, we should be more or less done with our growth capex plan, which exists today, by FY '26. Post that, it should normalize to around about INR100 crores of maintenance capex thereafter, with some inflation in that.

Moderator: The next question is from the line of Ravi Shah from VRS Capital.

Ravi Shah : Sir, I have two questions. Sir, what would be our outlook on the international patient business in South Mumbai center?

And are we seeing any recovery on that front?

Raj Gore: Yes. So Raj here. What we have seen is in terms of international business, we started the year very well. Our Q1 was one of the highest. Q2 onwards, with Bangladesh unrest, we saw impact.

Q3 was the lowest, as Indian government restricted the medical visas provided to Bangladesh Healthcare Global Enterprises Limited

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patients coming to India. So Q3 was our lowest. We expect Q4 to go back to Q2 level, and then going forward, in the coming year, we feel that, that should get normalized.

B. S. Ajaikumar: And Raj, is that a right statement -- I just want to add to that, that most of the hospital industry are experiencing the same issue. Is that a right statement?

Raj Gore: Yes. As you know, Bangladesh is the majority and the largest contributor to medical tourism business to Indian hospitals. This is an industry phenomenon across the country. And we're hoping that as political relations get normalized, the normalcy of flow of Bangladeshi patients to India will resume, and we will be back on track.

Ravi Shah : Sir, my second question would be also on the international side. So what is the overall contribution of international patients in terms of revenue on a company level? And what would be our outlook going forward for the same?

Raj Gore: Yes. So it is about 3.5% to 4%, and we expect it to maintain that going forward. As you know, we are having very aggressive domestic growth agenda. So, we're expecting that to continue to be 3.5%, 4% going forward.

Ashutosh Kumar: Just to add to what Raj said. This year we probably may be subdued because of these reasons. However, we expect it to get back to 3.5%

Moderator: The next question is from the line of Devang Patel from Sameeksha Capital.

Devang Patel: Sir, my question was on the margins for established centers where we have seen a 20% revenue growth and 14% EBITDA growth as per the PPT, implying there is on a Y-o-Y basis, lower EBITDA margins. This is despite MG Hospitals getting added with higher 24%, 25% margins, as you mentioned earlier. So, what is the reason that margins on a Y-o-Y basis are lower?

Ashutosh Kumar : So, we just explained. While we explained in the context of overall margin dilution. However, that holds true for our established centers as well. sequentially, we see revenue going down in quarter 3, giving us a disadvantage in terms of not enjoying operating leverage.

That prevented operating leverage in quarter 3 resulting in lower EBITDA, including MGM.

This is industry phenomenon and MGM is included in that. However, as we said that as we come in quarter 4, we have seen things getting normalized with our revenues also being higher compared to quarter 2 the way we are seeing it. We should see our margin expansion happening. So this was just a quarter 3 phenomenon, and we should be out of it in quarter 4.

Devang Patel: Sir, but q-o-q margin is -- you explained well. We understand the seasonality. I was asking on a Y-o-Y basis because seasonality would have affected last year also. On a Y-o-Y basis, EBITDA growth is lower than the revenue growth for established centers.

Ashutosh Kumar : So that's true. So in addition to that, we had also mentioned that what we have also seen, while the revenue q-o-q went down, we have seen that the revenue from pharmacy was relatively higher compared to our other modalities business, which typically is high gross margin business.

Now as this gets normalized in the upcoming quarter, quarter 4, we should see our margin getting Healthcare Global Enterprises Limited

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back. So it was primarily lower gross margin due to higher pharmacy business and lower operating leverage due to reduction in revenue.

Devang Patel: Okay. Sir, and the MG Hospital would have some drag at the PBT level after interest and depreciation. Could you

give that number?

Ashutosh Kumar : Sorry, we have not disclosed any asset level information so far. So we would abstain from it at this point of time.

Devang Patel: Okay. Sir, and finally, on the Milann business, that revenue continues to decline. If you could just talk about what our strategy here is to turn around that business and after the stake sale, would we be having a fresh look at this business?

B. S. Ajaikumar: As we all know, in the past, we have looked at Milann and what is the future of Milann in terms of divestment. But meanwhile, as we have explained, the promoter, the original founder of Milann exited and also her phase of no compete is over. In view of that, she has started her own few centers very close to our center. So because of that, particularly in Kumara Park and J.P. Nagar, we have seen a dip in the revenue.

But I'm happy to say that things are turning around. And when we look at the next few quarters, particularly the last month and also the next few quarters, we should climb back up to where we are and continue to show improvement. But at the same time, there is a decision where we want to see what time to divest it, we will be looking at that also as we move forward.

One of the areas we have seen also strength in Milann now is andrology. As we move forward, we are very happy to report that we have a strong andrology department in Milann which will really propel our growth. Because in the past, we have always looked at female fertility issues. But when you look at the data, nearly 40% to 50% it's male -related issues, which has been totally ignored in the fertility.

So we have taken that up very seriously. We are integrating the two male and female fertility.

So our centers will provide that kind of center of excellence we are trying to create, and also look at areas where they might have failed in the beginning, how we can integrate both and try to improve the outcomes. We are focusing like we have done in oncology, we are also focusing on the outcomes.

So this situation is a turnaround. We believe very strongly with this kind of advanced clinics what we are having, and we have a good opportunity to grow back. As we know in India, we believe infertility is becoming a major issue. Fertility will have to be addressed. For various reasons, which I have explained in the past, unfortunately, Milann went through a difficult cycle. And now I believe we are over that cycle, and we will see significant improvement as we move forward. And also, we will look at the opportunity to divest at the right time.

Moderator: The next question is from the line of Sakshi Pratap from Pratap Securities.

Sakshi Pratap : Wanted to ask two questions. Firstly, what would be our growth strategy going forward? Will it be acquisition led, if you could just throw some light on that?

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B. S. Ajaikumar: Going forward -- yes. Can you hear me?

Sakshi Pratap: Yes.

B. S. Ajaikumar: Going forward, our strategy will be, as I said, certainly making sure existing centers reach the full potential of growth. And we do see centers of excellence, full potential. We have new centers coming up in Bangalore and our hub -and-spoke model infusion clinics. This is where we see significant growth happening. But at the same time, we will certainly look at merger acquisitions as we did in Vizag, where we will look at where there are strategic opportunities for us to see a dedicated oncology for us to grow .

As we know, in merger acquisition, dedicated oncology centers are not that many. So we will look at that as well as the combination of greenfield, brownfield, very limited, but we look at existing centers growth also. All of this will be discussed appropriately with our new partner as we move forward. In this regard, I would like to see if Raj Gore, you want to add anything, please do add.

Raj

Gore: No, Dr. Ajai, you covered. As we've always told the market that with our brownfield expansion of capacities in our existing hospitals, we have enough headroom and with our dominant market share in each of our markets, enough potential to drive organic growth in our existing hospitals. That will continue to be the primary driver going forward, our organic plus brownfield growth. And then with the new partner, at an appropriate time, we will decide how to augment that with inorganic growth going forward.

Sakshi Pratap: And what would be the timeline for open offer?

Raj Gore : So I will try to answer. I think, look, the mandatory tendering of this offer is dictated by specific process and timelines laid down by the regulatory authority. It's the offer that KKR has to execute. And as and when they disclose more information as per the process that is laid out, you will get the answers in due course.

Moderator: The next question is from the line of Dhruv Shah from Dalal & Broacha.

Dhruv Shah : Sir, just one question on your debt level. So now it's at about INR1,500 -odd crores, and we've seen in the past that it has been increasing consistently. So, what kind of debt levels would you

be comfortable at, and when do we see debt repayment starting from?

Ruby Ritolia: Debt levels, currently, we are very comfortable with all the banking covenants that we have. What you mentioned includes capital leases. However, if we were to exclude capital leases, then our net debt, particularly from the banking, is about INR650-odd crores. We are very comfortable at this level and considering the growth and the capex investment that we have outlined for the current year as well as for the next year, we will be comfortable within our debt positions.

Dhruv Shah: And just one question regarding your ongoing capex . So, we stand with the current timeline, right? There is no delay in terms of opening of the ongoing capex , right?

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Ashutosh Kumar: We are broadly on time on operationalizing our centers. Yes.

Moderator: Thank you. Ladies and gentlemen, due to time constraint, we will take that as the last question. I would now like to hand the conference over to the management for closing comments.

Raj Gore: So, thank you once again for your active participation in this call. As I mentioned, we are happy that we will be able to enhance shareholders' value in the right way by focusing on doing the right thing for cancer patients. We are very gung -ho about the pote ntial of the organization and right now focusing on ending the year on a very strong note and planning for, again, a good year - on-year growth on top line and bottom line next year. We will keep you informed. Thank you for your support to the o rganization. Thank you.

Moderator: Thank you. On behalf of Health care Global Enterprises Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2025

May 30, 2025

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on May 26, 2025

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 46(2)(a) of SEBI (Listing Obligation and Disclosure Requirements)
Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on May 26, 2025, with Analysts/Investors to discuss the Audited Financial Results of the quarter and year ended March 31, 2025.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

HealthCare Global Enterprises Limited
HCG Tower, # 8, P Kalinga Rao Road, Sampangi Rama Nagar, Bangalore - 560027.
080 33669999 | info@hcgoncology.com | www.hcgoncology.com | CIN : L15200KA1998PLC023489

HealthCare Global Enterprises Limited
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"Healthcare Global Enterprises Limited
Q4 & FY '25 Earnings Conference Call"
May 26, 2025

E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 26th May 2025 will prevail

MANAGEMENT : DR. B.S. AJAIKUMAR – EXECUTIVE CHAIRMAN –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. RAJ GORE – CHIEF EXECUTIVE OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MR. ASHUTOSH KUMAR — ASSISTANT GENERAL
MANAGER – CORPORATE BUSINESS DEVELOPMENT –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
MS. RUBY RITOLIA – CHIEF FINANCIAL OFFICER –
HEALTHCARE GLOBAL ENTERPRISES LIMITED
SGA, INVESTOR RELATIONS ADVISORS –
HEALTHCARE GLOBAL ENTERPRISES LIMITED

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Moderator: Ladies and gentlemen, welcome to the Q4 and FY '25 Earnings Conference Call of Healthcare Global Enterprises Limited. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements do not guarantee the future performance of the company, and it may involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

Now, I hand over the conference to Dr. B.S. Ajaikumar, Executive Chairman of Healthcare Global Enterprises Limited. Thank you, and over to you, sir.

B.S. Ajaikumar: Thank you very much, and good morning, and warm welcome to all present on the Q4 and FY '25 earnings conference for Healthcare Global Enterprise. Today, I'm joined by our CEO, Mr. Raj Gore; Ruby Ritolia, our CFO; and senior management team, along with our SGA, our Investor Relations advisors.

We have concluded another good year, good performance, and very meaningful progress. Also, as you know, it's an exciting year for us with new investors coming. As we have already announced, we have CVC is diluting in place KKR is coming in, formalities are being completed. We look with the new investors for a long-term vision for HCG, which is very, very important to us.

HCG continues to be the leader in cancer care, quality of care outcome, standing as the only major health care institution with truly pan-India presence. Over the past year, we have strengthened this position by upgrading our infrastructure significantly a cross and also that's an M&A, details of which you will hear later on.

Over the past 2 decades, we have championed a distinctive integrated model that combines multidisciplinary clinical expertise with over nearly 350 to 400 doctors, advanced molecular and genetic diagnosis supported by robust hub-and-spoke model network. This comprehensive approach enables superior clinical outcomes powered by skilled clinicians, cutting-edge technology and most important data-driven decision-making in real time.

In the past, we used to say bedside to bench, and then a patient would get the necessary next patient would get. But today, I'm very happy to report we are pioneered in bedside to bench and bench to bedside, so the patients get the right treatment at the right time, very first time.

Looking ahead, we are making focused investments in next-generation capabilities, especially in early cancer detection and precision medicine. The most important part in any cancer treatment is proper staging diagnosis and the right treatment for the patient at the right time, so that the failures will become less.

In line with our commitment to cutting-edge diagnosis and research, we are pleased to share that we are acquiring the Orbitrap Astral Mass Spectrometer from Thermo Fisher Scientific, one of Healthcare Global Enterprises Limited

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the most advanced platforms globally for high-resolution mass spectrometry. In this regard, I'm also happy to read this advanced equipment is now the first time being released, and we are one of the first acquirers of this in the world. We are very proud. We have put together a solid team to see how we can go into the depth of cancer management and also acquire the necessary data for future research and development.

It will enhance our molecular and proteomic profiling capabilities. So, we are going from

genetics to proteogenomics, enabling our precise disease characterizat

ion and paving the way

for next -generation biomarker discovery, targeted therapies and repository in terms of old drugs in new bottle.

Our long-term commitment is to our patients, building a strong academic and research foundation. Through our fellowship programs, which is expanding rapidly and clinical research initiatives, we are nurturing the next generation of oncology leaders.

Together, we are shifting the narrative around cancer from fear to confidence, from complexity to clarity, from inequality to including access for all, so we can make cancer a lifestyle disease and people with cancer can have a good quality of life for long term.

I would like now to hand over the call to Mr. Raj Gore, our CEO, who will provide insights into our strategies moving forward and share the operational performance of the quarter and the year. Raj, handing over to you.

Raj Gore: Thank you, Dr. Ajai. Good morning. A very warm welcome to all the participants on the call.

We are proud to report a strong operational and financial performance this year. Excluding our Milann centers, the company's revenues grew by 20% in the quarter and 17% for the full year,

reaching INR571.1 crores for Q4 and INR 2,165.1 crores for FY '25.

Our adjusted EBITDA for the quarter was INR 106.9 crores, up 14% with margin of 18.3%, while for the full year, it stood at INR396.3 crores, a 17% increase with margin of 17.8%. This strong performance reflects the strength of our business model, brand and the expertise of our clinical teams and the efficiency of our advanced technology centers.

Before we dive into our operational highlights, I would like to take a moment to reflect on how cancer care in India is evolving and how HCG is playing a pivotal role in driving that transformation.

Cancer care in our country continues to face deep -rooted challenges, particularly when it comes to early detection, awareness and equitable access to quality treatment. Even today, far in many cases go unnoticed until they reach an advanced stage, especially in Tier 2 and Tier 3 cities where awareness levels and diagnostic infrastructure are still evolving.

At HCG, we have taken this challenge head on. Through focused awareness campaigns, screening initiatives and active community outreach, we are educating people about the importance of early diagnosis and preventive care. But awareness alone is not enough. We are backing it with action and infrastructure.

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We have made strategic investment in some of the most advanced technologies in the world, linear accelerator machines, PET -CT scanners, robotic surgery platforms, bringing precision personalization and world-class cancer treatment even to smaller cities across India.

So, patients don't have to travel long distances or face delays in getting the care they need. This integrated approach, combining education, access and innovation is already making a real difference. We are seeing more patients come in earlier. We are delivering better outcomes, and we are improving quality of life across communities.

Let me now walk you through some of the key operational highlights of FY '25 that are shaping HCG's performance and positioning us for continued leadership in the cancer care space. First, our expansion in Vizag through the addition of MG Hospital marks a significant milestone. With this acquisition, we have further consolidated our presence in the region and strengthened our position as the number 1 oncology care provider in the region.

Second, during the year, we added 5 state -of-the-art linear accelerator machines across multiple centers. LINAC machines are capital -intensive but essential for delivering precision radiation therapy. Our innovative pay-per -use model has allowed us to remain asset -light while still offering world -class treatment.

I'm happy to share that these machines are

seeing rising utilization rates, indicating both growing demand and increased trust in our clinical capabilities. Our emerging centers have also shown

strong performance, and we are beginning to see the results of the groundwork laid over the past few years.

Our South Mumbai center, which was previously impacted by drop in international patient flow, has made a remarkable comeback. We recorded year -on-year growth of 37% for the quarter

ending 31st March '25. Meanwhile, our Kolkata center delivered a robust 22% growth, fuelled by increasing local awareness, improved clinical offerings and better operational alignment. Both centers are now well positioned to contribute meaningfully to our overall performance.

We remain confident in the long -term potential of these emerging hospitals. As they continue to mature, we expect them to enhance our margin profile, especially as operating leverage begins

to set in.

On the digital front, we are seeing encouraging results. Digital revenue for FY '25 has doubled over the past year, and we are committed to scaling this further. We launched our official HCG

mobile app, a platform that enables to book appointments, consult doctor virtually, access reports and manage their care journey end-to-end. The response has been very positive. As we continue to digitize our patient experience, we expect digital revenues to become a major growing contributor to our top line in the coming years.

Looking ahead at FY '26, we have started the year on a strong note with the formal inauguration of our flagship HCG Cancer Center in Ahmedabad, 189-bedded state-of-the-art facility that marks a significant milestone in our expansion journey. This new center is designed to deliver comprehensive oncology care at scale, featuring 9 OPs, 34 ICU beds, 22-day care beds, 6

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dedicated bone marrow transplant units. This strategic capacity enhancement position us to handle, a projected 30% to 40% increase in patient footfall at this center.

Additionally, as a part of our continued efforts to deepen our leadership in key markets, we will commence operations of 2 new hospitals in Bangalore. The first unit is North Bangalore, a fast-growing urban cluster with rising demand for quality cancer care will come in second half of FY '26. This facility will help us tap into new catchment areas, while expanding access for patients in the northern corridor of the city.

The second facility will be in Whitefield. It is a strategic extension of our existing center of excellence in Bangalore. This expansion will act as an arm to our flagship unit, enhance patient experience and deliver specialized services with greater operational efficiency.

As informed earlier, we aim to expand in our existing markets by operationalizing over 900 beds across networks within next 3 years, including this year's addition and 350 beds that are fully invested but not yet operational. Backed by strong clinical backbone, a patient-first approach and a future-ready mindset, we are not just building hospitals. We are shaping a more hopeful, accessible and technologically advanced future for cancer care. With this, I hand over to Ms. Ruby, our CFO, for financial highlights.

Ruby Ritolia: Thank you, Raj, and good morning to everyone. As Raj mentioned, we've carried the momentum from previous quarter into the close of financial year 2025, and I'm pleased to report another strong performance that underscores both the resilience and scalability of our business model. For the quarter ended 31st March 2025, we recorded a robust 18% year-on-year growth in revenue, reaching INR 585 crores. And on a full year basis, revenue grew 16% to INR 2,223 crores, reflecting consistent execution and growing trust in our network. Our core HCG centers, excluding Milann, was the primary driver of this growth, delivering a 17% increase

in revenue,

accompanied by 17% growth in EBITDA, resulting in healthy EBITDA margin of 17.8%.

. We believe these achievements place us firmly ahead of industry averages, and we are optimistic about delivering even stronger growth in FY '26, both in terms of revenue and profitability. A few key operational metrics further reinforces this momentum.

Outpatient footfall rose by 13.8%, indicating higher patient engagement. In medical oncology, we saw a 24% growth in chemotherapy sessions, a clear indicator of growing patient demand.

Our LINAC machine utilization remained steady at 60% in quarter 4 of FY '25 with acquisition of 2 LINACs with MGM Hospital over the last 12 months.

Turning to the performance of our centers. For the quarter ended March 31, 2025, our established centers continued the steady upward trajectory with a 22% increase in revenue and 15% growth in EBITDA. Our emerging centers outperformed expectation with 32% revenue growth and a

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remarkable 44% increase in EBITDA, clearly reflecting the operating leverage at play as these centers scale.

Strong growth in our emerging center is a testament that these locations are not just growth drivers, but key strategic hubs in Tier 1 cities and access to large domestic and international patient base. The sustained recovery and performance will be instrumental in strengthening our margin profile going forward.

On the pricing and efficiency front, for the quarter ended 31st March '25, ARPOB across our network increased by 4%, reaching to INR 44,236. Established center recorded an ARPOB of INR 42,591 growing by 2.5%, while emerging centers delivered an impressive ARPOB of INR 66,755, growing by 12.4%.

Looking ahead, we plan a capex of INR286 crores in FY '26 in line with our network expansion and technology upgrade plans. Our effective tax rate for the quarter stood at 50.3% and for the year at 14.3%. We remain focused on improving efficiency, while maintaining compliance and governance standards.

Our PAT for the quarter was INR7 crores and for the year was INR 44 crores. As we communicated during the year, we added 2 new centers. The HCC new facility was operationalized and we acquired the new MG center in Vizag. Our expansion efforts led to ROU

creation of INR 275 crores, fixed assets addition of INR 400 crores and an increase in net debt of about INR250 crores. This together led to an increase in depreciation and interest together of about INR25 crores.

With scale -up of our expansion efforts, both in HCC and completion of our integration in Vizag center, we expect a higher return on these investments going forward. Before I take questions, I request Dr. Ajai to add anything further.

B.S. Ajaikumar: Thank you, Ruby. I just want to say Ruby has explained the rationale for our numbers, including the PAT. But in this regard, as we know, in the last 4 years, we have been in a consolidation mode. We have taken a call to expand with merger acquisition and including what Raj Gore mentioned about the greenfield centers. All of this, I believe, particularly with the new partner coming, KKR, who will be a solid foundation for future long-term growth of HCG.

So, the measures we have taken now are all very much in line with the long -term growth of HCG, and we really want to be in a dominant position to provide quality cancer care across India. So, with this, I would like to conclude, and I will hand over to the module people. Thank you very much.

And one other addition I want to add before you take is regarding the debt issue, we have communicated to the investors in the past that once the new investors, shareholders come, we will be looking at the possibility of primary equity coming into the system, and this will be decided in the coming few months. Thank you very much.

Ruby Ritolia: We can now take questions from the participants.

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Moderator: Thank you very much. First question is from the line of Gautam Rajesh from Everflow Partners. Please go ahead.

Gautam Rajesh: I have 2 major questions on the margins front. Margins are much lower than the hospital chains? Is that going to be a steady margin profile for our type of single specialty -based health care?

And my second question was what sort of margin profile do you then expect at a company level for the coming year and maybe the next 3 years?

Ashutosh Kumar: This is Ashutosh. For the margins, if I can draw your attention to Slide number 29, . You would notice that we have over 20% margin in our established centers, showcasing the strength of the platform. However, our emerging centers are a little over 10%. Together, we are delivering close to about 18% margin. As our emerging centers continue to grow, which is again reflected in last year's performance, you will see that our revenue grew by 32% and our EBITDA has grown by 44% in the same quarter for emerging centers, reflecting an increase in EBITDA margin as well. As they inch closer towards the EBITDA margin of the established centers, we expect our EBITDA margins to improve going forward.

Now with respect to the current quarter's margin, we have seen a slight decrease of almost about 40, 50 basis points in the EBITDA margin corresponding to same quarter last year. That is primarily because of higher contribution of medical oncology business, which is reflected in our chemotherapy share.

You would see in Slide number 30, that our chemotherapy volumes have grown by 24%, which is higher than other modality. I would like to just inform the participants here that while we practice comprehensive cancer care and all the modalities are equally important for us. From a financial outcome perspective, medical oncology modality is least profitable, but highly return accretive. So, the way we see it is as the medical oncology grows, it may have some pressure on our gross margin. But in the long run we expect higher operating leverage more than compensating for loss in gross margin eventually resulting in better returns as it is least capital intensive modality in oncology domain.

Gautam Rajesh: Okay, sir. So, like over the next 3 years, do we see improvement is there like a particular number or like you can quantify by any form of how much improvement can be seen as the emerging centers also catch up in margins?

Ashutosh Kumar: In our current portfolio, we should be expecting more than 20% in the future to come in the near future. However, as we operationalize new centres as part of our expansion initiative, overall we may see dilution till these centres scale up to steady state.

B.S. Ajaikumar: I just want to add Ashutosh. Ashutosh there that, please they have to keep in mind some of the

new centers in Bangalore, which they come, that can also have a stance on the margin. But is it good to say as the emerging centers also rack up, our margin, as we always indicated, will be in the low 20s. That is our initial goal.

Gautam Rajesh: Understood, sir. The low 20s would be initial goal.

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Moderator: We take the next question from the line of Yash Sinha from MIPL Family Office.

Yash Sinha: I just wanted to understand what our strategy was in terms of translating a lot of revenue growth and EBITDA growth that we've had over the past 2 or 3 years into tangible PAT growth going forward. What are the specific strategies that we are incorporating in order to do that in terms of maybe using the primary issuance to pay down some debt so that we can see some generation going forward?

Ashutosh Kumar: So as Ruby and Dr. Ajai also indicated in their speech, due to our recent investments towards expansion and acquisition, the cost of depr

eciation and interest has gone up leading to reduction

in PAT. However, as we scale up EBITDA from these investments, we expect PAT to normalize.

B.S. Ajaikumar: Also, Ashutosh, you want to comment on the interest -related PAT effect, how it will get improved this possibility of primary coming in?

Ashutosh Kumar: Right, Dr. and thanks for adding this up. As Dr. Ajai indicated, depending on the quantum of primary we infuse, and its short term allocation towards reduction of debt, it would help reduce interest cost and improve PAT. As the profitability improves, we would create headroom to leverage appropriately to fuel future growth.

Yash Sinha: Got it. Understood. Congratulations on another good set of number and all the best.

Moderator: We take the next question from the line of Devang Patel from Sameeksha Capital.

Devang Patel: Sir, the first question was on receivables, which are higher on a Y-o-Y basis. Could you throw some light on that?

Ruby Ritolia: Devang, so the receivables, we had a little higher during the year. Last year, we saw a lot of state getting into elections and there were code of conduct getting applied because of which some of the collections actually spilled over to April. So, in fact, April usually is the slowest in collection. But this year, there was difference. March, we did a highest ever collection that we could do in March, and that continued in the month of April itself also. So now we will see a more streamlined one with a normal increase in our credit sales, which is happening.

Ashutosh Kumar: Just to add on, if you look at our cash flow statement, I think last year, we had almost about INR 40 to INR45 crores of working capital increase against about INR80 to INR85 crores in the current year. -- the deficit of INR40 crores was made up in the month of April.

Ashutosh Kumar: So, there's not a large gap in the working capital if we include debtors collection in the month of April. It's just a timing issue,

Devang Patel: Will this be the new normal in terms of receivable days.

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Ruby Ritolia: So, as I said that the receivable days in the end of March was temporarily high. This already got corrected in April. So, I wouldn't say that the March number is the new normal. In fact, in terms of number of days, if I have to say, 105 to 107 should be the new normal, which we will maintain.

However, at the end of March, we were at somewhere around 115. So, we have covered up around 7 to 8 days in the month of April.

Ashutosh Kumar: Just one more point maybe, receivables from our newly acquired hospital in Vizag has also led to year on year increase.

Devang Patel: Okay. You explained a different point on the margins for the mature centers. Now, Vizag had a 35% margin in the earlier year. Did that not pull up our margins for the mature centers? What was the impact of Vizag in revenues and profits in the second half or for the full year?

Ashutosh Kumar: As Dr. Ajai indicated, we are transitioning right now and are in the process of integrating the facility. As part of our diligence findings some of the operating costs were not sustainable which has been corrected post acquisition as per HCG standards. That's point number one.

Second, In quarter 4, Odisha government pulled out its government scheme from non-Odisha states. And Vizag in Andhra Pradesh had been the largest beneficiary of treating patients from Odisha under the scheme.

This has resulted into a revenue loss of about INR4.5 crores per quarter had its impact on

EBITDA as well . However, we have taken appropriate initiatives to augment revenue from within the state . Further, we have strengthened the leadership at our hospital to fast track growth. In terms of financial outcome, MGM had a revenue of INR 50 crores in H2, with an EBITDA of INR9.5 crores.

Devang Patel: Wh

at could be the normalized EBITDA margin for this center going forward?

Ashutosh Kumar : As we scale up the revenue to its steady state, we expect EBITDA margin to be in the range of 25% to 30% .

Ashutosh Kumar : . In the current quarter, we also had onetime impact of some costs, which was identified during diligence. The adjusted EBITDA for MGM in H2 is INR 10.5 crores.

Devang Patel: Another question was on capex . There are 2 centers on which capex is mentioned in the slides.

Is there any other capex over and above these 2 centers for the next 2 years? And also given we are looking at primary equity infusion, is that only to reduce debt or we could look at further capex in the next 2 years after that?

Ashutosh Kumar : So, Ruby did indicate our current year's capex plan. I think it is in the range of about INR 280 crores. We already invested a little over INR 200 crores in last financial year. That includes close to about INR 100 crores of maintenance capex and large growth capex within our existing centers and markets are in Bangalore and Cuttack.

Outside of it, right now, we have not much plans in terms of further growth capex . So, this INR286 crores in the current year should reduce going forward with maintenance capex of about Healthcare Global Enterprises Limited

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INR100 crores and some growth capex would come , we will keep the market apprised of our growth plans and associated incremental capex at an appropriate time. However, we have given visibility in terms of how much capex we would do in the current year, which is close to about INR280 crores.

Devang Patel: Okay. Sir, and lastly, on the resolution of the earlier key investor giving bonuses directly to some of the employees, how many employees are covered? And what was the rationale for doing that? Was there an earlier agreement on that?

Raj Gore: So, I think all the relevant information have been disclosed at CVC. There was no direct agreement between company and the investor. And I think for the company to comment on this would be a little difficult.

B.S. Ajaikumar: Ashutosh, what is the question?

Ashutosh Kumar: The question, Dr. is that there was transaction bonus, which were paid to the employees by the shareholders. What was the thought process behind it? And is there any other arrangement, right?

B.S. Ajaikumar: It is purely a thought process of CVC. They felt because of their exit; they should reward the bonus for some employees. And for this, there was an approval by the majority of the minority shareholders. So, any questions they have on this regard, I would direct them to talk to CVC because we are not in a position to comment.

Moderator: Next question is from the line of Aditya Chheda from InCred Asset Management.

Aditya Chheda: So, you mentioned that, if I got it right in the initial comments, there were 900 beds additional over next 3 years and 650 are invested but not yet operational. Can you explain that better? Because there is a Slide 32, which says 85% of capacity beds are operational. So, if you can explain more on that, it would be helpful to know your plans going forward.

Ashutosh Kumar : So, Aditya, of the current capacity, I think we have almost about 200 beds, which are still not operational. In addition to that, we'll be investing in the remaining 600 to 700 beds over next 2-3 years.

Aditya Chheda: Got it. And the next question was in terms of cluster growth with the new Ahmedabad facility and facilities in Bangalore, Karnataka and Gujarat would be the ones which are expected to drive growth in the near term. Is it the right inference that these are higher ARPOB clusters versus the company average for us as of now?

Ashutosh Kumar : That's absolutely right. So, if you look at it , in the region where we are present in metros or Tier 1 cities, the ARPOB, is almost close to about 2x of what we generally see in Tier

2 cities. Our

ARPOB in Bangalore and Ahmadabad centres are the highest followed by Mumbai and Kolkata centre.

So, if you look at our presentation , emerging centers, which is primarily 2 centers in Mumbai and 1 center in Kolkata is delivering over INR 60,000 per bed compared to overall ARPOB of INR44,000.

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Aditya Chheda: Got it. So, this should equate into a high single -digit ARPOB growth for us directionally?

Ashutosh Kumar : Yes, that's right.

Moderator: We take the next question from the line of Kashish Thakur from Elara Capital.

Kashish Thakur: Sir, 2, 3 questions from my end. First question will be on the revenue which we generate from the digital channel. So, can you just shed some light that how it is and like how we are planning to grow it? And yes, it will be around that part.

Raj Gore: So, thank you for that question. Just to give a little bit of background, there are about 2 million new cancer patients every year in India in a population base of INR 140 crores. This is a widely spread thinly dense and fragmented customer base. One way of looking at it is a traditional way to reach out to them through traditional approaches of advertising, reaching out camps, et cetera.

But because it's such a widely spread segment, there are limitations on physical outreach.

But we know that consumer in every sector, patients want more information, want more power over their patient journeys, want to have more say in the decision-making, and they are using digital mediums to get informed, find information and to reach the right care provider. So basically, what digital platform gives us, is an ability to make patients easier to find us rather than we trying to find these 2 million patients among 140 crores population.

Now, we try to reach out to them through different digital channels, website, patient app, social media handles. There are some third-party channels. Our majority of revenue comes from our website and our social media handles. The way to drive it is search engine optimization, market after market, local. I think in last year, we have doubled our revenue.

We are still working on making our search engine optimization better at each geographic location. As that journey moves forward, we will be ranked higher in potential patient searches. And therefore, it gets us into their consideration set right away. And that we have seen from our experience.

As long as we are in their consideration set, our ability to convert them into a purchase decision or make them walk into our doctor's consultation room that conversion ratio historically has been very high for us because of our brand and clinical capabilities, et cetera. And as you know,

among health care, if there is any specialty where patients do not mind travel long distance to any corner of India or the world, it is cancer because it is a life and death.

So, we feel that as we invest our efforts, energy, resources in digital, we will be able to reach out to more and more diagnosed cancer patients across the country and even outside the country in our international markets. And this is a channel that will continue to grow for us going forward, contributing more and more revenue to our top line.

Kashish Thakur: The next question will be on the tax rate. What kind of tax rate should we expect for FY '26?

Ashutosh Kumar: Can you repeat the question, please?

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Kashish Thakur: I was asking about the tax rate, ETR. What kind of effective tax rate should we expect for FY '26?

Ruby Ritolia: I think we can expect about 30- odd percent to remain in that level during the next 1 year.

Kashish Thakur: Understood. Just wanted a clarity, like what kind of competition are you seeing in oncology space as many of your multi-specialty competitors have been

aggressively taking steps in this

direction. So how do you see the competition, especially in the markets outside Bangalore?

B.S. Ajaikumar: See, as you know, we are a focused specialty. We believe we have continued to garner higher share of market in city like Bangalore, Greater Bangalore. We are known as a destination. So, in our view, our growth, as you can see in our Bangalore sector, which is doing extraordinarily well. If you look at our main center of excellence in Bangalore, the growth, the margins have been extraordinary, generating a margin close to 28%, 30% with AR POB over INR 75,000, INR80,000.

So, this kind of high-quality growth will continue to happen in spite of whatever the number of centers they may be. The reason I say that is oncology has to be a destination. It is not a disease of convenience. So, in my view, that is how we started HCG as a single specialty, a single specialty hospital always will deliver better quality and outcome overall because of the way we have structured the ICU, the outcome, what we measure, the multi support system we give for the patient specific, for example, infection.

If you look at our mortality rate, let me give you one example. Across India, which I do audit of our mortality recurrence rates, when you look at our mortality in 2017 for 5,500 discharges, we had a mortality of 2.5% per month across India. But today, after we got into this auditing, today, our mortality for 13,000 discharges, 13,000 per month is less than 0.9%, below 1%. Our target should be 1%. Now we are trying to even lower that. I don't think anybody in the world has this kind of low mortality. I can tell

as an oncologist.

B.S. Ajaikumar: No, I just want to say we measure it, we put out of our data and do all that. And we also do data collection for research academics. So, when you talk about the competitive landscape, we feel honestly, they should be worried about us, not worried about them because the way we are expanding, putting together the quality of excellence, the quality we have today is unmatched. And I trained in Houston, MD Anderson, I can say we are equal to that very clearly. So, this is where we are with genomics, proteomics coming. So, I hope I answered your question.

Kashish Thakur: Yes, sir. Yes sir.

Raj Gore: Dr. Ajai, I just want to add 2 data points. Let me give an example of Bangalore. There is no other place where there are so many multi-specialty players are doing oncology and they have multiple hospitals doing oncology in Bangalore. And yet, we have grown our market share over the last 4 years. Number two, we entered in Kolkata and Mumbai market pretty late, and there were existing hospitals who are dominant player. But in both markets, we have gained largest market share over 4 years and climbed the market share ranking. And both are an example of why our single
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specialty focus is a better approach to specialize and focus and build confidence and trust of patients because this is the only thing that we do. We live, eat, drink oncology, and that gives us our best chance to give the most advanced and comprehensive treatment to our patients, and that's what is working for us till date.

Ashutosh Kumar: And just to add on, in this regard, we have not lost our market leadership in any of our locations in the last 3, 4 years in spite of competition coming in. The way we approach this and think about it is that India is an underserved market in terms of supply. And as the supply keeps growing, it's only going to help the best player providing quality care in that space.

Oncology is something which is a matter of life and death. And this is the only specialty among all other specialties in health care where second opinions are the highest. So once the patients are diagnosed that they have any indications of cancer, they try to figure out who are

the best

providers in that market to give them the best solution

B.S. Ajaikumar: Ashutosh, one other thing is it is not only like if a patient goes to a multi-specialty with abdominal pain and they found that they have a cancer, they may end up taking treatment there. But remember that significant number recurrence. When they get recurrence that is when they come to HCG. That is why I call it a destination. So that is how MD Anderson is, and that is how we have positioned ourselves as a destination, and that is why we will continue to grow.

Kashish Thakur: Understood, sir. Sir, just one last question. Why do we see our ARPOB still on the lower side? I still believe that oncology treatments are a bit on the expensive front. So, why our ARPOBs are so low?

Ashutosh Kumar: So, if I can take that question, Dr. Ajai, we have significant presence in Tier 2 cities. However, if you refer to our investor presentation slide 13, we have demonstrated a business model with similar profitability and return in Tier 2 markets compared to metro market in spite of ARPOB being lower.

While the ARPOBs in metro cities are higher, the throughput in terms of number of patients we treat in Tier 2 cities is higher on like to like capacity basis. This helps us generate similar profitability and return in Tier 2 cities compared to metro cities.

B.S. Ajaikumar: You may also want to make an observation, Ashutosh, on how our ARPOB is in our centers in urban areas and why the Tier 2 are maybe why it's lower and it is expected to be lower. So maybe you can make a comment on what is our ARPOB in that.

Ashutosh Kumar: Sure, sure. So, in the centre where we have full offering in terms of high-end therapies as well as being in metro, like, for example, Bangalore and Ahmedabad, the ARPOB is close to INR 1 lakh. followed by Mumbai and Kolkata, with ARPOB of about to about INR 60,000 to INR 70,000 in

So, these are the markets we have very high ARPOB. The other markets in tier 2 cities such as Odisha, Andhra, the ARPOB is about INR 30,000, INR 35,000 resulting into an overall ARPOB of INR 45,000 for the company
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Kashish Thakur: Understood, sir. Sir, just one last question. So, I might have missed what kind of ARPOB growth we might expect in FY '26?

Ashutosh Kumar: With continuous decrease in length of stay in the hospital coupled with high end treatment modalities we expect 7% to 8% increase in ARPOB

Moderator: Next question is from the line of Sakshi Pratap from Pratap Securities.

Sakshi Pratap: Just a couple of questions. So how was the performance overall for international patients? How do you think we have performed this year compared to last year given this year had a lot of geopolitical issues?

Raj Gore: Yes. So, Sakshi, the biggest contributor to India, not just for HCG, but for all hospital players in terms of international business is Bangladesh. And as you know, because of geopolitical issues between our 2 countries, India has reduced drastically the number of medical visas that India grants to Bangladeshi patients.

That is the major reason for the impact on international business across India, across players. That to some level has improved, but I think that will continue to be suppressed for the next few months till our relationship between 2 countries get completely normalized.

So that's the outlook in general for India. However, as even though our international business got impacted last year, we have more than made up for it through domestic channels. So international for us is a small part of our total business. Our core is domestic business, and we will continue to monitor international business situation. However, we will continue to work on making up for any reduced growth on international through domestic channel like last year .. So, we don't expect it to impact our revenue growth this year.

Sakshi Pratap: Understood.

B.S. Ajaikumar: I just want to add one more word to what Raj said. When we look at international, as we know, CCK Cancer Center, which is into radiation and medical oncology. I'm very happy to say that this center has been doing very good. We have installed a new radiation equipment, PET scan, and we are now in the process of possibly looking at a second LINAC in Nairobi. And we have also discussed at our Board concept node to see how we can really further improve there because the international opportunity in countries like Africa are significant.

And what we have shown is a significant growth, high profitability there with the margin reaching almost 24%, 25%. So, this is one other angle, which we will be addressing in the next few quarters with our new investor, and we will keep you definitely updated on.

Sakshi Pratap: Understood. And would you be able to provide the percentage contribution by international patients to our revenue?

Raj Gore: Around 3-4% of revenue is from international.

Moderator: Next question is from the line of Umang Gada from Avener Capital.

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Umang Gada: My question was more on the Milann side of the business. That business has evidently continued to struggle to grow. So, I just wanted an update from the management side that what are the strategies over there even maybe are we looking to spin off in the future or any strategies over there?

B.S. Ajaikumar: Yes. I can answer that. You're absolutely right. Milann have been through quite a tough time, particularly with our former partner self-starting units near our center. We have addressed this issue internally. We were just waiting for the new investor also to come. We have also discussed with them. So, we are definitely putting a proposal to divest. And hopefully, this will happen definitely this fiscal year, and we'll keep you updated.

Umang Gada: Yes, yes. I was, the answer was audible.

Moderator: Ladies and gentlemen, that was the last question. I would now like to hand the conference over to the management for closing comments.

Raj Gore: So, thank you so much for your continued interest in HCG. As informed by many of us during the call, it's a very exciting time for the organization. We have just come out of a very good operational and financial performance in FY '25. We've been sharing our plans for next 3 to 4 years continuously over the last few calls.

As with the incoming new investors, the organization is really excited about moving on from consolidation phase largely in last 4 years to a high-growth, high-performance phase in the coming future. And we will continue to keep you informed as and when those things materialize, and we have more information. Thank you so much. Have a good day.

Moderator: On behalf of Healthcare Global Enterprises Limited, that concludes this conference. Thank you for joining us. And you may now disconnect your lines.

Call: Dec 2025

December 11, 2025

Dear Sir/Madam,

Further to our intimation dated November 22, 2025, please see enclosed the Transcript of the Earnings Call held on December 04, 2025, with Analysts/Investors to discuss the Company's business strategy and long-term growth plans.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

Encl: a/a.

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Subject : Transcript of the Earnings Call held with Analysts/Investors on December 04, 2025

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 4 6(2)(oa) of SEBI (Listing Obligations and Disclosure Requirements)
Regulations, 2015

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"Healthcare Global Enterprises Limited 's
Virtual Investor and Analyst Meet "

December 4 , 2025

MANAGEMENT : DR. MANISH MATTOO - EXECUTIVE DIRECTOR & CEO

Healthcare Global Enterprises Limited
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Moderator : Ladies and gentlemen good day and welcome to Healthcare Global Enterprises Limited's Virtual Investor and Analyst Meet. Please also note that this session is being recorded. Before we begin, we would like to highlight that certain statements made during today's discussion may be forward-looking in nature. A disclaimer to this effect has been included in the presentation. We will begin the meeting with a short video followed by opening remarks from Dr. Manish Mattoo .

Welcome back, everyone. I now hand over the proceedings to Dr. Manish Mattoo .

Manish Mattoo : Good evening, everyone, and thank you for joining in. I am Dr. Manish Mattoo , CEO of HCG. I welcome you all to HCG's Virtual Investor and Analyst Meet. Today, I would like to take the opportunity to walk you through HCG's journey as leaders in cancer care, how we are thinking about our value proposition, our clinical differentiation, our patient care and outcomes, and more importantly, what is our vision for the future. How are we thinking ahead to advance our mission, backed by a clear strategy to compound our EBITDA with a disciplined approach to capital allocation . Today, HCG is India's largest cancer -focused oncology platform, with deep presence across India in metros as well as non -metros. Backed by 20 years of experience, HCG has built one of the most extensive and talented pool of clinicians in the country. Our patient -centric approach backed by focused tumour board approach and well -defined treatment protocols enable industry -leading gross mortality rate of 0.9%, which is one -half to one-third of other multispecialty peers. The thesis is simple : Specialist focus leads to better outcomes, stronger doctor performance, and strong economics. Today, HCG operates 25 hospitals across 10 states and 19 cities. And out of these 19 markets that we operate in, we have a leading position in 16 cities . Not just that, we have a comprehensive infrastructure spread across 2,500 beds, and we have the largest LINAC installation base of 38 LINACs in the country. Besides this, we have c.100 fully operating operative theatres and also 400 -plus experienced oncologists in our network. This depth allows us to consistently deliver great clinical outcomes, a good operating leverage, and access to some very interesting markets, which are growing quite well of late. If I look back 15 years, HCG's journey can be broken up into three phases. In the first phase, which was from 2010 to 2015, we really consolidated our base in Karnataka and also expanded to a few markets outside the state through our doctor partner model. This phase saw a revenue CAGR of 27% and an EBITDA CAGR of 30%. The next phase was from 2015 to 2020, which saw us expanding rapidly into eight new cities, eight new markets and we saw a shift from a doctor -operated model to a fully -owned model. And this phase gave us a revenue CAGR of 16% and an EBITDA CAGR of 14%.

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The last five years, from FY202 0 to FY202 5, have really been about consolidation, capital preservation, and operational efficiency. But this phase also saw a revenue CAGR of 15 % and an EBITDA CAGR of 18% , but throughout these phases, what has been important is the expansion of the capacity. Our platforms, our hospitals , almost doubled in size from 14 hospitals we went up to 25. Our LINAC installation base increased threefold. Our bed capacity increased fivefold and today, we are at 2,500 plus beds. Our revenue increased 15 -fold in this same time frame and grew at a CAGR of 15% and EBITDA grew at 16-fold at a CAGR of 18%. But while we were expanding across these years, one thing was very clear, that we continued to dominate the markets that we were in, and we continued to build a large talent pool and delivering great o

utcomes. Based on this historical context, we now stand at

a unique position of rising demand and constrained supply for high quality cancer care in India and HCG is exceptionally well positioned to capture this opportunity.

We are truly excited to have KKR as a majority shareholder and promoter. KKR brings with it a wealth of experience in building and scaling some of the world's leading healthcare platforms and something that is going to augment HCG's journey ahead. This partnership gives us access to global expertise, long -term capital, and proven operating frameworks, strengthening our focus on performance, governance, and discipline value creation. With KKR's support, we are now well positioned to scale and advance our mission of transforming

cancer care in India.

A little bit about myself, I am a trained anaesthesiologist with an MD from B . J Medical College Ahmedabad. I also have an MBA in Finance from Indian School of Business. I have had the privilege of working across healthcare majors like Apollo Hospitals and Fortis Healthcare, across geographies and functions. In my last role at Apollo Hospitals, I was overseeing a network of 2,000 beds in the time spent in Apollo, I have been able to double the business and increase EBITDA two-and-a-half times, essentially by deepening our clinical expertise, expanding our clinical programs and reach, and boarding several clinical teams who shared our mission and clinical vision. But these experiences have taught me some important things, that a strong focus on patients and clinicians can drive transformation growth , and I am truly delighted to be part of HCG and, in some form or shape, contribute in shaping the next wave of cancer care in India.

Before we get talking about HCG, it is important to understand the broader context of cancer care in India. Despite several advancements, cancer remains an underpenetrated and underserved disease. If you see on the left-hand side of slide #8, our cancer coverage by the National Cancer Registry is only 10%, whereas in China it is about 40% and US is about 90%. What that means is a large portion of population is beyond the formal cancer care ecosystem and that leads to underreporting. The fact that in 2020, only 56% of cancer cases were actually reported , a total 1.7 million cases were reported, whereas the real incidence was Healthcare Global Enterprises Limited

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about 3 million. By 2030, the number is going to swell up further. We are going to have a reported incidence of 4 million cases likely, but the real incidence will be around 6 million, which is a huge number.

Another challenge that we are facing is lack of early detection. As you can see in the slide, even in breast cancer cases, for example, which is a large part of cancer incidence in this country, only 29% cases come in at the right stage . The other big cancer incidences in lung and head and neck cancer cases, the reported incidence is even lower. Now, that leads to a high mortality to incidence ratio, which is quite serious for a country like us. This underscores the urgent need for immediate intervention, broader access, and consistent quality of care and this is leading to oncology becoming the fastest growing specialty driven by rising incidence, narrowing diagnosis gap, and improving access.

And as I was saying, it is not just an underpenetrated disease it is also an underserved disease in this country. As you can see on the left -hand side of the slide, we have only 0.3 PET scans and 0.3 LINACs per million population in this country and the similar corresponding number in US is 6 to 12 respectively. Now what that does is increase the burden of volume on our equipment. Today in India, for example, each LINAC takes care of 3,000 patients. In US, the number is about 400. On the right -hand side, you see a similar issue when it comes to capacity and talent. Ou

t of the overall 500 -plus districts that we have, only about 175 have comprehensive cancer centres . It is a big demand -supply gap. When it comes to oncologists, out of the need of 12,000 oncologists in the country, we only have 8,000 oncologists. Now, this is translating into tangible growth headroom for players with scale like HCG. And we can see cancer is a fast -growing speciality in this country, probably the fastest, growing at a 13% to 14% growth per annum, out of which 9% to 10% is driven by volume alone, because of factors like aging, demographic change, rise in risk factors, and so on and so forth, as well as affordability. So on the supply side, rapid improvements in technology and infrastructure are making advanced cancer care accessible, while on the demand side, because of these factors, the demand is increasing by the day and in this environment, HCG is at a vantage point because of our oncology -only focus, our clinical depth and pan -India presence, we are in a strong position not just to participate in this growth, but to lead this growth cycle. It is important to understand the disease as well, the disease that we are tackling, the disease that we are focused on.

Now, cancer is a complex, chronic, recurring, heterogeneous disease, and the disease landscape is changing every day. Now, it demands a super -specialized, focused approach to deliver superior clinical outcomes, which is very, very important because the margin for error in this specialty is very narrow for the patient. And HCG's model of a comprehensive cancer centre is especially suited for oncology because we integrate advanced diagnostics, genomics, proteomics, organ -based histopathology, theranostics, and a multi -specialty, Healthcare Global Enterprises Limited

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multi -modality treatment, and even post -care therapies like nutrition, rehabilitation, and so

on and so forth. And the result is a gross mortality rate of just 0.9%, which is industry -leading. As I had mentioned earlier, it is one half to one third of multi -specialty peer and it is something very difficult to replicate. So in summary on this slide, cancer requires a coordinated, multi -modality care only a focused, single -specialty platform can deliver this seamlessly and that is the foundation of HCG's clinical strength and market leadership. Globally too, if you look around and see which are the leading, the best cancer centres globally, who are the gold standard of care, you can think of a few. Whether it is Memorial Sloan Kettering, it is MD Anderson, and closer home, whether it is Tata Memorial, Rajiv Gandhi, and of course, HCG on the private side, they all have one thing in common all follow the single speciality model that delivers great outcomes for patients and have a superior brand recall. And HCG has demonstrated this effectively at scale over the last 20, 25 years, holding leading positions in 16 out of the 19 markets. In fact, as you can see on the slide, there was a study conducted in the US comparing outcomes in dedicated cancer hospitals versus multispecialty hospitals and it was clearly evident that the first -year mortality rate and a five -year survival rate were significantly better in dedicated cancer hospitals vis -a-vis multispecialty hospitals and this data, whether it is global or from the Indian context, reinforces our belief that when care is focused, concentrated and specialized patient outcomes improve. And that is the currency going forward that hospitals will have to build. It is not just the scale and footprint. It is going to be our clinical outcomes and quality of care and that is the approach guiding HCG's model and continuous focus on improving cancer care quality and consistency.

In fact, feedback from the medical community echoes the same thing, that their preference for single speciality model for referring their cases because

use of the factors that we mentioned

about comprehensiveness, clinical outcomes, focus, latest treatment options, and so on and so forth. Now, HCG has been driven by a strong clinical philosophy of delivering great clinical care, great outcomes, especially in high -need areas. While growth has been important to us, we have stood true to our values of ethical practice, integrity, and high quality. And because of a combination of strong clinical vision, mission, and our values, today, HCG is India's most trusted oncology platform based on the volumes and our reach and outcomes. Not only are we India's most trusted oncology platform, we are also the destination of cancer care in this country, as I mentioned, because of the shared volumes and outcomes, because we provide end -to-end cancer care, organ -specific, personalized care, through a fully integrated system. We have to understand that when a patient walks into HCG hospitals, every step in the patient's journey, whether it is diagnostics, whether it is genomics, molecular diagnostics, high -end theranostics, preventive oncology, or the whole spectrum of multimodality care through medical oncology, radiation oncology, surgical oncology, or the Healthcare Global Enterprises Limited

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post-care treatment options in rehabilitation and nutrition, and so on and so forth, or even targeted follow -up everything is delivered under one roof.

Even our histopathology today is organ specific and that is something very, very difficult to replicate. Hence, it is this comprehensiveness and completeness along with superior clinical outcomes that we have delivered consistently that makes us the destination for cancer care in this country. We have also been ahead of the curve when it comes to investing in technology. When India was delivering cancer care through cobalt machines, HCG was one of the first institutions to start LINAC installations. When LINAC became common, that is when HCG moved ahead and started installing Tomotherapy and CyberKnife units. And today, as we speak, we are going to install an MR LINAC in our upcoming hospital in North Bangalore. So suffice to say that we have always invested in cutting edge technology like AI and diagnostic, robotic surgery, genomics, adaptive radiation therapy that we have in our centres of excellence. All this while contributing to building the most technologically advanced oncology platform in the country. And this early and continuous investment in technology has not just delivered superior outcomes for our patients, they have also helped us attract and retain some of the best clinical talent as well.

One of HCG's strongest clinical differentiators has been the Tumour Board platform, the Tumour Board program, which we have been running consistently for the last 17 years, which ensures that every patient, every complex case, every challenging, rare, difficult case undergoes a multi -specialist review. And what that does is, this collaborative approach has delivered very superior and consistent clinical outcomes for us and setting us apart from our multi -specialty peers. Over the years, we have built a rich repository of insights through this Tumour Board platform with over 40,000 cases that we have discussed and about 400 oncologists and other specialists who have participated in this platform, in these discussions, come out with deep insights. And over a period of time, we have systematically embedded

these insights into our treatment protocols. And every year, hundreds and thousands of patients across the network benefit from these insights, and hence, again points out to the fact of delivering superior clinical outcomes. Hence, in that sense, our tumour board is not just a discussion forum it is really a powerful clinical engine, where oncologists and other specialists come together to tackle rare and challenging cases. And it is

also very important

to note that our platform does not just employ oncologists we have other specialists too taking care of patients and improving outcomes, like radiologists, intensivists, physicians, surgeons. We have the whole gamut of specialists across modalities coming together to take care of our patients and ensuring that they get the best under one roof. And these insights, like I was mentioning about the Tumour Board, have directly contributed to a low mortality rate, low relapse rate, and a high survival rate, a reflection of how well our clinical ecosystem is working for our patients. And you can see this whole clinical engine driving statistics too. Like I had mentioned, a comprehensive baselining, a deep baselining that we do for our

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patients on the diagnostic side, and when it comes to multidisciplinary decision-making through the Tumour Board, a multimodality treatment, and a targeted follow-up coming together, delivering great outcomes, as you can see on the right hand of the slide, in spite of a fourfold increase in our volumes over the last 10 years, our gross mortality rate has decreased by 74% to come down to less than 1% at 0.9%, which is really, really remarkable. In fact, our outcomes are perhaps at par with some of the best cancer institutes in the world. In a comparative study of five-year survival rates in a cohort of breast cancer patients, we found out that our survival rates were actually a couple of percentage points higher than the leading global cancer institute that I am talking about. Just goes on to show that not just in our country, in India, but globally also our outcomes are something to reckon with. And just before we move forward, a quick recap and overview of our Board and Management Team. Our Board brings together deep expertise, across finance, healthcare, investment, consumer sectors and governance, and combining clinical leadership, investment acumen, and strong audit and governance experience. And this diverse and highly experienced Board provides strategic oversight to us and is helping us strengthen HCG's performance and governance framework. HCG has built a strong team culture over the years and it has always been known for cohesive teamwork and integrity and that is something we are going to build on and evolve as it goes forward because it is very critical to have a high performing team, which is delivering on key metrics. Now over the past five months we have hired five senior leaders in critical functions including a Head of Clinical Strategy who will drive identification of high potential clinicians and onboarding them onto the network, a Chief Marketing Officer, whom we are hiring for the first time in HCG's history, who will drive brand amplification and patient engagement, a CIO who is going to drive the mandate of digital transformation, a Head of Investor Relations who is going to deepen stakeholder communication, and also Heads for International Business and Domestic Sales who are going to sharpen our commercial execution. Now, these additions who come from reputed organizations in the healthcare sector and come with diverse deep experience, combined with their existing leadership at HCG will bring together strong operational, clinical and strategic expertise positioning us well for the next wave of growth and transformation.

This slide reflects what HCG has really built over the last 20 to 25 years, which is a clear leadership position in cancer care by virtue of the large coverage we have, or by virtue of the operational capacity, by virtue of the volumes, clearly HCG is the leader in this space. As an example, if I were to tell you on the left hand side of the slide, if you can see, as compared to our closest competitor in this space, HCG, the installed LINAC base, we have three times that competitor, our revenue

is two-and-a-half times that competitor, and our clinical density measured by the number of clinicians oncologists we have on Board is actually two times just goes on to show that we are miles ahead of our closest competitors in Oncology. And not just

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that, we are ahead of other large multi-specialty players as well, underscoring the strength and scale of our focused model.

This slide captures our journey in figures largely and if you can see, we have had a proven track record of profitable growth over cycles as well. But particularly in the last five years, we have delivered consistent growth. In FY2025, we achieved a revenue of over Rs.2,200

crore at a CAGR of 15%. That growth was contributed by a good delivery of performance by our mature centres and also a ramp up of our relatively newer centres. Our EBITDA in the same time frame of FY20-25 actually doubled to Rs.396 crore and grew at a CAGR of 18% really through a sharper focus on operational efficiency, on cost discipline, and also the improving maturity of our network. And going forward as well, we expect to maintain a similar trajectory in the coming quarters.

I want to take a step back now and walk you through how we are thinking about tracking our business and structurally how we are thinking about our business. In the past, we classified our business as established and emerging, but as most of our centres have now matured or are near maturity, I think this metric or this classification no longer relevant and hence, we are moving to a new region-based, cluster-based framework, essentially grouping our markets into three main clusters, the South, the West, and the East and we have one international cluster which combines Milan as well and I will talk about that separately. And this cluster-based framework essentially will help us leverage regional synergies. It is also in line with our organizational structure and it will also help us understand the local dynamics better and help them address it so that we can improve accountability and performance.

Now, on the metric side that you can see on the right hand of the slide, we have simplified our performance tracking a little bit. Instead of the traditional multi-speciality metrics like occupancy and ARPOB, which do not fully represent our business because a large chunk of our business is on the outpatient side, like radiation and chemo modalities, we propose to focus on two new metrics. One is patient volumes, and the other is average realization per patient, or ARPP. Now, together, we are confident that these two metrics will give you a comprehensive view of all revenue streams and all modalities and help us track scale and realisation both. On an overall basis we will show you EBITDA margins for profitability and ROCE for capital efficiency and I will be discussing these metrics with you every quarter. I will now quickly introduce you to the clusters that we just spoke about.

The first cluster is the South cluster, which is a major cluster for us. It represents 10 hospitals across four states, Karnataka, Tamil Nadu, Telangana, and Andhra Pradesh, covering key cities like Bangalore, Hubli, Vijayawada, Vizag, and Ongole. And this cluster has, as I said, 10 hospitals, 870 operating beds, and 16 LINAC installations, and major markets being Bangalore and Vizag and I will talk about that a little later. And in FY2025, the cluster

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delivered a revenue of Rs.875 crore, growing at a 13% revenue CAGR since FY2020, driven by 11% volume growth in the same time period and a ARPP improvement of 2%.

This cluster has a centre of excellence in Bangalore, where we will deepen our presence by adding two more hospitals and dominate the landscape of the city as far as cancer is concerned and the other

major market of Vizag, where we already have a 50% market share, we want to deepen our presence in the surrounding areas. We want to increase the market share further. And we want, by doing more complex, more specialized work, by increasing our clinical depth, we want to become a centre of excellence in that city as well.

The second cluster is the West cluster, which represents 11 hospitals that we have across four states again of Maharashtra, MP, Gujarat and Rajasthan covering major markets like Mumbai, Ahmedabad, Nashik, Nagpur, Jaipur and Baroda so that is the West clusters. And this cluster has an operating bed base of 990 beds, and an overall revenue contribution of about 45% and the fact is that this West cluster has been a good engine for growth for us in the past and will continue to be. As you can see on the statistics, the overall revenue in FY2025 was Rs.990 crore from this cluster, and the revenue grew at 17% CAGR driven by a 10% increased growth in volumes, and a 6% growth in ARPP. And going forward, how we are thinking about this cluster is that it will continue to be a growth engine for us. The major market of Ahmedabad, where we have a centre of excellence, we will continue to maintain our leadership there. And also the other markets, like Mumbai, where we are forced to reckon with now the top market shares of that city. Mumbai will scale up, and so will Nashik and Nagpur. So that is how we are thinking about the West cluster.

The third cluster is our East cluster, which has three centres across Ranchi, Cuttack, and Calcutta, with an installed operating bed capacity of 280 and five LINAC installations. This cluster has also had a robust growth of 26% revenue CAGR since FY2020, driven by a 15% growth in patient volumes and 9% growth in ARPP. This cluster delivered a revenue of Rs.255 crore in FY2025. Going ahead, we have a major presence in Cuttack and Ranchi. We are the dominant player because of the trust that patients have on us or the clinical expertise that we have. So we will continue to dominate that market by expanding our capacity, work on that has already started. At the same time, we want to strengthen our positioning in Calcutta, which we foresee being the hub of growth for our East cluster in the days ahead.

On the international front, we have one centre in Kenya, which has a one LINAC installed. This centre has done really well for us in the last couple of years. We recorded a revenue of Rs.43 crore in FY2025. And this centre continues to do well this year as well and I think the momentum is going to continue. Besides growing organically, that centre also continues to feed high-complex work into our centres of excellence in India, like Bangalore and Ahmedabad. Lastly, as you are aware, Milann is our fertility business, which we acquired in Healthcare Global Enterprises Limited
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2019. And our focus in the last few quarters has really been about stabilizing operations, revenue, and profitability, which we have done now. But going forward, our focus will be on cancer care. And hence, we are evaluating several strategic options on the way ahead for Milan.

As far as our balance sheet is concerned, we have constantly been focused on deleveraging our balance sheet. In the last five years, our cumulative EBITDA was Rs.1,050 crore. With a healthy EBITDA to cash flow from operations profile of 72%, we have been able to generate Rs.754 crore of cumulative operating cash flow in the same time period. And our capex spend has been about Rs.659 crore, out of which Rs.353 crore has been spent on maintenance capex alone. Now, despite undertaking two acquisitions in Vizag and Indore, overall, our net debt to EBITDA ratio, which was 6.2x, in FY2020 has come down to 2.3x in FY2025. Now, how are we thinking about the future? I think it is a very, very important conversation. We are very

focused on several parameters going forward, and we have a clear strategy in our mind. We have four pillars of strategy that we have identified. The aim is really to improve clinical outcomes from where we are, because that is at the core of what we do. The focus is also on improving our growth and profitability, but all of it is underpinned by prudent capital allocation. So, I will give you a summary of our strategy going forward, and I will touch upon some of these pillars in detail in the subsequent slide. Now, our first strategic pillar is to optimize our existing network. Today, we have a large headroom for growth in our existing hospitals and we are going to deploy several measures to really unlock that growth. We are going to deepen our clinical presence, we are going to improve our quality of clinical talent, we are going to expand our clinical domains, we are going to unlock the potential of sales and marketing engine, we are going to amplify our brand and marketing efforts, and we are also going to scale our international business. Now, all these put together are definitely going to optimize our network going forward.

The second pillar is growth. While a big chunk of our growth is going to come from our existing network, we are anticipating a good chunk of growth coming from two other levers, two other channels. One is the brownfield expansion, which will happen in our key high potential markets like Ahmedabad, Cuttack, Baroda, Vizag so that is a big channel for our growth. And the other channel is greenfield expansion. We have identified 10 to 12 markets, 10 to 12 cities, which we feel are attractive, which fit into our strategic and financial framework where we feel HCG has a right to win, where we feel we can ramp up quite well and deliver on unit economics in a reasonable time frame. And out of these 10 to 12 cities, we are confident that in at least two to three cities, we should be able to start something in the next two, three years. And that will be the other engine of growth that we are looking forward to.

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The third strategic pillar is going to be our network efficiency, where we feel we have the potential to improve that as well. We have undertaken cost measures in the past, and we are continuing that journey in the future as well. We feel there are several levers that we can unlock to give us this efficiency. And also, we want to focus on operating leverage to improve our margins. We are also consciously moving in asset light adjacencies, investing in them, like day care and diagnostics, which we feel have high potential and are complementary to our business, and they can also help us grow in the days ahead. And last but not the least, that is a big lever for us, a big strategic pillar for us, is about enhancing patient experience. We are going to invest in upgrading our infrastructure. We are investing in digitizing patient experience inside the hospital and outside as well. We have invested in the past on our digital infrastructure, like a patient app, our CRM, our doctor portal. And we want to increase adoption so that we can get the benefits of that to improve overall patient satisfaction, but underpinning philosophy is going to be prudent capital allocation.

I will now walk you through some of the pillars in detail. This slide on growth potential of our existing hospitals is essentially talking about the revenue potential of these hospitals at

current pricing levels, assuming optimal occupancy and utilization. This metric while it may be new, is taking into account all the revenue streams of a particular hospital across OP, IP, along with the capacity limitations. And please remember, this is a point in time metric. And it will scale up as our price realizations improve. And you will notice across all the clusters, on average we are operating at about

50% to 60% of our overall revenue potential. So that is the key message I want to leave you with. These are hospitals where majority of the investments have happened, both in infrastructure and medical technology. And as we expand capacity at very incremental capex by unlocking the revenue and growth drivers that I spoke about in the previous slide, this metric is going to improve quite a bit in the future. This is another lever that we are going to focus on, optimizing our payor mix. Currently our cash insurance and corporate business is about 63% of our overall business. And our institutional business is about 34%. And that has happened over a period of time, the reliance on institutional business in some of our non-metro markets has been there historically because the idea was to ramp up those hospitals. Now that we have reached a maturity stage in most of our markets, we are going to consciously move away from this segment of the business by focusing on infrastructure upgrade, by getting on board expert clinical talent, really focusing on sales, marketing, brand amplification, improving patient experience. And with those measures, we are confident of improving our cash and insurance business quite favourably in future. The other lever that we are going to focus on is International. It has been in the past a good contributor to our overall sales, but is now at only 3% of our overall sales, which we feel is well below potential given HCG's clinical quality and brand equity. Over the next couple of quarters, we are going to focus on building our relationships in key markets, international markets like Africa, Middle East, Southeast Asia. We are going to work directly with the Ministries of Health in these markets. We already have existing relationships. We

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want to activate them further. We want to build on the digital platform to attract international patients. Also, the insurance piece is going to come in handy and get activated. We have, as I said, on-boarded an international sales leader who comes with extensive experience in this field and we are confident that with all this collective effort, we should be able to increase international sales channels contribution in the future.

I spoke about this in brief in, in my previous slide on strategic pillars, but as I had mentioned, a big chunk of our growth will come from our existing network, existing footprint, but we are looking at brownfield expansion and greenfield expansion. Greenfield expansion could come through two routes. One is that we set up our own hospitals, or if we have good value accretive acquisitions in the pipeline. So between brownfield and greenfield expansions, we are anticipating that we will add about 1,000 beds to our current capacity and add 10 additional LINACs in the future.

This slide actually displays how we are thinking about improving profitability and ROCE and how the drivers are already in place and how we see the shift happening in the next four to five years. If you can see the correlation in this slide, the relationship between scale and profitability is quite evident. In the top bracket, if you see, there are hospitals with a monthly revenue of Rs.10 crore plus. In the top left-hand corner, if you see. Now, these mature hospitals pre-corporate costs are delivering an EBITDA margin of 25% and a ROCE of about 27%, which is a strong benchmark for mature facilities in our network. Now, the other important thing is we have seen the progress in this bucket in the last couple of years. In FY20, we had just one centre in this bucket. Today, there are three. The second bucket, if you see below that topmost bucket, is a group of 11 hospitals today, which are in the range of Rs. 5 crore to Rs.10 crore monthly revenue. Now, this bucket is also delivering an EBITDA margin of 20% pre-corporate

cost and a ROCE of 12%, which is quite in a reasonable state as well. But the good thing about this bucket is that it is growing at a very healthy CAGR of 18%. Now, in all these hospitals, most of the investments have been made. So the growth that we are talking about is going to come at a minimal incremental capital spend. And as these hospitals show growth as we, at a network level push and drive these levers for revenue and volume growth in the range of high teens, they will migrate to the first bucket. So the hospitals in the second bucket will migrate into the first bucket over a period of time, because of this push on growth and volume, and eventually improving the overall profitability and ROCE of the company. Even our smaller centres which are doing less than Rs.5 crore per month, revenue per month, as you can see in the bottom most bucket, are improving and will show a steady improvement over a period of time as we put all these levers together to improve volumes and revenue.

As we look ahead, our vision is very clear to ensure that HCG remains at the forefront of cancer care in the country, combining scale, compassion and scalability. We aim not just to
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be the largest cancer player in this country, but to be the most trusted with patient experience and superior clinical outcomes at the heart of what we do. Our focus will continue to be on profitable growth, underpinned by disciplined capital allocation. We are confident of delivering revenue growth ahead of industry, supported by an operating leverage that will drive EBITDA growth faster than historical CAGR that we have seen. And at the same time, our ROCE is expected to improve as the profile of our higher yielding centres improves in the company and that reflects the strength of our model and our capital efficiency. In a sense, we are building a platform that is not only delivering outstanding clinical outcomes for patients, but is also creating sustained long-term value for all our stakeholders. Thank you so much for your patience and listening in. With that, I end my presentation, and I will be happy to take any questions that you may have. Thank you so much.

Moderator: Thank you very much, Sir. Ladies and gentlemen, we will now begin the question-and-answer session. We take the first question from Gaurav Tinani of Ambit Capital.

Gaurav Tinani : Thank you for the opportunity and thank you for the presentation, sir. First question would be, you have laid out a strategy, but if you could speak more about the future outlook for the business, what kind of revenue growth do you expect over the medium-to-long term? If you can speak more about that, please.

Manish Mattoo : Yes, sure, Gaurav. Thank you for the question. See, over the last five years, we have consistently delivered revenue growth of over 15% per annum, and that has outpaced the broader industry trajectory. Looking ahead, we aim to sustain if not exceed this momentum and continue to grow faster than the overall market that is a key direction that we are going in and this will be driven by a couple of factors first, we want to realise the full potential of our existing network. There is the brownfield expansion in play in our existing markets and high potential markets. And we aim to improve our case mix. We aim to improve our payor mix going forward and that will lead to higher realisation and this will be underpinned by the fact that we are building and focusing on centres of excellence in most of our markets. So all of this should lead to the number that I just alluded to.

Gaurav Tinani : Understood. Another question would be, how do you see our EBITDA margin trajectory going forward? How do you see EBITDA margin improving going forward,?

Manish Mattoo : Right. Yes that is an important question. And I would, again, allude to our historical growth

of EBITDA in the last five years particularly, our EBITDA CAGR has been 18% per annum. And we expect our EBITDA to grow at a faster pace than this historical CAGR and which will be driven by a couple of measures again. We will be sweating out our existing infrastructure. We will be bringing in operational efficiencies. There will be operating leverage that will come into play by improvement in our payor mix, by improvement in our
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case mix. And as I had said the investment, whether on the opex or capex side has already been made to a large extent in these centres and that should improve our EBITDA CAGR going forward. And as I had mentioned in my presentation, our mature centres doing more than Rs.10 crore of revenue are already delivering an EBITDA of 25% pre corporate cost. So as more and more centres come into this bucket, our EBITDA margins would grow at a healthy CAGR, more than the historical number that I just discussed.

Gaurav Tinani : Okay, so with higher growth and EBITDA growing faster, and operating leverage also kicking in, how do you expect ROCEs or return on capital employed to look like in the next three to five years? What is the sustainable ROCE that you see in the business or the medium to long term?

Manish Mattoo : Yes, thanks. Gaurav, for that question. And see, I would like to talk about the centres today, the mature centres who are delivering more than Rs.10 crore per month. Our pre-corporate ROCE for those centres is around 27% and it is a strong benchmark for mature facilities. And we have seen progress that these centres are increasing in number, and there will be more centres getting added to this. So overall, I feel we expect a trend of at least a 20% plus ROCE over the next five years.

Gaurav Tinani : Got it. Okay, I have more questions and I will join back the queue you, sir, but wish you best in this new role. Thank you.

Manish Mattoo : Thank you, Gaurav.

Moderator: Thank you. We take the next question from Param Desai of PL Capital.

Param Desai : Thank you for the opportunity. Manish, if you can just highlight that if you see a lot of multi - speciality hospitals are also stepping up the investments in the Oncology. So how are we positioned against them and what will be our value proposition?

Manish Mattoo : Thanks, Param. Again, a very relevant question. See, I think as I alluded to in my presentation as well, cancer is a very complex disease to handle and it requires a super -speciality focus, which we have delivered and shown that clinical outcomes that we deliver are far superior to multi -speciality hospitals. Now, it is incumbent on us to actually deliver this message to patients over and over again and reinforce on this, which we see happening today. We get a lot of patients for second opinions, third opinions that come to our centres because they believe that what we deliver is definitely superior. Now, as this message percolates further, because the incidence is rising, awareness is rising, we feel a lot of volume shift happening to our single speciality centres , more than multi -speciality. So our value proposition is really the integrated care that we deliver through our diagnostics, precision medicine, multimodality Healthcare Global Enterprises Limited

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care, great technology, which delivers superior outcomes. And I had mentioned this too. Globally, if you see, it has been proven time and again by dedicated cancer centres being regarded as the gold standard. So, while there is the cancer burden is increasing at a very brisk pace. I think we do not have enough quality care beds in the foreseeable future as well, despite our own plan to expand. There is room for growth for everybody but we definitely hold an edge as far as outcomes are concerned, as far as quality of life is concerned for

patients, and

as far as survival rate is concerned for patients. And that is something incumbent on us to communicate more aggressively to the market.

Param Desai : Got it. And we alluded that we will be doing 1000 -bed addition, largely brownfield but what will be our approach to the greenfield -led or say inorganic route? Because historically, some of the new centres we have started in the last few years have not performed to our expectations. So what will you do differently on that part?

Manish Mattoo : Yes. So I think it is important to identify the right markets, the right spec, so that we can tie in the right clinical talent coming in because that is important in the initial phase. And, from the learning of the past, and the due diligence that we have done, as I said, we have identified a couple of markets where we feel confident of them being attractive, whether it is high cancer incidence, low penetration of cancer care beds, or availability of clinical talent. So , we have a pipeline of these markets where we will at least be, looking actively in to expand. And based on our past experience, I think we will be getting the specifications right, the size of the hospital right, the talent pipeline right, so that we can ramp up fast and deliver on the unit economics that I mentioned in my presentation, because we definitely have to have those metrics in mind as we go into any market.

Param Desai : Got it. And what is the kind of capital outlay we envisage in the next three to five years to achieve the growth targets we have mentioned?

Manish Mattoo : So, Param, over the last five years, HCG has incurred a total capex of Rs.660 crore, out of which Rs.350 crore was towards maintenance capex. Now, as the size of the network has grown, we expect maintenance capex to be in the range of Rs.90 crore to Rs.100 crore annually. In addition, we would like to spend additional Rs.500 crore to Rs.600 crore over the next two, three years to fund expansion . Each centre with our specifications will cost us about Rs.100 crore or in that range , depending on the market . We want to spend capex on increasing our existing capacity. And also, most importantly, is to upgrade facilities and infrastructure so that they are in line with patient expectations, post which we expect a total capex to be around 30% to 35% of our EBIT DA.

Param Desai : Got it. That is all from my side. If I have more questions , I will join again. Thank you so much.

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Manish Mattoo : Thanks Param.

Moderator : Thank you. Our next question is from Sumit Gupta from Centrum Broking Ltd. Please go ahead.

Sumit Gupta : Hi, good evening. Thank you for the detailed presentation. Sir a few questions. First is on the payor mix. Like over the next three to four years, how do you plan to optimize the payor mix currently as like in FY2025, your institutional mix was 34%. So can we expect that to reduce? And within this, how much is CGHS?

Manish Mattoo : Gaurav, thanks for that question. In that institutional business, CGH S is in the range of 12% to 13% from the institutional business. And as far as optimizing the payor mix is concerned,

as I had mentioned, it will be driven by a couple of factors. One is the kind of infrastructure that we are upgrading it to. Second how are you focusing on optimizing and enhancing patient experience significantly? Cancer patients have special needs. How we understand those needs and deliver on improving their experience through multiple cycles that they come to the hospital for? Third is, how do we activate our sales and marketing engine aggressively, especially the marketing part? Because as we speak and amplify our message around our clinical outcomes, around our superior clinical sets, around our technology, I think we can see that shift happening. So I think these are the couple of measures. And fourth

is obviously

the corporate awareness part. I think we have to spend a lot of time and effort on building awareness in the right pockets for us. And fifth, I would say is the day care centres that I referred to. These are adjacencies that will help us enter micro markets of affluence, increase the feeder into our major centres, and they come at a marginal capex investment. So I think these four or five measures, I would say dominantly the execution rigour has to be much, much better and I am very confident that we should be able to optimize the payor mix with these measures in place.

Sumit Gupta : Understood. And so, going forward, how are you building the team below you to drive the growth? And second is, are you looking to raise equity to fund future endeavours ?

Manish Mattoo : So as far as the team is concerned, Gaurav, we have added some new roles in the team and we have on-boarded some senior leaders, from a reputed organization in the sector into the team. As I said, and I want to just reiterate that, roles like Head of Clinical Strategy to drive focus on identifying good clinical talent, good oncologists across our network and on-board them because it is a constant process. It is a long process. Every good clinical hire takes anywhere between six to eight months from the start. So I think to drive focus on that, we have got this experienced person on board who happens to be a very senior clinician himself. Second is our, the Chief Marketing Officer who is going to drive a lot of work around patient communication and brand amplification and other hires like CIO, Head of Investor Relations, Healthcare Global Enterprises Limited

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and two heads for International and Domestic Sales. These are the kind of roles that we have got good people to join us for. And that is with the existing team, combined with the existing team members who had already been here, I mean this team coming together should be able to drive the next wave of transformation.

Sumit Gupta : Thank you, also, are you looking to raise equity to fund future expansion or any growth, basically?

Manish Mattoo : So, we are evaluating our equity and debt ratio. And at an appropriate time, we will take an appropriate call on that.

Sumit Gupta : Thank you. All the best.

Manish Mattoo : Thank you. We will take our next question from Ritika Khandelwal from Perpetuity Ventures LLP. Please go ahead with your question.

Ritika Khandelwal: So how well equipped is HCG to deal with the comorbidities and other complications that comes with cancer? And also, what would be our radiation, surgical and medical oncology split?

Manish Mattoo : Okay, so Ritika, as I had mentioned in my presentation, while we are oncology focused, but we do not do just oncology. We have other specialists too in our centres who take care of our patients, like intensivists, gastroenterologists, physicians, surgeons, orthopaedists, we have the whole gamut because oncology today is a multi-modality treatment and oncologists alone cannot handle that. So we are well equipped from that perspective. And you can see that in our outcomes as well. I spoke about the mortality rate that cannot happen, that would not have happened if we did not have the whole gamut of services to address patient needs. So that is point number one. As far as your question on the split is concerned, radiation is today 20% of our overall revenue, medical is about 42%, and the balance is driven by diagnostics and surgeries, surgical.

Ritika Khandelwal: Okay, thank you. And also one more question. Are you planning on closing any centres which you have identified which is not doing that well?

Manish Mattoo : So, Ritika, we want to refrain from giving centre-wise guidance today. We are looking at a cluster-based approach to our business. And all the clusters have done well in the past. However, to answer this question spe-

cifically, there have been concerns in the past about

some of our centres. And we are looking at ways and means to improve profitability and improve the growth there, like expansion of our clinical our team members, improving our clinical programs, activating our sales and marketing, as I said, overall. So that work is happening at a network level and at a central level as well. And hopefully, that should give

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us the necessary desired outcomes. But as I said, the centres that you perhaps were alluding to form a very small fraction of our overall enterprise revenue and volume. And as I said, going forward, we will be talking about the cluster level structure that I spoke about in the presentation.

Ritika Khandelwal: Ok, makes sense. Thank you.

Manish Mattoo : Thank you, Ritika.

Moderator: Thank you. We have also received some questions via text from Mr. Varun Bang from Bandhan Life Insurance. His first question is, what does gross mortality rate mean in our industry? Is there a standard practice of calculating this number across industry?

Manish Mattoo : So, the mortality rate is actually calculated as the number of deaths inside the hospital over the total number of discharges that we have in that hospital in that specific time frame. So that is how it is calculated. And it is a standard metric that is calculated across all hospitals.

Moderator: Thank you, sir. He has two more questions. Can you help us understand the broader pricing strategy? How are we thinking about pricing at present? And is there an opportunity to improve or recalibrate service pricing in certain regions based on market response and value delivered?

Manish Mattoo : So, from time to time, we undertake an exercise to understand where we are, in terms of pricing vis-à-vis our competitors, and currently also we have undertaken that exercise which should get over in the next couple of weeks, which will give us our positioning in each market, and then the opportunities that lie in front of us but it is a very scientific exercise, and we want to rely on data coming from proper due diligence. So we will take an appropriate call on that once we have that research and we want to do that exercise annually and not just every two, three years.

Moderator: Thank you. And sir his last question is how is the current incentive structure for doctors designed? And are there plans to revisit or modify it going forward?

Manish Mattoo : I think many of our doctors are on a retainer fixed model. And I think that is the preferred model that we have in our system, people who are full timers with us. But of course, different markets have different nuances, like the Western cluster, many of the doctors prefer to be on a fee for service model, which is a reality and constraint of that market, and we have to align to that but in many of our centres , the model is a fixed retainer model and unless there is a necessity and need, we do not intend to change that going forward. And multimodality suits a full -time model. Since we have a multimodality model of clinical care, we want to stick to Healthcare Global Enterprises Limited
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that model so that doctors remain focused on patient outcomes and patient care and sometimes the incentive model can be an undesirable distraction.

Moderator: Thank you, sir. We will take the next question that is from Kunal Randeria from Axis Capital. Please go ahead with your question.

Kunal Randeria : Yes, good evening. And thank you for the opportunity, sir. So just, what explains the fact that some of the multi -specialty hospitals have seen their revenue growth to be slightly faster than your growth? Is it to do with the case mix? There may be some higher end surgeries, a lot of patients in metros prefer to go there and maybe follow up chemo or prefer to come to a city.

Is it something like that? Or what can you attribute it to?

Manish Mattoo : I think the 15% revenue growth that we have delivered at a network level, I would say is marginally higher than industry. There may be one odd player who is probably delivered a higher revenue growth because of the certain markets that they are present in. That could be one of the factors responsible.

Kunal Randeria : Sure. You in the presentation mentioned the number of centres that you have, right?

Manish Mattoo : Yes.

Kunal Randeria : But can you share because I am sure not all centres would be having all the services on offer because it would be very expensive, to have doctors across all modalities in one centre .

Manish Mattoo : Right.

Kunal Randeria : So, perhaps that could also explain the reason. But going forward, what should we take? So for example, the South, a Bangalore and a Chennai would have a lot more service offerings from your end vis -a-vis, let's say, Ongole or a Hubli or a Gulbarga. Would my understanding be correct?

Manish Mattoo : So the way we approach these is that in our all mature centres and all, we have comprehensive services and doctors are on full -time basis. But in some of our centres , like you said, Ongole and all, they will be on a visiting models , but see the comprehensiveness is not compromised.

In some of the smaller centres and all it may not be possible to get a full time endocrinologist, for example, but we will have a visiting endocrinologist, because at no point is clinical care compromised and I have said it again, at this cost of sounding repetitive. If you look at our mortality rates, they are far superior to multi-speciality hospitals without doubt. So, in none of our centres is comprehensiveness compromised and we have clinician availability and the full-time or the part-time mode I may just a model may just vary, but otherwise all the centres have these service offerings.

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Kunal Randeria : And just one more, if you do not mind, can you explain how the mature centres that is centres, let's say which are two to three years old, how do they grow and what are the growth drivers after let's say three years?

Manish Mattoo : 1x asset turnover in four to five years and that is the typical growth trajectory we take. Also a 15% to 20% EBITDA margin in the same time period and 15% to 20% ROCE in the same time period.

Kunal Randeria : I am sorry, I meant to ask not from a financial perspective, but after three years what are the drivers? Do you kind of, let's say, add more specialties there or, it is just a normal price hike driven what exactly, because I am assuming volumes will kind of taper off after three years.

Manish Mattoo : Yes, you are right. See, it is driven by a couple of levers. One is capacity expansion, right?

Second is addition of clinical teams. In many of our centres, we had to start with two medical oncologists in a centre of, say, 100 beds. Today, we have four. Third is specialty mix changing. In one of our centres, for example, we did not have a bone marrow transplant unit sometime back in the initial phases but after two, three years, we added the bone marrow transplant, which really contributed significantly to the revenue ramp up. Fourth is addition of technology from LINAC, we move to a CyberKnife, we move to a Tomotherapy, from regular surgeries we move to a surgical robot. And from chemotherapy, we move along that curve to immunotherapy, and so on and so forth. For example, infrastructure upgrade, payor mix change. So I would say all of these factors keep on getting added. And, every two, three years, and changing the revenue profile of the centre but I think essentially, it is about deepening our clinical teams and deepening our complexity of the programs that we run. So these are the two drivers that really improve the revenue

profile of the centres in every three, four years.

Kunal Randeria : All right. Ok, sir thank you very much and all the best.

Manish Mattoo : Thank you. We take the next question from Rahul Agrawal from EverFlow Partners. Please go ahead.

Rahul Agrawal : Thanks for the opportunity. Manish, you alluded to the growth outlook for the company. Can you also detail like what sort of growth do you expect from organic same centres that you have today versus the expansion in newer centres that you are planning? And second, over the next three odd years, what sort of EBITDA margin do you aspire to get to at a corporate level?

Manish Mattoo : Thank you for that question. See, as far as we are looking at the growth four or five years out, our big part of our incremental revenue, about 70% -75% of our incremental revenue is going

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to come from existing centres, right? 10% -15% is going to come from brownfield expansion, and about 8% -10% is going to come from the greenfield expansion that we have, planned for.

So that is the split of our incremental revenue coming in. As far as EBITDA margins are concerned, we are at 19% in Q2. We have delivered an EBITDA margin of 19% in Q2. And as I had said, based on our higher-than-market revenue growth and operating leverage, we expect the margins to grow faster than the historical trend. Obviously, it is upwards of 21%, 22%. We expect to be in that range at a corporate level.

Rahul Agrawal : All right, thank you. And also on the capex, I do not think I got the numbers right. Can you just clarify that again? Over the next three odd years, what sort of capex plans did you highlight? Did I get the number of Rs.600 crore right? And how much of that would be maintenance capex on existing centres versus what you expect as greenfield capex?

Manish Mattoo : So in the next two to three years, it is about Rs. 600 crore to Rs.700 crore, out of which about Rs.300 crore will be maintenance capex. And the rest will be for upgrading our infrastructure, increasing our existing capacity, and so on and so forth. So that is the number we are looking at.

Rahul Agrawal : Got it. Rs.600 crore over two to three years and Rs. 300 crore being maintenance and the rest being incremental facilities and brownfield.

Manish Mattoo : That is right. Thank you so much.

Moderator: Thank you. Our next question is from Rajas Joshi from Chrys Capital. Please go ahead.

Rajas Joshi : Yes, thank you. Firstly on some centres , as someone alluded to earlier, some centres , especially, for example, the South Bombay Centre , have been a drag on the profitability metrics. So, given how some cities like Mumbai , the treatment protocol is different, what would be your strategy going ahead for these specific cities and these specific centres to be precise?

Manish Mattoo : Thank you for your question. See as I had mentioned earlier as response to another question earlier also, we will refrain from giving central level guidance going forward but however, to quickly answer your question. South Mumbai has historically faced so me challenges in ramping up as compared to our other centres , but if you look at the overall Bombay cluster today, HCG is now among the top oncology players in that market. But at a network level, we are working through several initiatives to improve the performance of our centres , as I had mentioned, by augmenting the clinical talent, by introducing new programs, by improving our efficiencies in that centre , upgrading the infrastructure or technology, or international business, for example, which is very relevant for South Mumbai. All these put Healthcare Global Enterprises Limited

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together should help us turn these centres around. But as I said, we are looking at a regional level performance of the clusters and not necessarily at the cen

tre level.

Rajas Joshi : Understood. Secondly, on Milan n, you earlier alluded that looking at all the strategic options. So, what do we have a sense on the options available there?

Manish Mattoo : I think one of the options, while we are still evaluating, just to be very clear, could be divestment of this vertical. But again, not a definitive answer at this point in time, because we are evaluating various options.

Rajas Joshi : And lastly, on margins. So you have given a lot of qualitative colour o n margins. And you also answered, the previous participant a bit on the number side. So I mean, breaking down the margin expansion into probably cost and reali sations. How, is there room for price hikes per se in some of our existing regions when we benchmark ourselves to our peers?

Manish Mattoo : That is right. There is, in our mature markets, there is definitely an opportunity and we will keep evaluating these opportunities every year. And that some similar exercises actually is already underway right now as we speak across the network.

Rajas Joshi : Okay and if I look at some of your peers, right, their margin profile is somewhere in the, let 's say, 25% to 35% odd range. And you just mentioned that, you are looking at somewhere close to around 21% -22%. So, I mean, what would really be driving that delta from 22% to let 's say 25 , 30 odd percent? I mean, any specific reasons that you would like to call out for that and how we can further move up the ladder there?

Manish Mattoo : So in near term, as I had given the margin guidance, that is the near term guidance of 21% to 22%. But as you see, some of our mature centres are delivering 27% -28% margin as well on a pre -corporate cost basis and that is the long term vision for the company overall. And in my presentation, I alluded to the path towards that as well. So with more focus on these levers, I am sure we are hopeful of driving towards that number in the long term.

Rajas Joshi : I understood. Got it. All right. And so this margin profile that you earlier alluded to, that is for the next three odd years, you reckon?

Manish Mattoo : I would say this journey is around four to five years. That is the timeline we have in mind for delivering these margins similar to mature centres on a pre -corporate cost basis .

Rajas Joshi : Okay. And in terms of costs related margin, so be it in terms of procurement or otherwise, any leve rs there on that side as well? I mean, that you would like to exploit and something coming from there as well probably?

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Manish Mattoo : That is right. Procurement is going to be also one of the levers that we are going to work on to improve margins.

Rajas Joshi : Okay. Alright. Thanks a lot.

Moderator : Thank you. We take the next question from Ankush Mahajan from Sanctum Wealth. Please go ahead.

Ankush Mahajan : Sir, most of my questions are answered. Just trying to understand, sir, what kind of reali sation growth exactly we can expect in upcoming years? What kind of the inpatient volume growth that we can look for the existing centres in the upcoming years?

Manish Mattoo : Overall, we are looking at 4% to 5% reali sation improvement every year. And the volume growth will be in the region of 9% to 10%. And that is what we are anticipating in the near

to midterm.

Ankush Mahajan : And sir this Rs.600 crore of capex plan, how could we see the funding about this Rs.600 crore capex and debt position in next three to four years?

Manish Mattoo : Net debt to EBITDA ratio is two -and-a-half times today and we are comfortable with that right now. And a lot of this capex funding is going to come from internal accruals, because we are mindful of this leverage ratio. We are able to fund our growth capex through a combination of free cash flow and available borrowing headroom, because we want to prudently maintain our net

debt to EBITDA threshold at 2 to 2.5x on a pre -Ind AS basis in the near term.

Ankush Mahajan : Thank you sir. That is from my side.

Moderator : Thank you. We take the next question from Dheeresh Pathak from Whiteoak Capital . Please go ahead.

Dheeresh Pathak : Yes. Hi. Thank you. I am referring to the slide that you had shared, slide number 38, which gives a cut off , the facilities in terms of revenue per month, right? So you show about Rs.10 centres which are less than Rs.5 crore a month and 11 centres which are between Rs.5 crore to Rs.10 crore a month. So what is the vintage of these two buckets and are there particular reasons that you identified that despite long vintage they are in this bucket? Is there a particular reason or is just that these are not matured? Because my impression was that not many new assets have been added in the portfolio. So the vintage alone cannot be the reason. There must be other reasons. So what are the other reasons?

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Manish Mattoo : The bucket which has centres doing more than Rs.10 crore per month are about 10 years plus vintage and the middle bucket that I mentioned are about six to seven years.

Dheeresh Pathak : What about the last bucket?

Manish Mattoo : It is less than five years that mostly but it is not just a function of vintage alone, it is also a function of size. So some of the centres may be old but may be smaller in size and hence delivering less than Rs.5 crore per month revenue. Ongole for example is an example which is an old centre but delivering less than it because of the size of the hospital.

Dheeresh Pathak : Okay , thank you for that answer. There is one slide, slide 35 which talks about the revenue potential so at an aggregate portfolio level the yellow bar on the right hand side if I understand this correctly you are saying that we are doing Rs.2 ,200 crore revenue in FY2025 and that is only 56% of our potential revenue right.

Manish Mattoo : At the current pricing levels.

Dheeresh Pathak : That you mean at a theoretical level or at a revenue which is at, something which is practically doable?

Manish Mattoo : It is practical and doable because we have taken optimal occupancy and utilization. We have not taken 100% occupancy and 100% utilization. These are practically doable revenue numbers at existing price points.

Dheeresh Pathak : Given what is the time frame that you have in mind to achieve that potential then?

Manish Mattoo : Four to five years.

Dheeresh Pathak : Four to five years. Why is that? Because I know it sounds, maybe I do not understand the nuance of the business that much, but given that a lot of our assets, most of the assets would be at least five years old vintage. So a lot of the hospitals that we are seeing, they will start going towards mature economics towards five years. So, and you are saying there is demand supply gap, there is higher incidence of cancer. So why is it going to take longer? Because these are not new assets you are talking about. These are most assets which are more than five years old. And correct me if I am wrong. So why does it take five years out then if there is so much demand supply gap, good brand equity, good clinical outcomes?

Manish Mattoo : So I think the timeline I gave is on the outer side. Of course, in some of the centres where the investments in some of the markets which are expanding rapidly, in some of the centres where the investment has already been made, this centre -wise realization can happen sooner than that. But at a network level, because some of the centres will require some upgradation, some

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of the centres will require some payor mix optimization, which may take some time but that time frame that I gave you was at an outer level. But we could achieve, this potential,

a higher

potential in some of the centres in the near term as well in next two to three years, like, for example, Ahmedabad, we have transitioned to a new hospital there with 100 new beds and

all. So I think that that will also take some time, but largely at a network level, that is the timeline I had in mind, but some centres will mature faster.

Dheeresh Pathak : Thank you so much.

Moderator : Thank you. Sir we have received a text question from Mr. Rishit Parikh from Nippon India Mutual Fund. The first question is, capital has been a constraint in the past. How do you plan to address that? Split of 1K beds between green and brownfield, which hospitals have scope?

Manish Mattoo : Yes. So as far as capital is concerned, we have actually conserved capital in the last five years. Our net debt to EBITDA ratio, if you see, has come down quite significantly in the last five years. And to answer your next question, the split between brownfield and greenfield on those 1,000 beds, I think brownfield will add about 700 beds through that route. And in markets like Ahmedabad, Vizag, Baroda, Cuttack, so that is our brownfield expansion plan. And as far as greenfield expansion is concerned, in newer cities like Pune, Surat, Varanasi, Kanpur, to name a few, while it is just indicative, we have identified 10 to 12 cities in that bucket, that will be about 200 to 400 beds.

Moderator : Thank you, sir. His second question is do we have capital availability for greenfield or acquisitions and is there a capex number for those investments? Is there a timeline to reach 21%-22% margins?

Manish Mattoo : Yes for our greenfield expansion will largely happen through our internal accruals so we are very clear about it. And as far as margin forecast is concerned, as I had mentioned earlier, that is the timeline of roughly three years that we have in mind to reach that number.

Moderator : Thank you. Sir we have a question from Shyam Srinivasan of Goldman Sachs (India) Securities Pvt Ltd. His question is, you have served previously as Head of Apollo Karnataka. What are the key differences on operations, clinical, focus culture that you noticed when you took over this leadership role at HCG. What are key learnings that you plan to bring from your previous experience?

Manish Mattoo : Well, I think culturally, both organizations are very similar, if you ask me, because the focus is on hiring of good clinical talent, developing clinical programs, high on ethics and integrity, and really focusing on clinical outcomes. And also, expansion at a pan India scale. These are similarities. Differences on account of the fact that Apollo is a multi-specialty chain vis-à-vis Healthcare Global Enterprises Limited
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vis, HCG being a single speciality oncology-focused chain, there are some differences in terms of the variables at play in both these organizations in terms of scale and size. But other than that, if you ask me, not much of a difference as far as the core DNA is concerned. Both organizations are focused on driving outcomes. Both organizations are compassionate and focus on compassion, scale, and clinical excellence. So that has been my learning in the last five months that I have been here.

Moderator: What are the key learnings that you plan to bring from your previous experience?

Manish Mattoo : Yes, I think the big learning in my stint at Apollo was that how to deepen clinical programs, and how to make organizations attractive for clinicians to come on board. What kind of support do the clinicians need really, when they transition from a different organization to yours? How do you help promote them? How do you help them with technology, the right infrastructure in place? And how do you really scale up medical programs? That really was my learning. And that was the reason we were able to deliver what we were able to deliver in that three, four-year

time frame that I was with Apollo. And how do you leverage the brand?

How do you really create brand resilience around clinical outcomes, clinical excellence, outreach. So many learnings, really to speak, which I am hoping I will be able to implement effectively at HCG.

Moderator : Thank you. Ladies and gentlemen, that brings us to the end of question-and-answer session. I now request Dr. Mattoo to add a few closing comments. Over to you, sir.

Manish Mattoo : Thank you so much, everyone. I think it was a wonderful interaction and a very interesting and insightful set of questions. I really enjoyed presenting HCG's vision to you and I look forward to interacting with you in the future and really deliver on the numbers that we have spoken about, really excited about the path that HCG is taking now in partnership with KKR and I look forward to more such fruitful and engaging interactions. Thank you so much. Have a good evening.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Feb 2026

HealthCare Global Enterprises Limited

Reg Off: HCG Towers #8, P. Kalinga Rao Road, Sampangi Ram Nagar, Bengaluru - 560 027

Corp Off: #3, G -Floor, Tower Block, Unity Buildings Complex, Mission Road, Bengaluru - 560 027, Karnataka, India

91 80 4660 7700 | Email -query@hcgel.com | Website - www.hcgoncology.com | CIN: L15200KA1998PLC023489

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February 13 , 2026

National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Dear Sir/Madam,

Subject : Transcript of the Earnings Call held with Analysts/Investors on February 09, 2026

Stock Code : BSE – 539787, NSE – HCG

Reference :

Regulation 30, Part A of Schedule III of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Please find attached herewith the Transcript of the Earnings Call held on February 09, 2026 , with Analysts/Investors to discuss the Unaudited Financial Results of the Company for the quarter and nine months ended December 31, 2025.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Request you to take this on record.

Thanking you,

For HealthCare Global Enterprises Limited

Sunu Manuel

Company Secretary & Compliance Officer

Encl: a/a.

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Healthcare Global Enterprises Limited

Q3 & 9M FY26 Earnings Conference Call

February 09, 2026

Moderator Ladies and gentlemen, good day, and welcome to the Healthcare Global Enterprises Limited Earnings Conference Call. I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you, sir.

Anoop Poojari Thank you. Good afternoon, everyone, and thank you for joining us on Healthcare Global Enterprises' Q3 and 9M FY '26 Earnings Conference Call. We have with us Dr. Manish Mattoo, Executive Director and CEO of the company. We would like to begin the call with opening remarks from Manish, following which we will have the forum open for an interactive question -and-answer session.

Before we start, I would like to point out that some statements made in today's discussion may be forward -looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier. I would now like to invite Manish to make his opening remarks.

Manish Mattoo Thank you, Anoop. Good afternoon, everyone, and thank you for joining us today.

I hope you have had a chance to review our Q3 & 9M FY26 results presentation.

We are pleased to be engaging with you once again following our recent strategy

interaction as we continue to focus on strengthening HCG's core oncology platform and driving consistency across clinical and financial outcomes. Over more than two decades, HCG has established itself as a focused oncology network with a Pan -India presence, deep clinical expertise and a multidisciplinary model of care. Consistent clinical outcomes supported by technology and tumour -board -led decision -making remains central to how the organization operates. Building on this foundation, our strategy is centred on creating the most trusted outcome -led oncology platform in India through a single specialty model. Our emphasis remains on optimizing the existing network by deepening clinical programs, improving utilization and mix and strengthening referral and patient engagement. Growth is expected to be driven primarily through existing centres , supported by brownfield expansion in high potential markets and calibrated greenfield

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additions over time. This approach is underpinned by a calibrated capital allocation framework, a sustained focus on patient experience and operating leverage as scale builds across the network. Before we get into the financials, I want to briefly highlight the clinical capability that underpins HCG's performance and long -term moat. HCG operates a comprehensive cancer care model, and our strength lies in managing high complexity cases at a large scale. In the last quarter, this capability was demonstrated across specialties. As an example, in radiation oncology, we treated a tumour completely encircling the optic nerve using most advanced version of Cyberknife, which achieved a tumour reduction without any loss of vision, highlighting the precision of our advanced radiosurgery platforms. In medical oncology, we achieved a complete response in a rare BRCA2 -positive, HER2 -positive PIK3CA mutated breast cancer, which is a subtype that typically responds poorly to treatment. This reflects our depth in molecular oncology and personalized treatment protocols. From a governance and research standpoint, our K. R. Hospital received NABH accreditation for its ethics committee; a highly selective recognition that strengthens our credibility in advanced clinical trials and research -led care. In surgical oncology, our robotic platforms continue to expand the boundaries of curative care. A 97 -year -old patient with advanced colon cancer underwent robotic R0 resection; and in another case a 10 -hour robotic prostate surgery involving rectal encroachment also achieved R0 resection. In neuro -oncology, for example, we performed diffusion tensor image -guided awake brain surgeries in critical regions, maximizing tumour removal while preserving neurologic function. Alongside clinical excellence, we are also focused on sustainable and responsible oncology. We launched a green radiation oncology initiative to reduce energy consumption and carbon footprint and introduce low -dose restorative radiotherapy for inflammatory and degenerative joint conditions, opening up new evidence -based applications of radiation therapy. Importantly, this clinical strength is now being efficiently monetized through our digital engine. In Q3 FY26 , digital revenue grew 26% year -on-year despite a meaningful reduction in paid media spend. Our own channels strengthened materially. The website remained the primary contributor at 67% of the overall digital contribution, while the mobile app scaled 4.5x to contribute 13% to our digital revenues. Campaign efficiency improved sharply with campaign -led revenue growing 38% year -on-year and increasing its share from 20% to 27%. While traffic declined due to external disruptions in Google Ads, outpatient volumes grew 26% and inpatient volumes grew 37%, clearly indicating higher

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conversion, better case mix and superior demand quality. We also reduced aggregator dependence with their share declining from 12% to 9%, improving revenue quality and long -term margins. Taken together, this reflects a repeatable, system -driven model where clinical excellence creates trust and digital capability converts that trust efficiently into high -quality volumes and dominant market share in core catchments. Turning now to our Q3 financial and operating performance. Q3 is typically seasonally softer for the health care services industry. Despite this, HCG delivered revenues of INR 633 crore, representing a year -on-year growth of 13.4%. Excluding the fertility business, HCG delivered revenues of INR 618 crore, supported by healthy patient volume growth of 8% year -on-year, underscoring

resilient demand for oncology care across the network.

In addition to volume momentum, revenues benefited from improving utilization across centres and a steady mix of complex cases. This demonstrates continued progress in strengthening clinical

programs and expanding service depth across hospitals, particularly in larger and more mature clusters.

During our strategy discussion, I had mentioned that our existing network has achieved about 60% of its revenue potential, and there continues to be headroom to grow within these hospitals, which we are seeing in our quarter-on-quarter performance.

Average revenue per patient, including our fertility business during Q3FY26 stood at approximately INR 84,000, marking a year-on-year increase of 5%. ARPP trends were influenced by case mix dynamics and regional payor mix variations alongside the ongoing scale-up of services across the network.

Adjusted EBITDA for Q3FY26 was INR 111 crore, reflecting a 20% year-on-year growth with EBITDA margins expanding to 17.5% from 16.5% during Q3 FY '25. Margin expansion during the quarter was driven by operating leverage as the utilization of the hospitals improved. This was achieved through continued investments in clinical talent, technology and infrastructure, which remain essential to supporting long-term growth. Going forward, margin expansion at HCG is expected to be driven by scale benefits and a gradual improvement in case and payor mix.

From a regional perspective, performance across clusters remained healthy. In the South, the cluster delivered a 9% year-on-year revenue growth, driven by continued momentum at our Bangalore cluster and Vizag, despite temporary disruptions in Andhra Pradesh related to the state-sponsored scheme. These disruptions impacted volumes during the quarter but were resolved within the

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quarter and the region is now back on track.

Excluding Andhra Pradesh, the South cluster grew volumes at 11% year-on-year in Q3 FY '26. During this period, the case mix continued to improve, supported by a higher contribution from high-end medical oncology work, including immunotherapy and CAR-T cell therapies, particularly at the Centre of Excellence in Bangalore. In addition, a relatively favourable payor mix with lower exposure to scheme volumes in Andhra Pradesh supported ARPP growth for the cluster. On the growth front, preparations for launch of our North Bangalore and Whitefield Green projects are progressing well. The North Bangalore facility with a planned capacity of over 120 beds is expected to commence operations by the end of Q4 FY '26, with clinician hiring largely completed. It will be the first in Bangalore to offer MR-LINAC technology, creating a strong clinical and technology-led differentiation.

Additionally, we will also be adding about 20 beds to our existing Centre of Excellence in Bangalore by reconfiguring the specs of the hospital, and we believe this would further add to our profitable growth.

In the West, the cluster delivered strong revenue growth of 17% year-on-year, supported by robust patient inflows across hospitals in Gujarat and Maharashtra. Performance was led by healthy demand across centres and continued strengthening of our regional footprint. Volume growth during Q3 FY26 stood at 11% year-on-year, driven by the expanded capacity in Ahmedabad, addition of key clinicians and focused sales and marketing initiatives. These factors together supported higher throughput and improved utilization across hospitals, contributing to the overall momentum in the cluster.

Moving to East. The cluster reported a 12% year-on-year revenue growth, driven by strong patient inflows in Cuttack and Ranchi alongside the continued ramp-up of operations in Kolkata. Volume growth during Q3 FY26 stood at 16% year-on-year, supported by sustained demand and expanding reach across catchment areas. ARPP, however, declined by 3% year-on-year, primarily due to transition in the Odisha state government scheme and a case mix change. However, this impact was largely offset by a strong volume growth.

During Q3 FY26, cluster leadership in the East

was further strengthened with the

appointment of a new regional business head, bringing over three decades of experience in East India markets, which is expected to support execution and

performance consistency going forward. I am also happy to announce that in line with our earlier communication of brownfield expansion in existing high-potential centres, we will be adding about 60 beds to our Cuttack Hospital, which is

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expected to be operationalized by the end of FY '27.

Overall, the company exited December on a strong footing across clusters with 9M FY '26 revenue at INR 1,893 crore and a year-on-year growth of 16% and adjusted EBITDA of INR 346 crore with a year-on-year growth of 20%. The EBITDA margin for the YTD period is 18.3%, an improvement of 60 basis points versus the same period last year. We expect operating momentum to continue into the March quarter, supporting solid full year performance.

In terms of returns, pre-tax ROCE stood at 13.3% for the 9 M FY '26. We expect ROCE to continue to trend higher over time as incremental growth can be achieved with relatively limited capital deployment, enabling returns to improve as fixed costs are absorbed more efficiently. Our focus remains on achieving the full potential of our existing network, disciplined capital allocation and a faster ramp-up of newer centres, which together provide a clear pathway to sustained improvement in returns.

I would like to reiterate that there is sufficient headroom and incremental growth in existing centres to improve ROCE. Also, in the near to medium term, a number of centres will reach threshold size, enabling this margin and ROCE expansion. From a broader perspective, it is important to emphasize that while quarterly performance can vary with differences in scale, utilization and seasonality, the structural drivers of the business remain firmly intact. Demand for high-quality cancer care continues to grow faster than supply and the resilience we have seen even during a seasonally softer quarter reinforces our confidence in the underlying strength of the business.

To summarize, Q3 FY26 represented a steady financial and operating execution, continued progress across regional clusters and improving scale and clinical depth. We remain focused on scaling responsibly, strengthening clinical leadership and improving operating leverage over time while staying anchored to our core objective of delivering superior outcomes for patients. With that, I will conclude my opening remarks and would request the moderator to open the floor for questions. Thank you.

Moderator Thank you very much. We will now begin the question-and-answer session.

The first question is from Ali asgar Shakir from Motilal Oswal Mutual Fund. Please go ahead.

Aliasgar Shakir Yes, thanks for the opportunity. Hi Manish. Thank you so much for the detail and the explanation of the quarter and outlook. I have three questions. First question is on growth. Now, in the previous call, the analyst call that you did, you had

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indicated that you would aim to grow at about 15%+. Now when I break it up, there are three - four levers in which you are exploring growth. Point number one, you mentioned even today that the potential of the existing facility is only 60%.

So, there is a lot of scope to grow from here, right?

Second thing is, we have a plan to add about 1,000 beds from, I think, close to about 2,500 -2,600 beds. So, cumulatively, if I see the operational bed where we are probably 2,000 -odd to the overall bed capacity that we will increase to probably about 3,500 that could be about close to almost over three to four years, a CAGR of approximately high teens growth that we can get from here, plus ARPP growth, as you have mentioned, about 5% - 7%.

So, when I add all of this, I see that there is a potential to grow at probably about 22% - 25% ann

ually for the next three - four years. So, when you are guiding 15%+ growth, is it that we are a bit conservative over here? Or are there any risks in the business because of which you are building lower growth that we are not able to see? If you can just throw some light here.

Manish Mattoo Thanks for that question. I think our, of course, ambition is obviously to grow higher than what we have committed. But looking at historical trajectory, looking at the competitive intensity, the expansion will not happen at once. I mean, it will be in a phased manner. Looking at the pricing sensitivity around cancer care, we believe a 15%+ growth should definitely be achievable. But of course, we will gun

for -- our ambition is -- or aspiration is to do more than that.

Aliasgar Shakir Got it. So, I am not asking probably from a short term, but from a three -year point of view, certainly growth could be much higher, right? With the capacity addition that we are doing and the potential in our existing facility.

Manish Mattoo We continue to maintain our guidance of 15% +. But as I said, our aspiration would be to do better than that. But as of now, the guidance stays the same.

Aliasgar Shakir Got it. Second question on margin. Again, here as well, the room to improve in terms of margin from probably about the mid -high teens that we are, to close to about where the competition is, upward of above 25% is fairly high. So, while I understand that, I mean, there is a lot of existing capacity that we are trying to fill. But would you say that in the long term, probably three to four years, the potential to reach mid -25% is there? Or inherently, do you think that the stand -alone facility would have lesser potential for margin improvement?

Manish Mattoo So, Ali, as I said earlier also, if you see the trajectory this year, the fact is that we started at 17.5%, are trending at 18.5% for the year so far. And the trajectory will continue to be upwards. So, our aspiration, of course, is to be 23% - 24% plus in the three -to-four -year period, and we are very confident of delivering that.

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But comparing exactly like -to-like to multi -specialty, that is one way to look at it. The other way is to see how the trajectory of the margin is, from 17.5% to go up to 24% - 25%, I think is quite steep, which we are gunning for. And I think that trajectory is what I think should be interesting rather than the absolute margin itself. And I have said it in the past also, some of our mature centres are at 26% - 27%, and they have delivered that margin consistently over the last couple of years. And we are sure that eventually, the organization will converge towards that.

Aliasgar Shakir Understood. This is very clear and very useful. Last question, sir, you have announced Board meeting for Rights Issue . And I am just trying to understand, I mean, our leverage position is very, very comfortable. And the growth plans also that we have, I think, should be very well managed. So, if I do the math, probably about September, we are about INR 680 crore, right, of net debt.

And probably if the kind of growth guidance that we have, we should be landing at a pre -Ind AS EBITDA of upwards of INR 500 crore probably next year, which is, I mean, less than 1.5x of net debt to EBITDA, which should definitely take care of our payments coming up for Vizag next year plus any maintenance capex.

And still leave you with at least INR 200 crore to INR 300 crore of cash available for further growth capex. So, I am just trying to understand what is the reason for this fund raise? And do we have any concrete plans to, I mean, utilize these funds for growth?

Manish Mattoo So, Ali, we believe that an equity infusion at this stage will strengthen the company's balance sheet and the capital structure. And that is directionally why the rights issue is there. But at this point in time, that

is all I can divulge. But very

shortly , we will share the specifics after our Board meets.

Aliasgar Shakir Understood. Thank you so much. Really helpful. I will come back in the queue.

Moderator The next question is from Nitin Agarwal from DAM Capital. The next question is from Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal Hi. Thanks for taking my question. Manish, on our growth plans, which are -- I mean, when you think about growth, obviously, apart from when you talked about the brownfield and the greenfield, so which are -- if you can probably just characterize what are the sort of areas in the current units in the current network where you would like to add capacities over the next couple of years? And what kind of characteristically are the newer markets, you probably would want to reach out to in terms of adding up new units?

Manish Mattoo Yes. Thanks, Nitin, for your question. See, I mean, we are very confident of our

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platform, which is the USP of our platform, which is to deliver clinically superior outcomes, treat high complex cancer cases and deliver the best clinical outcomes in the country today as far as cancer care is concerned. And hence, as I said, marrying that with the existing capacity, there is enough headroom for growth in our existing hospitals at our current utilization rates.

So, the growth will happen across the centres , whether it is South, East or West,

the major cities being Bangalore, AP, Mumbai, Ahmedabad, Ranchi and Kolkata. Those will be our main drivers for growth. But in addition to that, I mentioned earlier also, there are two very exciting greenfield projects coming up in Bangalore itself, which is our core market and which has done very well in the past, one in North Bangalore and one in Whitefield. Between the two of them, we will be adding about 150 beds.

Now besides that, we are also looking at newer markets. There is a pipeline of cities that we are looking at in the West, which is a growing territory for us. And also, some of them in the South, too, and that was in the investor deck. So, I would say there are 4 - 5 cities too beyond our existing markets where we feel we can grow. And the brownfield expansion, as I said, will happen across our major centres in Ahmedabad, in Bangalore, in Cuttack and in Mumbai as well.

Nitin Agarwal Okay. And if you can just give us a little bit more update on how the progress has been in the business in the Mumbai and the Kolkata markets, because I think these were two large metros where we have entered relatively recently, and there has been always this question around what could be our competitiveness versus the larger multi-specialty players in the bigger cities?

Manish Mattoo Mumbai market has done exceptionally well for us. We have grown quite strongly in the whole of last year at high teens. And not just in revenue terms, but in EBITDA terms as well, it is been an exponential growth for us. And we feel Mumbai market will continue to deliver for us in the years to come. So hence, the brownfield expansion in that market is on the cards. In Kolkata, I mean, overall, the East market has done well for us.

Kolkata, we have kind of consolidated our position now. It will need infusion of additional clinical talent because we feel East market, Kolkata being the central and eastern part of the country, has a lot of potential. We made a change in the leadership there, and we are confident that with that change, we should be really able to ramp up our Kolkata business, too.

Nitin Agarwal And when you look through our network, what proportion of the network, if there is a way to characterize that, I mean, where we are probably significantly below our, what you call optimal EBITDA levels, where there is an opportunity rather to really move up meaningfully. So, what proportion of our revenue would you

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characterize

would probably fall in that bracket?

Manish Mattoo So, see, both East and South clusters are doing very well for us as far as EBITDA is concerned, some centres are delivering more than 25% - 26% of EBITDA. The West, we have seen a significant ramp-up happen in the last year or so, which continues to be slightly lower than our average EBITDA. But I think with our growth plans, our investment plans in that market, we should be able to ramp it up closer to our average company EBITDA, even exceed that number.

Nitin Agarwal And lastly, on -- once you sort of have taken over the business, while there are these unit level opportunities which are really there, but are there any sort of corporate level -- network level low-hanging fruits that you can target in terms of profitability improvement that you can probably highlight?

Manish Mattoo Yes, of course. So, we have started work on that already. In the last four -five months, we have started work on several value creation initiatives around revenue leakage, reducing revenue leakage, improving conversions. There is work that is being done on procurement optimization, and we have seen that flow into our margin and EBITDA as well.

We are working on enhancing our clinical density. So, these are three -four initiatives which we are driving from the corporate level, and we are already seeing impact of that in our expanded EBITDA margins across clusters. We are also, across our network, we have a large pool of data coming in from patients. We see about 70,000 - 80,000 patients annually that we treat in our network, which is a huge pool of data. We are using that to drive more data analytics and AI, improve our outcomes, and that is another initiative that we are driving besides the ones that I mentioned earlier.

Nitin Agarwal Thank you so much.

Moderator Thank you. The next question is from Devang Patel from Sameeksha Capital. Please go ahead.

Devang Patel Sir, could you delve a bit into what factors led to slower growth in the Southern cluster during Q3FY26? And again, if I heard you correctly, you mentioned excluding AP, the growth was 11%, whereas in first half, the growth was almost

19%. So, would our growth, excluding these one-off factors, be again similar to H1? That is my first question.

Manish Mattoo So, Devang, thank you for that question. As I mentioned, our growth, excluding the normalizing for the AP effect, was 11% year-on-year. Bangalore cluster did really well. The strike did impact our volumes because the Vizag market is a big market for us where we are the dominant players. The strike continued for

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twenty - twenty-five days, and it did impact the subsequent month volumes as well because for cancer, that is the kind of nuance we have.

For a YTD basis, you are looking at a growth of 16% volume growth year-on-year, which also had a base effect of MG Hospital Vizag, which we acquired last year. But going forward, we expect the growth that we have mentioned about 8% - 10% definitely happening in the subsequent quarters with Vizag ramping up and Bangalore continuing to do well.

Devang Patel Sorry, I did not understand, what kind of continuing growth we would see in this cluster?

Manish Mattoo So, going forward, volume growth of about 10%, we are expecting from this market in the subsequent quarters. This will further get augmented as North Bangalore gets operationalized at the end of this financial year.

Devang Patel Okay. Understood. My second question was on gross profit margin. In the previous two - three years, it was hovering around 75%. And this quarter or last two - three quarters, it is been trending down and it is at 72% now. Could you explain what is leading to lower gross profit margin?

Manish Mattoo So, Devang, it is largely driven by our case mix change, which is favouring medical oncology. The ramp-up in medical oncology and pharmacy costs has been

slightly

margin dilutive, but we are working towards increasing our case mix and improving our payor mix, which should offset that in the ensuing quarters. And - but the benefit of medical oncology is that we also draw operating leverage from it, which contributes positively to our EBITDA. And our main focus is obviously to drive EBITDA margins upwards. And there, I feel it is net-net a positive impact.

Devang Patel Okay. Just lastly, I request that with the change in reporting format, we have done away with bed or occupancy-related metric. If you could just help us add that in the presentation to give us a sense of capacity utilization for every quarter.

Manish Mattoo Yes. So, Devang, so as we had mentioned, going forward, we will stick on two metrics, which is largely around volume and ARPP for cancer patients because they have a long-term association or dependence on the hospital. So, the long-term value realization is of more importance to us than the episodic treatment, which gets captured in ARPOB. Hence, we would like to stick to these metrics, and we will continue to report progress on them every quarter.

Devang Patel Fine, sir.

Moderator Thank you. The next question is from Gaurav Tinani from Ambit Capital. Please go ahead.

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Gaurav Tinani Hi. Good day. So, first question is, have we seen any impact of the new CGHS norms, excluding treatment of some high-value chemotherapy drugs during Q3 FY26? And secondly, also with the GST cut on a lot of these medications, has that impact also been felt in this quarter on the revenues and EBITDA, please?

Manish Mattoo Yes. Thanks, Gaurav, for the question. So, I would say as far as GST is concerned, there was a marginal impact on the top line as well as margin and we were able to offset that by some initiatives that we took within the organization around pricing and consumption. So that's been offset because of that.

As far as CGHS is concerned, yes, there was some immediate impact, but we have been able to offset that too by changing consumption patterns in our hospitals.

And also, the fact that overall, at an overall company level, it does not contribute much to our business for now, it is a neutral impact, so to speak. A lot of work has happened on these two fronts to make it -- make sure that it does not impact us going forward as well.

Gaurav Tinani Got it. Separately, with this strike in Andhra, can you just quantify how many days did that kind of affect the business? And did that also have an adverse impact on the working capital also being stretched at least for Q3 with some receivable days going up and that payment should happen in the next few quarters?

Manish Mattoo So, it did have a material impact on our volumes, because about -- we lost about twenty -twenty five days of working, and that did have an impact on the November volumes as well. But now that growth is coming back to us. I mean, that effect has been normalized. As far as receivables are concerned, in fact, we have got a good flow of money coming in from that segment.

The fact that Q3 had a billing of about INR 25 crore, but we had a net receivable addition benefit of INR 5 crore. So, net-net, there is a positive flow from the government side as far as the receivables are concerned. And Q4 also looks very good from that perspective for us for that market.

Gaurav Tinani Got it. Very encouraging. Last two clarifications. So, I think first one would be, there was a -- within this quarter, there was an announcement of onetime amendment to the founder's contract of INR 2 crore. So, is that onetime impact in the Q3 results? And secondly, it is a resolute margin for this quarter despite these disruptions or transient impacts. Q4 with the new Bangalore Greenfield coming on, would that be a temporary drag that you see in Q4 margins vers us Q3?

Manish Mattoo So, to your first question, the impact of that

amendment will come in Q4. We have

not taken that in Q3. And to your second question, obviously, Q4 is trending better than Q3. And our hope is that it will be the best quarter for us this financial year.

But the impact of North Bangalore, we will only be able to see in Q1 coming in

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because from the time of operationalizing to the business impact will probably take us a quarter.

Gaurav Tinani Got it. Thank you so much. All the best. I will join back the queue.

Manish Mattoo Thank you, sir.

Moderator Thank you. Next question is from Sumukh from Korman Capital. Please go ahead.

Sumukh Hi. So, my question is on the operating margins and the ROCE sir. So, it is been historically low when I compare it to a multi -specialty hospital. So, is there anything structurally different in cancer care specialty hospital because it is more equipment heavy and you have relatively smaller hospitals, which is leading to lower margins and ROCE's?

Manish Mattoo Yes. So, see, traditionally, if you see historically, many of our Centres of Excellence have done extremely well when it comes to ROCE. They are clocking an ROCE of 28% - 29%, close to 30% also. So, structurally, I do not think there is anything wrong. It is just that 50% of our centres, if you see today they are around eight to ten years old.

And that is why the impact that we will see in the ensuing years as they mature and we see more operating leverage coming in, the ROCE will significantly improve. And we have stated that as well. So, I feel it is not a question of any fundamental issues with the business. It is a question of the maturity of our centres that is impacting the ROCE to this level.

And second point is that with our capex outlay that we have planned now for the next four -five years, after FY '27, the EBITDA growth rate will outpace the capex deployed -- capital deployed growth rate by a good margin, and that will flow directly into ROCE. So, we feel we are confident of delivering the ROCE margins that we have committed in our Investor Day deck of about 20% in the next four to five years.

Sumukh Okay, sir. And this 30% margin, sir, at what maturity would the hospital ideally be achieving this?

Manish Mattoo So, we have seen that when a centre crosses INR 10 crore per month revenue, the ROCE impact improves significantly. And we have seen that trend in the last year as well. So, going forward, as more and more centres mature, I feel from a current 13.5%, we should be progressively moving towards 20% margin at a company level. By maturity, I mean the revenue threshold, not the age of the hospital.

Sumukh Okay. Thank you.

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Moderator Thank you. The next question is from Rajat Srivastava from Tata Mutual Fund. Please go ahead.

Rajat Srivastava Thanks for taking my question. Manish, my first question is that what percentage of your revenues is coming from patients who have probably had the first line of treatment or first treatment in some multi -specialty hospitals and for whatever reason, they were not happy with their treatment, so they have come to HCG for

that. And I just want to understand how much of your revenues are coming from those patients? Because you claim that probably you are one of the best in cancer care.

Manish Mattoo Yes. Rajat, very interesting question. So, actually, a good chunk of our patients come in after having taken an opinion at a multi-specialty hospital. I can come back to you with the exact figures, but my estimate is around 45% patients come, which is a large chunk. But going forward, we want to kind of focus on the first opinions also strongly because that is a big pipeline that we see growing. While our Centres of Excellence will get the second and third opinions, which is really the moat that we build on. But I think going forward, this will come down a bit, while the overall pie will increase.

Rajat S

Rajat S All right. And also, what percentage is medical tourism for you? And probably if you can tell us what are you doing to promote that? Because that turns out to be a big thing for Indian health care system.

Manish Mattoo Currently, it is about 3.5%, Rajat, and our endeavour is to take it to about 7% in the next four years. The way that we are thinking about is that there are some traditional markets which have channelled into HCG hospitals. And today, oncology comprises about 25% - 30% of our overall - of the overall medical value travel in the country. So, it is the largest, fastest-growing specialty on MVT on an overall basis, and we feel this segment is going to grow even further. And hence, we are deploying the resources, both people and infrastructure in our major six to seven markets to draw into this growth.

Rajat Srivastava But any reason, Manish, that this number is at 3% right now? Because I think for other hospitals, it is closer to 10% already. So, was it not a focus area for us till now and it is now? Like, can you elaborate on it?

Manish Mattoo So, yes, I mean, for us, domestic has done extremely well, and that was the focus area. But we feel that since oncology, post-COVID particularly, has grown rapidly. And hence, our focus is now on leveraging on this channel as much as possible. And one more factor why we have not done so well is that our centres, except for Bangalore centre are not located in metros, which typically get the bulk of these patients like Delhi and Chennai, for example.

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So, with now our expansion in Bombay, for example, and Kolkata expanding its footprint and increasing in size and Bangalore, of course, is our Centre of Excellence, we are confident of increasing this piece of business for the organization.

Rajat Srivastava Great. Manish, if I can squeeze one more. So, you mentioned that your bed centres are operating at 25% - 26% EBITDA margin levels and probably closer to 30% ROCE. Can you tell us what is the number of beds - out of the 2,200 beds which we have currently, how many beds would be operating at that level?

Manish Mattoo So, Rajat, if you can -- I can come back to you with the exact number, but our investor deck has a reference to this, so you may want to check that. Otherwise, we can come back to you with details, more specific details. I do not have that number offhand. The exact number of beds.

Rajat Srivastava Got it. All right. Thanks, Manish and all the best.

Moderator Thank you. The next question is from Aditya Khemka from InCred Asset Management. Please go ahead.

Aditya Khemka Hi, good afternoon. Beyond the two facilities in Bangalore, those are mentioned in the investor presentation, can you share some more granular details on where we should expect the incremental beds to come from?

Manish Mattoo So, Aditya, the incremental beds will come from across the organization, as an example, Vizag; Kolkata; Cuttack, I have mentioned; Ranchi will have some bed expansion; KR Road, which I said Bangalore, our flagship; Ahmedabad; Bhavnagar and Borivali. So, I would say across seven-eight centres, we will see bed expansion varying between 20 - 60 beds.

But the large chunk will come from Bangalore. The largest chunk will come from Bangalore.

Aditya Khemka Right. Over the last 3 years, incremental part of our revenue was from the established centres and the mix was north of 80% - 85%. On our outlook on directional improvement in margins, how do we think about balancing growth and margins going forward? Whether we should be balanced about the fact that there is going to be material growth in new beds, which could impact EBITDA margins going forward?

Manish Mattoo So, going forward, about 80% of our margin growth, in absolute terms is going to come from existing centres . About 10% - 12% is going to come from brownfield expansion in the cities that I mention

ed and about 5% - 6% will come from greenfield expansion, which is going to be a very calibrated and prudent approach as far as capital allocation is concerned.

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Aditya Chheda Got it. Those are my questions. Thank you.

Moderator Thank you. Participants who wish to ask questions, may press star and one. The next question is from Aryama an Pawar from Prudent Investment Managers. Please go ahead.

Aryama an Pawar Yes. Hi, sir. So, you had called out the 15% + sales growth number. So, can you maybe just break this down into ARPP growth and volume growth? That will be really helpful. And maybe if you could call out future capex for FY '27 and '28, that will be really helpful?

Manish Mattoo So, our -- the breakup for the 15% + is about 10% will come from volume growth and about 5% will come from ARPP growth. For capex this year, our expected closure is about INR 275 crore - INR 280 crore of capex and next year, probably about 10% - 12% higher than that.

Aryama an Pawar Okay. Sure. Thank you.

Moderator Thank you very much. That was the last question in queue. I would now like to hand the conference over to the management team for any closing comments.

Manish Mattoo Thank you so much. I would thank everybody who participated and asked some very insightful questions. And it was a wonderful interaction and hope that we continue to exchange these insights from time -to-time, not just on these calls and look forward to mee ting you again next quarter. Thank you so much. Have a wonderful day. On behalf of team executives, we would like to thank you all. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: May 2026

HealthCare Global Enterprises Limited

Reg Off: HCG Towers #8, P. Kalinga Rao Road, Sampangi Ram Nagar, Bengaluru - 560 027

Corp Off: #3, G -Floor, Tower Block, Unity Buildings Complex, Mission Road, Bengaluru - 560 027, Karnataka, India

91 80 4660 7700 | Email: query@hcgel.com | Website - www.hcgoncology.com | CIN: L15200KA1998PLC023489

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May 26, 2026

Dear Sir/Madam,
National Stock Exchange of India Limited,
Compliance Department,
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai - 400051,
Maharashtra, India BSE Limited,
Compliance Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai - 400001,
Maharashtra, India

Subject : Transcript of the Earnings Call held with Analysts/Investors on May 20, 2026 to discuss the Audited Financial Results of the Company for the quarter and year ended March 31, 2026

Stock Code : BSE – 539787, NSE – HCG

Reference :
Regulation 30 read with Schedule III of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Please see attached herewith the Transcript of the Earnings Call held on May 20, 2026, with Analysts/Investors to discuss the Audited Financial Results (both Standalone & Consolidated) of the Company for the quarter and year ended March 31, 2026.

This is also available on the website of the Company - <https://www.hcgoncology.com/investor-relations/>

Kindly take the intimation on record.

Thanking you,
For HealthCare Global Enterprises Limited

Sunu Manuel
Company Secretary & Compliance Officer

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Healthcare Global Enterprises Limited
Q4 & FY26 Earnings Conference Call
May 20, 2026

Moderator: Ladies and gentlemen, good day, and welcome to the Healthcare Global Enterprises Limited Earnings Conference Call.

I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you, sir.

Anoop Poojari: Thank you. Good afternoon, everyone, and thank you for joining us on Healthcare Global Enterprises Q4 and FY26

Earnings Conference Call. We have with us Dr. Manish Mattoo, Executive Director and CEO of the company; and

Mr. Ravi Gothwal, Head of Investor Relations. We would like to begin the call with opening remarks from Manish, following which we will have the forum open for an interactive question -and-answer session. Before we start, I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier. I would now like to invite Manish to make his opening remarks.

Manish Mattoo: Thank you, Anoop. Good afternoon, everyone, and thank you for joining us today. I hope you have had the opportunity to review the results presentation for the fourth quarter and full year ended March 2026. FY26 has been an important year for HCG. It was a year in which we strengthened the core of our oncology platform, sharpened our strategic focus, improved the resilience of our operating model, and took several steps to position the company for its next phase of profitable growth. Particularly, a couple of things that I would like to share.

FY26 was a year of profitable broad -based growth. Quality of growth improved supported by higher volumes, better realization discipline, and ongoing payor mix optimization. We sharpened our focus on oncology, strengthened the balance sheet, and are deploying capital into higher returns markets. FY27 growth levers are also visible through our planned initiatives.

Before we get into the update on financial performance, I would like to take a minute and highlight a few milestones achieved during

this quarter and FY26. First, we commenced operations at our North Bangalore facility. This is a strategically important addition in one of HCG's core markets and strengthens our presence in the rapidly growing northern corridor of Bangalore. The facility has been designed as a comprehensive oncology center with capabilities across medical, surgical, and radiation oncology, nuclear medicine, PET -CT imaging, and BMT. It also brings MR -Linac technology to the Bangalore market, creating a clear clinical and technology -led differentiation for HCG. Second, HCG's clinical differentiation continues to be reflected in the complexity and uniqueness of cases managed across its network. During the year, our teams delivered several highly specialized interventions, underscoring the

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depth of our multi -disciplinary capabilities and our innovation -led approach to oncology care. These included a rare robotic -assisted endoscopic urological procedure, which was subsequently published in an international journal; the management of complex dual primary malignancies, including a case involving base of tongue cancer followed by aggressive metastatic prostate cancer, where discordance between pathology markers and clinical findings made the diagnosis particularly challenging; and an ultra -personalized immuno -radiation strategy for a centenarian patient with advanced cancer, in which the team combined immunotherapy with PULSAR radiotherapy and achieved meaningful tumor regression , despite the patient's age and clinical complexity.

In addition, advanced approaches such as cytoreductive surgery with HIPEC continue to be deployed across the network for complex ovarian cancer cases, further reinforcing HCG's ability to manage highly specialized oncology interventions through coordinated multi -disciplinary care. These examples are not isolated successes. They reflect the culture of clinical excellence and scientific thinking embedded within HCG. Increasingly, our ability to combine specialized expertise, tumor board -led care, advanced technology, and personalized treatment approaches are helping us manage high -acuity, high -complexity cases that strengthen both patient outcomes and HCG's position as a differentiated oncology platform.

Third, we have further strengthened the management team. I am pleased to welcome Sanjeev Kumar Saxena as Chief Financial Officer ; and Ravi Gothwal as Head of Investor Relations. Both bring with them relevant experience in healthcare services and will add depth to our finance, capital allocation, and stakeholder engagement capabilities. Ravi is present with us on the call today and Sanjeev will be joining the team shortly at the end of this month. With these appointments and with the regional and other corporate leadership additions which we have made throughout the year, we now have a stronger execution engine to support the next phase of growth.

Fourth, we successfully completed the rights issue of INR425 crore. The strong response from shareholders reflects our continued confidence in HCG's strategy and long -term opportunity. I would like to thank all our shareholders for their support.

The capital raised has strengthened our financial foundation and gives us greater flexibility to invest behind our long-term priorities, including capacity expansion, clinical infrastructure upgrades, technology investments and selective growth opportunities, while continuing to maintain a disciplined return -focused approach. This also positions us well for any organic, inorganic expansion in the future.

Finally, we have taken a strategic decision to sharpen our focus on oncology. In line with this, the board has approved the sale of Milann, our fertility business, to Invi ga Healthcare Fund. The transaction was signed yesterday and is expected to close within Q1 of FY27.

The transaction values Milann at an enterprise valuation of INR63 crore. The equity consideration for 100% of the business, including certain transition services support is INR37.6 crore, of which 75% will be received upfront at closing and the balance 25% over 18 months. We believe this is consistent with our stated strategy of focusing management bandwidth and capital on HCG's core oncology platform. I will be happy to address any questions on this transaction during our Q&A session.

Alongside clinical capability, we are also strengthening the digital layer of the business. Our objective is to improve the quality of patient acquisition, strengthen conversion, enhance patient experience and reduce dependence on external channels over time.

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Digital is increasingly becoming an important enabler for HCG, particularly in urban and se

mi -urban catchments,

where patients and families actively seek credible clinical information before deciding where to initiate treatment. By strengthening owned channels, improving campaign efficiency and building a more seamless patient journey, we are working to convert the trust created by our clinical brand into high-quality volumes across the network.

Turning now to our financial performance. For Q4 FY26, HCG delivered revenues of INR652 crore, reflecting 11.3% year-on-year growth. For the full year FY26, revenues stood at INR2,545 crore, representing 15% year-on-year growth. Excluding the fertility business, full year revenues grew 15%, supported by 12% growth in patient volumes and a 3% improvement in average revenue per patient.

This growth reflects the strength of our distributed oncology network. We saw healthy patient inflows across regions, better utilization in several centers and continued progression of hospitals into higher revenue cohorts. Importantly, the growth has been broad-based rather than dependent on a single center or cluster, which reinforces the scalability and resilience of the platform.

The South cluster contributed approximately 39% of FY26 revenues and grew about 13% for full year. Growth was supported by continued momentum across Tier 2 centers and steady performance in key markets. Q4 growth was moderated by softer medical value travel and intentional paring down of low-margin business. However, the cluster remains well-positioned with the launch of North Bangalore, planned bed additions at the Bangalore COE, and planned initiatives to improve medical value tourism.

The West cluster remained our largest revenue contributor, accounting for approximately 45% of FY26 revenues and grew at about 14% for the full year. Performance was led by strong patient inflows across Gujarat and Maharashtra, capacity expansion in Ahmedabad, clinician additions, and focused commercial initiatives.

The East cluster contributed approximately 11% of FY26 revenues and grew at about 11% for the full year. Cuttack and Ranchi continue to perform well, while Kolkata is expected to benefit from on-going clinician onboarding. The cluster offers meaningful growth potential as utilization improves, clinical team in Kolkata ramps up, and the planned Cuttack expansion progresses over time.

Our international business in Kenya, while still a smaller part of the portfolio, delivered strong growth over the year. Revenue grew 39% year-on-year for Q4 and 71% for the full financial year '26, supported by ramp-up in radiation oncology, PET services, and stronger patient inflows.

From a profitability perspective, FY26 reflects the benefits of scale, maturing centers, and improving network productivity. Adjusted EBITDA for Q4 FY26 stood at INR125 crore, up 17% year-on-year, with margins expanding by 90 bps to 19.2%. For the full year, adjusted EBITDA stood at INR471.1 crore, also up 19% year-on-year, with margins improving by 70 bps to 18.5%. This margin improvement is driven by operating leverage, better utilization, throughput optimization, and disciplined cost management.

Our pre-tax ROCE improved to 14%, supported by higher utilization, center maturity, and disciplined capital allocation. We continue to believe that as more centers move into higher revenue buckets and incremental growth is generated from existing infrastructure, HCG has a clear pathway to improving both margins and return over time.

On the balance sheet, the rights issue and sustained cash generation have strengthened our financial position. FY26 operating cash flow conversion remained healthy at 75% and net debt reduced meaningfully to 1.4x during the

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year. This gives us the ability to pursue growth with greater flexibility while remaining prudent on leverage and capital allocation.

Looking ahead, our priorities remain clear. We will continue to optimize the existing

network by improving

utilization, strengthening referral ecosystems, enhancing patient throughput, and deepening high-complexity clinical programs. On the expansion front, our near-term emphasis is on brownfield opportunities in high-potential centers, where incremental capital can be deployed with faster ramp-up and better returns.

Selective greenfield additions will be pursued where the market opportunity, clinical need, and strategic fit are compelling. The launch of North Bangalore and Whitefield, the planned Cuttack expansion, and other identified brownfield additions across the network are all consistent with this approach.

With that, I will conclude my opening remarks and request the moderator to open the floor for questions. Thank you.

Moderator: Thank you. We will take the first question from the line of Himanshu Binani from Anand Rathi Share and Stock Brokers Ltd.

Himanshu Binani: Yes, so, thank you for taking my questions and congratulations for a good set of numbers, sir. So my first question

was regarding the revenue growth. So given Q4 is typically a seasonally strong quarter for the business, so how one should actually interpret the 11% revenue growth reported during this quarter?

Manish Mattoo: Thanks, Himanshu, for the question. You are right, Q4 is typically a stronger quarter than other quarters. See, but

you have to see it in two aspects. One is for the year, the revenue growth has been 15%, and in Q4, there were two factors, one was there was a conscious paring down of low-margin business.

You know, I think we have been stating it for a couple of months now and, we have taken measures to bring down the low-margin business, which has impacted the revenue growth. There was also a factor of the Middle East conflict impacting the medical value travel to our West and South cluster, which additionally impacted the revenue growth. However, both these factors are being worked upon.

You know, -Vinayak Nayak, who's joined to lead international business as Head of International Sales, and the rest

of the team are working aggressively to bring back the international business to the earlier estimates. And also, this transition from low -margin to high -margin will be actually normalized over the next couple of quarter . And in the medium term, our estimate remains close to the 15% growth projections that we had given earlier.

Moderator: Thank you. We will take the next question from the line of Abdulkader Puranwala from ICICI Securities Limited .

Abdulkader Puranwala: Yes, thank you for the opportunity. So my first question is with regards to the shifting of the Whitefield project.

Can you help us understand what exactly, you know, we meant about an alternate location? And subsequent to that, you know, when we are talking about adding 200 beds at the five locations, can you help us quantify exactly, you know, how many beds are we adding across each of these cities?

Manish Mattoo: Yes, thanks, Abdul. So to your first question, the Whitefield existing infra that we have is slightly smaller as per

our specs. Whitefield is a much bigger market and we feel there is going to be enormous demand in that market

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and we want to be prepared for that from an infrastructure perspective. Hence, we are looking at an asset which will provide us at least 120, 130 beds.

And that is why, you know, there is this alternate location that we are seeking to realize the potential of that large market. So that is one. As far as brownfield expansions are concerned, so I will give you center -wise breakup. In Cuttack, we are adding 75 beds. In Ranchi, we are adding 30 beds. In Vizag, we are adding 50 beds and in Bhavnagar , we are adding 20 + beds. So that put together should 200 + beds over the next 24 months.

Abdulkader Puranwala: Understood, sir. And sir, in terms of, you know, the fundraise and then now we are also divesting

Milann centers.

So sir, I mean, going ahead, how should we look at the debt part of the business? I mean, is there any target internally in terms of net debt to EBITDA or net debt to equity that we are immediately targeting for the next one or two years, considering, you know, we also plan to add some beds here?

Manish Mattoo: So the net debt to EBITDA ratio has, come down favourably. Earlier it was 2.2 x, 2.3x. It is come down to 1.4 x as

of today and this gives us enough headroom to invest in capacity addition, expansion, brownfield, technology additions and all. And we will continue to be mindful of that ceiling. Internally, we have kept it at 2.5 x and I think in the medium to long term, that is where it will be.

Abdulkader Puranwala: Got it. And sir, just last one if I may . So among, you know, the four regions where we are operating, just

directionally if you could help understand that, you know, where is the scope you see, you know, for further margin expansion in terms of, you know, the delta which can be brought into for the next two to three years?

Manish Mattoo: I think we have identified opportunities for margin expansion across clusters. But largely, I think it will come from

West and East more, because South is a mature cluster with margins already 25% plus. But we will see opportunities for margin expansion all across.

Abdulkader Puranwala: Okay. And sir, just one more follow -up from my end. So when we talk about a 15% kind of growth expected for

next year and, you know, subsequent to that, and if I look at your current occupancy at the southern cluster, you know, that is already slightly higher. So would it be fair to assume that ahead, you know, we would have after Bangalore gets added, we would still have some headroom for new bed addition into this one?

Manish Mattoo: So the growth will happen through multiple levers, not just occupancy enhancement. You know, we have identified

multiple levers for growth. One is, of course, North Bangalore is going to be a big driver for growth. There will be Whitefield. There is brownfield expansion that I already alluded to. Plus, there is work happening on clinical talent addition. You know, in Q4 alone, we have added about 23 oncologists to our network and we will see the growth coming from that addition in the subsequent quarters.

There is also work happening on sales acceleration. We are doubling our Day -Care centers. Currently we have five, we are adding about 8 to 10 Day-Care centers over the next 24 months across multiple markets. There is also work happening on ORC outreach. Then, of course, on the cost side, there is opportunity on procurement, on leases that we are exploring. So I think collectively there are many levers that are play at play here which will help us expand margin meaningfully over the next year.

Abdulkader Puranwala: Got it, sir. I have no more questions, so I will get back in the queue.

Moderator: Thank you. We will take the next question from the line of Himanshu Binani from Anand Rathi Share and Stock

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Himanshu Binani: So sir, on the cluster -wise revenue we see, so we have seen that the Western cluster has reported the highest

revenue while the utilization has been like around 50% versus the consolidated 58% sort of like utilization. So just wanted to understand the utilization as in what exactly the utilization you are talking about is the occupancy or is the overall hospital equipment's included?

Manish Mattoo: Center utilization is a slightly different metric from occupancy, and it factors in other metrics also like the chemo

beds utilization, the OT utilization, Radiation machine utilization. So it is overall, I would say, a blended metric capturing the potential revenue going forward. And you are right, in Ahmedabad, for example, at 50% center utilization, there is headroom for growth in a very rapidly expanding and a

very important market for us.

Himanshu Binani: Okay. And sir, lastly from my side, if I actually look into Slide number 18 on the ramp-up of the new facilities

moving up the value chain both in terms of revenue and margins in FY26. So can you please elaborate which of these centers have actually moved from the low bucket to the upper bucket and ahead which ones as per the management have the highest possibility of moving up the value chain going forward?

Manish Mattoo: As we had stated earlier also, we are for now refraining from giving center-wise details, I would appreciate if we

can stick to that. But suffice to say that both the buckets are growing as per our expectations and will continue to grow further.

Himanshu Binani: Sure, sir. Thank you.

Manish Mattoo: But suffice to say that both the buckets are growing as per our expectation and will continue to grow further.

Moderator: Thank you, sir. We will take the next question from the line of Nitish Rege from ChrysCapital.

Nitish Rege: Yes. So, sir, you mentioned that, you are going to do selective greenfield expansion. Any update on the greenfield expansion pipeline other than the two Bangalore facilities? Any geographies we are particularly focused on for this expansion?

Manish Mattoo: So Nitish, there are at least 10 to 12 cities that we are evaluating options in and depending on market potential,

location availability, talent pipeline availability, we will be finalizing, you know, few locations ever so often. But as of now, there are several examples I can give you between a Pune, Surat, Coimbatore, Nellore, Lucknow, Jalandhar. There are several options that we are evaluating to finalize our greenfield options.

Nitish Rege: Okay. And in terms of, you know, inorganic growth, are we in talks in advanced talks with any target companies?

Any update on that front?

Manish Mattoo: There are I mean, options being evaluated currently also, but there are no major updates to be shared with the community right now.

Nitish Rege: Okay, okay. And just, you know, how do you see the margin trajectory evolving? Do you believe the company can do another 100 basis points improvement in EBITDA margin let's say in FY27? And, you know, what's our outlook on the shift in the payor mix? Where do you think, you know, by what time will this optimize for us?

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Manish Mattoo: So this last couple of quarters, we have seen the payor mix move up favourably by 100 basis points. And the margin

expansion also, if you just take Q4, has expanded by about 90 basis points versus last Q4. And with the measures that we have taken and that we will continue to take both on the quality of revenue side and other things that I mentioned earlier, and on the cost side, the margin expansion is achievable and we have demonstrated that over the last two quarters. So I am quite confident of delivering on that estimate that we had shared earlier.

Nitish Rege: Okay, okay. Thank you so much.

Moderator: Thank you. We will take the next question from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Thanks for taking the question. Manish, so from your perspective since the time you have taken over the business.

If you can, you know, sort of just, you know, tell -- let us know in terms of the way the business was being run, you know, prior to the promoter changeover and management change. I mean, where are the big opportunities that you see, you know, from a discrepancy perspective where, you know, we can create maximum amount of value for the business. And where are the primary low-hanging fruits on this account that you would like to pursue, you know, as we go forward here?

Manish Mattoo: Yes, thanks for your question. I do not want to comment on what was the earlier priority and all, but definitely growth was there. But today, as we look at growth, we are focused on profitable growth.

There is focus on revenue

growth, but we want it to come in a holistic way. So the focus is on margin expansion, improving returns, improving quality of earnings, and stronger cash generation, which we clearly have seen in Q4 and, very confident that the trend will continue.

And the levers that I mentioned earlier also about holistically looking at improving the quality of revenue that we do, improving the clinical talent addition, focusing on clinical differentiation. I think that is our biggest moat and that we will continue to build across the markets. We want to use our geographic expansion to our advantage, being present in 18 cities, with 25 hospitals, that is a huge network and we want to leverage on it.

Coupled with that, you know, the new expansion that I mentioned, North Bangalore, expansion in the other territories that I mentioned, addition of clinical talent, sales acceleration, expansion through Day-Care centers, outreach center. I think there are several levers that we are working on today. As I said, focus being not just on growth, but being on profitable growth, being on improving returns and stronger cash generation.

The growth has come through in this quarter also. If I look upon for the full financial year, it is driven by better center utilization, by improving clinical productivity, by focusing on costs. So I think coupled with these factors, I would say the priority is on holistic profitable growth going forward.

Nitin Agarwal: And Manish, you know, on the point that you mentioned about focus the priority being more profitability -focused

driven growth rather than just chasing growth for top-line perspective. But does that put us at a disadvantage versus, you know, some of our, you know, larger peers, multi-specialty hospitals who have got the ability to invest more aggressively behind oncology? And that seems to be honestly the trend across all the listed hospital chains where oncology investments are a key priority for -- from investment perspective across most of those guys.

Manish Mattoo: I think we are very confident of our clinical differentiation and the moat that we have created over 35 years. We

are leaders in most of the markets that we operate in. And while there will be competition, but I think we have enough created enough legacy, outcomes, and brand recall in the market to sustain that growth. Secondly, there is

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an inherent demand that is being generated in the market, you know, quite rapidly because of higher incidence of cancer.

So I would say we will continue to grow, quite well in the next couple of years given the fact that we have such a strong legacy and the growth levers that I mentioned before. So I do not think that is going to be a challenge.

Nitin Agarwal: Lastly on the growth part, you know, 15 -20% growth that you have guided to, given our revenue base, is there a

possibility that it can get meaningfully stepped up from here? What can really drive up, if assuming that kind of inflection can come in our growth from?

Manish Mattoo: I think, as we deepen our presence in different markets, there is potential for a higher growth or, you know, inorganic

expansion through that route. I mean, we can ramp up the growth further.

Nitin Agarwal: Okay. Thank you so much.

Moderator: Thank you. We will take the next question from the line of Santosh from FundVeda.

Santosh: First of all, congratulations on your good set of numbers that you have delivered and I see clear efficiencies that you are building in in the hospital. My question is regarding the interest cost. So, we have been hovering around INR40 crore of interest cost each quarter. So, with the rights issue funds and remaining funds, do we expect that interest cost would reduce in the upcoming quarters?

Manish Mattoo: You are right after the rights issue and, you know, we have repaid some long-term debts, we do expect some

improvement, some reduction in the interest cost. Our average interest cost is at 8%

Santosh: Okay. Got it. Thank you, sir.

Moderator: Thank you. We will take the next question from the line of Shashank Goyal from Vyomara Capital.

Shashank Goyal: Thank you so much. My question is, what sort of occupancy are the various hospitals in our system operating at?

Manish Mattoo: For the for your benefit, if you refer investor presentation slide 17, we have highlighted that a center utilization of

58% today, which reflects future growth potential. I mean, that is the metric we are tracking now against occupancy because we feel this is actually a blended metric that combines our utilization across modalities and this gives a better picture of how you should view our utilization rate overall. So, we are at 58% of our growth potential.

Shashank Goyal: Okay. And my next question is, like, can we continue to deliver same growth for the next two, three years on the

in the hospitals which are already there or will we start running into the constraints of capacity?

Manish Mattoo: I think for the next couple of years, our existing hospitals with expansion within them will deliver at least 75% to

80% of our growth and the rest will come from newer centers.

Shashank Goyal: Okay, okay. Thank you. That is all from my side. Thanks.

Moderator: Thank you. We will take the next question from the line of Devang Patel from Sameeksha Capital Private Limited.

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Devang Patel: My first question was on growth in revenue per patient. You mentioned earlier that we are reducing lower-margin

business. So why is that not reflected in a better growth on realization? And secondly, in the West also, the trend across quarters is there is a tapering down of growth. If you can talk about on these.

Manish Mattoo: See, as I stated earlier, growth is not our only objective. Growth has to be accompanied by margin expansion, returns, better returns, improving quality of earnings, and, you know, stronger cash generation, which we have demonstrated can come even at a lower revenue. So, we have to consciously, you know, we have taken those calls consciously, which may have a short-term impact on revenue, but will have a long-term beneficial effect on all the other metrics that I mentioned. And that is how it is an intentional strategy to go in that direction and that is what we will continue to do for some time. And that we have done across clusters, actually, South, West, and East, with

equal success.

Devang Patel: When you guide to 15% revenue growth, would you be looking at 5% inflation, medical inflation going forward?

Manish Mattoo: Yes, Over time, we will replace the low margin business with a higher -margin, higher -realization business, which

will improve ARPPs in the medium to long term.

Devang Patel: Right. My second question was on capex for 200 beds over next two years. What would be the amount you would

be spending just on the growth part of capex?

Manish Mattoo: So on these are all brownfield expansion and average cost is about INR 45 lakh per bed. So that is about INR90 -

100 crore for 200 beds.

Devang Patel: Okay. And lastly on the EBITDA to CFO conversion at 75% this year was lower than, you know, the trend we have seen earlier. Are there any one-offs in this? Could we expect an improvement going forward in our working capital cycle?

Manish Mattoo: Yes. So when you adjust for ESOPs settlement that, the CFO is actually 87%. So that is the one-off.

Normalized

for that, the EBITDA to CFO is actually 87%.

Devang Patel: Right. Thank you so much.

Moderator: Thank you. We will take the next question from the line of Pranav Chawla from Ambit Capital Private Limited.

Pranav Chawla: Congratulations for a good quarter. Sir, I had a couple of questions. One regarding slide number 18 where we have

highlighted margins for various

buckets where we classify. If I refer the same to our FY 25 presentation, we are

seeing a dip in INR5 crore to INR10 crore revenue per month hospitals as well as we are seeing a sharp decline in margins for the less than INR5 crore bucket hospitals as well. Can you elude to what has led to that dip?

Manish Mattoo: So that is because, you know, there are several centres moved from third bucket to the second one and second to the first one.

Pranav Chawla : So this is primarily due to moving of cohorts?

Manish Mattoo: Thanks right.

Pranav Chawla: So the balance hospitals that are still operating at this less than 5% bucket, are these the Mumbai assets that we

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were referring to in the past and is there a scope for margin improvement in this bucket?

Manish Mattoo: It includes that.

Pranav Chawla : So is there scope for margin improvement in this bucket?

Manish Mattoo: So there is scope to improve margin and ROCE and, of course, you will see improvement in the subsequent quarters.

Pranav Chawla: Okay. Got it, sir. Sir, and one thing, is it possible for you to quantify what percentage of revenue that you had to

forego or the low -margin business that we foregone over this fiscal so that we get a better like -on-like comparison of the business?

Manish Mattoo: Right now we may not be able to answer that question.

Pranav Chawla: Okay, absolutely no problem. Sir, lastly on the Milann transaction, can you give us more color regarding the thought

process behind divesting this asset to Inviga Healthcare Fund?

Manish Mattoo: So first, let me answer the question why it was divested and then I will answer the second part of the question. See,

Milann has been part of HCG for 13 years, you know, and then we had a majority stake in 2013 and then it became a fully -owned subsidiary in 2020. Over the past few years, Milann's operating footprint has, you know, steadily declined.

The number of centers has come down from 9 to 6 and today the business operates at a low EBITDA margin and remains only marginally above break -even on a post -rent basis. So, reviving and scaling the platform would require significant management bandwidth and primary capital infusion, both of which are outside HCG's current strategic focus because we are focused on developing the primary oncology network, core oncology network.

So, HCG then decided to exit this non -core business and transfer it to a buyer who was better positioned to infuse growth capital, scale the business independently, and carry forward the Milann legacy. And why it was sold to Inviga Healthcare Fund. See, we initiated a comprehensive sale process, a competitive sale process where HCG shortlisted Inviga as the preferred buyer and the focus was on maximizing valuation and ensuring that the transaction, you know, was done with certainty and it facilitated a smooth transition for Milann's employees as well.

So, following a comprehensive evaluation of the offers received and after obtaining the requisite approvals from the Board and the Audit Committee, HCG proceeded with finalization of the with Inviga Healthcare Fund.

Pranav Chawla: Got it. Sir, and from a three to five -year lens, can you highlight how many beds do you plan to reach by FY30 or

FY29, whichever you are comfortable with at this moment? I assume this will primarily be Brownfield in nature, the count that you will refer to.

Manish Mattoo: So, we have maintained that we will be adding about 1,000 beds by FY30. 400 of those beds will come through

Greenfield expansion and about 600 through the Brownfield expansion.

Pranav Chawla: Got it, got it. Perfect. Thank you so much, sir. All the best.

Moderator: Thank you. We will take the next question from the line of Anubhav Sanghal from Anand Rathi Share and Stock

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Anubhav Sanghal: Hi, Manish. Congratulations on a good set of numbers. My ques

tion is regarding the new North Bangalore facility

where you have added the MR -Linac. Just wanted to understand, is MR -Linac on an ownership basis or is it a pay -per-use?

Secondly, how are you looking at the ROCE trajectory in the Bangalore region with the addition of North Bangalore?

Manish Mattoo: So, Anubhav, MR -Linac is on a pay -per-use basis to create an asset -light model for us. Second is that overall, while

Bangalore KR, the flagship hospital, is operating at about 30%+ ROCE, and as company we expected to hit a 20% ROCE over next four to five years .

Anubhav Sanghal: All right. Thank you.

Moderator: Thank you very much. Ladies and gentlemen, we will take that as the last question. I now hand the conference back

to the management for the closing comments. Thank you and over to you, sir.

Manish Mattoo: Thank you so much. Thank you so much for joining us and I look forward to our next interaction. In case any other

questions have remained unanswered, please reach out to Ravi or me as you deem appropriate. Thank you.

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